



DELORIS ANDA NIELSEN
Deloris lives with obesity
Denmark



Novo Nordisk – a focused healthcare company

Investor presentation
First six months of 2025

Agenda

Progress on Strategic Aspirations 2025

Commercial execution

Innovation and therapeutic focus

Financials

Forward-looking statements

Novo Nordisk's statutory Annual Report 2024, Form 20-F, any quarterly financial reports, investor presentations and written information released, shown, or oral statements made, to the public in the future by or on behalf of Novo Nordisk, may contain certain forward-looking statements relating to the operating, financial and sustainability performance and results of Novo Nordisk and/or the industry in which it operates. Forward-looking statements can be identified by the fact that they do not relate to historical or current facts and include guidance. Words such as 'believe', 'expect', 'may', 'will', 'plan', 'strategy', 'transition plan', 'prospect', 'foresee', 'estimate', 'project', 'anticipate', 'can', 'intend', 'target' and other words and terms of similar meaning in connection with any discussion of future operating, financial or sustainability performance identify forward-looking statements. Examples of such forward-looking statements include, but are not limited to:

- Statements of targets, future guidance, (transition) plans, objectives or goals for future operations, including those related to operating, financial and sustainability matters, Novo Nordisk's products, product research, product development, product introductions and product approvals as well as cooperation in relation thereto;
- Statements containing projections of or targets for revenues, costs, income (or loss), earnings per share, capital expenditures, dividends, capital structure, net financials and other financial measures;
- Statements regarding future economic performance, future actions and outcome of contingencies such as legal proceedings; and
- Statements regarding the assumptions underlying or relating to such statements.

These statements are based on current plans, estimates, opinions, views and projections. Although Novo Nordisk believes that the expectation reflected in such forward-looking statements are reasonable, there can be no assurance that such expectation will prove to be correct. By their very nature, forward-looking statements involve risks, uncertainties and assumptions, both general and specific, and actual results may differ materially from those contemplated, expressed or implied by any forward-looking statement.

Factors that may affect future results include, but are not limited to, global as well as local political, economic and environmental conditions, such as interest rate and currency exchange rate fluctuations or climate change, delay or failure of projects related to research and/or development, unplanned loss of patents, interruptions of supplies and production, including as a result of interruptions or delays affecting supply chains on which Novo Nordisk relies, shortages of supplies, including energy supplies, product recalls, unexpected contract breaches or terminations, government-mandated or market-driven price decreases for Novo Nordisk's products, introduction of competing products, reliance on information technology including the risk of cybersecurity breaches, Novo Nordisk's ability to successfully market current and new products, exposure to product liability and legal proceedings and investigations, changes in governmental laws and related interpretation thereof, including on reimbursement, intellectual property protection and regulatory controls on testing, approval, manufacturing and marketing, and taxation changes, including changes in tariffs and duties, perceived or actual failure to adhere to ethical marketing practices, investments in and divestitures of domestic and foreign companies, unexpected growth in costs and expenses, strikes and other labour market disputes, failure to recruit and retain the right employees, failure to maintain a culture of compliance, epidemics, pandemics or other public health crises, the effects of domestic or international crises, civil unrest, war or other conflict and factors related to the foregoing matters and other factors not specifically identified herein.

For an overview of some, but not all, of the risks that could adversely affect Novo Nordisk's results or the accuracy of forward-looking statements in the Annual Report 2024, reference is made to the overview of risk factors in 'Risks' of the Annual Report 2024.

None of Novo Nordisk or its subsidiaries or any such person's officers, or employees accept any responsibility for the future accuracy of the opinions and forward-looking statements expressed in the Annual Report 2024, Form 20-F, any quarterly financial reports, investor presentations, and written information released, shown, or oral statements made, to the public in the future by or on behalf of Novo Nordisk or the actual occurrence of the forecasted developments.

Unless required by law, Novo Nordisk has no duty and undertakes no obligation to update or revise any forward-looking statement, whether as a result of new information, future events, or otherwise.

Important drug information

Victoza® and Ozempic® are approved for people with type 2 diabetes only

Saxenda® and Wegovy® are approved for people with overweight and obesity only

Strategic Aspirations 2025 | Highlights first six months of 2025

Light blue indicates developments in Q2 2025

 Purpose and sustainability (ESG)	Progress towards zero environmental impact - CO₂e emissions¹ increased by 31% compared to first six months of 2024 Adding value to society - Medical treatment provided to 42.8 million people living with diabetes and 2.9 million people living with obesity Being recognised as a sustainable employer - Share of women in senior leadership positions has increased to 43% from 41% end of June 2024	 Innovation and therapeutic focus	Further raise innovation bar for Diabetes treatment - Ozempic® positive opinion by the EMA for PAD Develop superior treatment solutions for Obesity - Advancement of sc and oral amycretin to phase 3 - CagriSema phase 3b REDEFINE 11 trial initiated - Sema 7.2 mg EU submission - Septerna license agreement for oral small molecules Strengthen and progress Rare Disease pipeline - Alhemo® US approval and CMHP positive opinion Establish presence in CV & Emerging Therapy areas - Coramitug phase 2 trial successfully completed
 Commercial execution	Diabetes value market share at 32.6% (-1.4 %-p)² Obesity care sales of DKK 38.8 billion (+58% at CER) Rare disease sales of DKK 9.5 billion (+15% at CER)	 Financials	Sales growth of 18% (CER) Operating profit growth of 29% (CER) Free cash flow of DKK 33.6 billion and 36.5 billion returned to shareholders

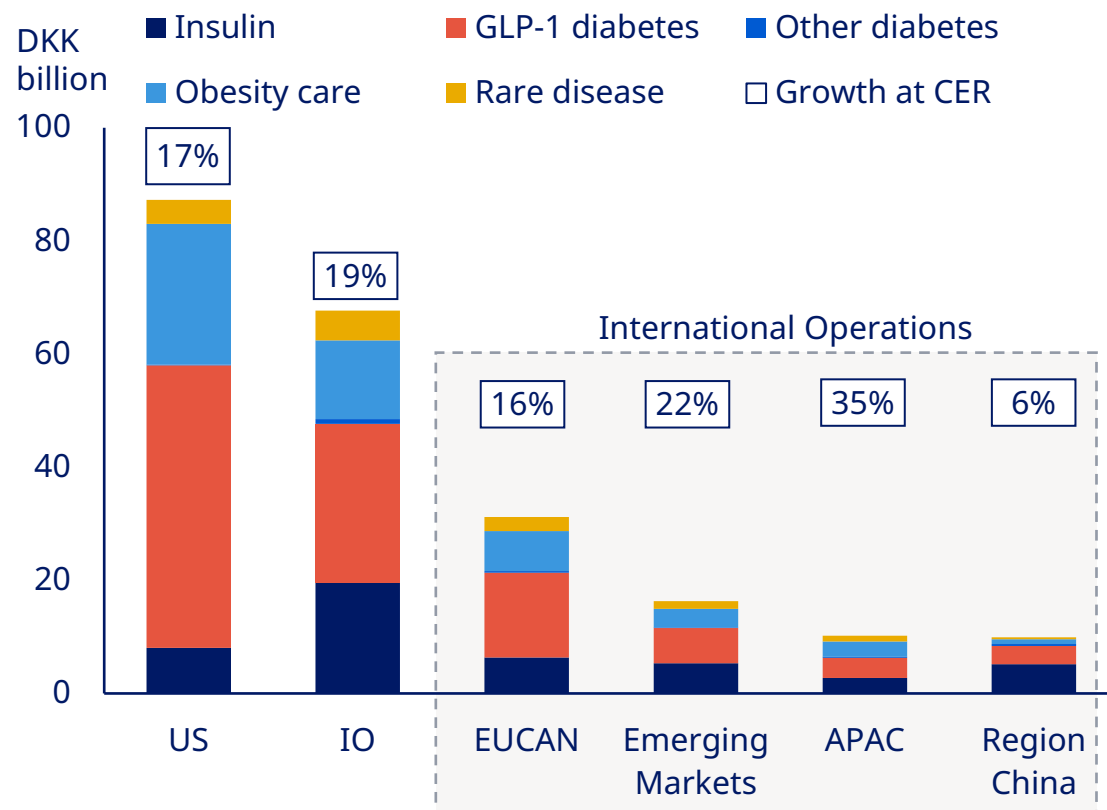
¹Scope 1, 2 and 3; ²MAT (Moving Annual Total) value market share

CagriSema: cagrilintide 2.4 mg and semaglutide 2.4 mg; CER: Constant exchange rates; CO₂e: CO₂ equivalents; CV: Cardiovascular; EMA: European Medicines Agency; EU: European Union; JP: Japan; MASH: Metabolic dysfunction-associated steatohepatitis; PAD: Peripheral arterial disease; Sc: Subcutaneous; Sema: Semaglutide; US: United States

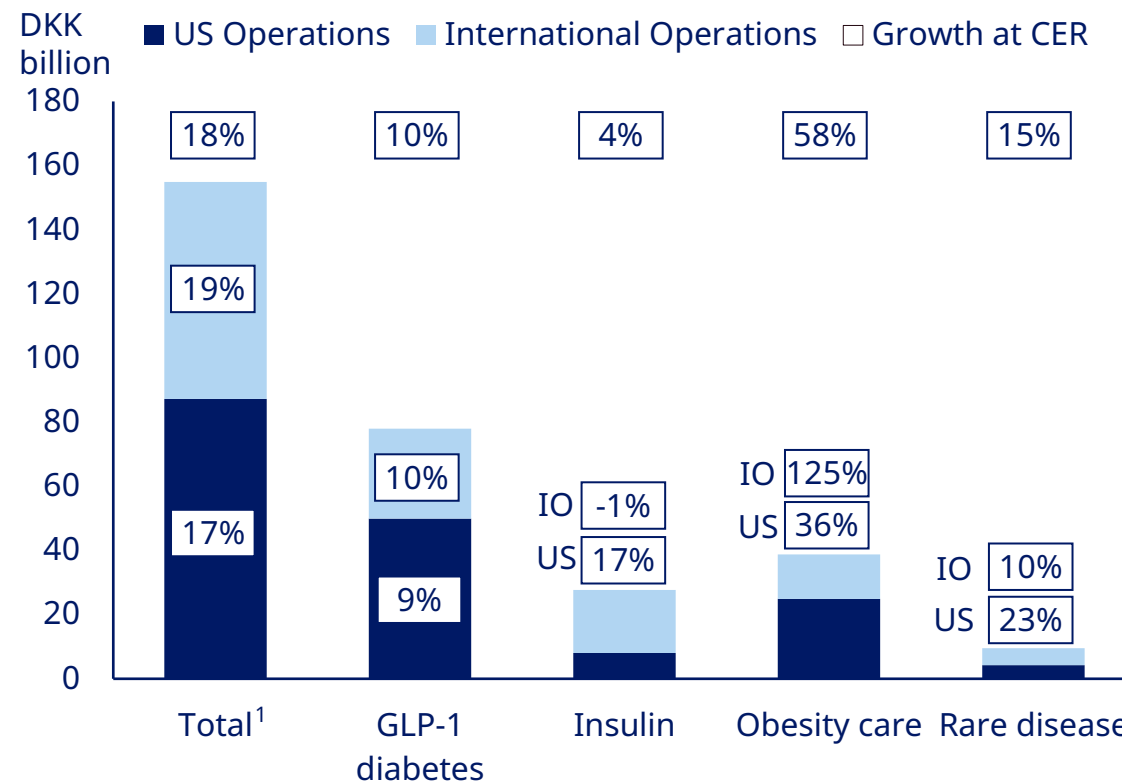
Note: The strategic aspirations are not a projection of Novo Nordisk's financial outlook or expected growth.

Sales growth of 18% driven by both operating units

Reported geographic sales split for first six months 2025



Reported therapy area sales and growth for first six months 2025

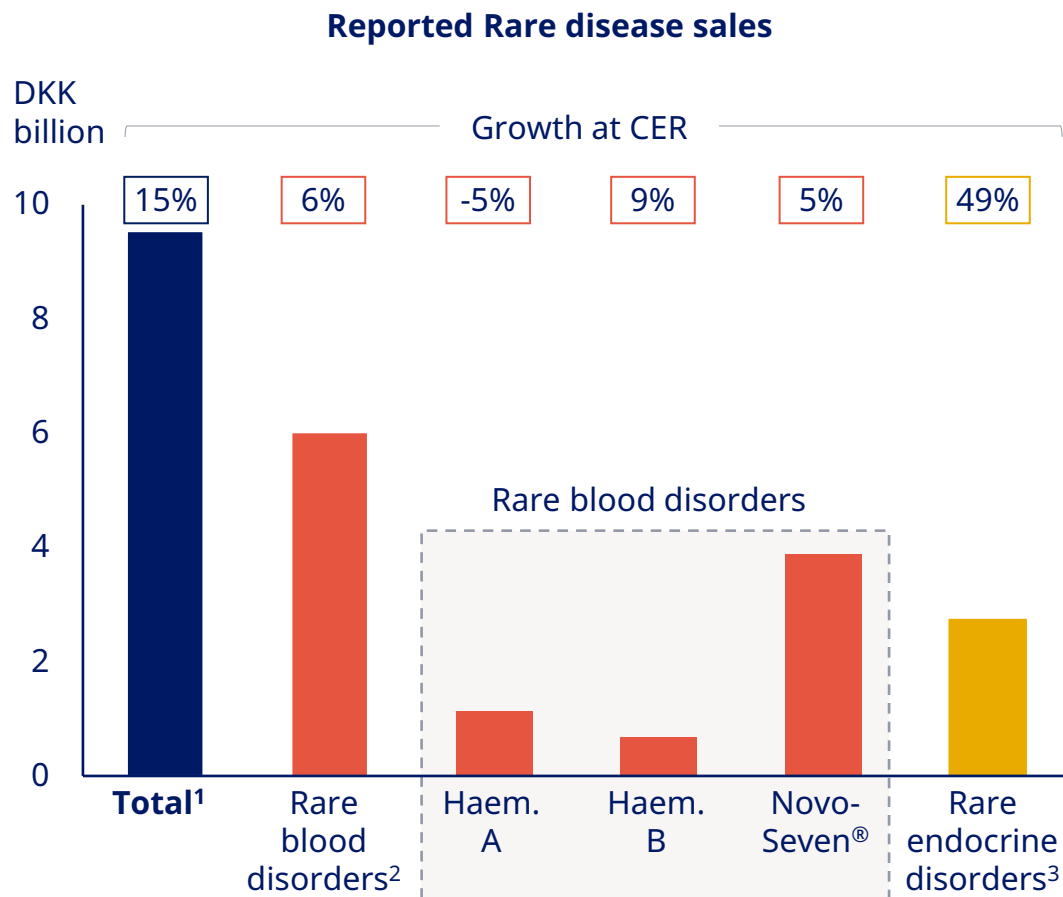


¹Other diabetes' is included in Total

APAC: Japan, Korea, Oceania and Southeast Asia; CER: Constant exchange rates; Region China: Mainland China, Hong Kong and Taiwan; Emerging Markets: mainly Latin America, Middle East and Africa; EUCAN: Europe and Canada; IO: International Operations; US: United States

Note: Unless otherwise specified, sales growth rates are at CER

Rare disease sales increased by 15%



Rare disease sales performance

Rare disease sales increased by 15%:

- Sales in US Operations increased by 23%
- Sales in International Operations increased by 10%

Rare endocrine disorders sales increased by 49%:

- US Operations increased by 67%, driven by Norditropin® and Sogroya®
- International Operations increased by 30%, driven by Norditropin® and Sogroya®

Rare blood disorders sales increased by 6%:

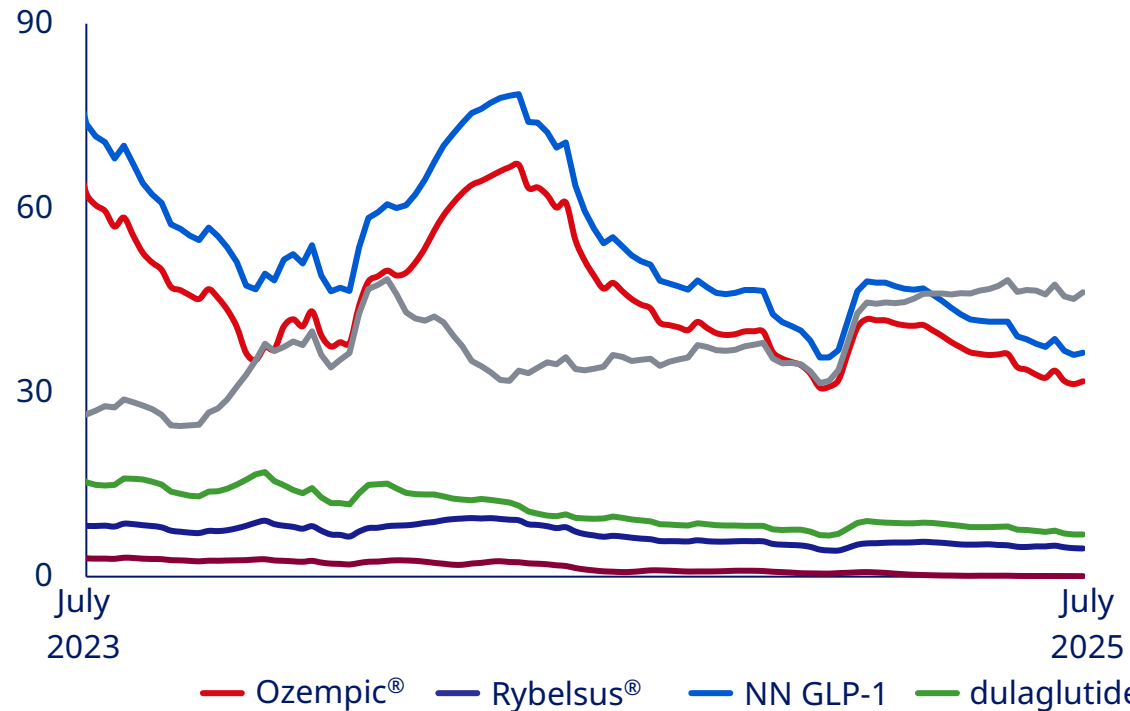
- US Operations increased by 6% driven by increased NovoSeven® and Alhemo® sales
- International Operations increased by 6% driven by increased sales of haemophilia B and Alhemo®

¹Total includes "Other Rare disease", which consists of primarily Vagifem® and Activelle® ²Comprises Sogroya®, NovoSeven®, NovoEight®, Esperoct®, Refixia®, NovoThirteen® and Alhemo® ³Primarily Norditropin® and Sogroya®
 CER: Constant exchange rates; Haem. A: Haemophilia A; Haem. B: Haemophilia B; IO: International operations; US: United States
 Note: NovoThirteen® is not shown for Rare blood disorders breakdown, only for the total bar. Unless otherwise specified, sales growth is at constant exchange rates

US diabetes GLP-1 class growth slowing compared to prior years

US GLP-1 diabetes weekly NBRx prescriptions

Weekly NBRx
scripts ('000s)

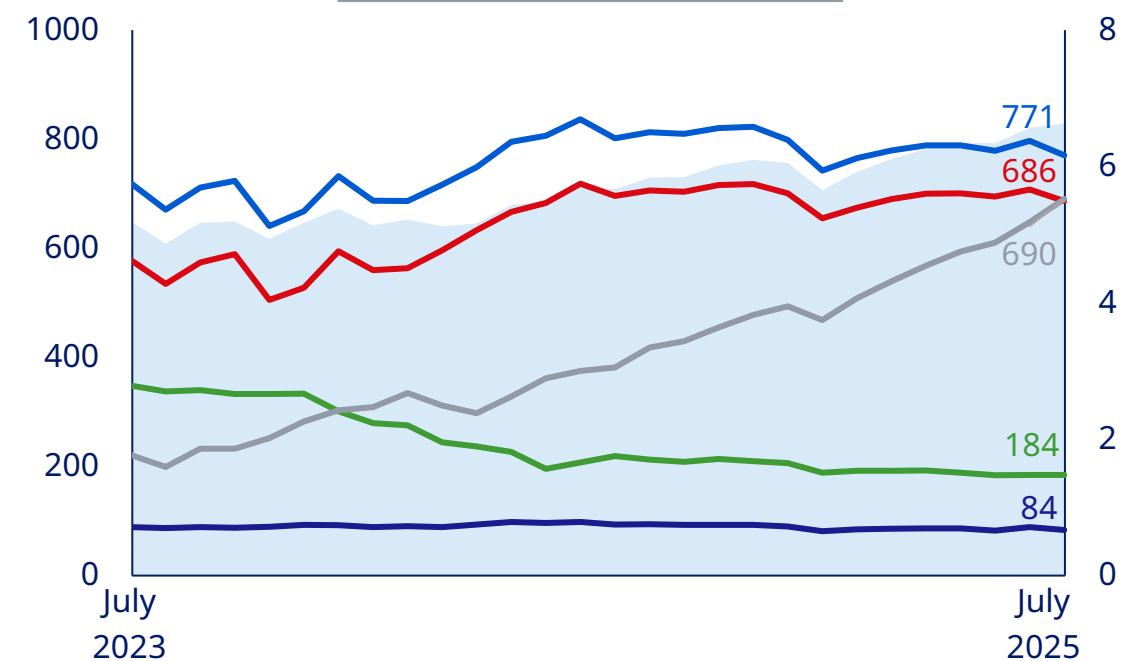


US GLP-1 diabetes TRx market share

TRx scripts
('000s)

Total GLP-1 SUs
(millions)

Class growth ~15%

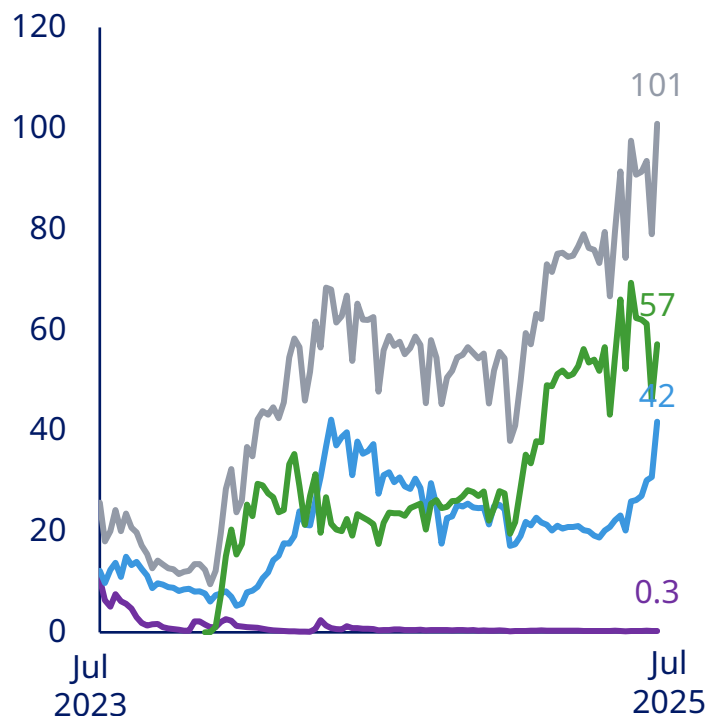


NBRx: New-to-brand prescriptions; NN: Novo Nordisk; Scripts: Prescriptions; SU: standard units; TRx: Total prescriptions; US: United States
 Note: Class growth calculated based on SU volume for diabetes GLP-1 as May'25-July'25 vs May'24-July'24 (Rolling 3-month average)
 Source: IQVIA Xponent Plantrak, NBRx and TRx data from week ending 18th July and 25th July, respectively. Each data point represents a rolling four-week average.

US branded anti-obesity medication market expansion continues, while GLP-1 compounding continues

Branded AOM NBRx in the US¹

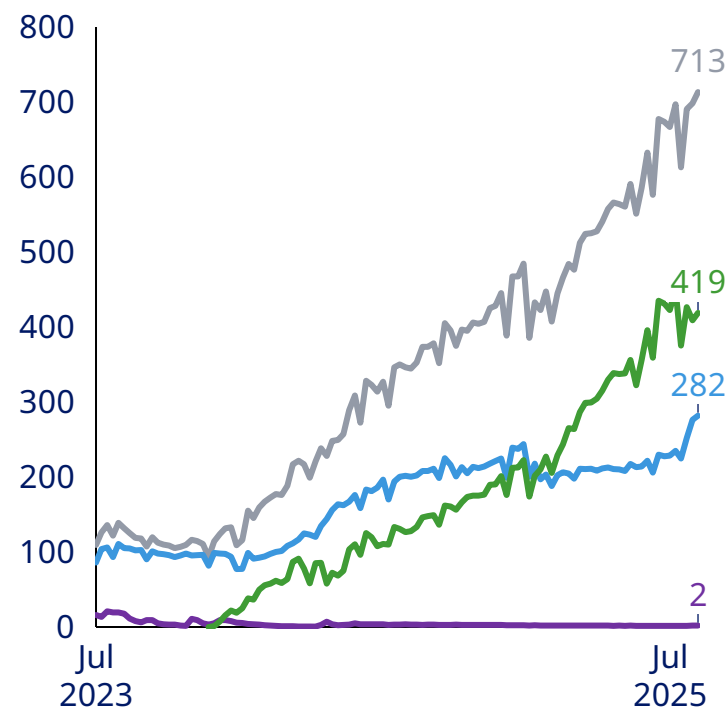
NBRx scripts ('000s)



— Saxenda® — Wegovy® — tirzepatide — Branded AOM market

Branded AOM TRx in the US¹

TRx scripts ('000s)



Branded AOM class grew >160%²

Compounding

- Novo Nordisk is focused on actively preventing unlawful and unsafe compounding

Commercial execution

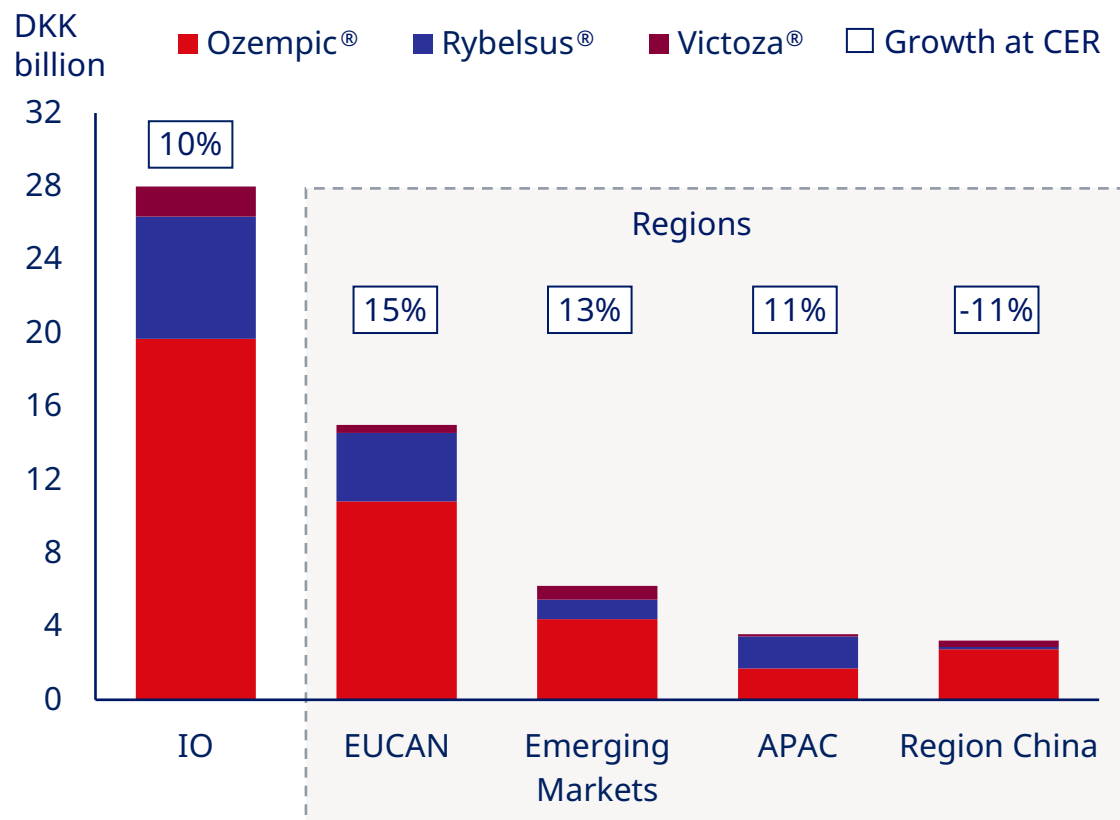
- Cash channel expanded from 4% to ~10% of TRx since January 2025
- CVS national template formulary conversion ongoing
- MASH decision still expected in Q3 2025
- Wegovy® supply available to meet demand in US

¹ Each NBRx and TRx data point represents one week of data. IQVIA Xponent 11 Jul 2025 for NBRx and IQVIA NPA weekly, 25 Jul 2025 for TRx, including NovoCare Pharmacy TRx starting with week-ending 18 July 2025. ²Class growth based on IQVIA 25 July 2025 volume data, MAT.

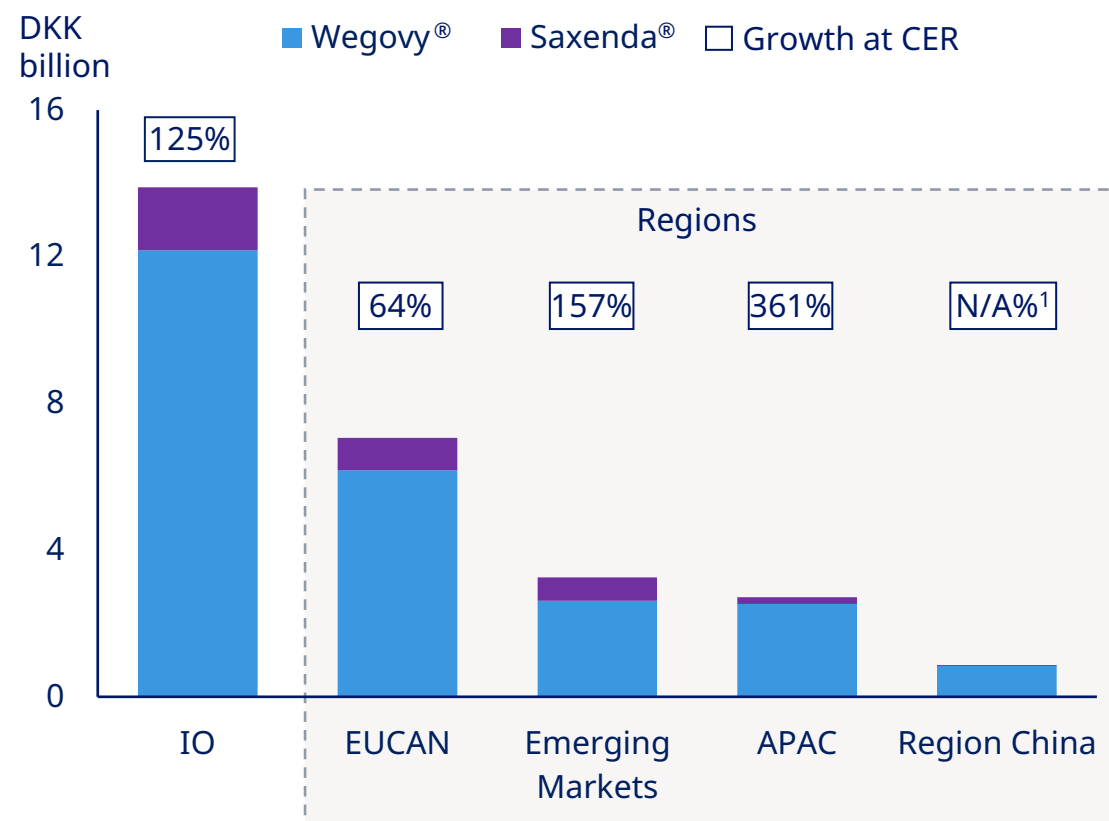
AOM: Anti-Obesity Medications (includes Wegovy®, Saxenda®, Zepbound®, Qsymia® and Contrave®); HFpEF: Heart failure with preserved ejection fraction; MAT: Moving annual total; TRx SU: A one-month prescription supply; US: United States

International Operations sales growth of 19% driven by GLP-1 Diabetes and Obesity care

Reported GLP-1 Diabetes care sales and growth for first six months 2025



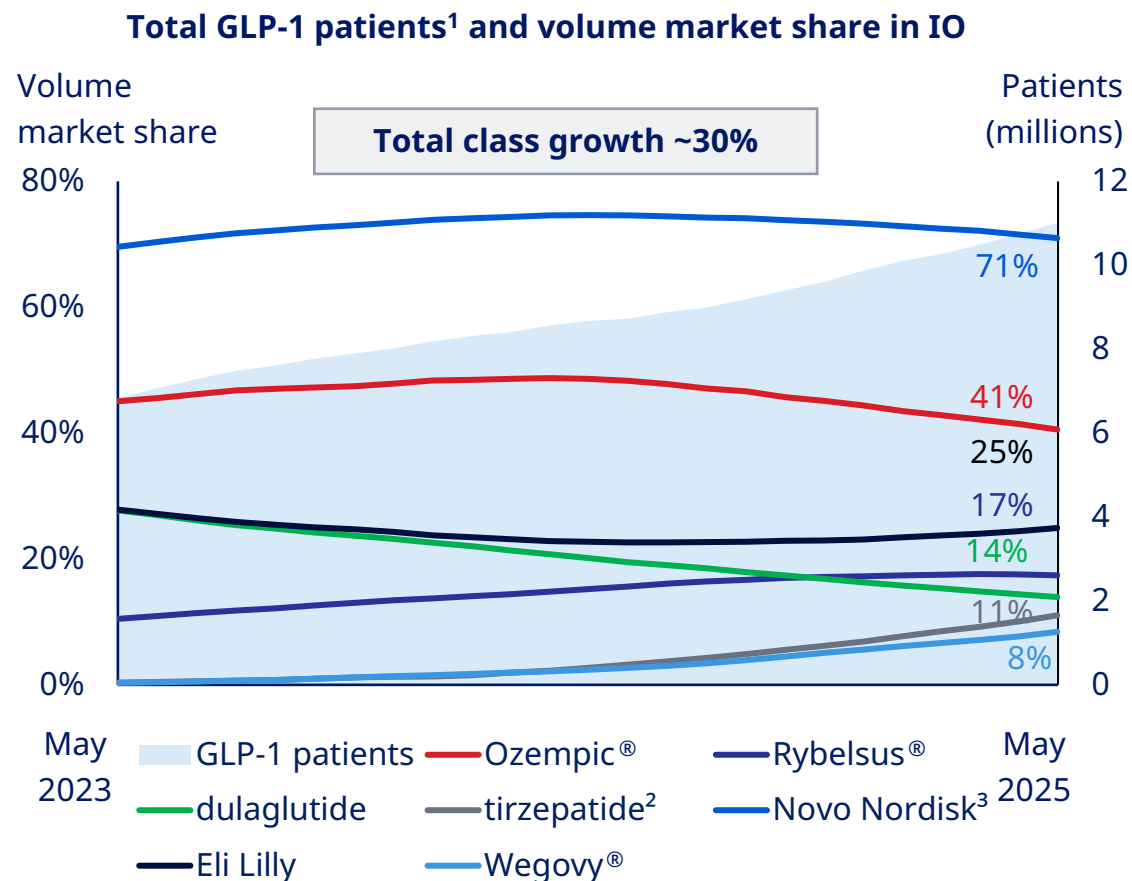
Reported Obesity care sales and growth for first six months 2025



¹No comparator for first six months 2025

APAC: Japan, Korea, Oceania and Southeast Asia; CER: Constant exchange rates; China: Mainland China, Hong Kong and Taiwan; Emerging Markets: mainly Latin America, Middle East and Africa; EUCAN: Europe and Canada; IO: International Operations

Total GLP-1 class market share of 71% in International Operations



IO total GLP-1 performance

Diabetes GLP-1

- Rybelsus® launched in more than 40 countries
- Ozempic® launched in around 80 countries with promotional focus resumed, reflecting improved supply

Obesity

- Wegovy® launched in around 35 countries
- MASH indication submitted in JP in May 2025
- Semaglutide 7.2 mg submitted in EU in July 2025
- Roll-out of Wegovy® in additional countries expected in H2 2025

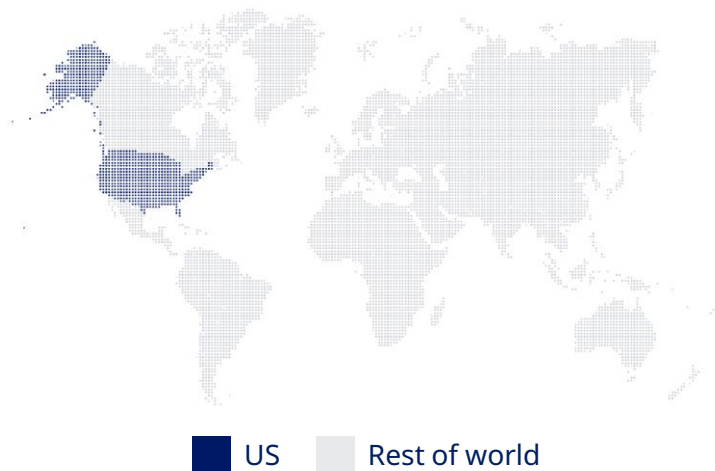
¹GLP-1 patients across Diabetes and Obesity care ²In IO countries, tirzepatide is categorised under GLP-1 diabetes only, despite having indications for Diabetes and Obesity in most launched countries ³Includes Victoza® and Saxenda®
IO: International Operations; JP: Japan

Note: Market share and patient numbers are based on countries with IQVIA coverage. GLP-1 class growth calculated as Mar'24-May'24 vs Mar'25-May'25 (Rolling 3-month average)

Source: LHS: IQVIA MAT, May 2025 (Spot rate). Volume packs are converted into full-year patients based on WHO assumptions for average daily doses; Market values are based on the list prices. RHS: International Diabetes Federation: Diabetes Atlas 11th edition, 2025, World Obesity Atlas 2024

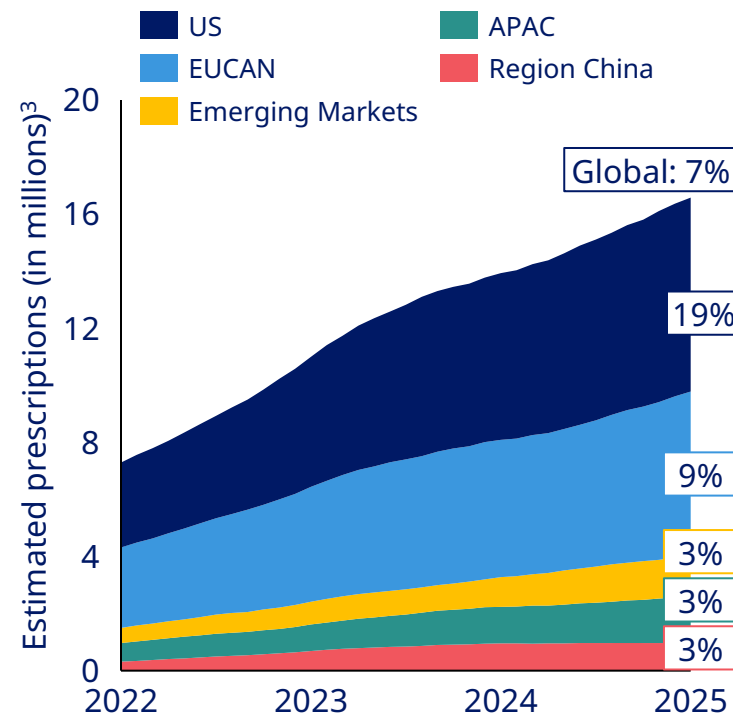
The high unmet need in diabetes and obesity and low market penetration to-date makes unlocking the market a key priority

Global diabetes and obesity unmet need

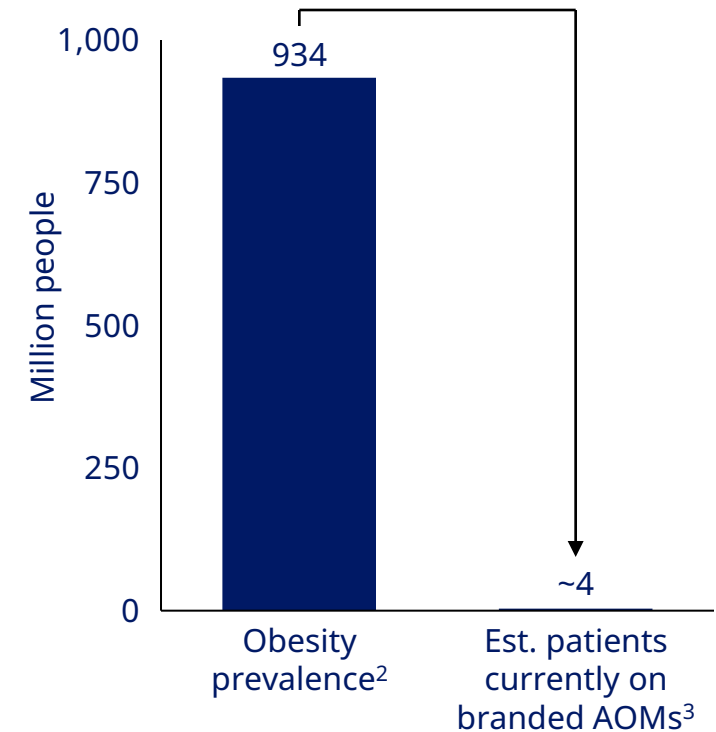


- >550 million people live with diabetes globally, with over 90% outside of the US¹
- >900 million people with obesity globally, with around 90% outside of the US²

Globally, ~7% of total estimated diabetes prescriptions are for a GLP-1



Less than 1% of people with obesity globally are treated with branded AOMs



¹Diabetes Atlas 11th edition, 2025, including Type 1 and Type 2 Diabetes. ²NHANES (2013-2014, 2015-2016, 2017-2020, 2021-2023), UN World Population Prospects report, WHO, IDF World Diabetes Atlas, World Obesity Atlas and PADAWA Analysis. ³Based on IQVIA MIDAS, May 2025 data - In ex-US countries, tirzepatide is categorised under GLP-1 diabetes only in IQVIA data, despite having indications for diabetes and obesity in most launched countries in IQVIA.

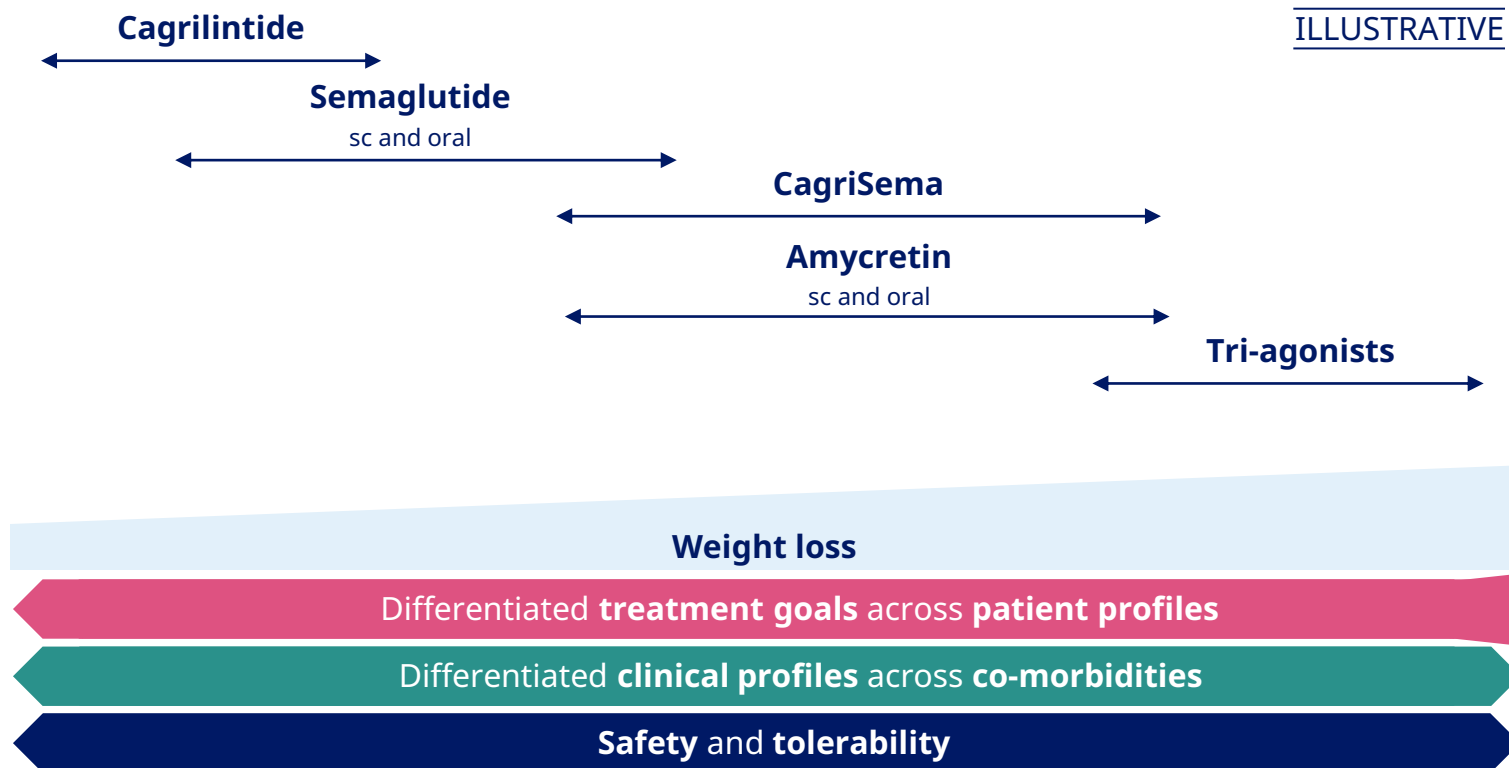
APAC: Japan, Korea, Oceania and Southeast Asia; AOM: Anti-Obesity Medications; Emerging Markets: mainly Latin America, Middle East and Africa; EUCAN: Europe and Canada; Region China: Mainland China, Hong Kong and Taiwan; US: United States.

Note: the estimated GLP-1 share of prescriptions is based on volume packs from IQVIA. Volume packs are converted into full-year patients/prescriptions based on WHO assumptions for average daily doses or if not available, Novo Nordisk assumptions. It is possible for a patient to have a prescription for more than one diabetes treatment.

Novo Nordisk's obesity portfolio addresses the future segments and patient preferences of the obesity market

Addressing unmet needs across patient segments via a focus on weight loss and differentiated clinical profiles¹

ILLUSTRATIVE



Examples of future patient segments



BMI
35–40

BMI
40–45

BMI
45–50

+ Age and gender differences

+ Lifestyle considerations

+ ORC clinical profiles

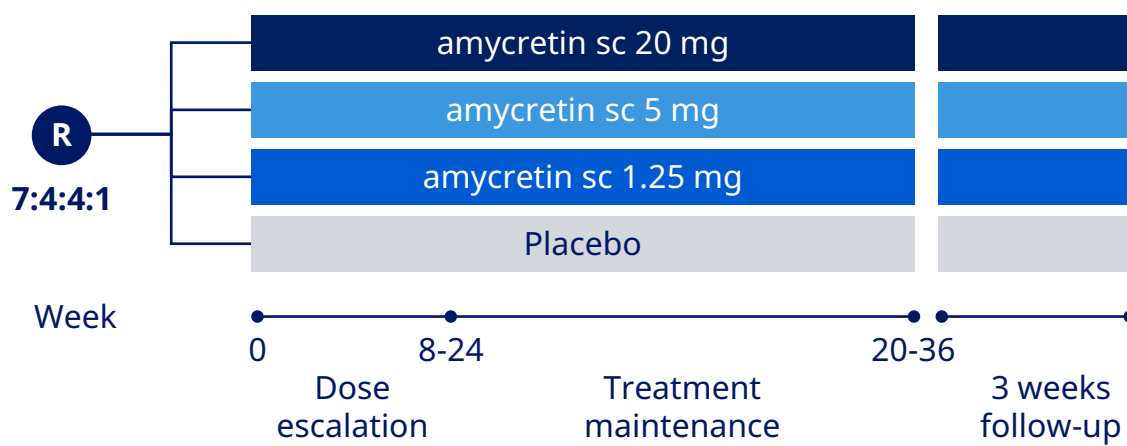


¹Illustrative, not exhaustive of full obesity pipeline

BMI: Body mass index; CVD: Cardiovascular disease; HF: Heart failure; MASH: Metabolic Dysfunction-Associated Steatohepatitis; OA: Osteoarthritis; ORC: Obesity related comorbidities; Sc: Subcutaneous

Amycretin to advance into phase 3 based on the successful completion of phase 1b/2a trial

Dose response part of the amycretin sc phase 1b/2a trial



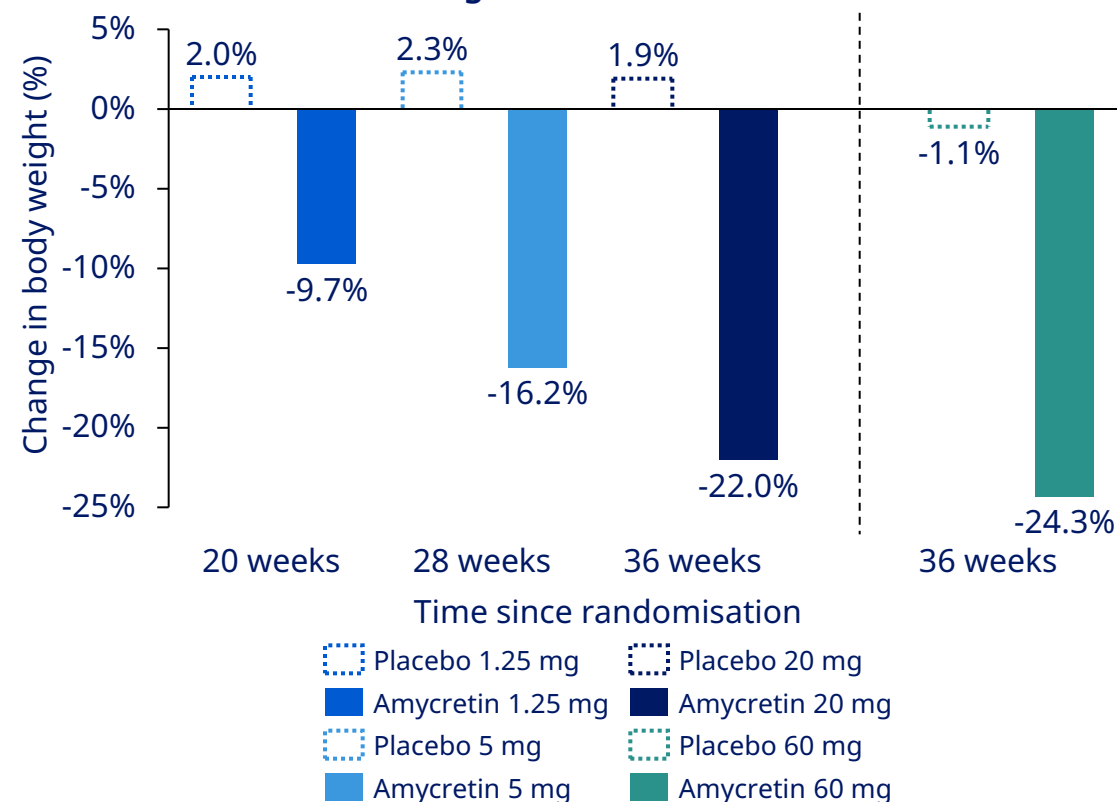
Trial objective

- Investigate safety, tolerability, pharmacokinetics and efficacy of amycretin sc in participants with overweight or obesity

Endpoints

- Primary: Number of treatment emergent adverse events
- Secondary: Relative change in body weight, AUC, c_{max} , t_{max}

Estimated body weight loss in dose response arms and 60 mg dose escalation arm¹



¹NN9490-7613. Dahl K et al., Lancet 2025, 406(10499):149-162. In total, 125 participants were randomized to sc amycretin (n=101) or placebo (n=24). Dose escalation arm examined multiple ascending doses of once-weekly sc amycretin up to 60 mg, and dose response arm examined multiple ascending doses up to a 12-week maintenance dose of 20 mg, 5 mg and 1.25 mg.

AUC: Area Under the Curve; BMI: Body mass index; c_{max} : maximum (peak) plasma concentration; HbA_{1c}: Haemoglobin A_{1c}; MAD: Multiple ascending dose; Sc: Subcutaneous; t_{max} : time to reach maximum (peak) plasma concentration

Note: Amycretin is a unimolecular GLP-1 and amylin receptor agonist.

AMAZE is a comprehensive phase 3 development programme for sc and oral amycretin expected to start in Q1 2026

Selected amycretin phase 3 trials in obesity programme

AMAZE 1 WL in Obesity

- **80-week** vs. placebo (incl. 52-week ext. phase)
- **Primary endpoint:** Weight loss

AMAZE 2 WL in T2D

- **80-week** vs. placebo
- **Primary endpoint:** Weight loss

AMAZE 3 OSA

- **80-week** vs. placebo
- **Co-primary endpoint:** AHI/WL

AMAZE 5 Knee OA

- **80-week** vs. tirzepatide
- **Co-primary endpoint:** WOMAC/WL

AMAZE 9 Oral amycretin

- **72-week** vs. Placebo
- **Primary endpoint:** Weight loss

2026

2027

2028

Potential future trials

Phase 3 development programme

- Evaluate multiple maintenance doses
- Evaluate subcutaneous and oral route of administration
- Evaluate key obesity related comorbidities

Potential to investigate the benefits of amycretin across obesity related comorbidities, such as:

ASCVD

Heart failure

CKD

Knee Osteoarthritis

Obstructive sleep apnea

R&D milestones

■ Clinical milestones¹
■ Regulatory milestones¹

	Project	Q2 2025	Q3 2025	Q4 2025
Diabetes care	CagriSema			Phase 3 results (REIMAGINE 3)
	Oral/Sc amycretin			Phase 2 results
	OW GIP/GLP-1			Phase 2 results
Obesity care	Oral sema 25 mg			US decision
	Sema 7.2 mg		✓ EU submission	
	CagriSema	✓ Phase 3 initiation (REDEFINE 11)		
	Triple (tri-agonist)		Phase 1 results	
	Cagrilintide			Phase 3 initiation
	Oral/Sc amycretin	✓ Advancement to phase 3		
	Amylin 355			Phase 1 results
Rare Disease	Sogroya®	✓ US submission ²	✓ JP submission ²	
	Mim8		US submission	EU submission
	Alhemo®		✓ US approval ³ ✓ EMA positive opinion ³	
CETA	EVOKE (AD, sema 14 mg)			Phase 3 results
	Coramitug (ATTR-CM)		✓ Phase 2 results	Phase 3 initiation
	Zalfermin (FGF21)	✓ Phase 2 results		
	ESSENCE (MASH, sema 2.4 mg)	✓ JP submission	US decision	

¹Expected to be published in the given quarter or in the subsequent quarterly company announcement. ²Non-replacement indications. ³Without inhibitors.

AD: Alzheimer's disease; ATTR-CM: Transthyretin amyloid cardiomyopathy; CagriSema: cagrilintide 2.4 mg and semaglutide 2.4 mg; CETA: Cardiovascular & emerging therapies; EMA: European Medicines Agency; EU: European Union; GIP: Gastric inhibitory polypeptide; FGF-21: Fibroblast growth factor 21; JP: Japan; MASH: Metabolic dysfunction-associated steatohepatitis; OW: once-weekly; Sema: Semaglutide; US: United States; Sc: subcutaneous

Financial results – in the first six months of 2025

In DKK million	First six months of 2025	First six months of 2024	Change (reported)	Change (CER)
Sales	154,944	133,409	16%	18%
Gross profit	129,208	113,219	14%	16%
<i>Gross margin</i>	83.4%	84.9%		
Sales and distribution costs	(32,425)	(28,190)	15%	15%
<i>Percentage of sales</i>	20.9%	21.1%		
Research and development costs	(21,998)	(24,772)	(11%)	(11%)
<i>Percentage of sales</i>	14.2%	18.6%		
Administration costs	(2,536)	(2,314)	10%	11%
<i>Percentage of sales</i>	1.6%	1.7%		
Other operating income and expenses	(9)	(163)	N/A	N/A
Operating profit	72,240	57,780	25%	29%
<i>Operating margin</i>	46.6%	43.3%		
Financial items (net)	(1,402)	(530)	N/A	N/A
Profit before income tax	70,838	57,250	24%	N/A
Income taxes	(15,301)	(11,793)	30%	N/A
<i>Effective tax rate</i>	21.6%	20.6%		
Net profit	55,537	45,457	22%	N/A
Diluted earnings per share (DKK)	12.49	10.17	23%	N/A

Financial outlook for 2025

	Expectations 6 August 2025	Expectations 7 May 2025
Sales growth – at CER	8% to 14%	13% to 21%
Sales growth - reported	Around 3 percentage points lower	Around 3 percentage points lower
Operating profit growth – at CER	10% to 16%	16% to 24%
Operating profit growth - reported	Around 5 percentage points lower	Around 5 percentage points lower
Financial items (net)	Gain of around DKK 1.6 billion	Gain of around DKK 0.9 billion
Effective tax rate	21% to 23%	21% to 23%
Capital Expenditure (CAPEX)	Around DKK 65 billion	Around DKK 65 billion
Free cash flow¹	DKK 35 to 45 billion	DKK 56 to 66 billion

¹Excluding impact from business development

CER: Constant exchange rates

Note: The financial outlook assumes of a continuation of the current business environment and given the current scope of business activities and has been prepared assuming that currency exchange rates remain at the level as of 31 July 2025

Strategic aspirations 2025



Purpose and sustainability (ESG)

- Progress towards zero environmental impact
- Being respected for adding value to society
- Being recognised as a sustainable employer



Innovation and therapeutic focus

- Further raise the innovation bar for Diabetes treatment
- Develop a leading portfolio of superior treatment solutions for Obesity
- Strengthen and progress the Rare disease pipeline
- Establish presence in Cardiovascular & Emerging Therapy areas



Commercial execution

- Strengthen Diabetes leadership - aim at global value market share of more than 1/3
- More than 25 billion DKK in Obesity sales by 2025
- Secure a sustained growth outlook for Rare disease



Financials

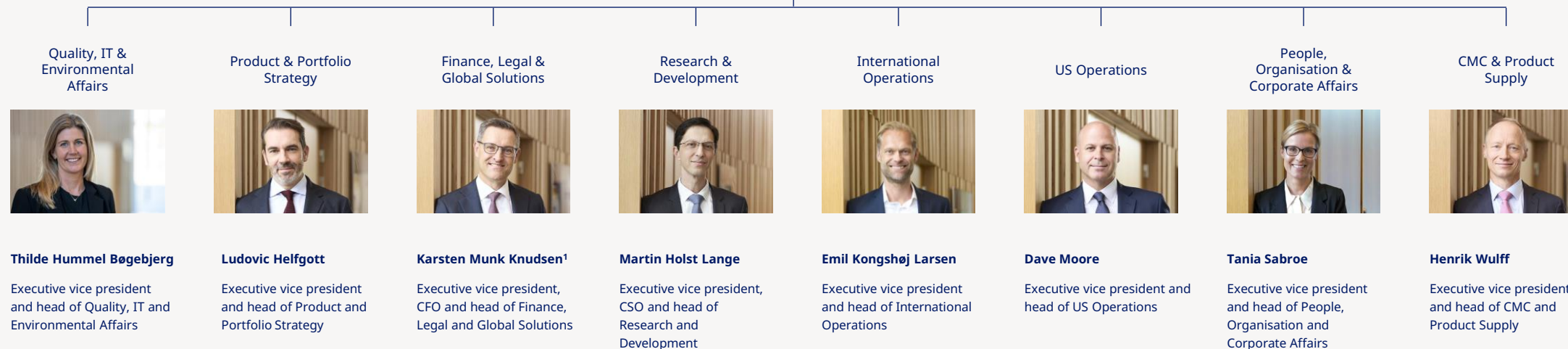
- Deliver solid sales and operating profit growth
- Drive operational efficiencies across the value chain to enable investments in future growth assets
- Deliver free cash flow to enable attractive capital allocation to shareholders

Executive Management as of 7 August 2025



Maziar Mike Doustdar¹

President and CEO



¹Registered as executive with the Danish Business Authority

CEO: chief executive officer; CFO: chief financial officer; CMC: Chemistry, Manufacturing and Control; CSO: chief scientific officer; US: United States

Investor contact information

Share information

Novo Nordisk's B shares are listed on the stock exchange in Copenhagen under the symbol 'NOVO B'. Its ADRs are listed on the New York Stock Exchange under the symbol 'NVO'.

For further company information, visit Novo Nordisk on:
www.novonordisk.com

Upcoming events

5 November 2025	Financial results for the first nine months of 2025
4 February 2026	Financial statement for 2025

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Appendix

Novo Nordisk corporate strategy

Diabetes care

GLP-1

Insulin

Obesity care

Rare disease

Cardiovascular & Emerging Therapy Areas

Regional information

Financials and Product Supply

Sustainability

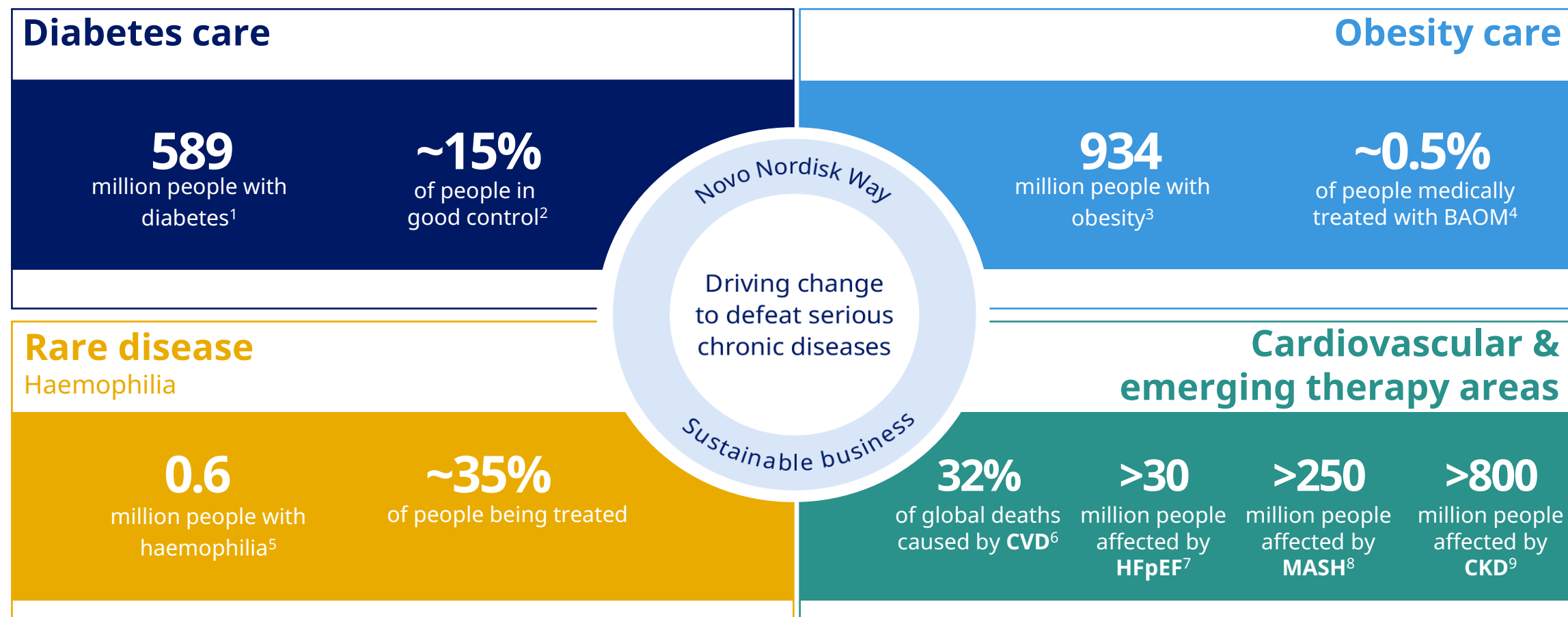
Novo Nordisk Corporate Strategy



Diabetes and obesity remain the key priority areas in the corporate strategy

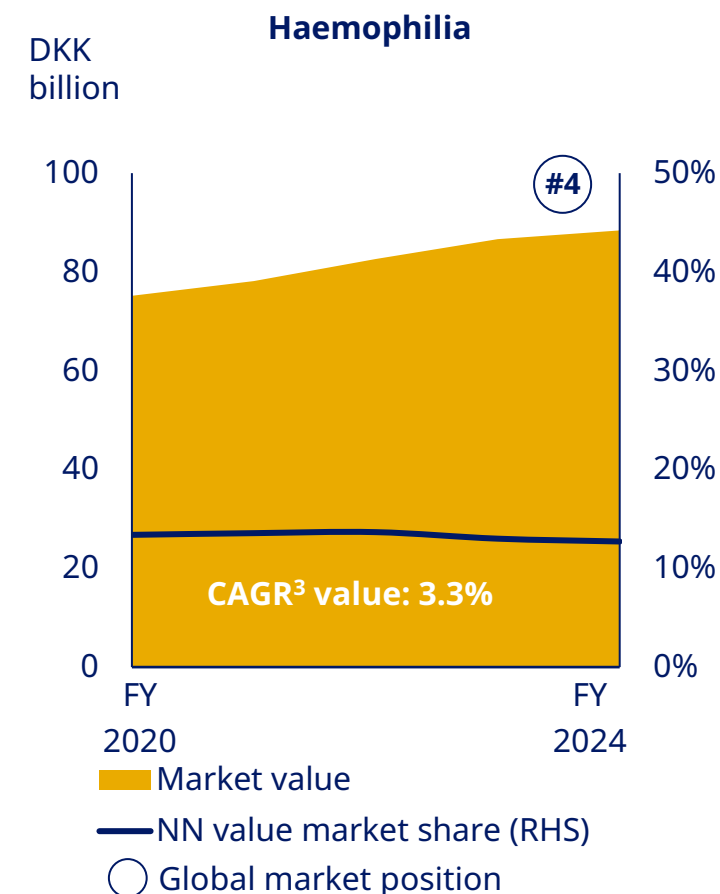
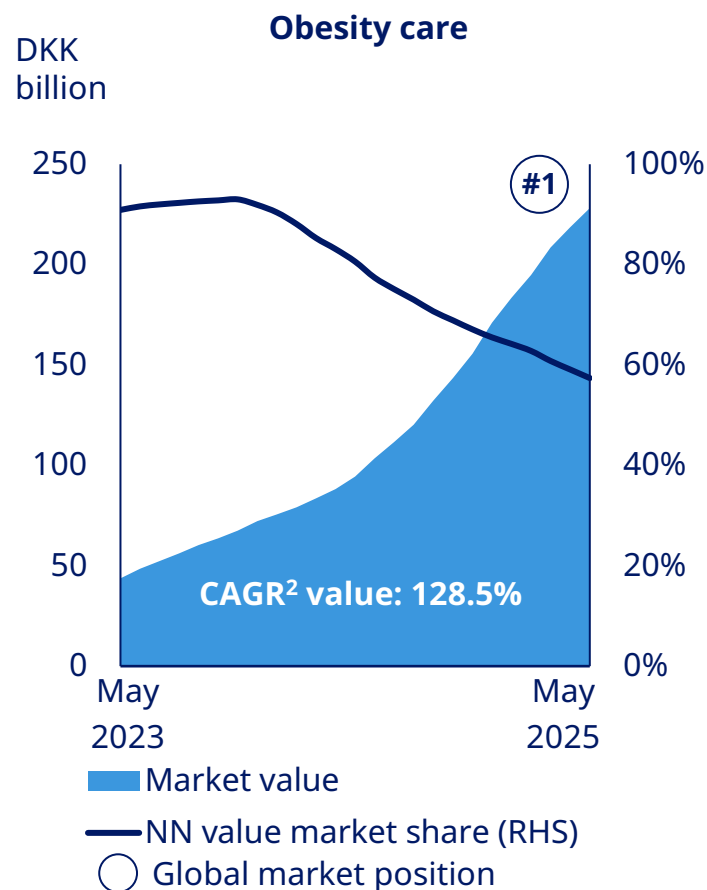
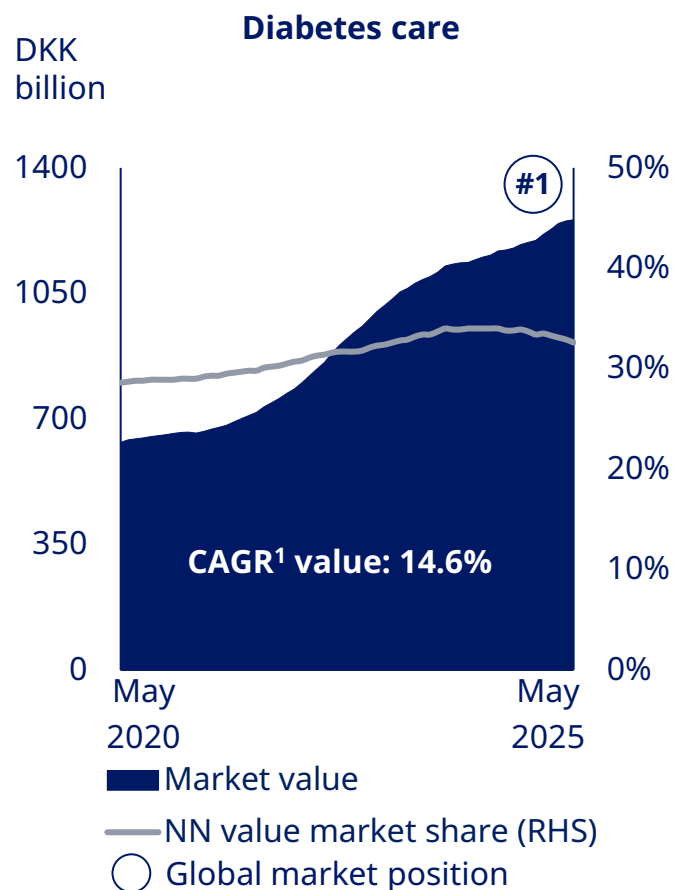
Therapy area priorities	Portfolio focus	Investment approach
1 Diabetes Obesity	Broad and deep	Key investment focus
2 CVD RBD	Multiple targets in key segments	Invest to build competitive pipelines
3 MASH RED CKD	Selective, based on potential and synergies	Targeted investment allocation
4 AD/PD	Opportunistic and trigger-based	Targeted investment allocation

Innovation starts with addressing unmet needs, improving outcomes and reaching more patients



¹International Diabetes Federation: Diabetes Atlas 11th edition, 2025; ²Real-world studies indicate between 30-55% of patients reach HbA_{1c} target <7% .e.g. <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4388968/>, taking 42.5% in good control of treated people; ³NHANES (2013-2014, 2015-2016, 2017-2020, 2021-2023), UN World Population Prospects report, WHO, IDF World Diabetes Atlas, World Obesity Atlas and PADAWA Analysis; ⁴IQVIA as of Nov'24 ⁵WFH annual survey 2020 (120 of 147 countries responded); Prevalence by calculating expected number of patients using 20.9 per 100,000 in haemophilia - Identified patients as proxy for receiving some sort of treatment; ⁶WHO. Cardiovascular Diseases 2023; ⁷Chris J Kapelios et al Cardiac Failure Review 2023;9:e14; ⁸Younossi ZM et al. Hepatology. 2023;77:1335-1347; ⁹Kovesdy CP. Epidemiology of chronic kidney disease: an update 2022. Kidney Int Suppl (2011). 2022 Apr;12(1):7-11
BAOM: Branded Anti Obesity Medication; CKD: Chronic kidney disease; CVD: Cardiovascular disease; HFpEF: Heart failure with preserved ejection fraction; MASH: Metabolic dysfunction-associated steatohepatitis; WHO: World Health Organization

Novo Nordisk has leading positions in diabetes, obesity and haemophilia



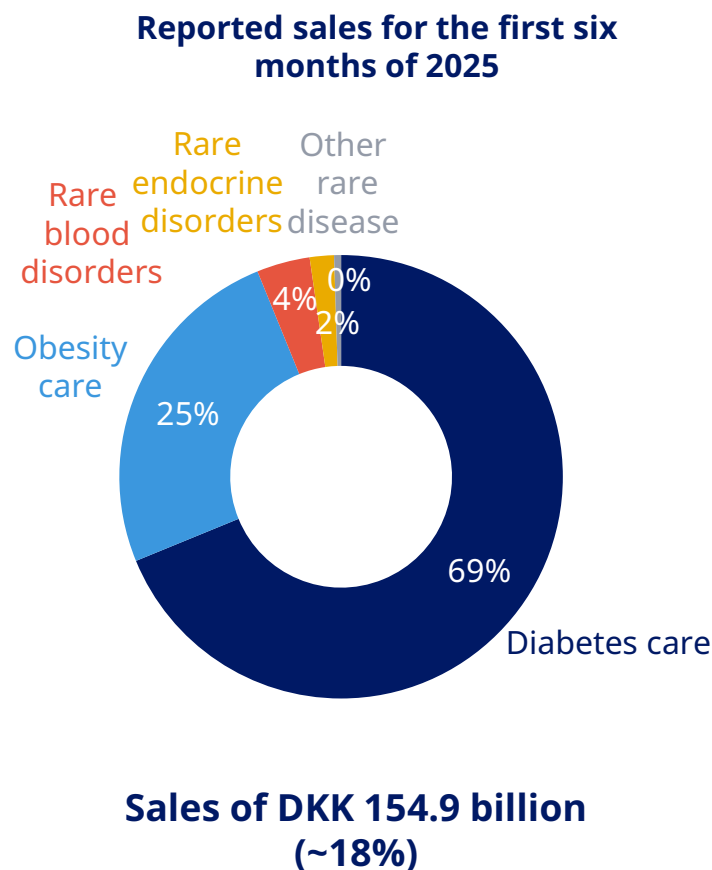
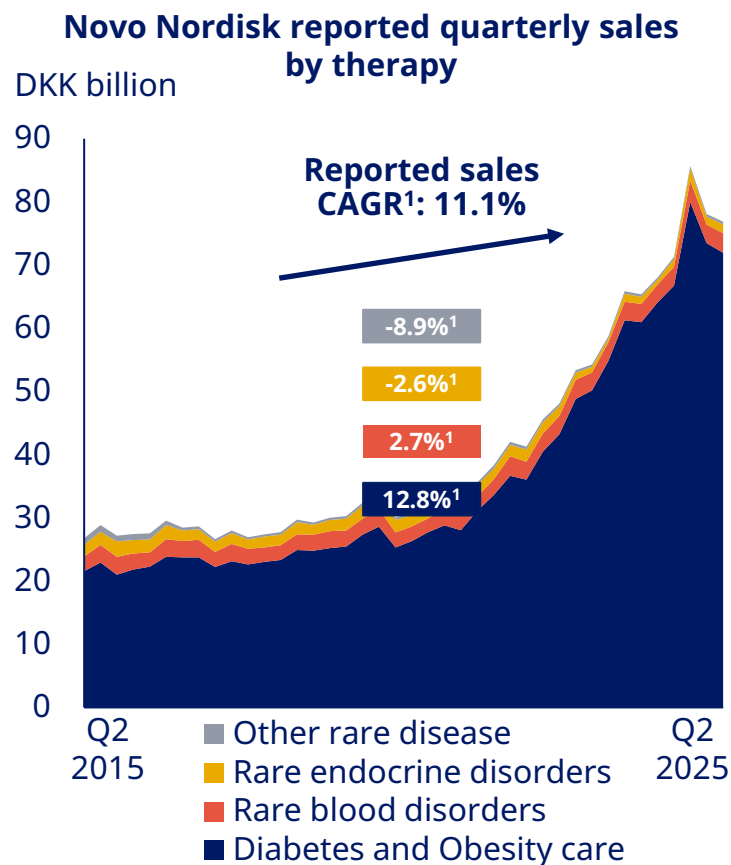
¹CAGR for 5-year period ²CAGR for 2-year period ³CAGR for 5-year period

NN: Novo Nordisk; RHS: Right-hand side

Note: Annual sales figures for haemophilia A, B and bypassing agent segments, plasma derived products excluded Feiba®

Source: Company reports for haemophilia market; IQVIA MAT, May 2025; Note: Market values are based on the list prices

Sales growth of 18%, driven by the GLP-1 portfolio for diabetes and obesity treatment



Reported sales and growth breakdown for the first six months of 2025

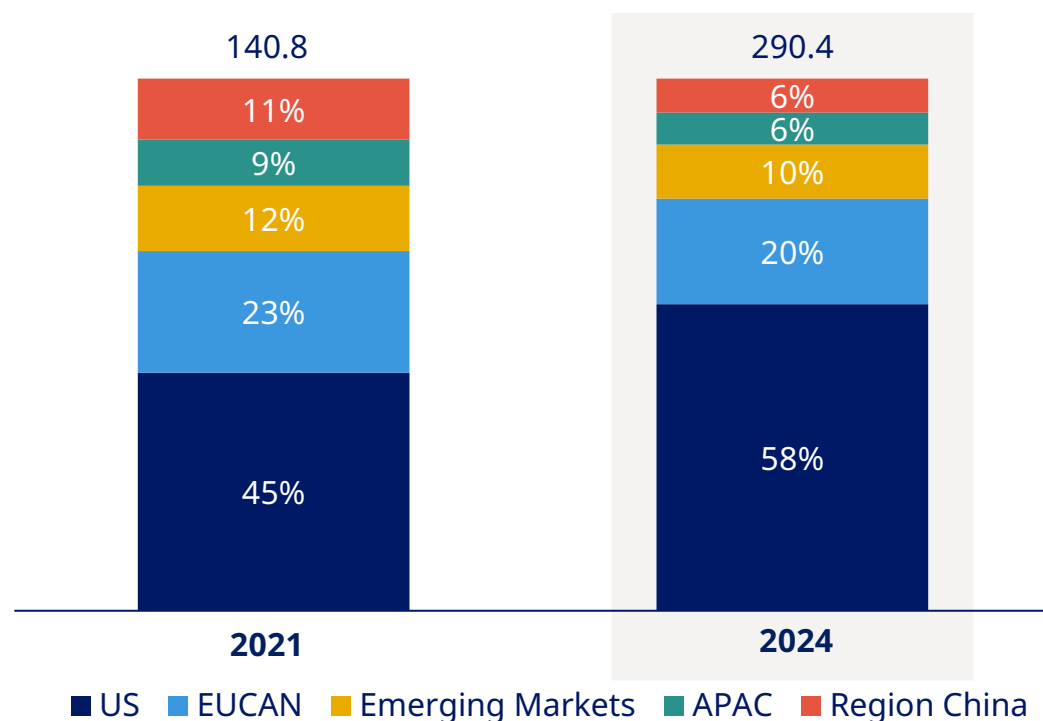
Therapy	Sales (mDKK)	Growth	Share of growth
Injectable GLP-1 ²	66,592	10%	27%
Rybelsus®	11,348	5%	3%
Total GLP-1	77,940	10%	30%
Total insulin³	27,743	4%	4%
Other Diabetes care ⁴	927	-16%	-1%
Total Diabetes care	106,610	8%	33%
Obesity care ⁵	38,796	58%	62%
Diabetes and Obesity care	145,406	18%	95%
Rare blood disorders ⁶	6,017	6%	1%
Rare endocrine disorders ⁷	2,732	49%	4%
Other Rare disease ⁸	789	4%	0%
Rare disease	9,538	15%	5%
Total	154,944	18%	100%

¹CAGR for 10-year period ²Comprises Victoza®, Ozempic® ³Comprises Awiqli®, Tresiba®, Xultophy®, Levemir®, Ryzodeg®, NovoMix®, Fiasp®, NovoRapid® and human insulin ⁴Primarily Novonorm®, needles and GlucaGen® HypoKit® ⁵Comprises Saxenda® and Wegovy® ⁶Comprises NovoSeven®, NovoEight®, NovoThirteen®, Refixia®, Esperoct® and Alhemo® ⁷Comprises Norditropin® and Sogroya® ⁸Primarily Vagifem® and ActiVelle®
Note: Sales numbers are reported in Danish kroner; Growth is at constant exchange rate; Refixia® and NovoThirteen® are launched as Rebinyn® and TRETEN®, respectively, in the US

Sales growth of 18%, driven by both US Operations and IO with 17% and 19% sales growth respectively

Historic and reported sales by geography

DKK billion



Reported sales and growth breakdown for first six months of 2025

Regions	Sales (mDKK)	Growth	Share of growth
International Operations	67,665	19%	46%
EUCAN	31,212	16%	19%
Emerging Markets	16,334	22%	13%
APAC	10,209	35%	12%
Region China	9,910	6%	2%
US Operations	87,279	17%	54%
Total sales	154,944	18%	100%

APAC: Japan, Korea, Oceania and Southeast Asia; China: Mainland China, Hong Kong and Taiwan; Emerging Markets: mainly Latin America, Middle East and Africa; EUCAN: Europe and Canada; IO: International Operations; US: United States

Note: Numbers may not add up to 100% due to rounding; Growth at Constant exchange rates; Sales numbers are reported in Danish kroner

Source: Quarterly company announcement

Novo Nordisk holds solid patent protection and competitive advantages

Novo Nordisk's position is protected by patents and value chain setup

EU/US patent protection¹

OZEMPIC® semaglutide injection	2031/32 ²
RYBELSUS® semaglutide tablets	2031/2032 ^{2,3}
Fiasp® fast-acting insulin aspart	2030 ⁴
esperoct® turoctocog alfa pegol	2034/32 ²
Xultophy® insulin degludec/liraglutide [rDNA origin] injection	2028/29
TRESIBA® insulin degludec [rDNA origin] injection	2028/29
RYZODEG® 70% insulin degludec and 30% insulin aspart [rDNA origin] injection	2028/29
refixia®	2027/28
ONCE-WEEKLY SOGROYA® somapacitan	2036/34

Novo Nordisk holds competitive advantages compared to biosimilars



Research & Development

- Need to show comparability in PK/PD trials
- Strict regulatory requirements in the EU and the US
- Requirement for both drug and device offering



Commercialisation

- Large and fragmented target audience
- Cost pressure from payers
- On-going conversion to next-generation drugs and slow market dynamics



Manufacturing

- Economies of scale
- Upfront CAPEX requirements with delayed ROI
- Decades of experience with high volume production of core yeast and mammalian API platforms

¹List does not include all marketed products ²Current estimates. Wegovy® patent identical to Ozempic® patent ³Tablet formulation and once-daily treatment regimen are protected by additional patents expiring in 2031-2034 ⁴Formulation patent; active ingredient patent has expired

API: Active pharmaceutical ingredient; CAPEX: Capital expenditure; PD: Pharmacodynamic; PK: Pharmacokinetic; ROI: Return on investment

Core capabilities together with additional drug modalities open up new opportunities across therapy areas

		Core Novo Nordisk capabilities			Modalities accelerated via partnerships & acquisitions	
		 Proteins/ Peptides/mAB	 siRNA	 Cell Therapy	 Small Molecules	 Gene Therapy
Therapy areas	Diabetes	✓	✓	✓		✓
	Obesity	✓	✓		✓	
	CVD	✓	✓	✓	✓	✓
	RBD	✓	✓		✓	✓
	MASH	✓	✓	✓		
	RED	✓	✓			✓
	CKD	✓	✓	✓	✓	
		✓ Active pipeline		✓ Exploratory		

CKD: Chronic kidney disease; CVD: Cardiovascular disease; mAB: Monoclonal antibody; MASH: Metabolic dysfunction-associated steatohepatitis; RBD: Rare blood disorders; RED: Rare endocrine disorders; siRNA: Small interfering ribonucleic acid
 Note: Currently active means Novo Nordisk is currently pursuing research projects, while exploratory indicates active early exploration activities and/or partnerships initiated

siRNA platform expected to deliver and mature across therapy areas in alignment with corporate strategy

Progress with the siRNA platform



12 phase 1 trial initiations with GalXC™ since 2017



Rivfloza™ the first Novo Nordisk siRNA drug, approved in 2023

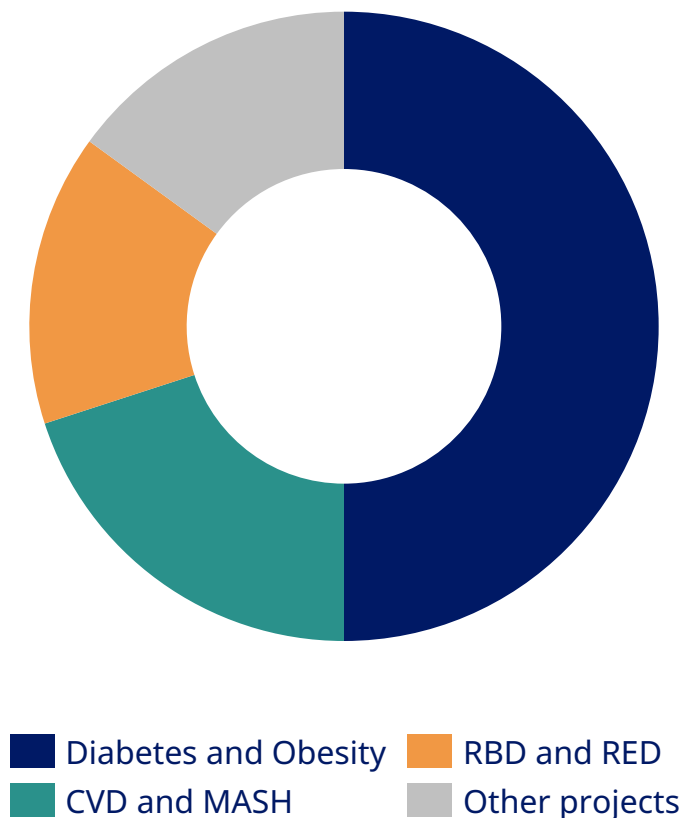


3 phase 1 trial initiations with GalXC-Plus™



More than 50% of upcoming phase 1 trials expected to be with GalXC-Plus™

Distribution of siRNA portfolio projects



Phase 1 initiation ambition with siRNA

3

... phase 1 initiations on average per year across disease areas with the siRNA platform is **on track**

Phase 1 aspiration of bringing more targets from research to development faster is on track for 2025

Key drivers increasing number of phase 1 initiations



Increased investments across portfolio



Target discovery engine delivers targets that are relevant to human disease

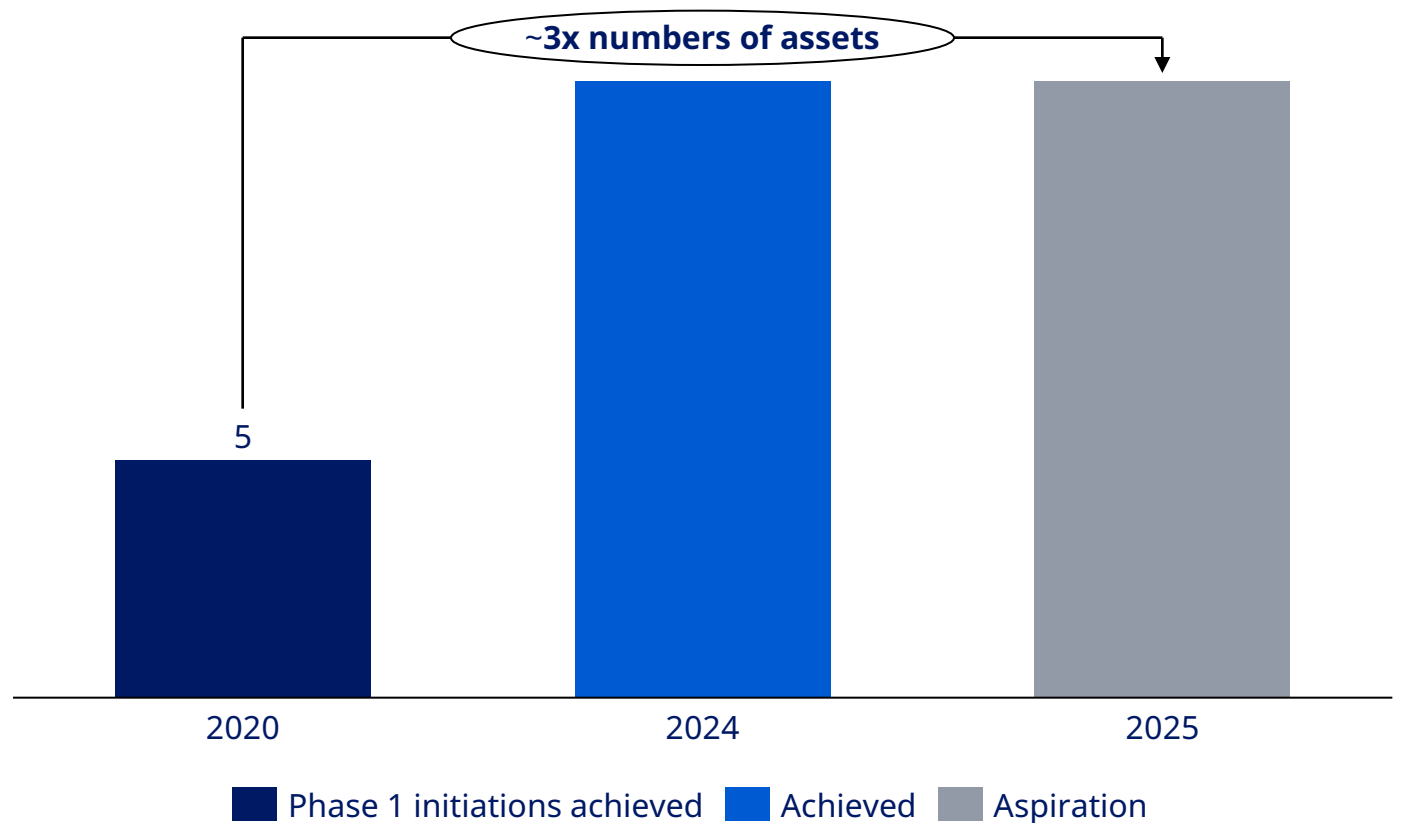


Leverage AI/digital capabilities throughout drug discovery process

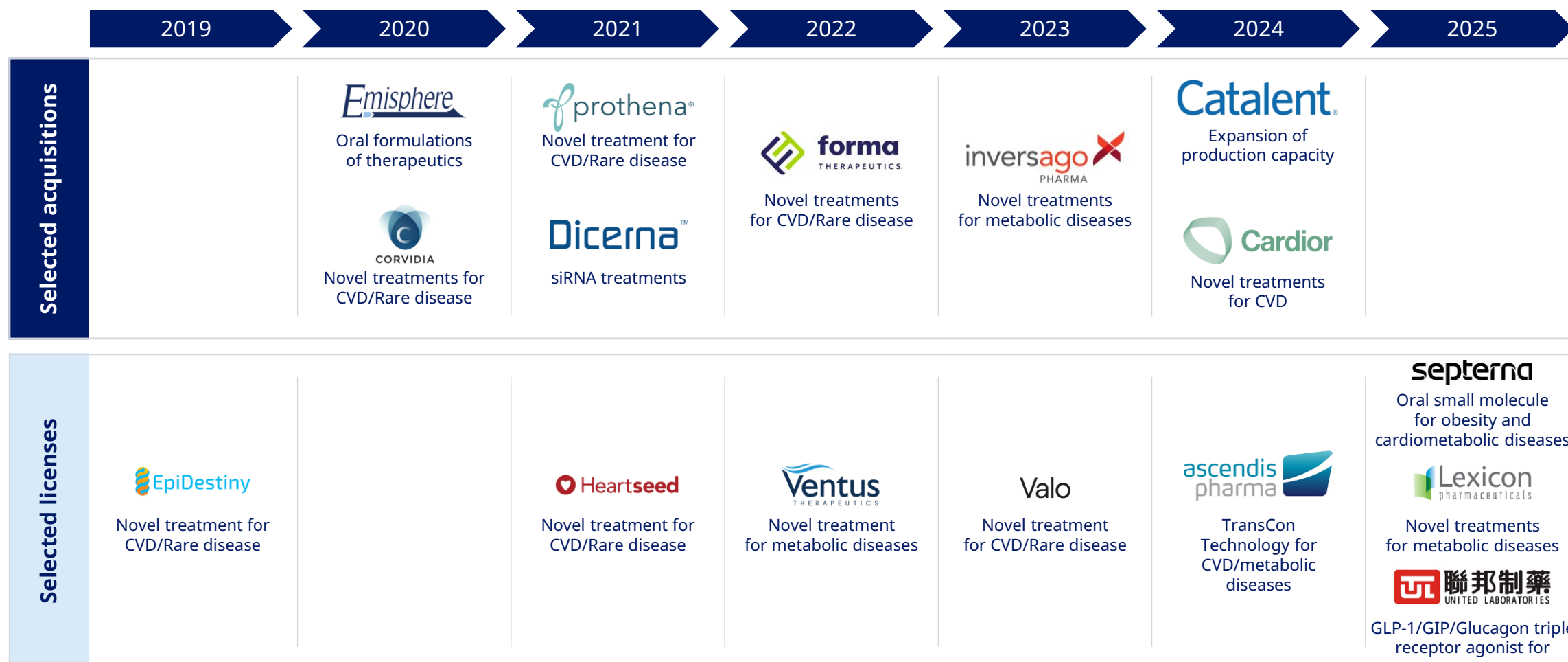


Early pipeline growth delivers more phase 1 opportunities

Number of phase 1 initiations in 2020 and aspirations towards 2025



Partnerships and acquisitions support future research and development



Pipeline supports significant growth opportunities across all four strategic focus areas

PHASE 1	PHASE 2	PHASE 3	SUBMITTED	APPROVED
NN1644 – GSI	NN9541 – OW GIP/GLP-1 co-agonist	NN9388 – CagriSema	NN1436 – Insulin Icodec ²	Tresiba®
NN1471 – Pumpsulin	NN9506 – FUSE ¹⁰	NN9924 – Oral Semaglutide 25 and 50 mg ¹	NN1535 – Icosema ¹	Xultophy®
NN9041 – DNA Immunotherapy	NN9440 – Monlunabant	NN9838 – CagriSema	SOUL – Oral semaglutide 14.0 mg CVOT ⁶	Awiqli® ⁵
NN9490 – Sc. Amycretin	NN9490 – Sc. Amycretin	NN9932 – Oral Semaglutide 25 and 50 mg ⁸	STRIDE – Semaglutide 1.0 mg in PAD	Levemir®
NN9487 – Oral Amycretin	NN9487 – Oral Amycretin	NN9536 – Semaglutide 7.2 mg ¹	STEP HFpEF – Semaglutide 2.4 mg ⁷	Ryzodeg®
NN9638 – Amylin 355	NN9440 – Monlunabant	NN6535 – Oral Semaglutide 14.0 mg in AD	NN9931 – Semaglutide 2.4 mg in MASH ⁹	NovoMix®
NN9839 – Amylin 1213	NN9505 – FUSE ¹⁰	NN6018 – Ziltivekimab in ASCVD	NN7415 – Concizumab, HA/HB ³	Fiasp®
NN9662 – Triple	NN6706 – CDR132L	NN6018 – Ziltivekimab in HFpEF		NovoRapid®
NN9559 – UBT251 (GGG tri-agonist)	NN6019 – ATTR Cardiomyopathy	NN6018 – Ziltivekimab in AMI		Rybelsus®
NN6582 – LXR(a) in MASH	NN7533 – NDec in SCD	NN7769 – Mim8 in HA		Ozempic®
NN6581 – MARC1 in MASH	NN7536 – Etavopivat in Thalassemia	NN7535 – Etavopivat in SCD		Victoza®
NN9003 – Stem Cells in HF		Other PHASE 3 trials		Wegovy®
NN9001 – Stem Cells in PD		FOCUS – Semaglutide 1.0 mg in diabetic retinopathy		Saxenda®
NN6022 – Ventus NLRP3i in CVD				NovoSeven®
NN6537 – CNP in HF				NovoEight®
NN6705 – NLRP3 in MASH				Esperoct®
NN7442 – Inno8				NovoThirteen®
NN7614 – TMPRSS6 RNAi				Refixia®
				Alhemo® ¹¹
				Rivfloza® ⁴
				Norditropin®
				Sogroya®

Diabetes care
 Obesity care
 Rare blood disorders
 Rare endocrine disorders
 Cardiovascular & Emerging therapy areas

¹Submitted to EMA ²CRL received in the US ³Submitted to EU for HA/HB ⁴Approved for PH1 by FDA ⁵Approved in the EU, China, Canada, Australia, Switzerland and Japan ⁶Submitted in US and EU ⁷Re-submitted in US with data from FLOW and SOUL in January 2025. STEP HFpEF label update reflected in EU label based on positive CHMP opinion received in Q3 2024 ⁸Submitted in US for 25 mg ⁹Submitted in US, Japan and EU ¹⁰In collaboration with GE Healthcare ¹¹Approved in US for Hwl and HA/HB and EU for Hwl
 AATLD: Alpha-1 Antitrypsin Deficiency-associated Liver Disease; AD: Alzheimer's Disease; ANGPTL3: Angiopoietin-like protein 3; AMI: Acute myocardial infarction; ASCVD: Atherosclerotic Cardiovascular Disease; ATTR: Transthyretin amyloidosis; CKD: chronic kidney disease; CVOT: Cardiovascular outcome trial; FGF-21: Fibroblast growth factor 21; GHD: Growth hormone disorder; GSI: Glucose Sensitive Insulin; HA: Haemophilia A; HF: Heart failure; HFpEF: heart failure with preserved ejection fraction; Hwl: Haemophilia with inhibitors; LXR(a): Liver X receptor alpha; MARC1: Mitochondrial amidoxime reducing component 1; MASH: Metabolic dysfunction-associated steatohepatitis; MDS: myelodysplastic syndrome; OM: Once monthly; OW: Once weekly; PAD: Peripheral arterial disease; PD: Parkinson's Disease; PH: Primary hyperoxaluria; SC: Subcutaneous; SCD: Sickle cell disease; Sema: Semaglutide

Diabetes care

Disease and market

GLP-1 segment

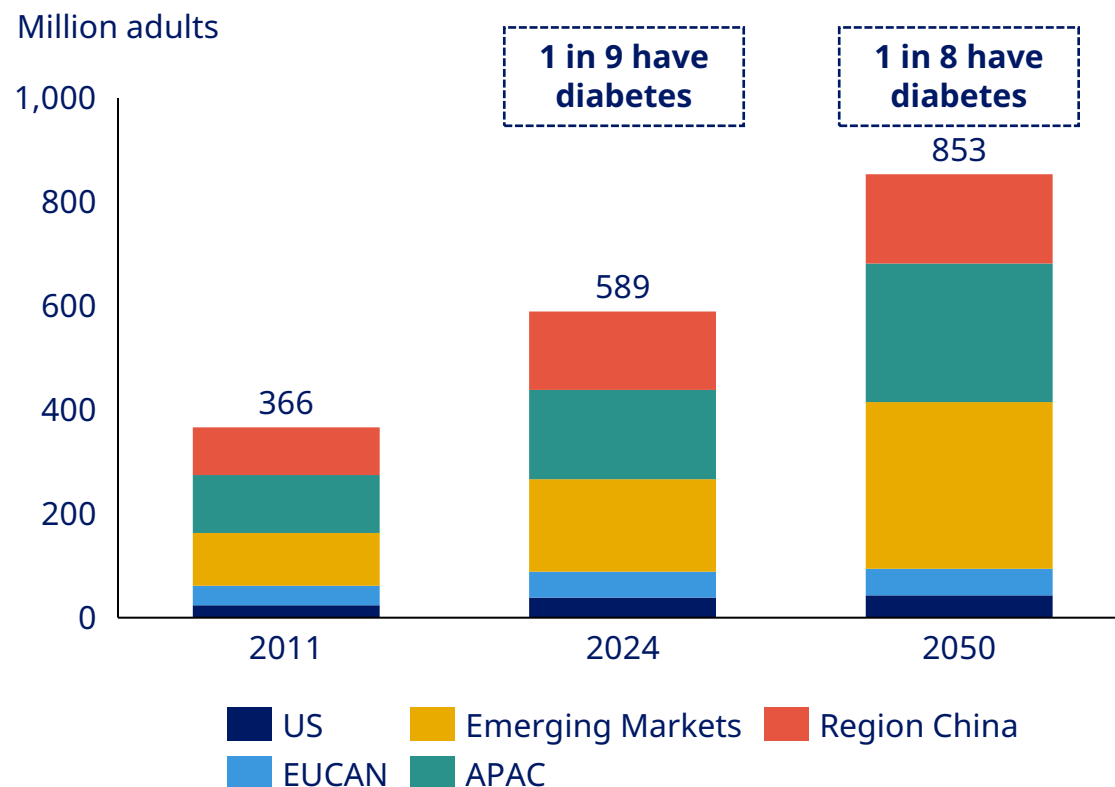
Insulin segment



SIMONE LENSBOLE
Simone lives with type 2 diabetes
Denmark

Diabetes is a serious chronic disease with increasing prevalence worldwide and multiple associated comorbidities

In 2050, ~850 million adults are expected to live with diabetes



High unmet medical need remains within T2D and the associated comorbidities¹



Mortality:

8 years shorter life expectancy



Cardiovascular disease:

>30% people with T2D affected



Chronic kidney disease:

up to ~40% of people with T2D affected²



Peripheral artery disease:

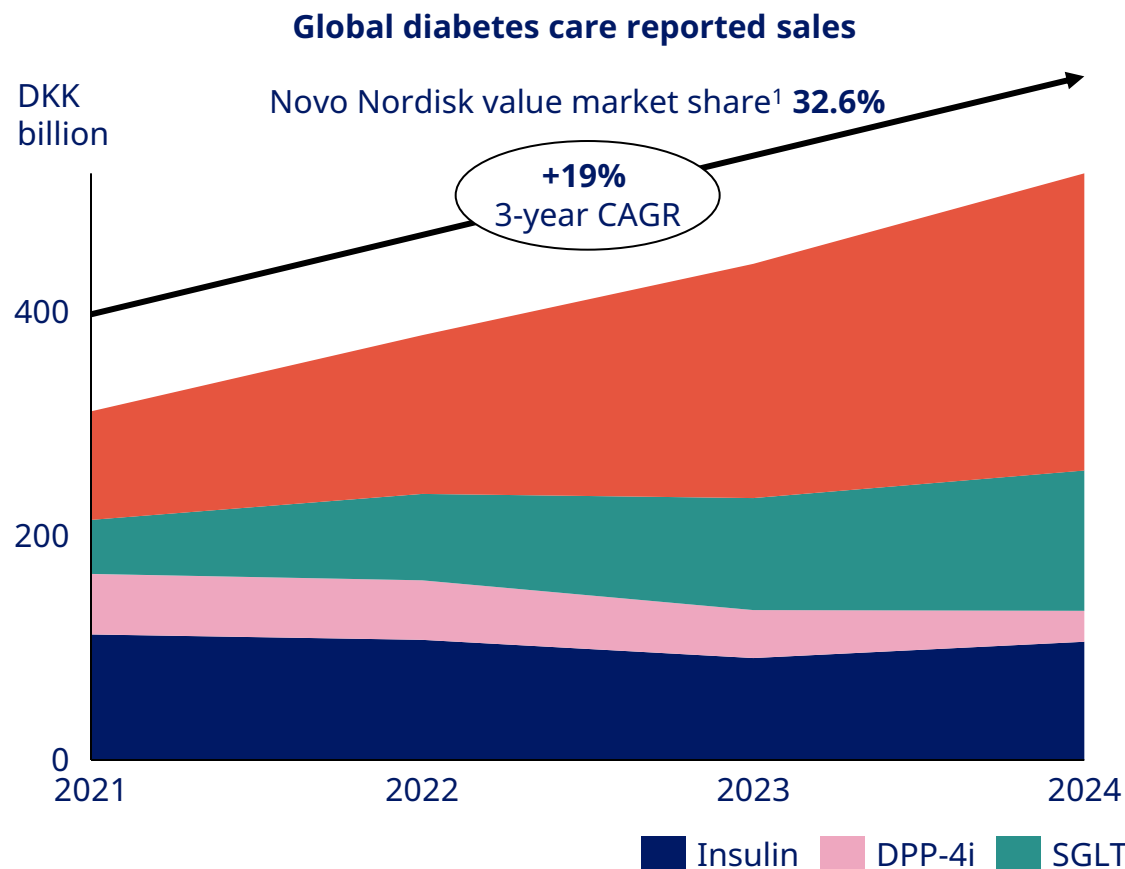
>200 million people affected globally of which 20-30% have T2D

¹ADA. Diabetes Care 2022;45:S1-S264; ²Cosentino F, et al. EJH 2020;41(2):255-323

APAC: Japan, Korea, Oceania and Southeast Asia; Emerging Markets: mainly Latin America, Middle East and Africa; EUCAN: Europe and Canada; Region China: Mainland China, Hong Kong and Taiwan; T2D: Type 2 diabetes; US: United States

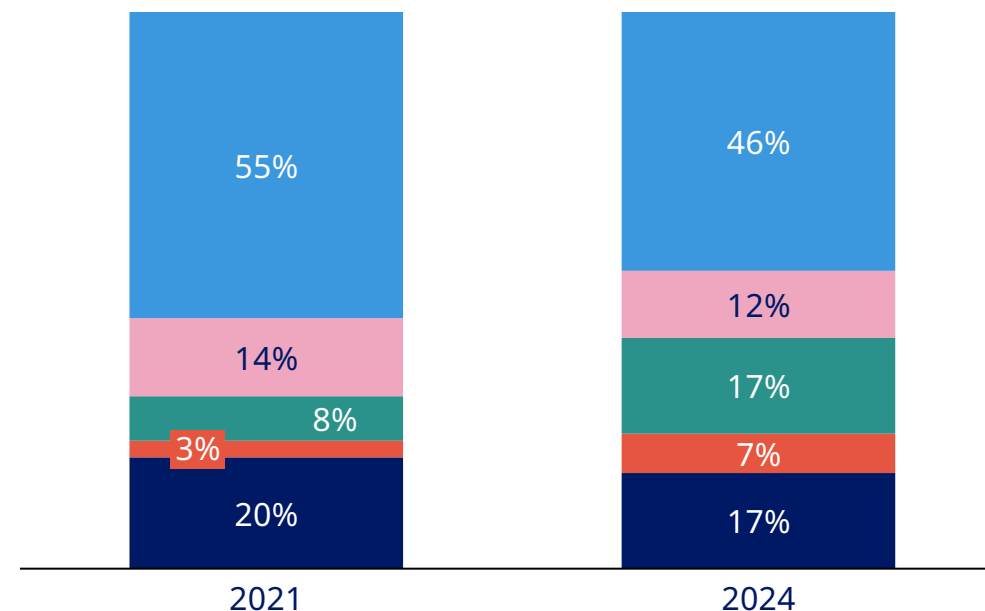
Source: Diabetes Atlas 11th edition, 2025

Novo Nordisk is the global leader in the growing diabetes market



Volume growing ~6% with more people using GLP-1s and SGLT-2is

Estimated prescription share per treatment category²



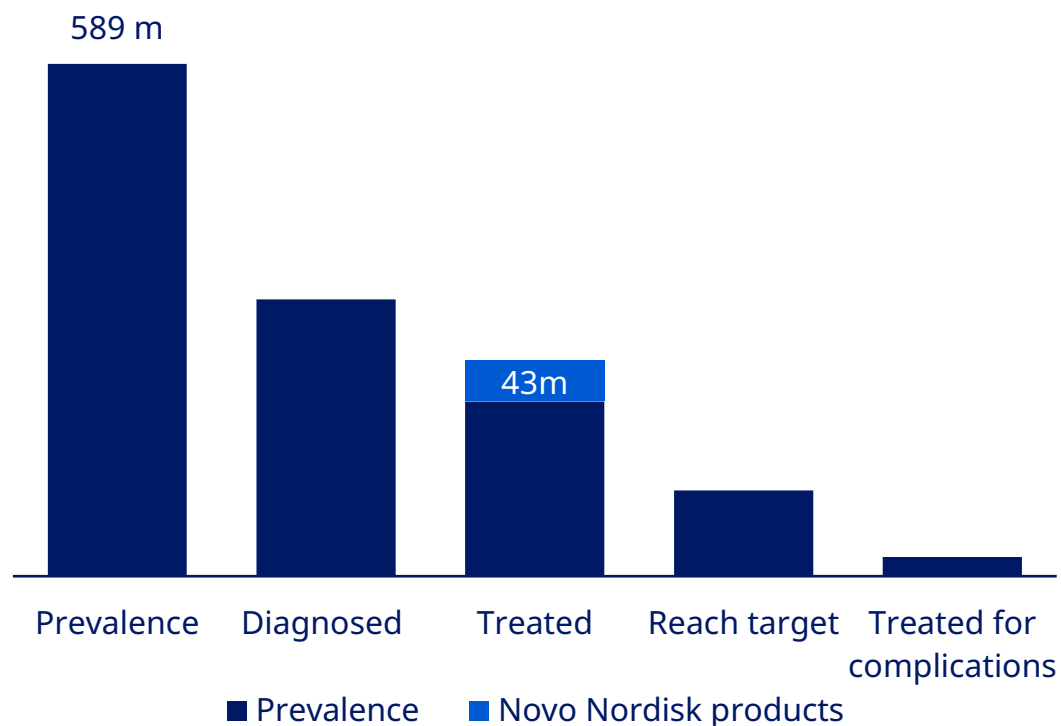
¹Based on IQVIA MAT, May 2025; ²2025 does not add to 100% due to rounding

CAGR: Compound annual growth rate; DPP-4i: Dipeptidyl peptidase 4 inhibitor; OAD: Oral anti-diabetic; SGLT-2i: sodium-glucose co-transporter-2 inhibitor; SU: Sulfonylurea; Trad.: Traditional; TZD: Thiazolidinedione
Note: GLP-1 + basal insulin combination sales are included in insulin; Traditional OADs include metformin, SU and TZDs

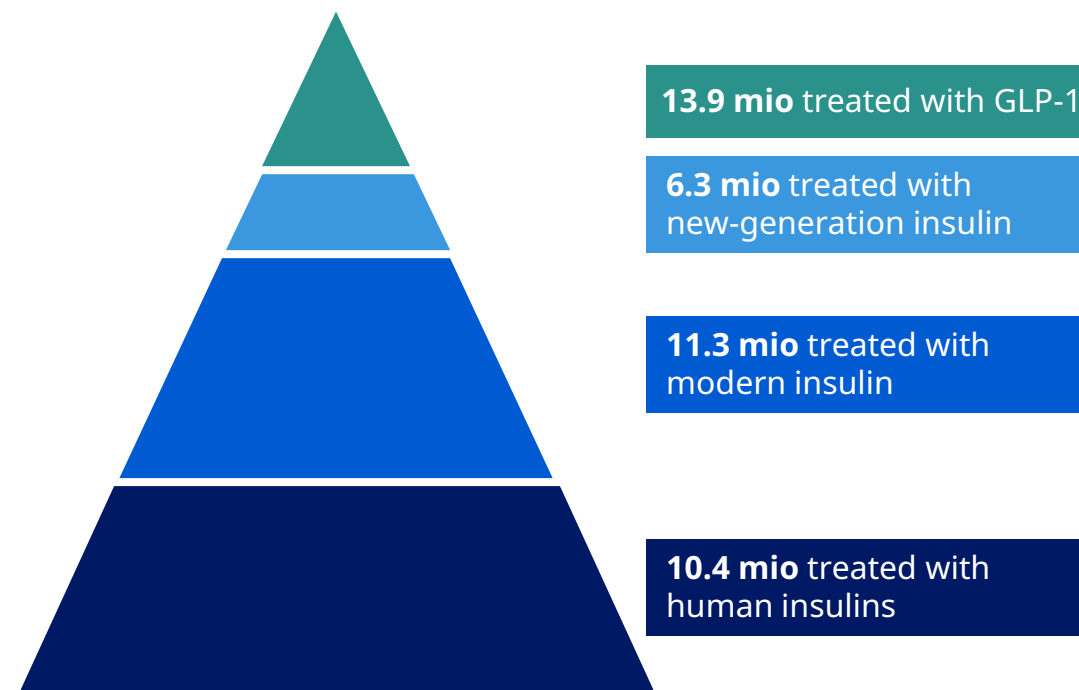
Source: Company reported sales for insulin, GLP-1, SGLT-2i and DPP-4i, 2024 vs 2023; Estimated patient share, IQVIA MAT, Feb 2025

The unmet need within diabetes care remains large with too few patients reaching glycaemic target and treated for complications

1 in 2 adults go undiagnosed and more treated patients should reach their HbA_{1c} target



Of the 589 million, 43.0 million¹ people are treated with Novo Nordisk diabetes products



Source: Diabetes prevalence and diagnosed are based on Diabetes Atlas 11th edition, 2025; Treated is based on IQVIA patient data; real-world studies indicate between 30-55% of patients reach HbA_{1c} target <7% .e.g. <https://www.ncbi.nlm.nih.gov/pmc/articles/PMC4388968/>

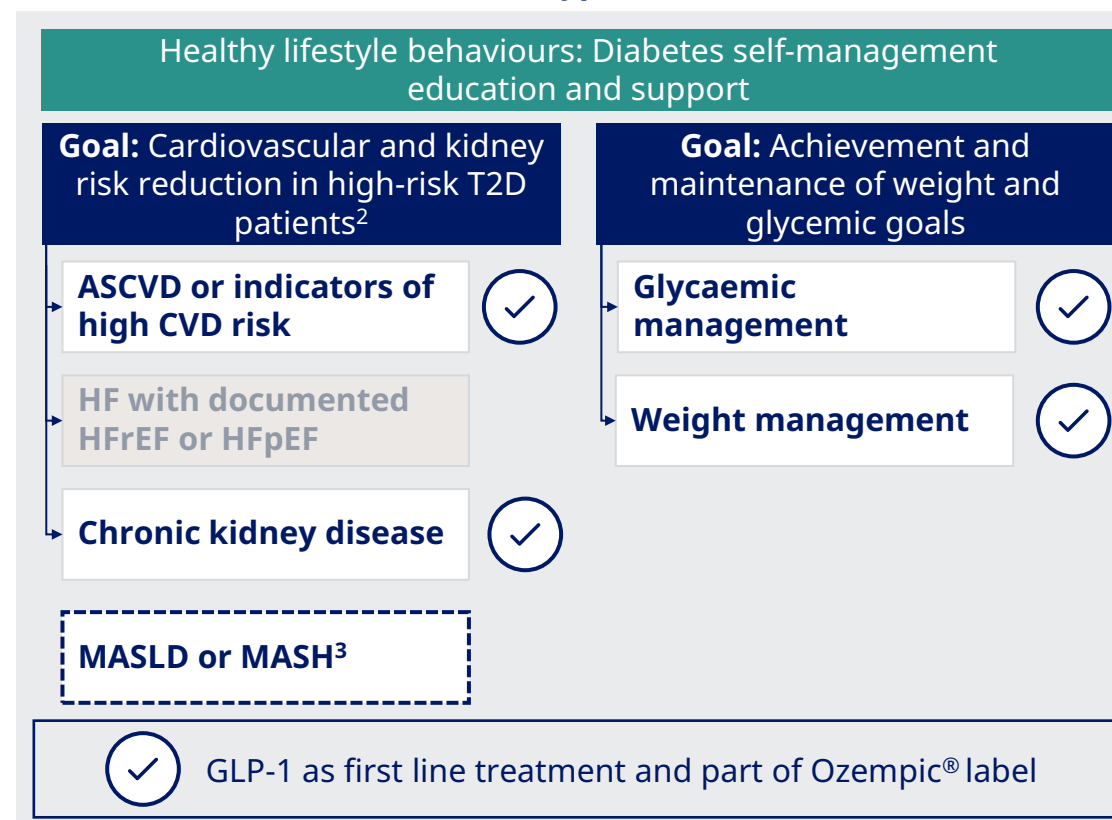
¹In addition to the above-mentioned product classes, other diabetes care constitutes the remainder of people treated with Novo Nordisk products; Estimated number for full-year 2024 (total available in Novo Nordisk Annual Report 2024)
Source: Novo Nordisk Annual Report 2024 (WHO designated daily dose methodology is applied to convert sales into patients reach)

GLP-1s have positive effects beyond glycaemic control reflected in the treatment guidelines

Medications for treatment of type 2 diabetes

Class	Efficacy	Hypo risk	Weight change	Cardiovascular effects	
				ASCVD	HF
Metformin	High	No	Neutral	Potential Benefit	Neutral
Sulfonylurea	High	Yes	Gain	Neutral	Neutral
TZDs	High	No	Gain	Potential Benefit	Increased risk
DPP-IV inhibitors	Intermediate	No	Neutral	Neutral	Potential risk
SGLT-2 inhibitors	Intermediate	No	Loss	Benefit	Benefit
GLP-1	High	No	Loss	Benefit/Neutral ¹	Neutral
Long-acting insulin	High	Yes	Gain	Neutral	Neutral
Fast-acting insulin	High	Yes	Gain	Neutral	Neutral

2025 ADA guidelines for pharmacologic treatment of adults with type 2 diabetes



¹Benefit: dulaglutide, liraglutide, semaglutide; Neutral: exenatide once weekly, lixisenatide; ²eGFR < 60 mL/min/1.73 m² OR albuminuria (ACR ≥ 3.0 mg/mmol (30mg/g)). Repeat measurement is required to confirm CKD; ³If additional CV/kidney risk reduction/management of other metabolic comorbidities/glycemic lowering is needed

ADA: American Diabetes Association; ASCVD: Atherosclerotic cardiovascular disease; CKD: Chronic kidney disease; CVD: Cardiovascular disease; EASD: European Association for the Study of Diabetes; FDA: The US Food and Drug Administration; HbA_{1c}: Haemoglobin A_{1c}; HF: Heart failure; HFrEF: Heart failure with reduced ejection fraction; HFpEF: Heart failure with preserved ejection fraction; Hypo: Hypoglycaemia; MASH: Metabolic dysfunction-associated steatohepatitis; MASLD: metabolic dysfunction-associated steatotic liver disease; TZDs: Thiazolidinediones; T2D: Type 2 Diabetes; US: United States

Source: Adapted from: "Standards of Medical Care in Diabetes – 2022" Supplement 1, p.133; diabetes.org. American Diabetes Association.

Innovation is the focus for strengthening leadership in diabetes

Approach to diabetes innovation



Expand focus beyond HbA_{1c} to
cardiometabolic and renal
outcomes



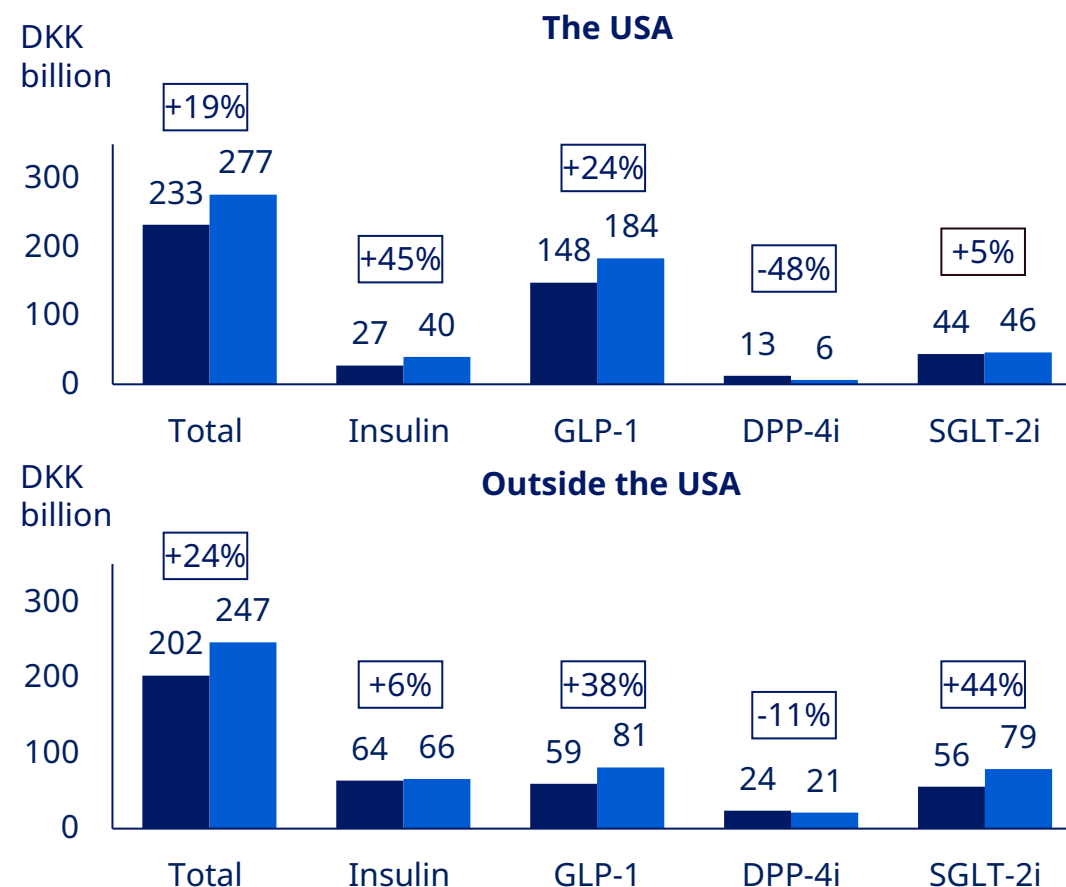
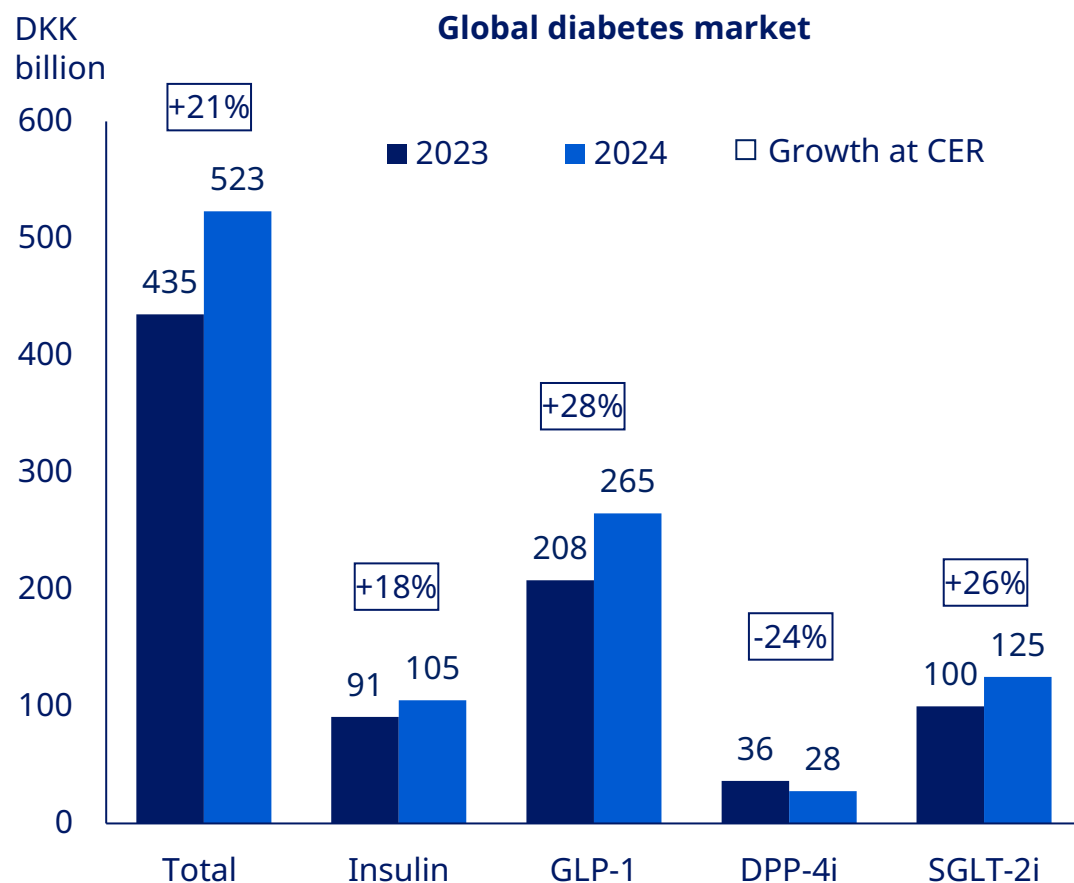
Continue exploring preventative
and curative treatments

Novo Nordisk's product portfolio covers all three treatment segments

Key products	Oral anti-diabetic	Injectable GLP-1	Insulins
	 semaglutide tablets	 ONCE-WEEKLY semaglutide injection	Icodec¹ Once-weekly insulin IcoSema¹
		 liraglutide injection	  insulin degludec (rDNA origin) injection fast-acting insulin aspart
Pipeline ²	Oral semaglutide 25/50 mg³ Oral amycretin	CagriSema Sc amycretin OW GLP-1/GIP	 

¹Currently under regulatory approval; ²Pipeline references phase 2 ready and phase 3 assets ³Oral semaglutide 25 mg submitted in US
 GIP: Gastric inhibitory polypeptide; HbA_{1c}: Haemoglobin A_{1c}; OW: Once-weekly; Sc: Subcutaneous

The total branded diabetes market has a global value of DKK ~523 billion annually

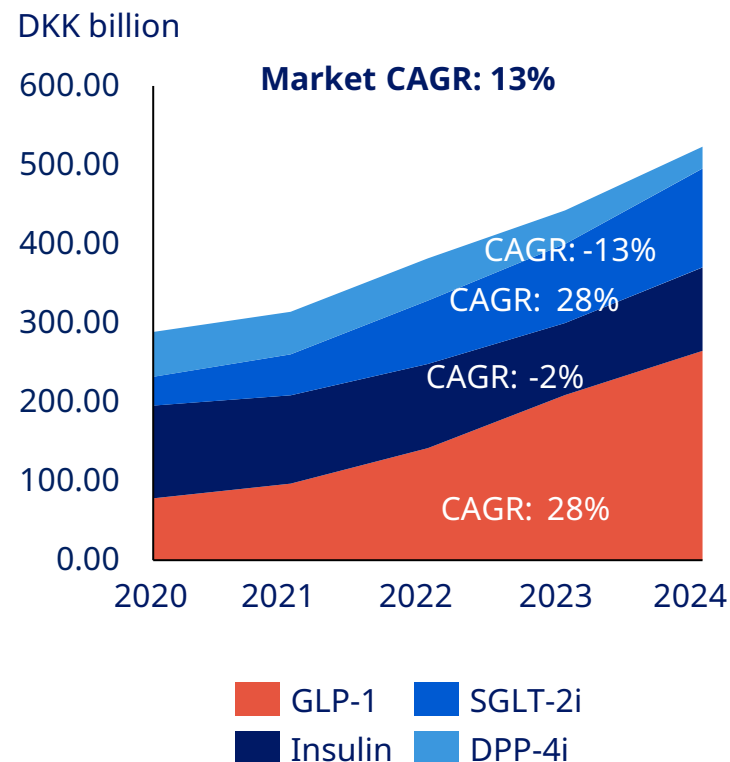


Note: The segment value is based on reported figures, whilst the market growth is under constant exchange rate (CER). For Novo Nordisk the diabetes growth includes Insulin and GLP-1, excluding 'other diabetes care'.

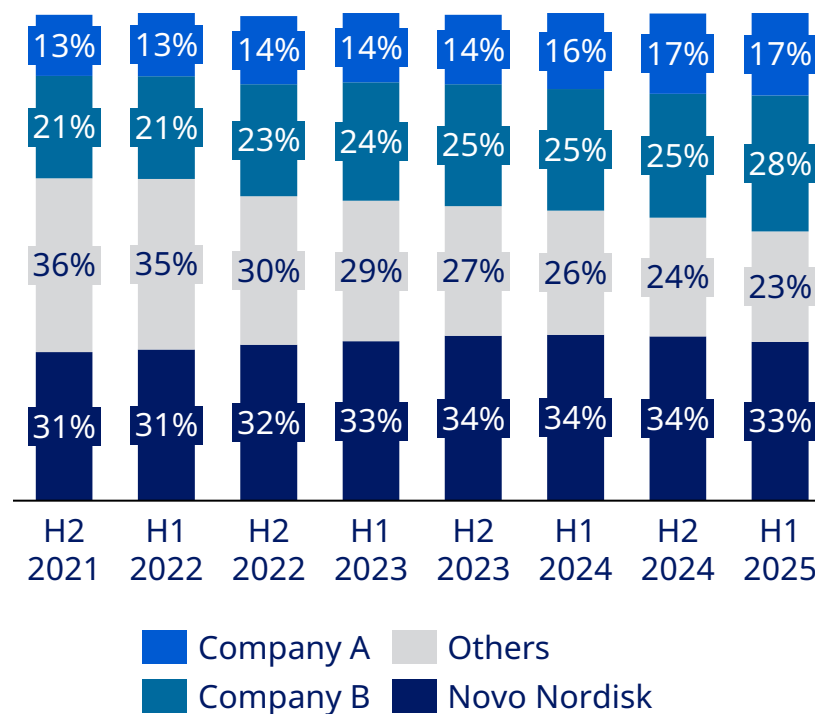
Source: Company announcements as of Q4 2024; 2024 data based on Q1 2024 to Q4 2024 and 2023 data based on Q1 2023 to Q4 2023

Novo Nordisk has a leadership position within the growing diabetes market

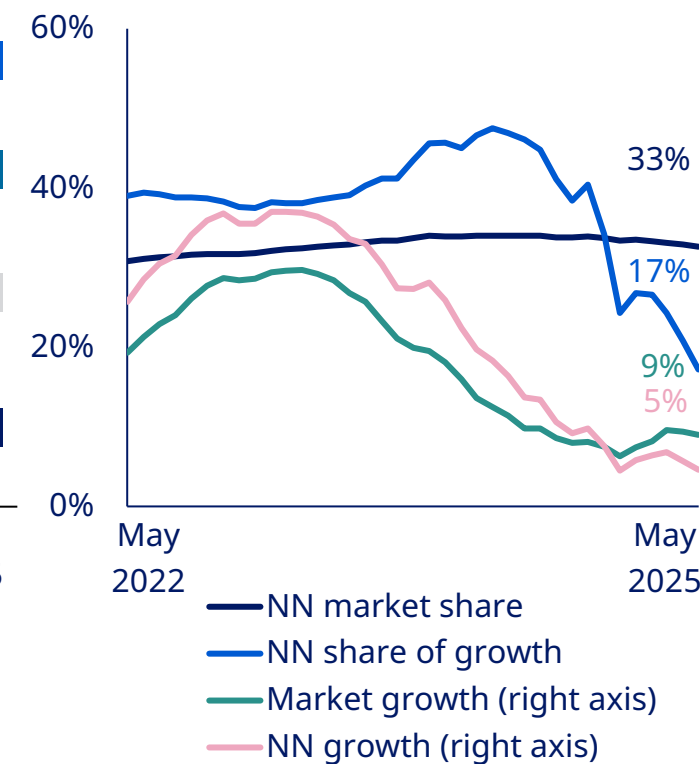
Global diabetes market by treatment class¹



Novo Nordisk remains global diabetes value market leader



Novo Nordisk market share and share of growth



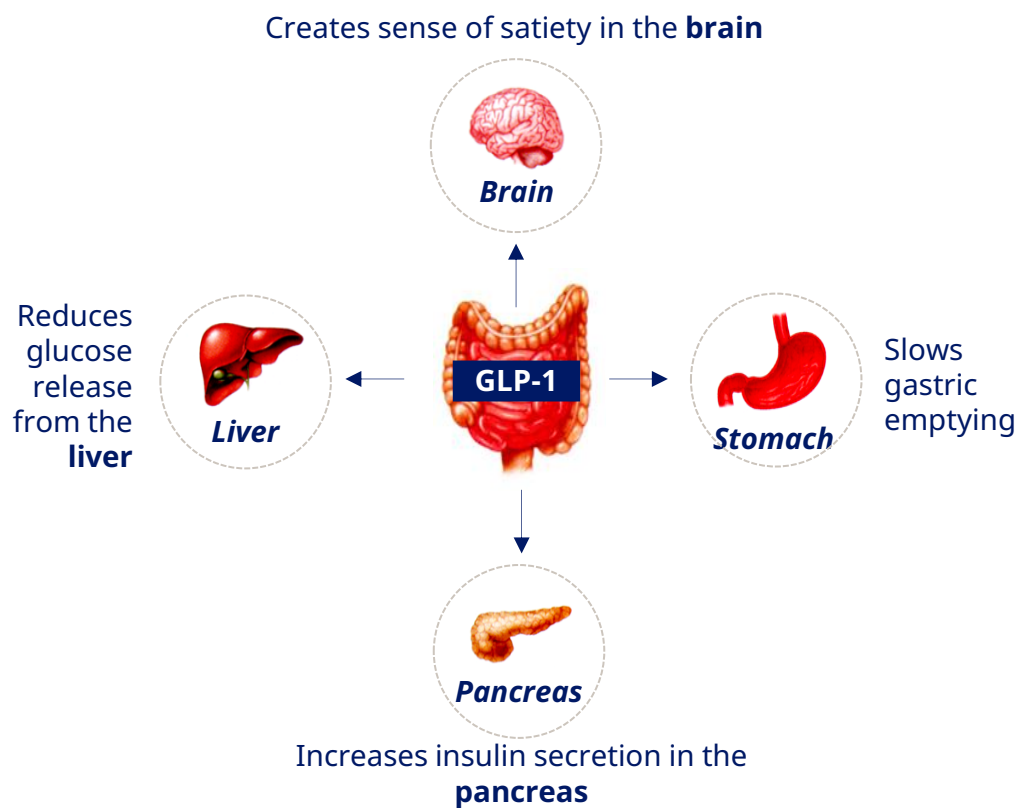
¹Data is based on company reported sales. Data does not include Galvus and generic metformin, sulphonylureas or thiazolidinedione

NN: Novo Nordisk

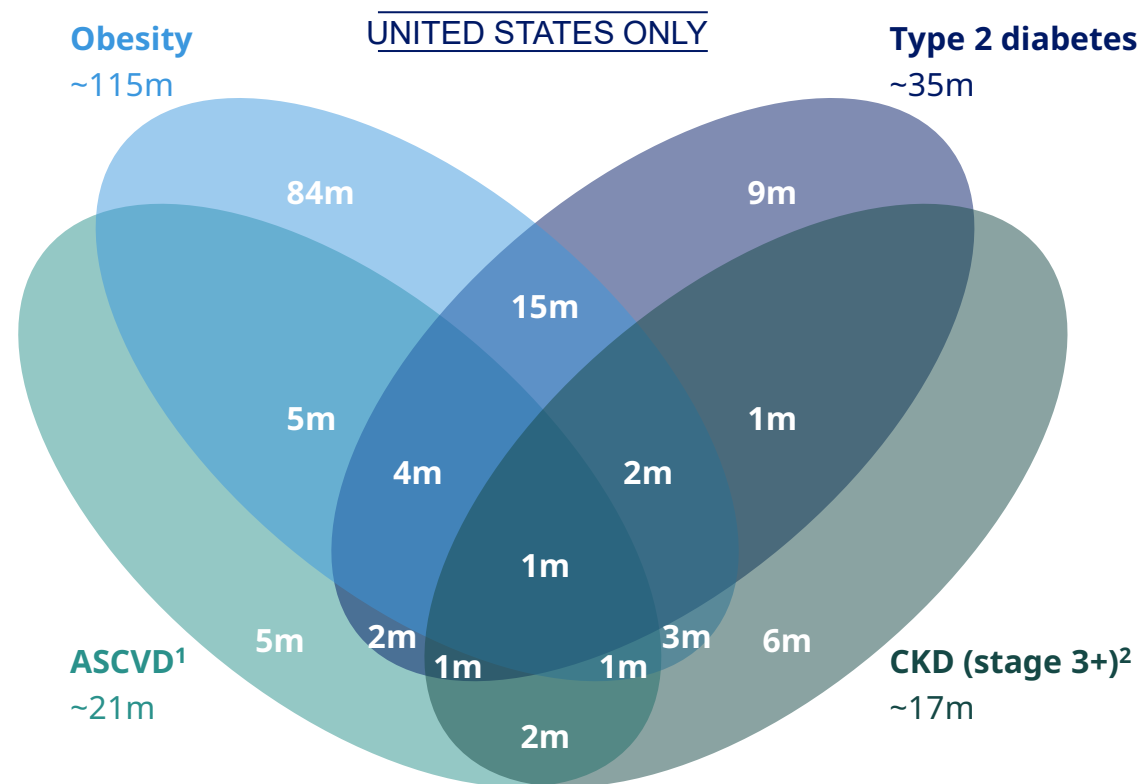
Source: IQVIA May 2025 value figures Note: IQVIA data can be inflated due to use of list prices. Due to contractual obligations competitor names are not disclosed. Company A and B represent actual companies

GLP-1 mechanism of action and potential therapeutic opportunities

GLP-1 mechanism of action



Patient overlaps for key focus areas in type 2 diabetes



¹Myocardial infarction, stroke and coronary heart disease ²eGFR <60 ml/min/1.73m² ³On top of cardiovascular standard of care

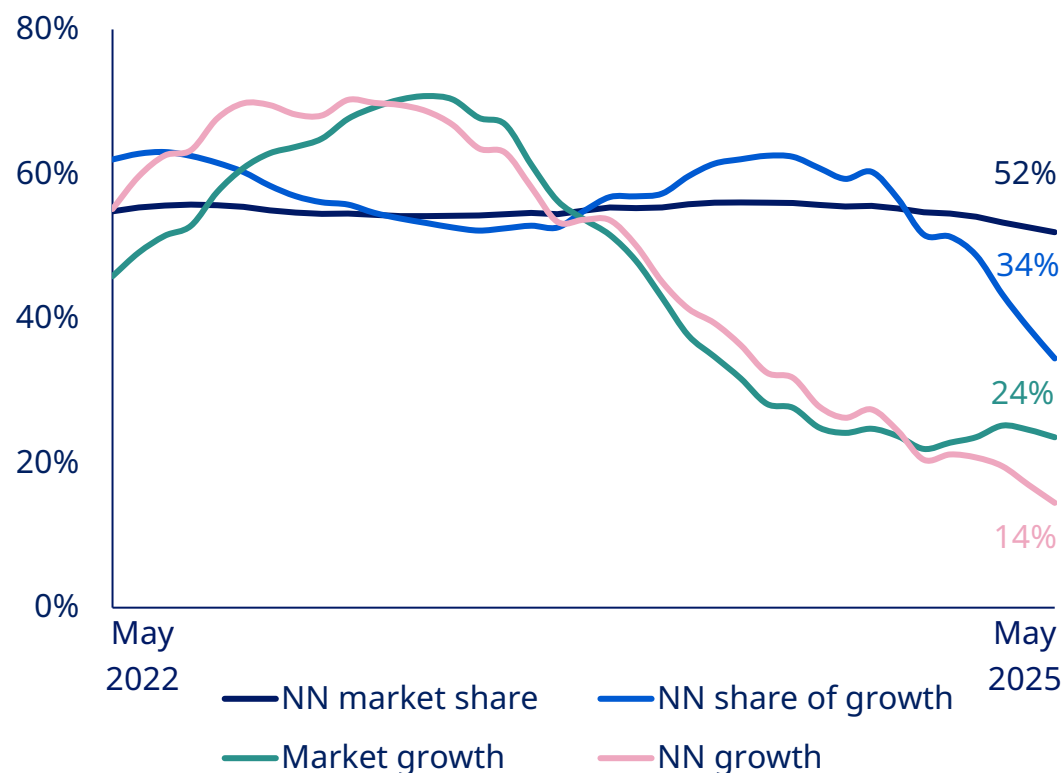
ADA: American Diabetes Association; ASCVD: Atherosclerotic cardiovascular disease; CKD: Chronic kidney disease; CV: Cardiovascular; EASD: European Association for the Study of Diabetes; HbA_{1c}: Haemoglobin A_{1c}; HF: Heart failure; HFREF: Heart failure with reduced ejection fraction; HFpEF: Heart failure with preserved ejection fraction

Note: Prevalence overlaps have been estimated on patient-level data from NHANES. Post-estimation adjustments have been undertaken to match certain key metrics as reported by publicly available sources. Numbers are rounded

Source: NHANES (waves 2003-2004, 2013-2014, 2015-2016 and 2017-2020); UN World Population Prospects 2022; International Diabetes Federation: Diabetes Atlas 10th edition, 2021; World Obesity Atlas 2023

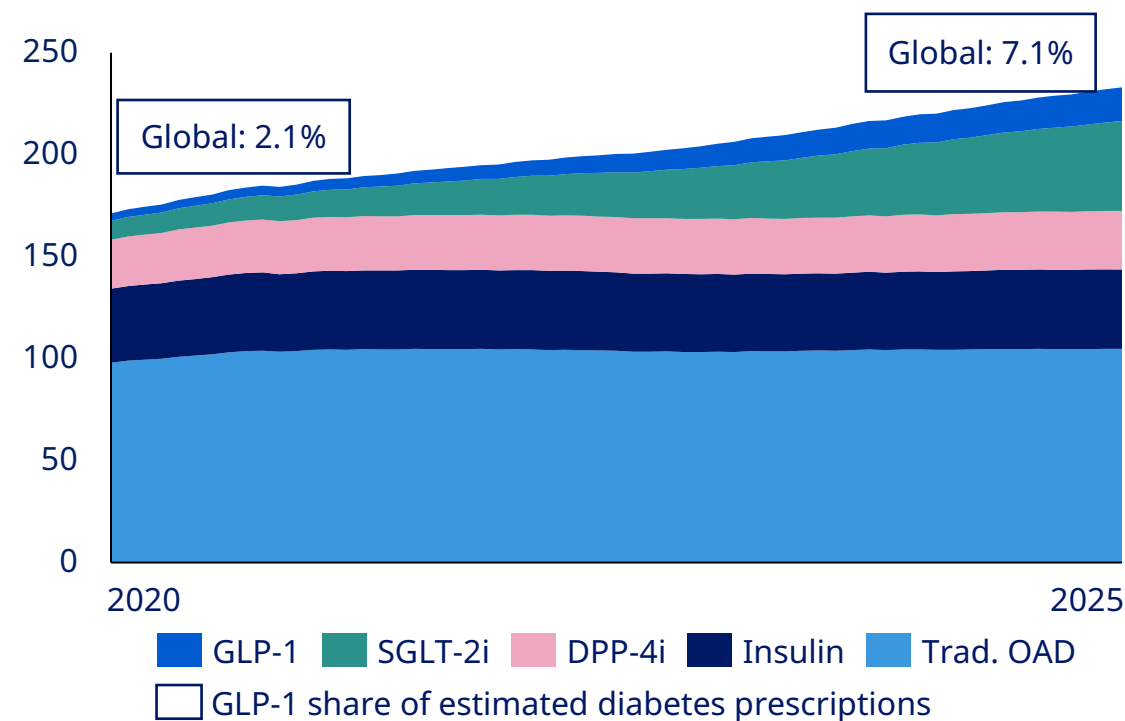
Novo Nordisk has 52% of the global GLP-1 market, while GLP-1 penetration of diabetes volume varies across regions

GLP-1 market growth and Novo Nordisk market share



GLP-1 share of total estimated diabetes prescriptions¹ is 7.1%

Million prescriptions¹

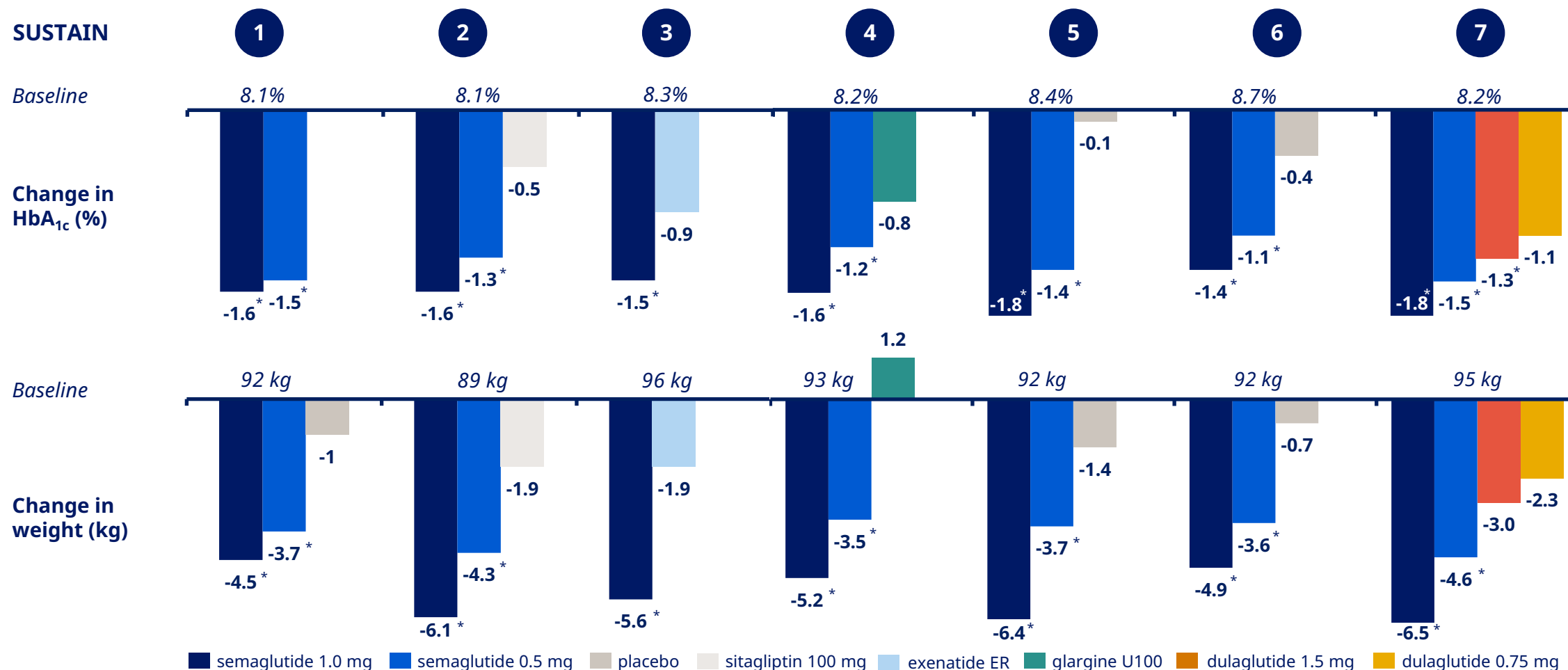


¹The estimated GLP-1 share of prescriptions is based on volume packs from IQVIA. Volume packs are converted into full-year patients/prescriptions based on WHO assumptions for average daily doses or if not available, Novo Nordisk assumptions

Source: IQVIA MAT volume (Spot rate), May 2025; Market values are based on the list prices

Source: IQVIA MAT value (spot rate), May 2025; Market values are based on the list prices

SUSTAIN trials with subcutaneous semaglutide



*Statistically significant; SUSTAIN 1: QW sema vs placebo in drug-naïve people with T2D; SUSTAIN 2: QW sema vs sitagliptin 100 mg QD in people with T2D added to 1-2 OADs; SUSTAIN 3: QW sema vs QW exenatide ER 2.0 mg in people with T2D added to 1-2 OADs; SUSTAIN 4: QW sema vs QD insulin glargine in people with T2D added to 1-2 OADs; SUSTAIN 5: QW sema vs placebo in people with T2D added to insulin; SUSTAIN 6: QW sema vs placebo, added to standard-of-care; SUSTAIN 7: QW sema vs QW dulaglutide 75 mg and 150 mg in people with T2D added to 1-2 OADs. ER: Extended-release; QW: once-weekly; QD: once-daily; sema: semaglutide; T2D: type 2 diabetes, OAD: oral anti-diabetics

Semaglutide 2.0 mg s.c. brings patients needing treatment intensification to target

Phase 3 trial, SUSTAIN FORTE, completed and label application approved in the US and the EU

Estimand	Trial product estimand		Treatment policy estimand	
Once-weekly semaglutide	2.0 mg	1.0 mg	2.0 mg	1.0 mg
HbA _{1c} reduction	2.2%*	1.9%	2.1%*	1.9%
Body weight reduction (kg)	-6.9*	-6.0	-6.4	-5.6
HbA _{1c} < 7.0% ¹	68%	58%		

¹ADA recommended treatment target

*Statistically significant

S.c.: subcutaneous

Data from SUSTAIN FORTE



Semaglutide 2.0 mg showed superior HbA_{1c} reduction with more patients reaching target¹ versus semaglutide 1.0 mg

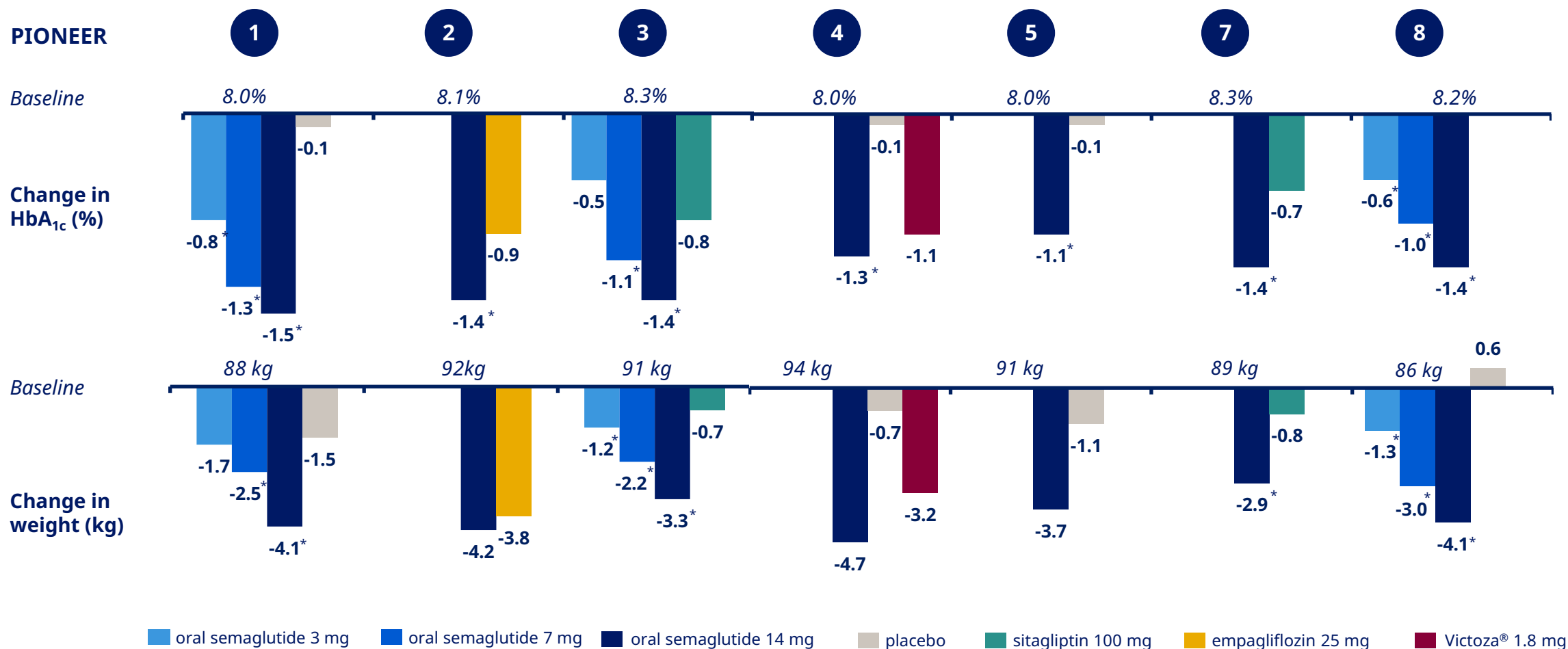


Semaglutide 2.0 mg appeared to have a safe and well-tolerated profile
Gastrointestinal adverse events were similar for semaglutide 1.0 mg and 2.0 mg



Label expansion application approved in the US, JP and the EU

PIONEER programme with oral semaglutide

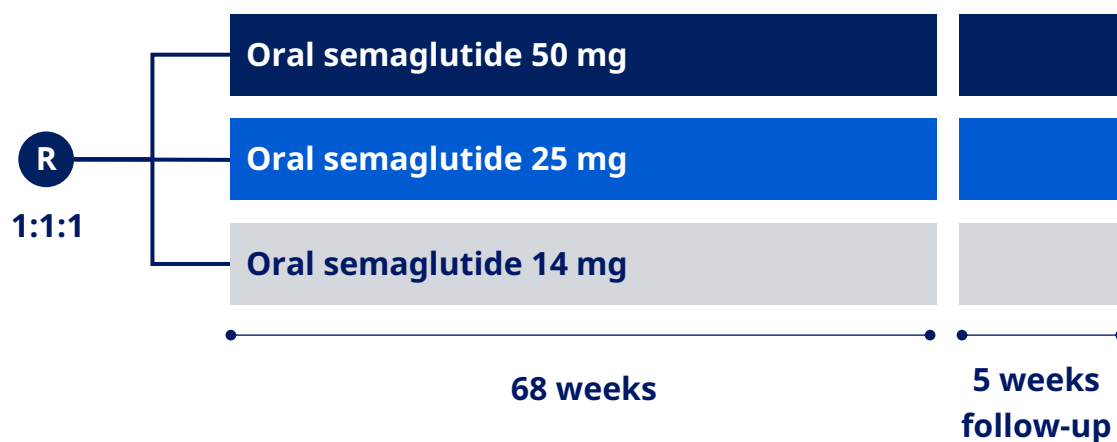


ER: Extended-release; QW: once-weekly; QD: once-daily; oral sema: oral semaglutide; T2D: type 2 diabetes, OAD: oral anti-diabetics; CV: Cardiovascular

Note: PIONEER 9 and PIONEER 10 were Japanese studies and PIONEER 6 was a CV safety study. * Statistically significant based on the trial product estimand; PIONEER 1: QD oral sema vs placebo in people with T2D treated with diet and exercise only; PIONEER 2: QD oral sema vs empagliflozin 25 mg in people with T2D; PIONEER 3: QD oral sema vs sitagliptin 100 mg in people with T2D; PIONEER 4: QD oral sema vs Victoza® 1.8 mg and placebo in people with T2D; PIONEER 5: QD oral sema vs placebo in people with T2D and moderate renal impairment; PIONEER 7: QD oral sema using a flexible dose adjustment based on clinical evaluation vs sitagliptin 100 mg in people with T2D; PIONEER 8: Effects of QD oral sema vs placebo in people with long duration of T2D treated with insulin

PIONEER PLUS achieved its primary endpoint and demonstrated statistically significant HbA_{1c} reduction vs oral sema 14 mg

Oral semaglutide 25 mg and 50 mg vs 14 mg in subjects with T2D



Primary endpoint:

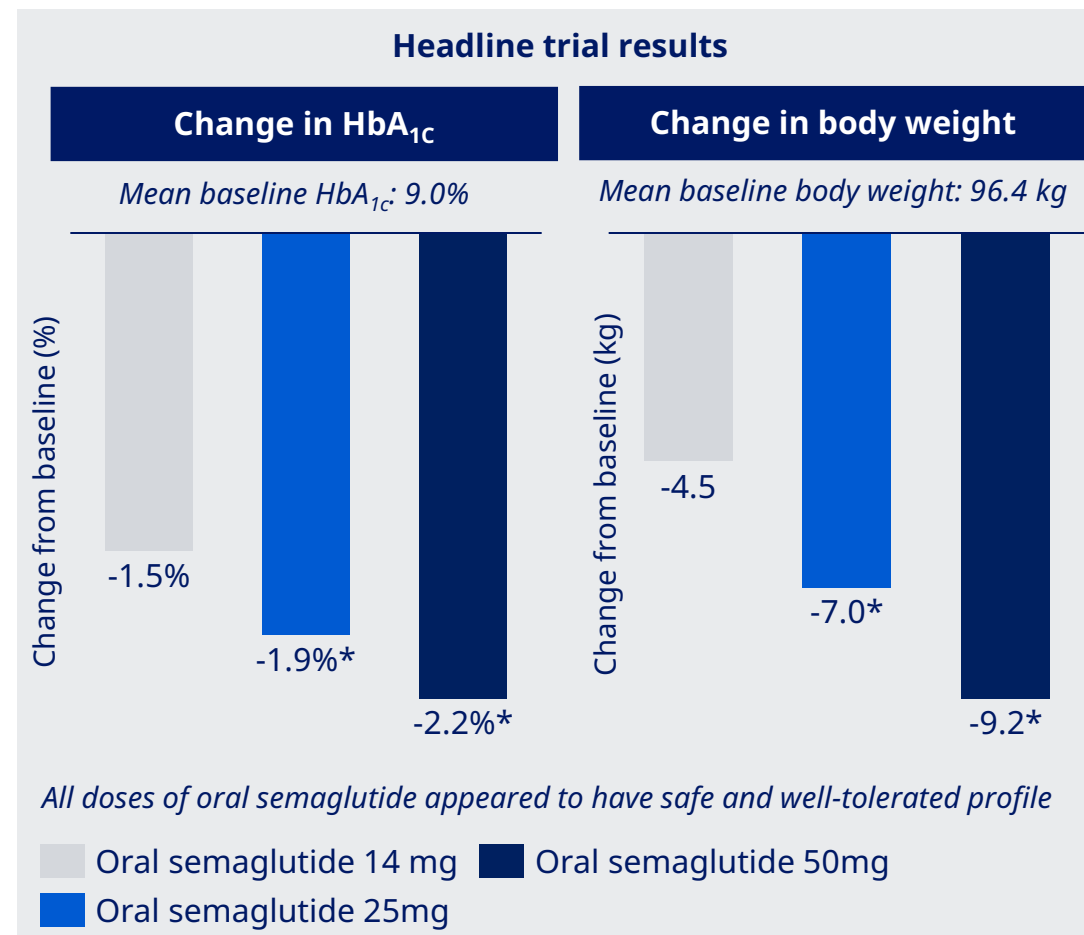
- Change from baseline to week 52 in HbA_{1c}

Secondary endpoint:

- Change from baseline to week 52 in body weight

Inclusion criteria (1,606 participants):

- Type 2 Diabetes
- HbA_{1c} 8.0 - 10.5%
- BMI ≥25 kg/m²
- Stable dose of 1-3 OADs (metformin, SU, SGLT-2i or DPP-4i¹)

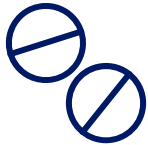


*Statistically significant/superior vs oral semaglutide 14 mg; ¹DPP-4i terminated at randomization

T2D: Type 2 diabetes; HbA_{1c}: Glycated haemoglobin; BMI: Body Mass Index; OADs: Oral antidiabetic drugs; SU: Sulfonylurea; SGLT-2i: Sodium-glucose cotransporter-2 inhibitors; DPP-4i: dipeptidyl peptidase-4 inhibitors

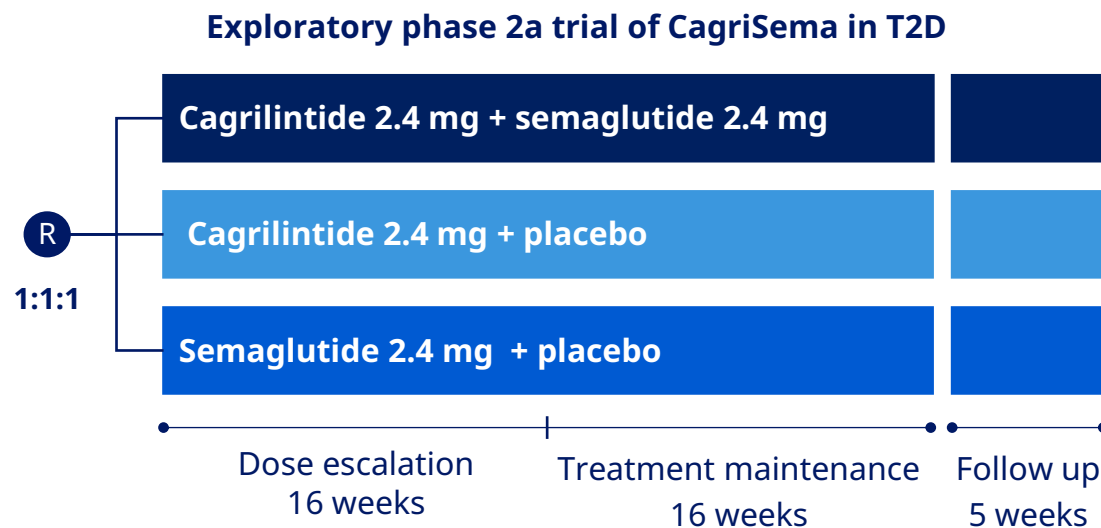
Note: Trial product estimands shown; Trial objective: To compare the safety and efficacy of 25 and 50 mg oral semaglutide with 14 mg oral semaglutide once daily in people with type 2 diabetes

Semaglutide has produced a comprehensive body of evidence and clinical outcome data for a GLP-1 in type 2 diabetes

 Semaglutide sc 1.0 and 2.0 mg	Glycaemic control*	MACE outcome	PAD outcome
	2.2%-p Reduction HbA _{1c} ¹ SUSTAIN FORTE	26% Reduction in MACE ² SUSTAIN-6	13% Improvement in MWD ³ STRIDE
	Body weight* 	Kidney outcome 	All-cause mortality
	7.2% Reduction in body weight ¹ SUSTAIN FORTE	24% Reduction in Major Kidney Disease Events ⁴ FLOW	20% Reduced risk of all-cause death ⁴ FLOW
 Oral semaglutide 14, 25 and 50 mg	Glycaemic control*	Body weight*	MACE outcome
	1.9/2.2%-p Reduction HbA _{1c} ⁵ PIONEER PLUS	7.0/9.8% Weight loss ⁵ PIONEER PLUS	14% Reduction in MACE ⁶ SOUL

*Trial product estimand; ¹P. Frias, SUSTAIN FORTE, Lancet, 2021 (9):563-574; ²Steven P Marsoe, SUSTAIN-6, N Engl J Med 2016;375:1834-1844; ³Marc P Bonaca, STRIDE, Lancet, 2025 ;405(10489):1580-1593; ⁴Vlado Perkovic et al, FLOW, N Engl J Med 2024;391:109-121; ⁵Vanita R Aroda, PIONEER PLUS, Lancet 2023 402(10403):693-704; ⁶Darren K. McGuire, SOUL, N Engl J Med 2025;392:2001-2012
HbA_{1c}: Haemoglobin A_{1c}; MACE: Major adverse cardiovascular events; MWD: Maximum walking distance; PAD: Peripheral artery disease; Sc: Subcutaneous; T2D: Type 2 Diabetes; %-p: Percentage points

Phase 2 trial for CagriSema in people with type 2 diabetes was successfully completed in Q3 2022

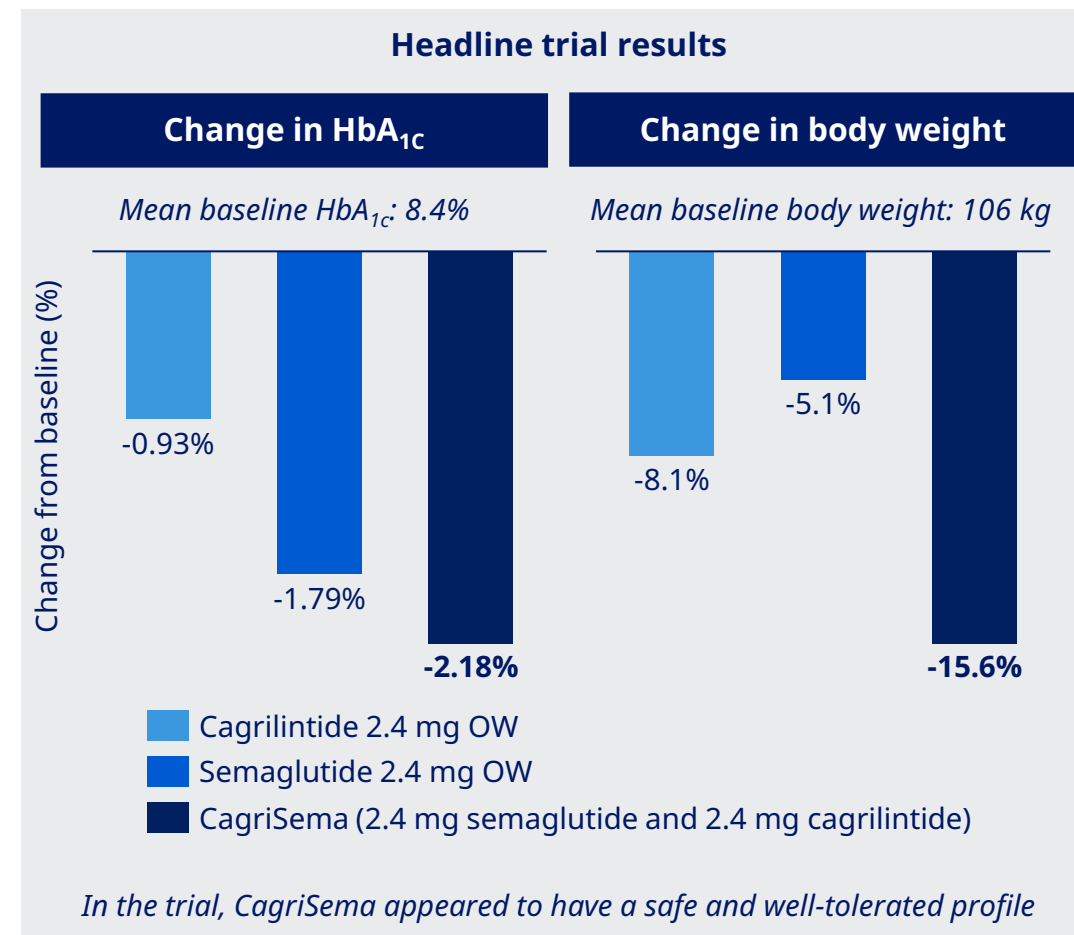


Primary endpoint:

Change from baseline (week 0) to week 32 in HbA_{1c}

Inclusion criteria (92 people):

- Type 2 diabetes
- HbA_{1c} 7.5–10.0%
- Metformin +/- SGLT2i
- BMI ≥27 kg/m²



T2D: Type 2 diabetes; BMI: body mass index; HbA_{1c}: Glycosylated haemoglobin; OW: Once-weekly

Note: Trial product estimands shown; Trial objective: To compare the effect of co-administered (separate injections) semaglutide and cagrilintide versus semaglutide in subjects with T2D inadequately controlled on metformin with or without SGLT2 inhibitor

Phase 3 trial programme with CagriSema in type 2 diabetes, REIMAGINE, was initiated in Q3 2023

CagriSema characteristics



CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and semaglutide 2.4 mg



Phase 3a programme with CagriSema in T2D:

- Aims to confirm efficacy and safety across four global trials
- Expected completion during 2025/2026

Global phase 3 trial programme

REIMAGINE 1 vs placebo

- **180 patients** with T2D
- **40-week** vs. placebo
- **Primary endpoint:** HbA_{1c}

REIMAGINE 2 FDC trial

- **2700 patients** with T2D, MET +/- SGLT-2i
- **68-week** vs. semaglutide, cagrilintide and placebo
- **Primary endpoint:** HbA_{1c} and bodyweight

REIMAGINE 3 Add-on to insulin

- **270 patients** with T2D, Basal insulin +/- MET
- **40-week** vs. placebo
- **Primary endpoint:** HbA_{1c}

REIMAGINE 4 H2H vs tirzepatide

- **1000 patients** with T2D, MET +/- SGLT-2i
- **68-week** vs. tirzepatide
- **Primary endpoint:** HbA_{1c} and bodyweight

REDEFINE 3 CVOT – shared with obesity programme

- **7000 patients¹**
- **Event driven**
- **Primary endpoint:** 3-point MACE

2023

2024

2025

2026

¹165% of patients with T2D, 35% without T2D

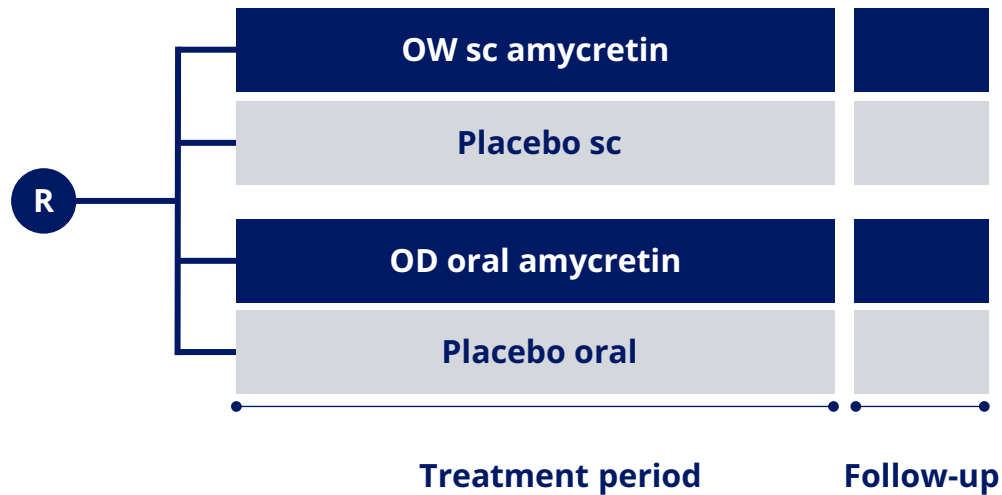
FDC: Fixed dose combination; T2D: Type 2 Diabetes; H2H: Head-to-head; CVOT: Cardiovascular outcomes trial; 3P: Three point; MACE: Major adverse cardiovascular event; MET: Metformin; SGLT-2i: sodium-glucose co-transporter-2 inhibitor

Note: CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

Amycretin phase 2 trial with oral and subcutaneous administration in people with type 2 diabetes has been initiated

Phase 2 amycretin trial design

ILLUSTRATIVE



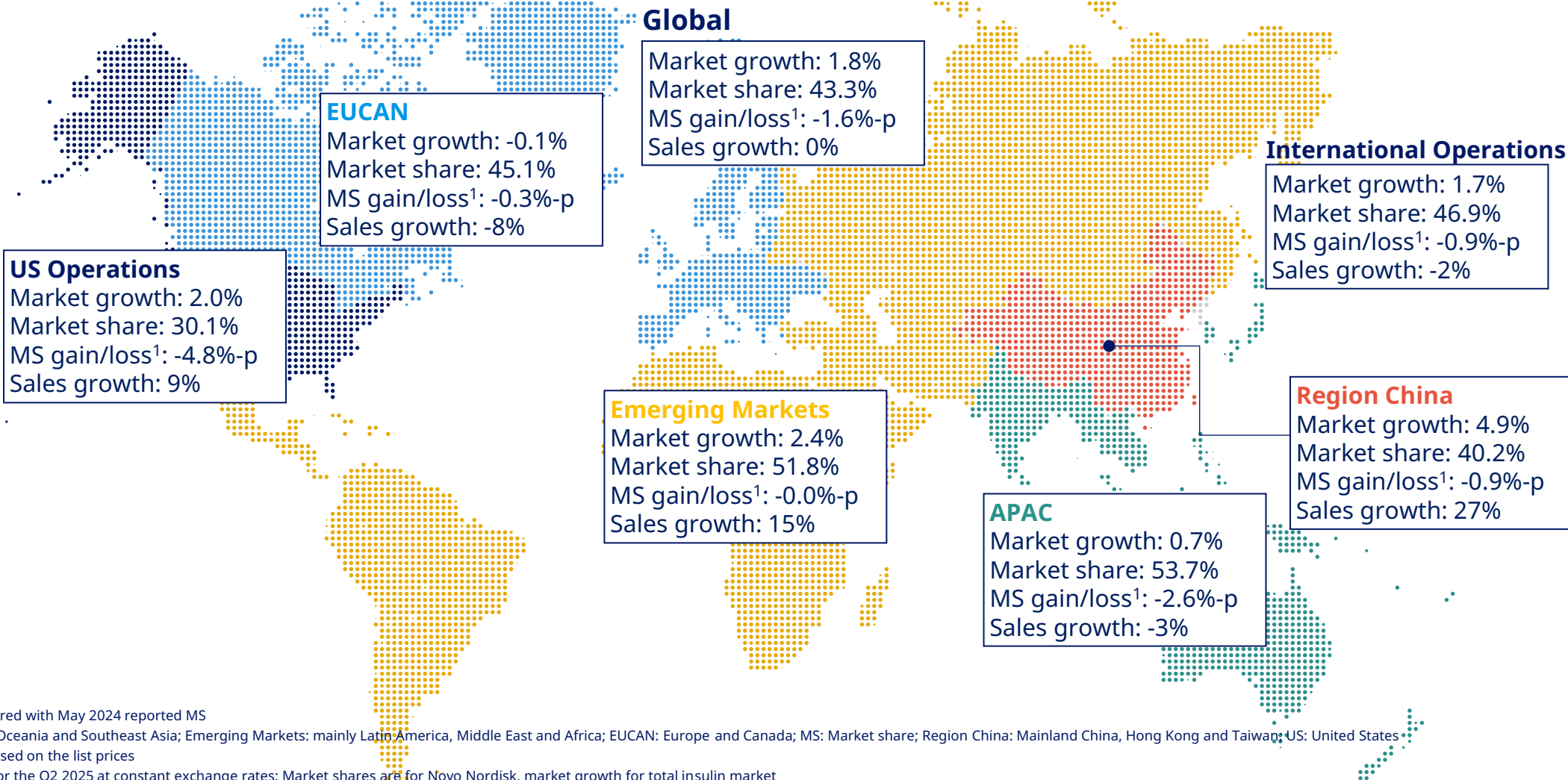
Objective

- Demonstrate the dose-response relationship of amycretin for change in HbA_{1c} from baseline in participants with type 2 diabetes

Proposed key endpoints

- Change in HbA_{1c} (%-point) from baseline
- Relative change in body weight (%) from baseline

Novo Nordisk global insulin volume market leadership at 43.3% and the global insulin volume market increased by 1.8%



¹MS gain/loss compared with May 2024 reported MS

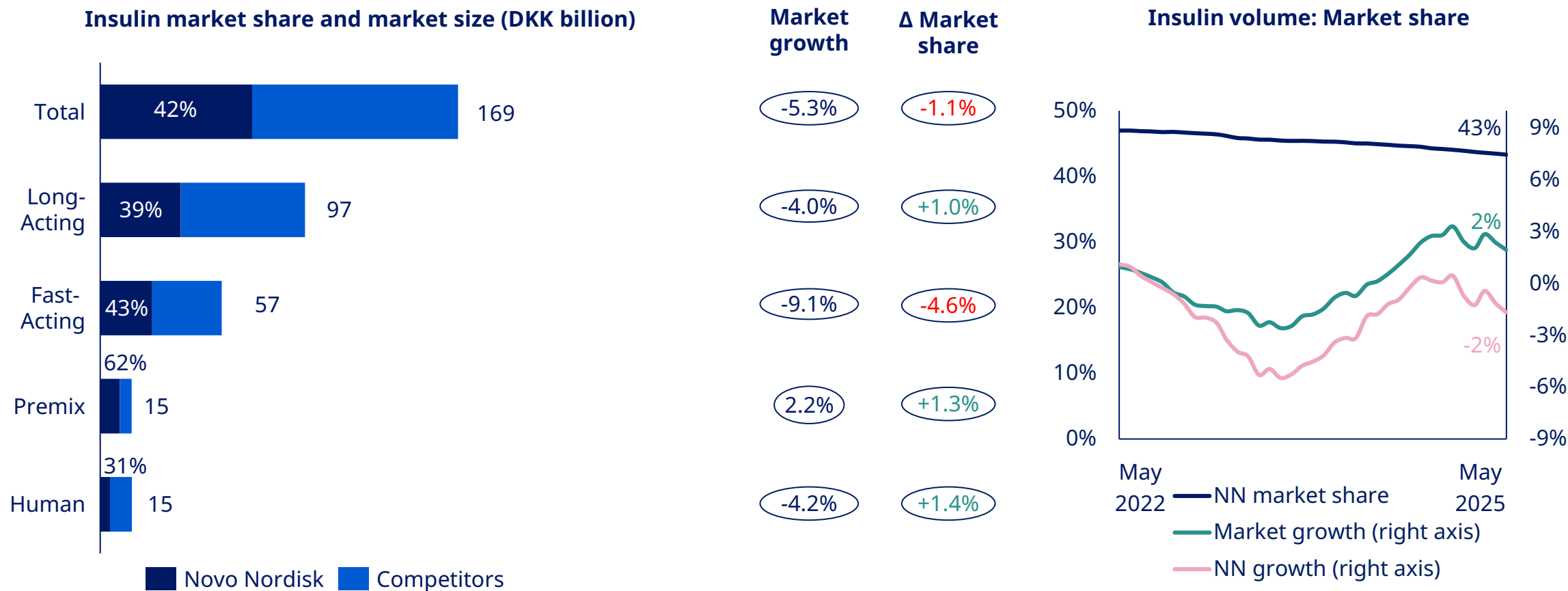
APAC: Japan, Korea, Oceania and Southeast Asia; Emerging Markets: mainly Latin America, Middle East and Africa; EUCAN: Europe and Canada; MS: Market share; Region China: Mainland China, Hong Kong and Taiwan; US: United States

Market values are based on the list prices

Note: Sales growth for the Q2 2025 at constant exchange rates; Market shares are for Novo Nordisk, market growth for total insulin market

Source: IQVIA MAT, May 2025 volume figures

Insulin market size and Novo Nordisk volume and value market share



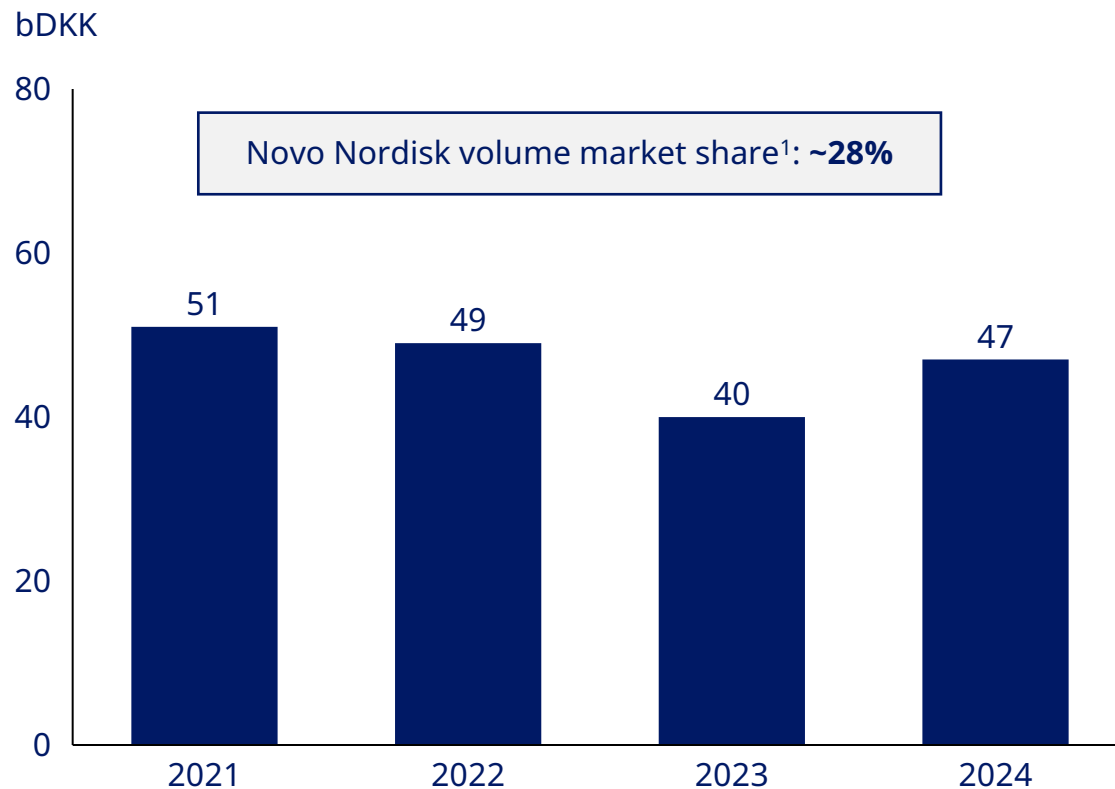
NN: Novo Nordisk

Note: LHS graph - Value, RHS Graph - Volume, MAT, all countries; Share of growth not depicted due to too high numbers ; Market values are based on the list prices

Source: IQVIA, May 2025

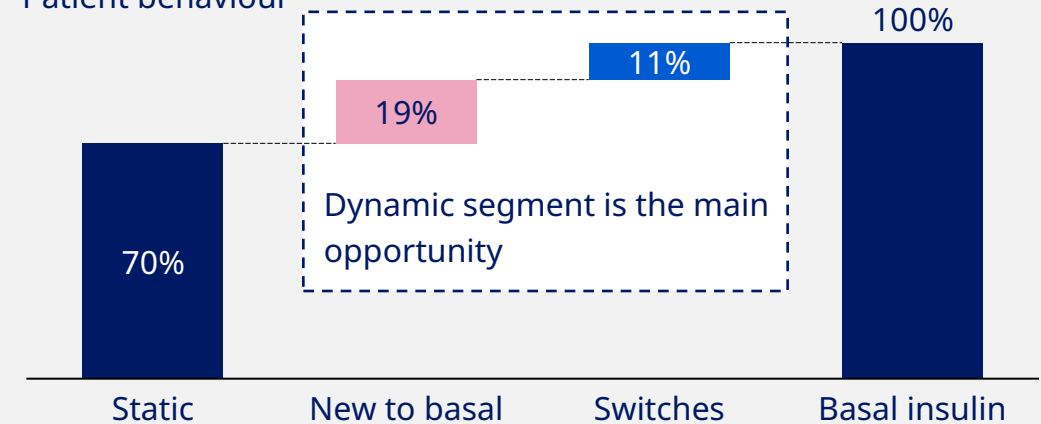
Insulin icodec holds potential to be the insulin of choice for people living with type 2 diabetes starting basal insulin treatment

Today's global basal insulin market is sizeable



The opportunity for insulin icodec

Patient behaviour



Insulin icodec reduces basal insulin inj. from 7 to 1 per week



Many patients delay insulin initiation >2 years due to dosing frequency



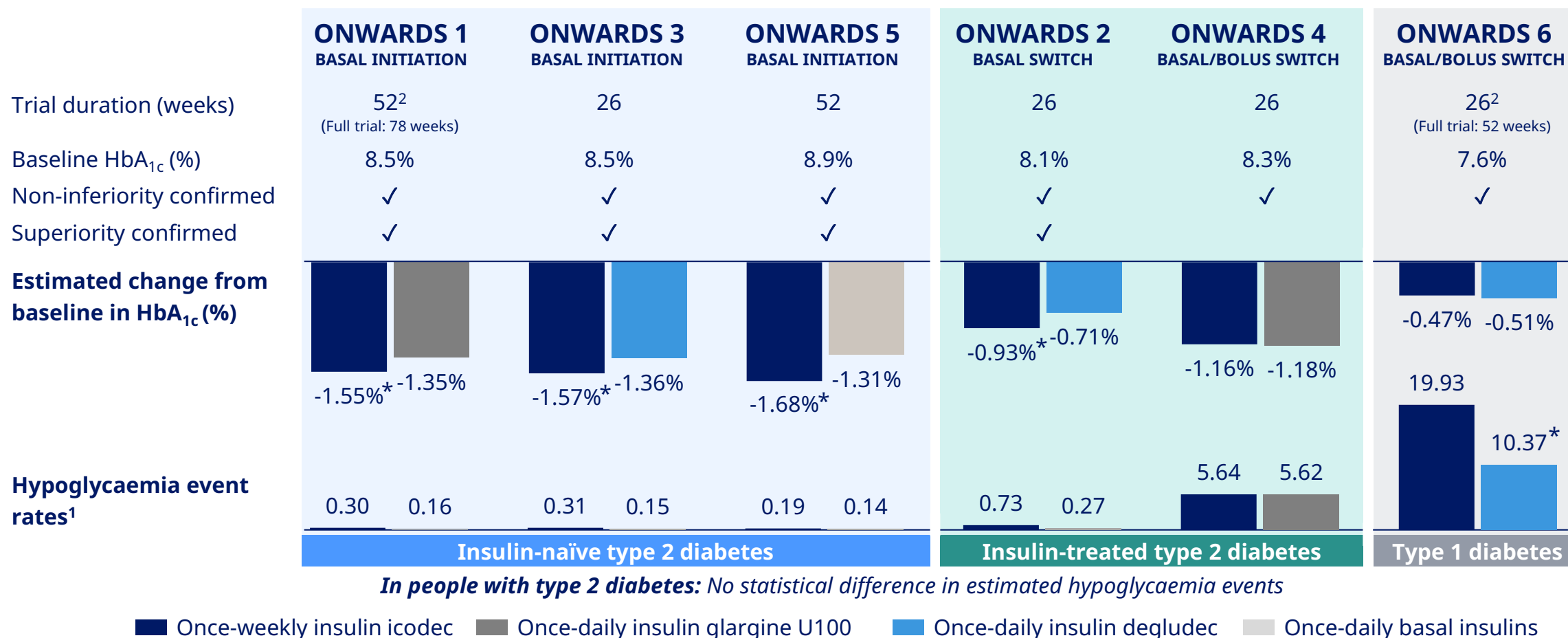
HCP and patient preference for once-weekly treatments

¹IQVIA MAT, May 2025

HCP: Health care professional; Inj.: Injections

Source: Company reported sales; Novo Nordisk market research

Once-weekly insulin icodec appeared to be effective and to have a safe profile in the phase 3 ONWARDS programme



*Statistically significant. 1 Severe or clinically significant hypoglycaemia events (blood glucose <3 mmol/L) per patient year, included for end of trial/end main phase in-trial. 2 Duration refers to trial main phase.

ONWARDS 1: QW insulin icodec vs QD insulin glargine U100 both with non-insulin anti-diabetic treatment in insulin-naïve people with T2D; ONWARDS 2: QW insulin icodec vs QD insulin degludec in people with T2D switching from a QD insulin; ONWARDS 3: QW insulin icodec vs QD insulin degludec in insulin-naïve people with T2D; ONWARDS 4: QW insulin icodec vs QD insulin degludec both with mealtime insulin in people with T2D treated with basal and bolus insulin; ONWARDS 5: QW insulin icodec vs QD basal insulin with an app providing dosing recommendation in insulin-naïve people with T2D; ONWARDS 6: QW insulin icodec vs QD insulin degludec both with mealtime insulin in people with T1D
T1D: Type 1 diabetes; T2D: Type 2 diabetes. Note: Overview refers to primary end-points in main phases of trials

Phase 3 trial programme for IcoSema in T2D, COMBINE

IcoSema characteristics



IcoSema is a fixed dose combination of insulin icodec and semaglutide

- Simple and convenient once-weekly injection



Phase 3a programme with IcoSema

- Aims to confirm efficacy and safety across three global trials
- All pivotal trials successfully completed
- Novo Nordisk submitted for regulatory approval in H2 2024 in the EU

Focused phase 3 trial programme

COMBINE 1 Post-basal insulin

- **Initiated in Q2 2022**
- **1290 patients*** previously on basal-insulin
- **52-week** vs. insulin icodec
- **Prim. endpoint:** HbA_{1c} superiority
- **Sec. endpoint:** Weight / hypo superiority



COMBINE 2 Post-GLP-1

- **Initiated in Q2 2022**
- **680 patients*** previously on GLP-1 RA
- **52-week** vs. semaglutide 1.0 mg
- **Primary endpoint:** HbA_{1c} superiority



COMBINE 3 Basal insulin intensification

- **Initiated in Q4 2021**
- **680 patients*** previously on basal insulin
- **52-week** vs. insulin glargine + insulin aspart
- **Prim. endpoint:** HbA_{1c} non-inferiority
- **Sec. endpoint:** Weight / hypo superiority



COMBINE 4 Post OAD

- **Initiated in Q1 2024**
- **475 patients*** previously on at least 2 OADs
- **40-week** vs. OD insulin glargine
- **Primary endpoint:** HbA_{1c} superiority

Trial ongoing

2021

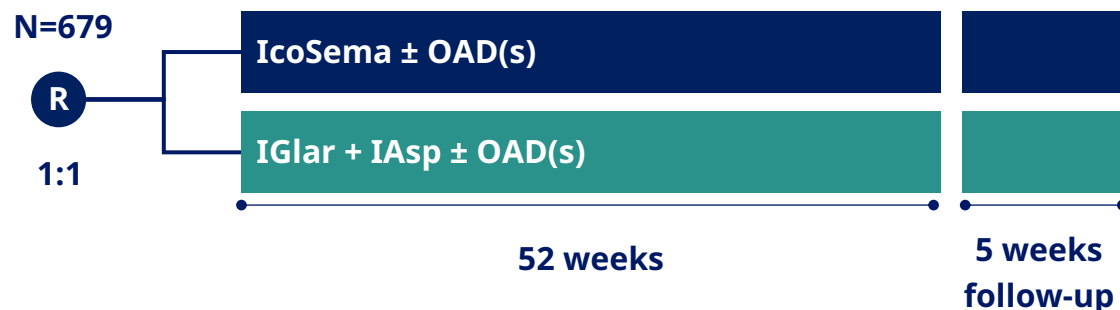
2022

2023

2024

Phase 3a trial (COMBINE 3) with IcoSema successfully completed

IcoSema vs Insulin glargine U100 and insulin apart in subjects w/T2D

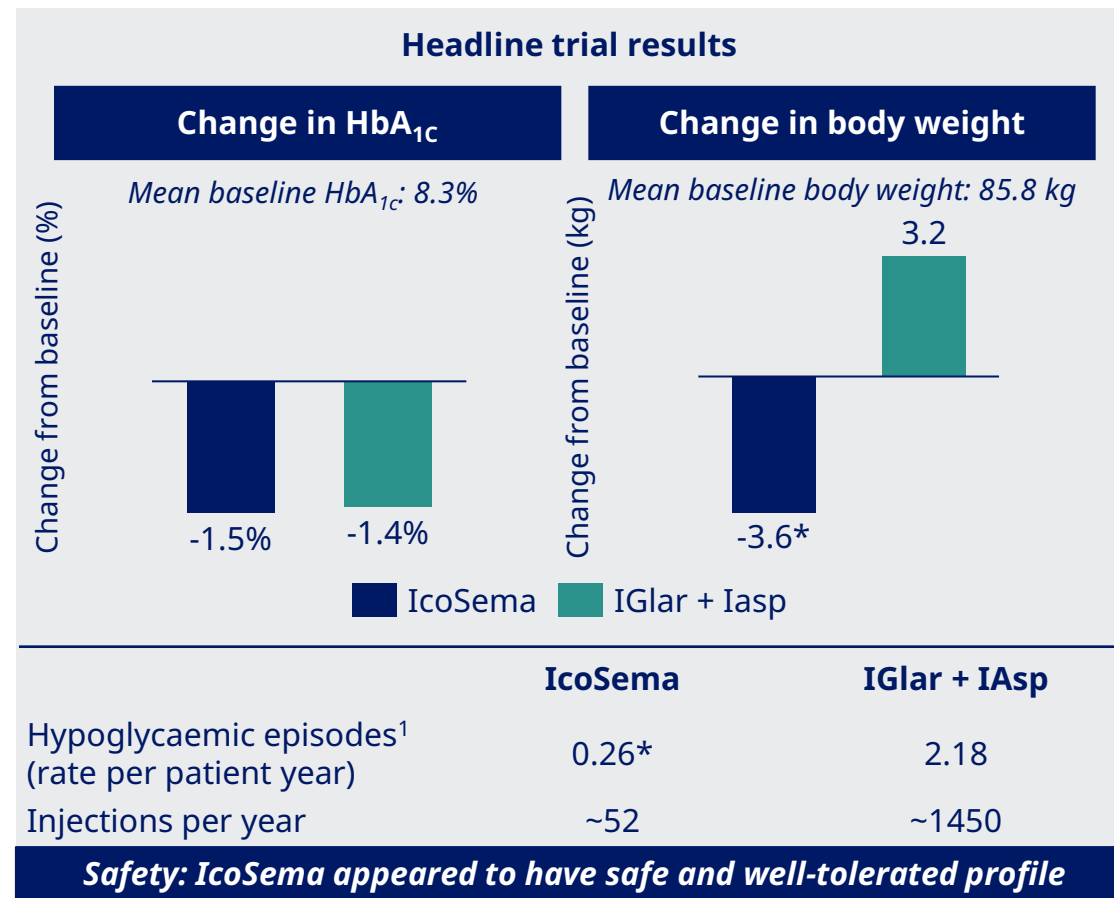


Primary endpoint:

- Change in HbA_{1c} from baseline to week 53

Confirmatory secondary endpoints:

- Change in body weight from baseline to week 52
- Number of hypoglycaemic¹ episodes from baseline to week 57



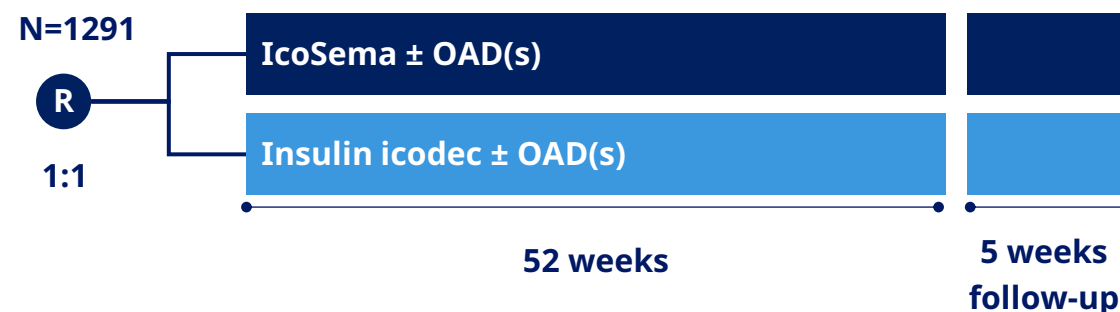
*Statistically significant/superior vs. Insulin glargine U100 and insulin apart. ¹ Level 2 and 3 hypoglycaemic episodes with blood glucose below 3.0 mmol/L

T2D: Type 2 diabetes; HbA_{1c}: Glycated haemoglobin; BMI: Body Mass Index; OADs: Oral antidiabetic drugs.

Note: Trial objective: To confirm efficacy and compare safety of once weekly IcoSema compared with daily insulin glargine combined with insulin apart, both treatment arms with or without OADs in participants with T2D inadequately controlled with daily basal insulin

Final pivotal phase 3 trial with once-weekly IcoSema successfully completed

COMBINE 1 - IcoSema vs Insulin icodec in subjects with T2D



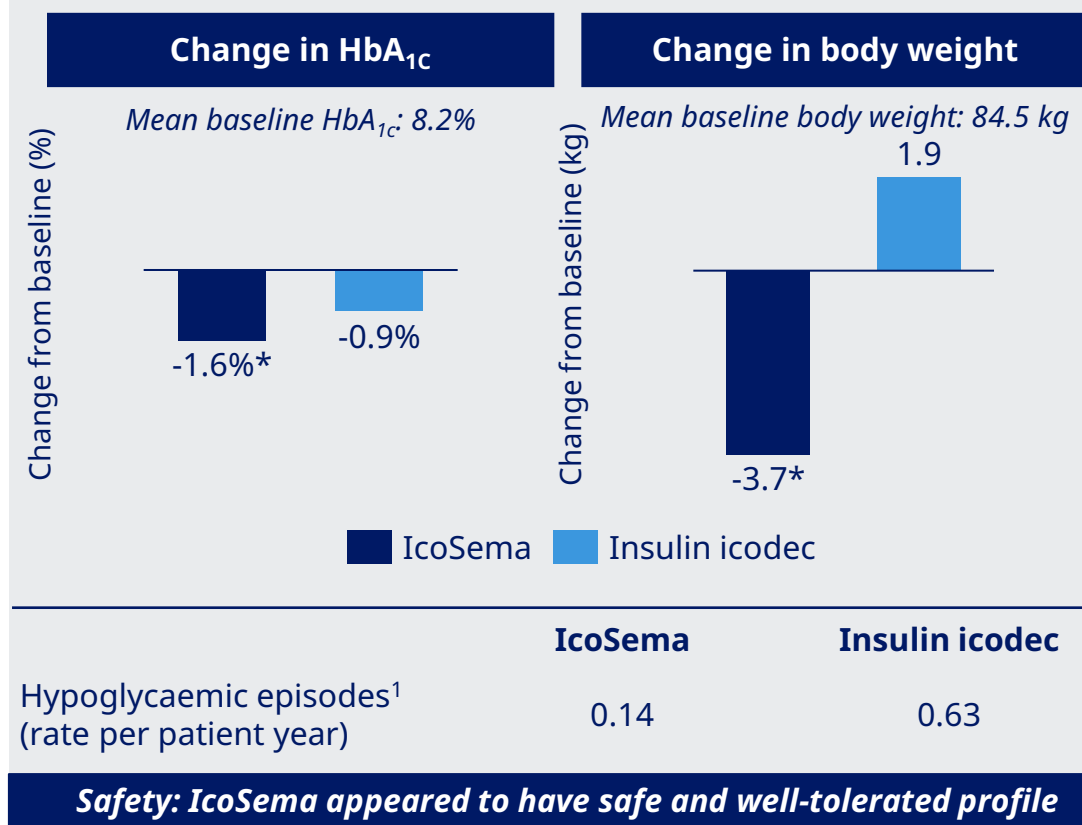
Primary endpoint:

- Change in HbA_{1c} from baseline to week 52

Secondary endpoints:

- Change in body weight from baseline to week 52
- Number of level 2 or 3 hypoglycaemic¹ episodes from baseline to week 57

COMBINE 1 headline trial results



*Statistically significant/superior vs. Insulin icodec. Data shown for HbA_{1c} and body weight is the treatment policy estimand ¹ Level 2 and 3 hypoglycaemic episodes on-treatment observation period.

HbA_{1c}: Glycated haemoglobin; IcoSema: a combination of basal insulin icodec and semaglutide; OADs: Oral antidiabetic drugs; R: Randomisation; T2D: Type 2 diabetes;

Trial objective: To confirm efficacy and compare safety of once weekly IcoSema compared with once weekly insulin icodec, both treatment arms with or without OADs in participants with T2D inadequately controlled with daily basal insulin

Development pipeline addresses unmet need in diabetes care by further raising the innovation bar

Further raise the innovation bar

Our key focus areas



Address significant unmet need



Develop next-generation treatments



Continued generation of outcomes data

Diabetes development pipeline¹

Diabetes

Project	Phase
GLP-1 diabetes²	<i>Marketed</i>
Long-acting insulins³	<i>Marketed</i>
Premix insulins⁴	<i>Marketed</i>
Fast-acting insulins⁵	<i>Marketed</i>
Awikli^{®6}	<i>Marketed</i>
Icosema	Submitted
SOUL (oral semaglutide 14.0 mg CVOT)	Submitted
STRIDE⁷ (semaglutide 1.0 mg in PAD)	Submitted
oral semaglutide⁸ (25 mg and 50 mg)	Phase 3 completed
CagriSema (2.4 mg/2.4 mg)	Phase 3 ongoing
sc. amycretin OW and oral OD	Phase 2 ongoing
monlunabant	Phase 2 ongoing
OW GIP/GLP-1	Phase 2 ongoing
FUSE⁹ - Peripheral focused ultrasound	Phase 2 to be initiated
GSI	Phase 1 ongoing
DNA immunotherapy	Phase 1 ongoing
Pumpsulin	Phase 1 ongoing

¹Human insulins and other diabetes care not included in development pipeline overview ²Includes Rybelsus®, Ozempic®, and Victoza® ³Includes Tresiba®, Xultophy®, and Levemir® ⁴Includes Ryzodeg® and NovoMix® ⁵Includes Fiasp® and NovoRapid®

⁶Launched in five countries in IO ⁷EMA adopted a positive opinion for an updated Ozempic® label based on STRIDE data ⁸Submitted to EMA ⁹In collaboration with GE Healthcare

CB1R: Cannabinoid receptor 1; CKD: Chronic Kidney Disease; CVOT: Cardiovascular Outcome Trial; GIP: Gastric inhibitory polypeptide; GSI: Glucose Sensitive Insulin; OD: Once-daily; OW: Once-weekly; Sc.: Subcutaneous

Obesity care

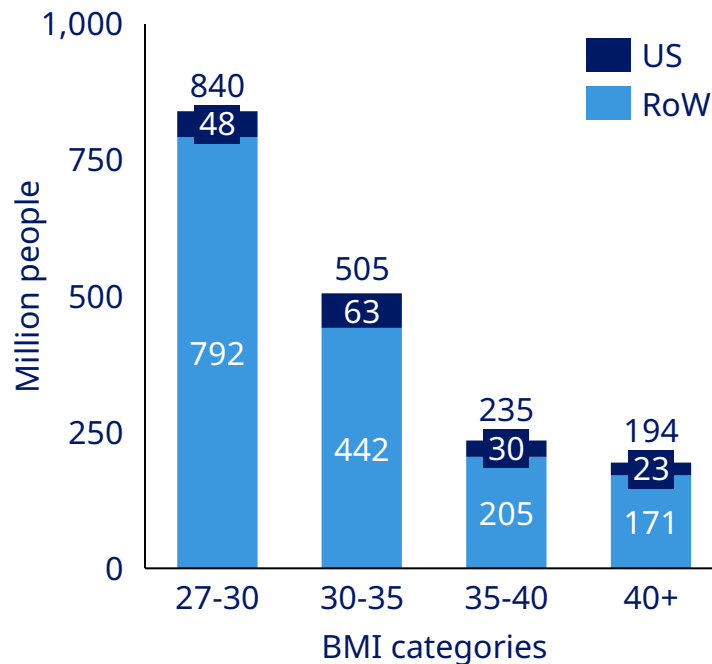
Obesity disease background
Obesity market development
Innovation



MICHAEL PETERSEN
Michael lives with obesity
Denmark

Obesity is a serious chronic disease with a large unmet medical need that requires innovative treatment options

More than 1.7 billion people is living with overweight or obesity globally



Obesity is associated with more than 200 different complications



Metabolic



Cardiovascular



Mechanical

Life expectancy decreases as BMI increases

Likelihood of reaching age 70 per BMI group from a baseline age of 46¹



Normal BMI
80%



BMI 35-40
60%



BMI 40-50
50%

Today

- Few treatment options available: <1% of global obese population on a branded AOM
- 2025 ACC clinical guidance for weight management in patients where treatment may provide CV benefit

¹Prospective Studies Collaboration, Whitlock G, Lewington S, et al. Body-mass index and cause-specific mortality in 900,000 adults: collaborative analyses of 57 prospective studies. Lancet. 2009

AOM: Anti-obesity medication; BMI: Body mass index; RoW: Rest of world; ACC: American College of Cardiology

Source: NHANES (2013-2014, 2015-2016, 2017-2020, 2021-2023), UN World Population Prospects report, WHO, IDF World Diabetes Atlas, World Obesity Atlas and PADAWA Analysis

Novo Nordisk's innovation is focused on addressing weight loss magnitude as well as emerging patient needs and comorbidities

Building a leading portfolio

Our key focus areas



Body weight loss



Co-morbidity impact



Safety and tolerability



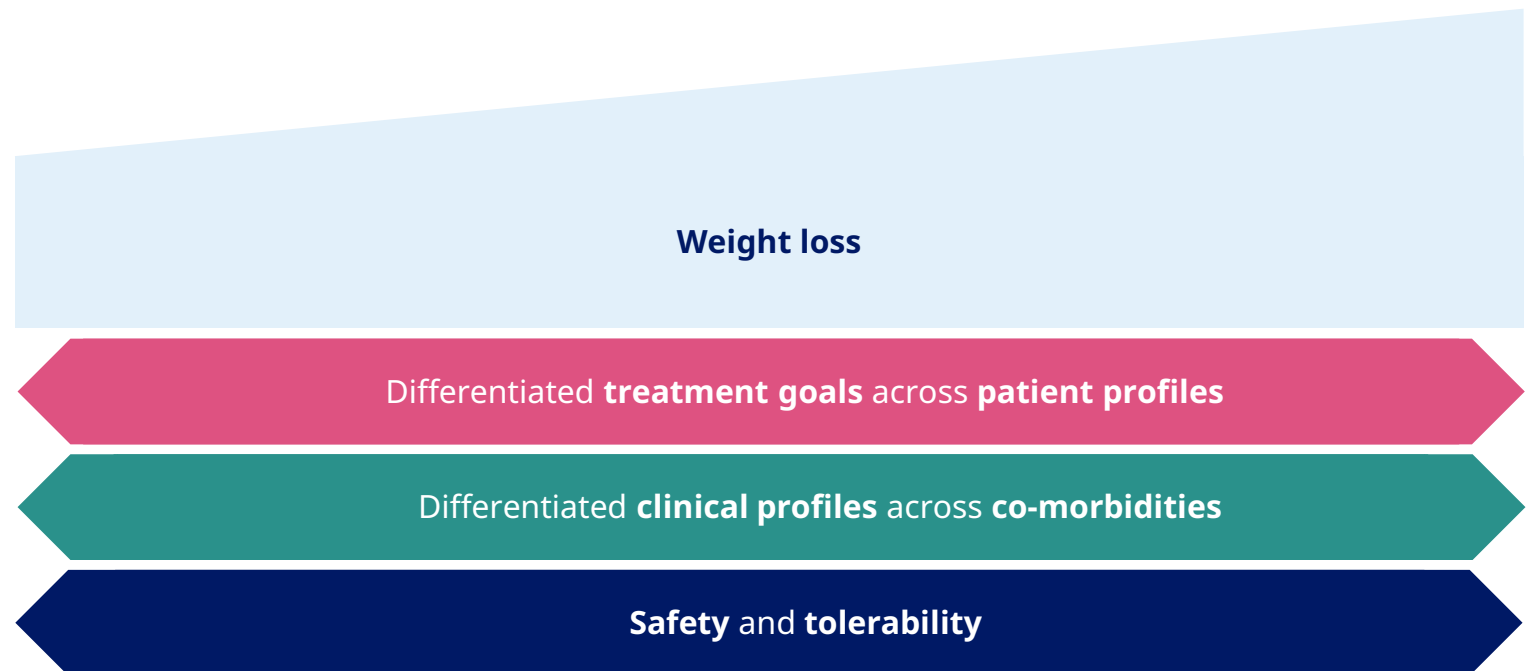
Composition of weight loss



Dosing frequency

Addressing unmet needs across patient segments via differentiated clinical profiles

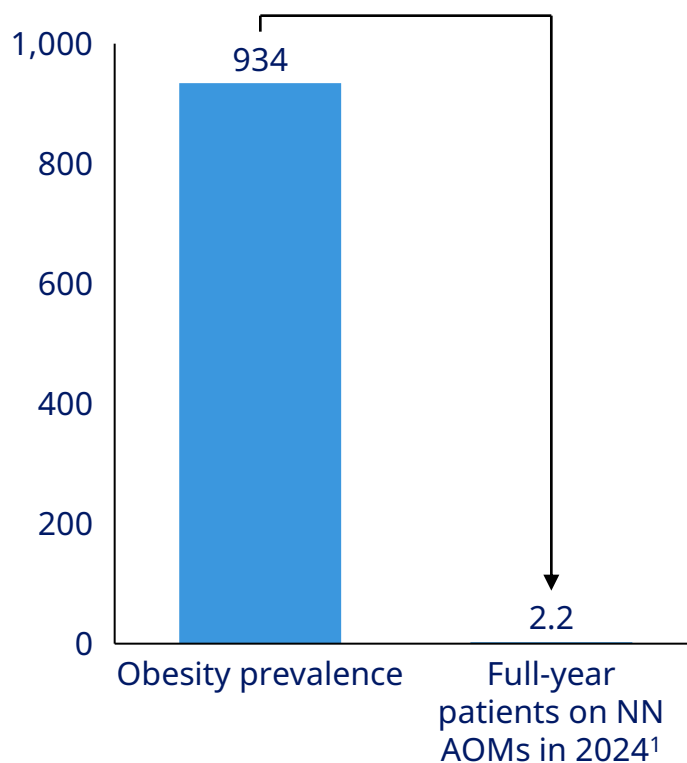
ILLUSTRATIVE



With the launch of Wegovy® in 2021 a lot changed, yet the large unmet need in obesity remains

Few people are treated for obesity today

Million people



Key market changes since the Wegovy® launch in 2021

	Patients 	Prescribers 	Payers 
Before	Needs to be activated Low adherence eg due to tolerability, affordability and treatment expectations	Consider treating obesity Sporadic local guidelines	US: Limited willingness to cover AOMs IO: Mostly out-of-pocket
After	Decision-maker with consumer like behaviour Increasing adherence as barriers are addressed, but still not chronic care	Treat obesity Sporadic local guidelines	US: Good commercial coverage IO: Mostly out of pocket, but increased levels of reimbursement

¹The number represents the estimated full-year patients reached with Novo Nordisk products as outlined in the 2024 Annual Report

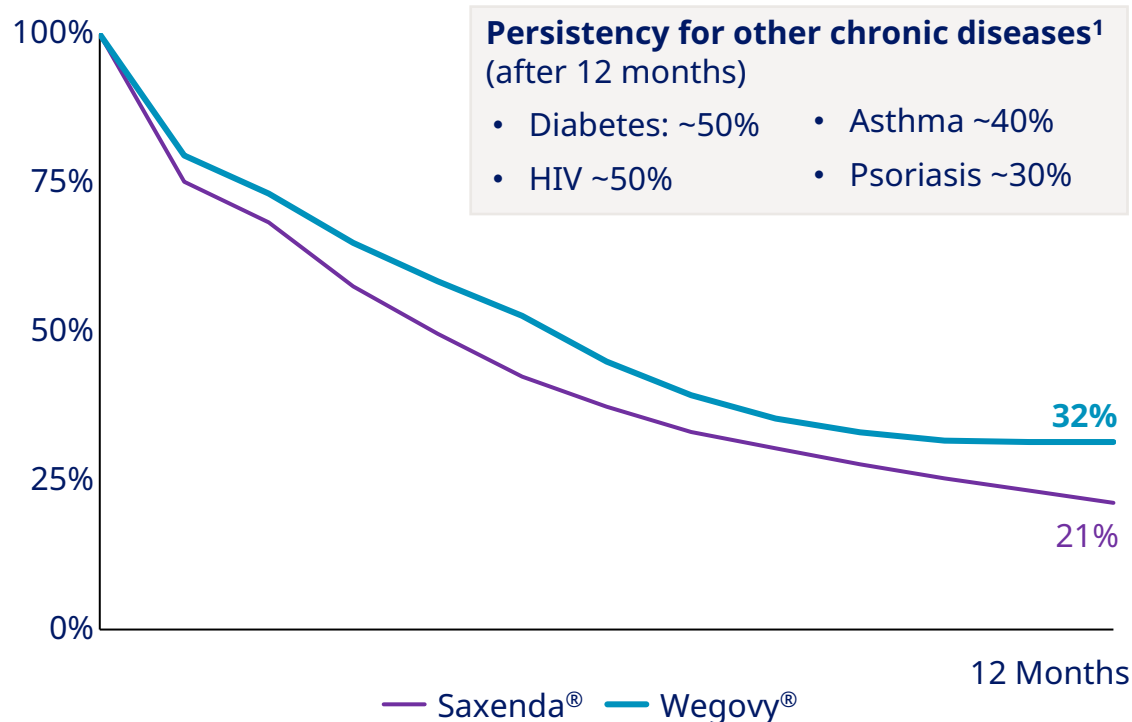
AOM: Anti-obesity medications; IO: International Operations; NN: Novo Nordisk; US: United States

Source: NHANES (2013-2014, 2015-2016, 2017-2020, 2021-2023), UN World Population Prospects report, WHO, IDF World Diabetes Atlas, World Obesity Atlas and PADAWA Analysis, Novo Nordisk Annual Report 2024

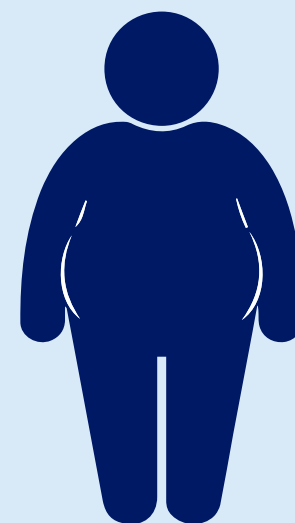
Novo Nordisk is broadening focus from solely weight loss to improving health for patients with overweight or obesity

Patient persistency on anti-obesity medications after 12 months

Patients remaining on treatment (%)



Characteristics for patients on Wegovy® in the US



≈ 86% naïve to AOM treatment



78% female

Age

Average of 48 years



Average BMI of 37



Patients on Wegovy® with type 2 diabetes diagnosis: 7%



With comorbidities:
 ≥1: 75% ≥2: 51% ≥3: 31%



Average Wegovy® stay time >6 months²

¹Hichborn, et al. (2018). Improving patient adherence through data-driven insights. McKinsey & Company; ²Average Wegovy® stay time >6 months despite supply constraints based on real world data, patient cohort included those initiating therapy between Oct '21 and Mar '22, followed for 1 year;

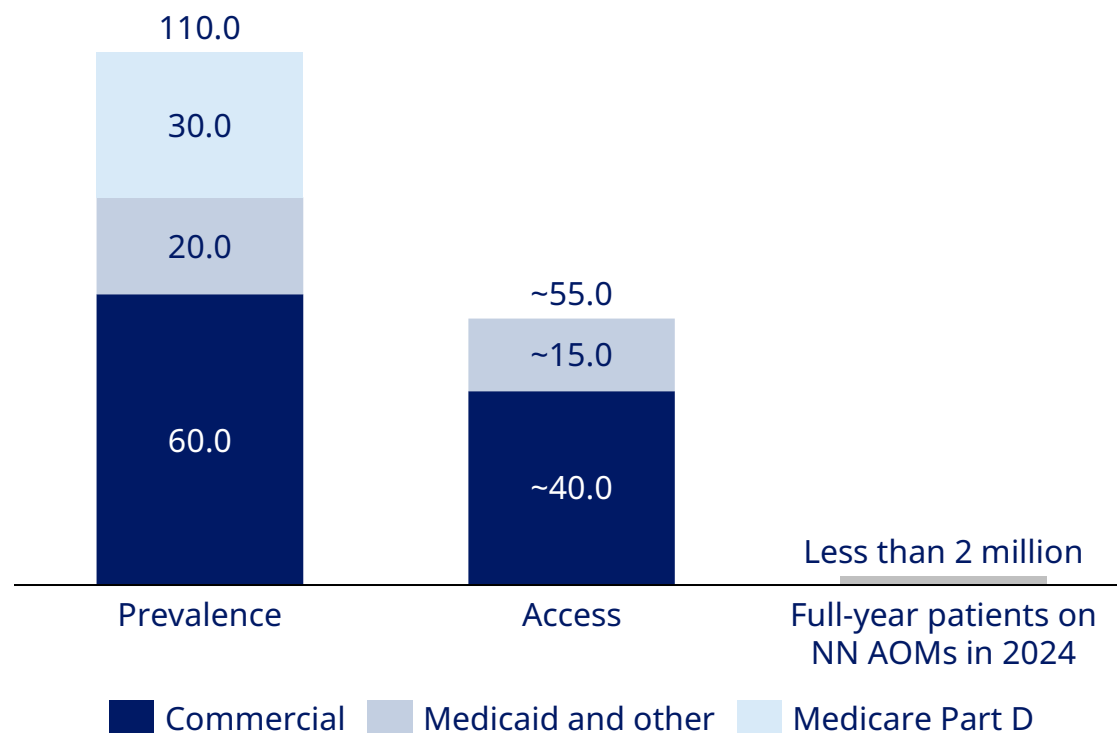
AOM: Anti-obesity medications; BMI: Body mass index; HbA1c: Haemoglobin A1c; HIV: Human Immunodeficiency Virus; US: United States

Source: IQVIA LAAD, AOM Rx, 12 months ending November 2024; Real world evidence based on prescription data

Novo Nordisk has broad and affordable access to 55 million people with obesity for Wegovy®

~55 million people with obesity have Wegovy® coverage in the US

People with obesity (millions)



Progress across all channels in early 2025

Commercial

- ✓ Broad formulary access and continued employer opt-in
- ✓ > 85% of patients pay \$50 or less per prescription

Medicaid and other

- ✓ **Federal coverage:** Examples include DoD, veteran affairs, and Indian Health service
- ✓ **Medicaid states:** Coverage of Wegovy® for CV patients continues to grow; >30 states programs cover Wegovy®

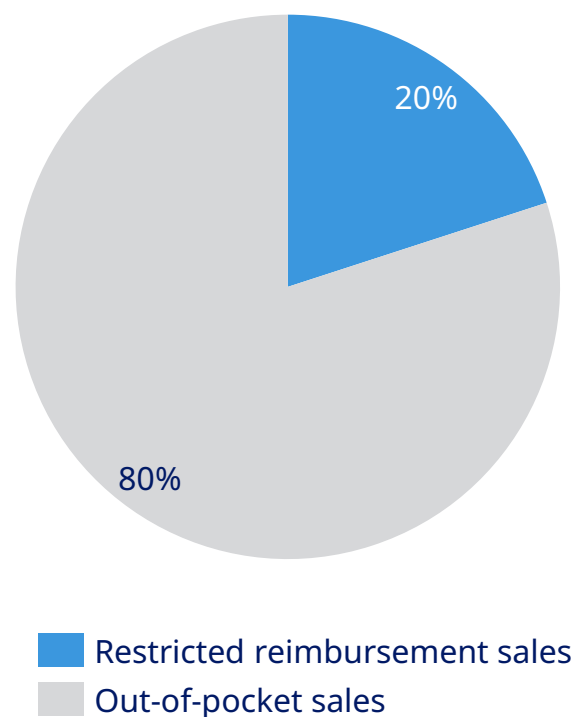
Medicare Part D

- Reimbursement of AOMs for obesity still prohibited by law
- CMS now allowing reimbursement in Part D for AOMs with a CV indication

Anti-obesity medications are expected to be mostly out-of-pocket, with SELECT as key lever to improve reimbursement

Majority of IO AOM sales are currently OOP

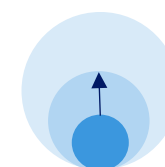
INDICATIVE



Current AOM reimbursement examples

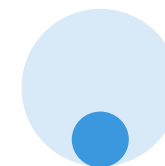
UK	BMI ≥35 or BMI ≥ 30 with ORC
COL	BMI ≥30 with two ORCs
CH	BMI ≥28 with ≥1 ORC or BMI ≥35
15 countries have selected reimbursement for Saxenda®	

SELECT could improve access to Wegovy®



Wegovy® reimbursed

Leverage SELECT to expand or improve market access



Wegovy® not reimbursed

Use SELECT to open or re-open reimbursement negotiations

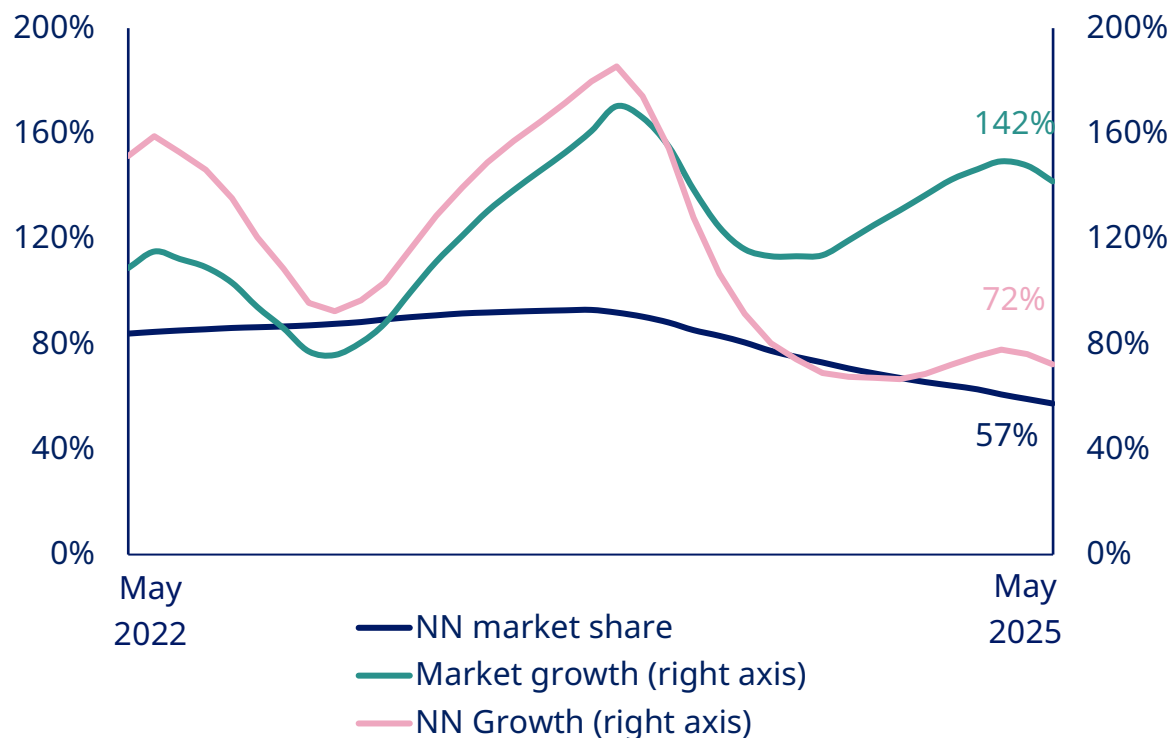


Out-of-pocket

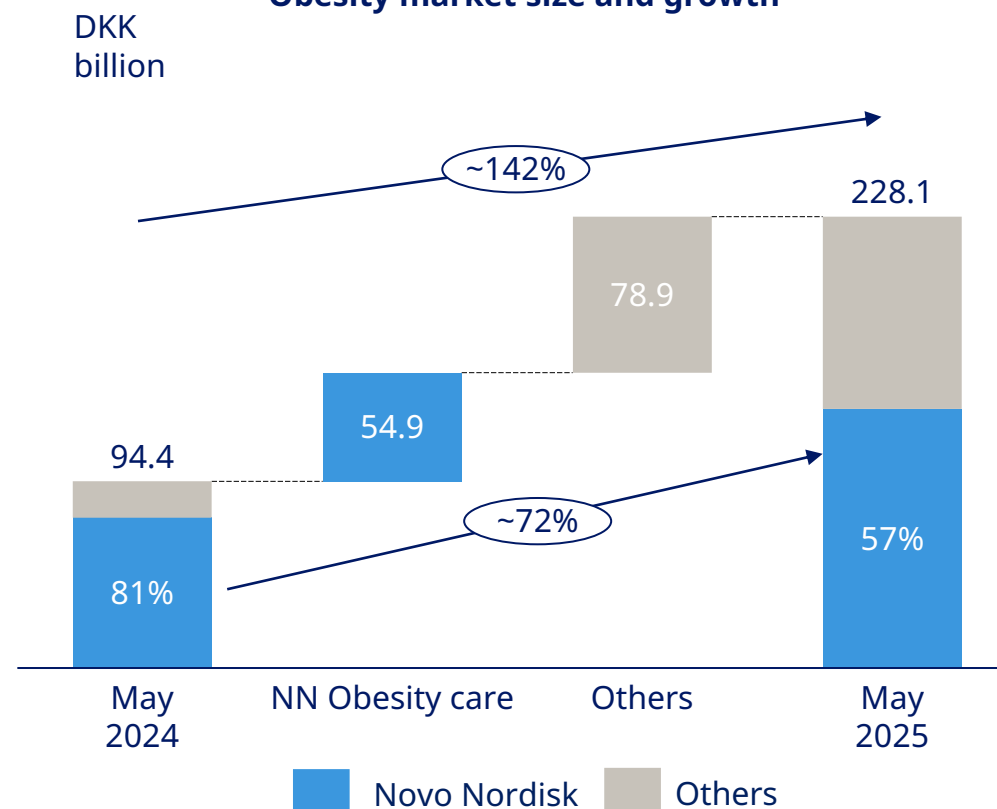
Increase willingness to pay in out-of-pocket markets

Global obesity market growth has been accelerating with Novo Nordisk capturing the majority of growth

Obesity market growth and Novo Nordisk value market share



Obesity market size and growth



In clinical trials, semaglutide 2.4 mg has demonstrated an impact on comorbidities that overlap with obesity

Weight loss

REDEFINE 1 (CagriSema)



22.7% weight loss¹

STEP 1 trial (Wegovy®)



16.9% weight loss¹

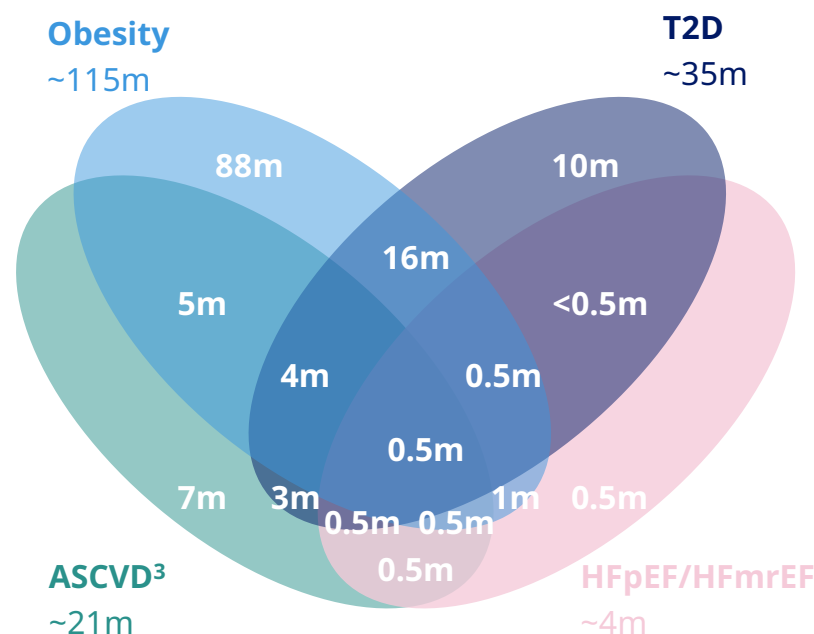
SCALE 1 trial (Saxenda®)



7.4% weight loss²

Disease overlap in the United States

UNITED STATES ONLY



Obesity-related comorbidities

SELECT trial



20% MACE risk reduction

STEP HFpEF trial



KCCQ-CSS score ETD: 7.8
(semaglutide 2.4 mg vs placebo)

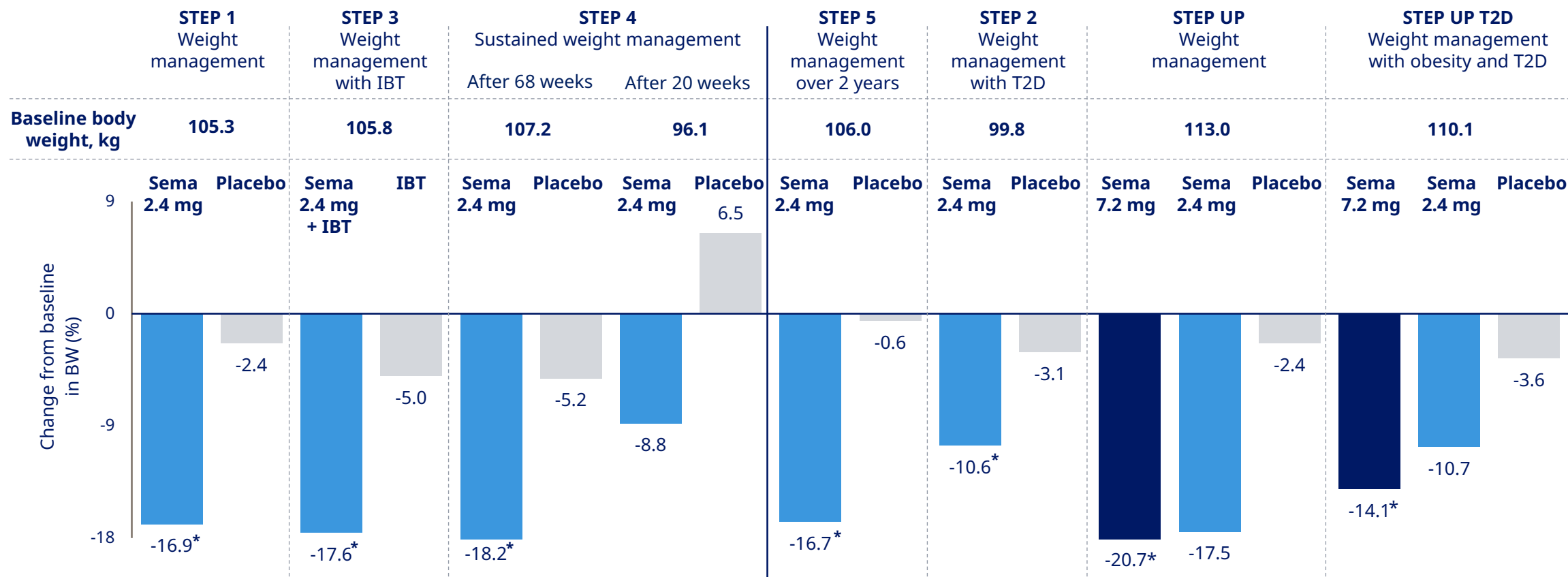
Knee osteoarthritis trial



41.7 WOMAC pain score reduction

¹Trial product estimand; ²Treatment policy estimand; ³Myocardial infarction, stroke and coronary heart disease; ASCVD: Atherosclerotic cardiovascular disease; MACE: Major adverse cardiovascular events; ETD: Estimated treatment difference; HFpEF: Heart failure with preserved ejection fraction; HFmrEF: Heart Failure with Mid-Range Ejection Fraction; WOMAC: The Western Ontario and McMaster University Osteoarthritis index. Note: Prevalence overlaps are estimated on patient-level data from NHANES. Post-estimation adjustments have been undertaken to match certain key metrics as reported by publicly available sources. Numbers are rounded. Source: NHANES (waves 2003-2004, 2013-2014, 2015-2016 and 2017-2020); UN World Population Prospects 2022; International Diabetes Federation: Diabetes Atlas 10th edition, 2021; World Obesity Atlas 2023

Across the STEP and STEP UP trials, a weight loss of up to 20.7% was reported for people treated with sc semaglutide

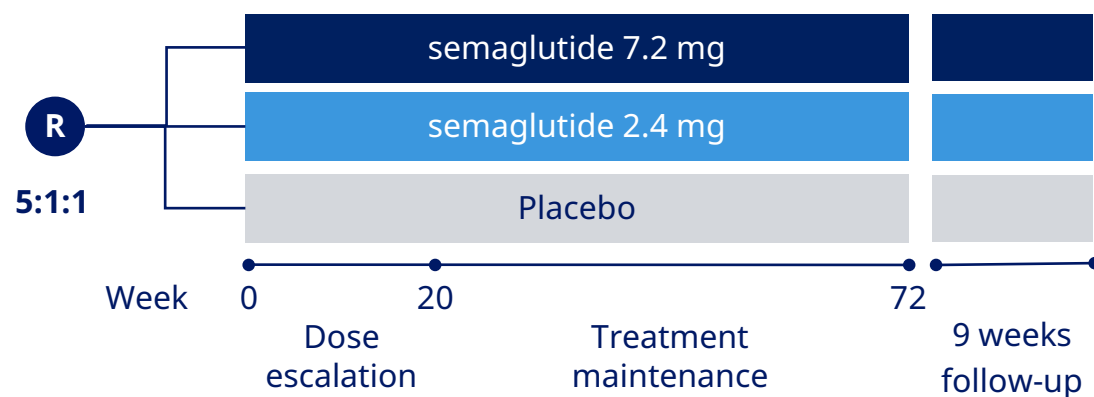


*P-value <0.0001, based on the trial product estimand (secondary statistical approach): treatment effect if all people adhered to treatment and did not initiate other anti-obesity therapies

BW: Body weight; IBT: Intensive behavioural therapy; Lira: Liraglutide; Mgmt.: Management; SC: subcutaneous; Sema: Semaglutide; T2D: Type 2 diabetes

In STEP UP, semaglutide 7.2 mg achieved 20.7% weight loss and around one third of participants achieved $\geq 25\%$ weight loss

STEP UP enrolled 1,407 people with obesity¹



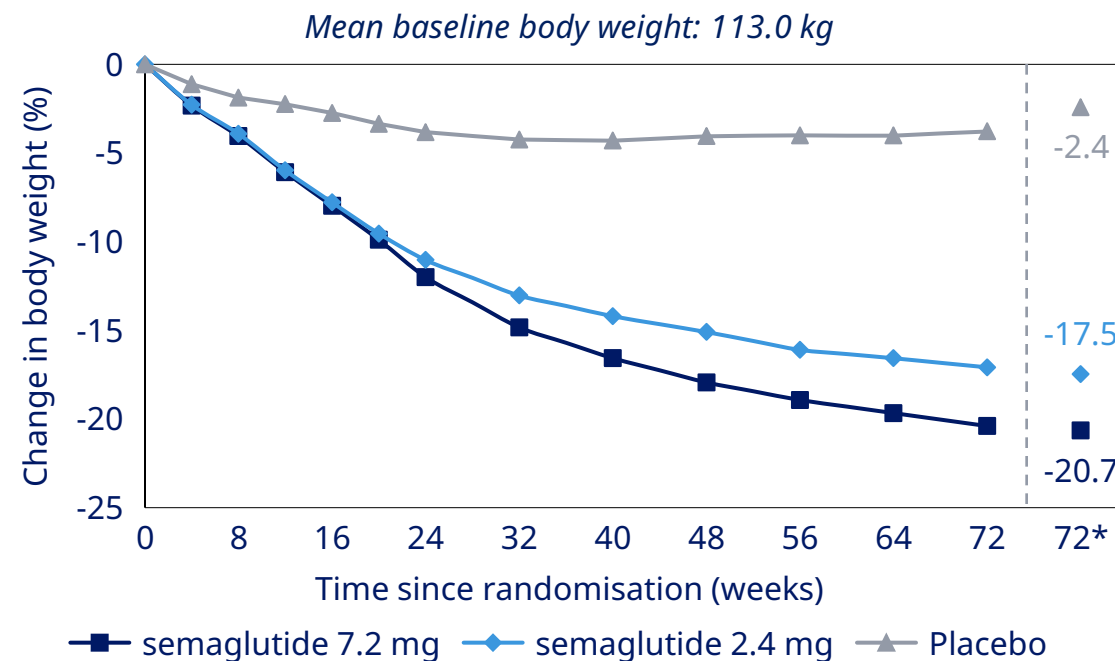
Trial objective

- Confirm superiority of sema 7.2 mg vs placebo

Co-primary endpoint

- Relative change in body weight (%) from baseline to 72 weeks
- Achievement of $\geq 5\%$ weight loss

Weight loss for semaglutide 7.2 mg in STEP UP trial



Categorical weight loss with sema 7.2 mg

$\geq 20\%$ WL reduction

50.9%

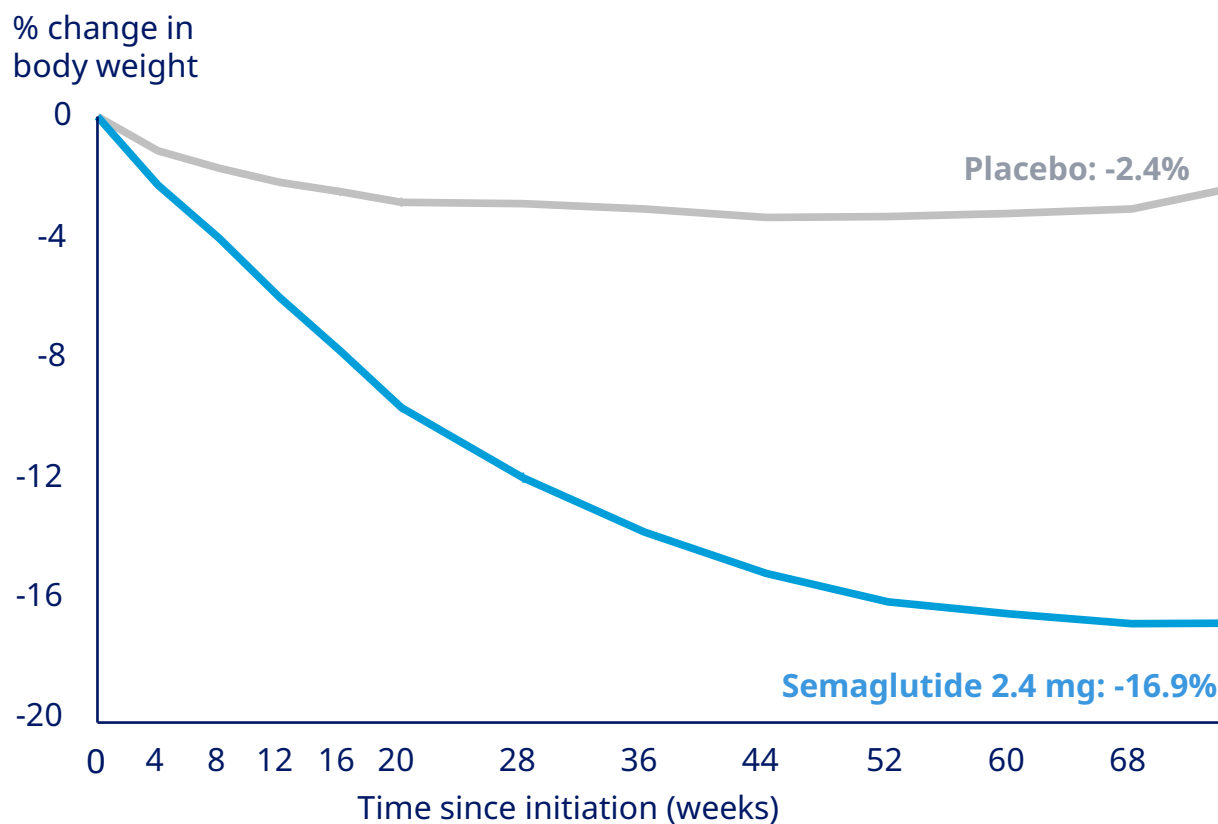
$\geq 25\%$ WL reduction

33.2%

*Estimated means. ¹BMI: ≥ 30 kg/m². Excludes diabetes diagnosis or HbA_{1c} $\geq 6.5\%$
 BMI: Body mass index; HbA_{1c}: Haemoglobin A_{1c}; Sema: Semaglutide; WL: Weight loss
 Note: data shown is trial product estimands
 Source: Novo Nordisk data on file

In STEP 1, people treated with semaglutide had a superior weight loss of up to 16.9%

The pivotal STEP 1 trial showed greater than 16% weight loss



Data from STEP 1



- Average age 46
- 74.1% women
- Average BMI - 37.9 kg/m²



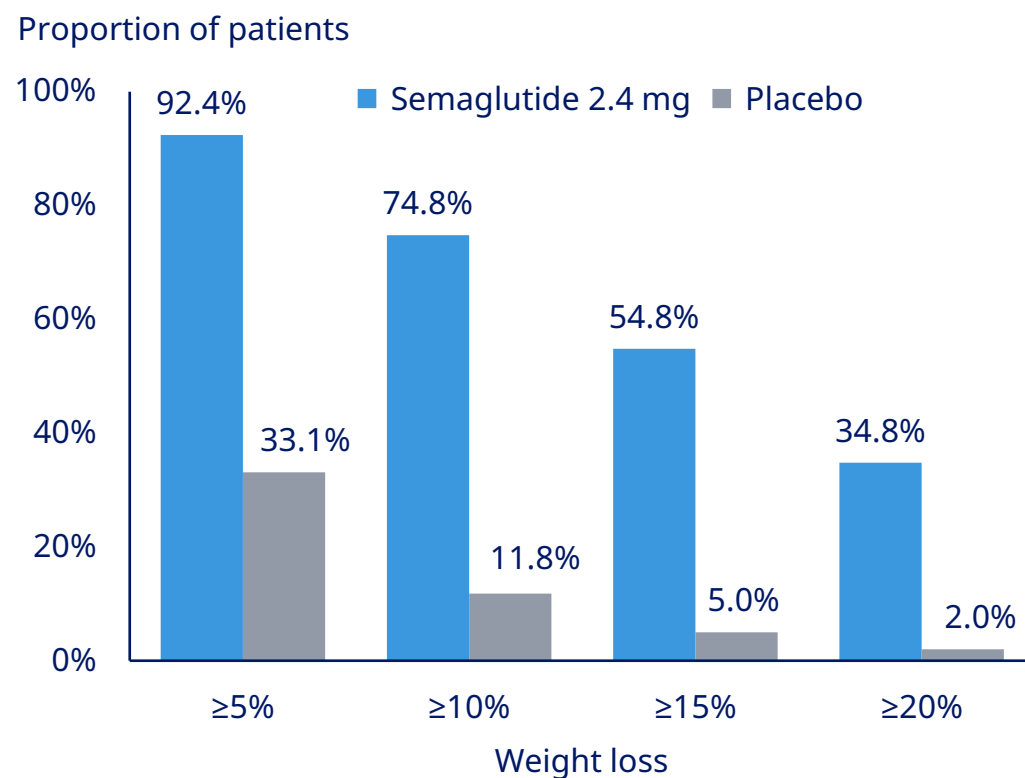
Improvements in lipid profile as well as C-reactive protein



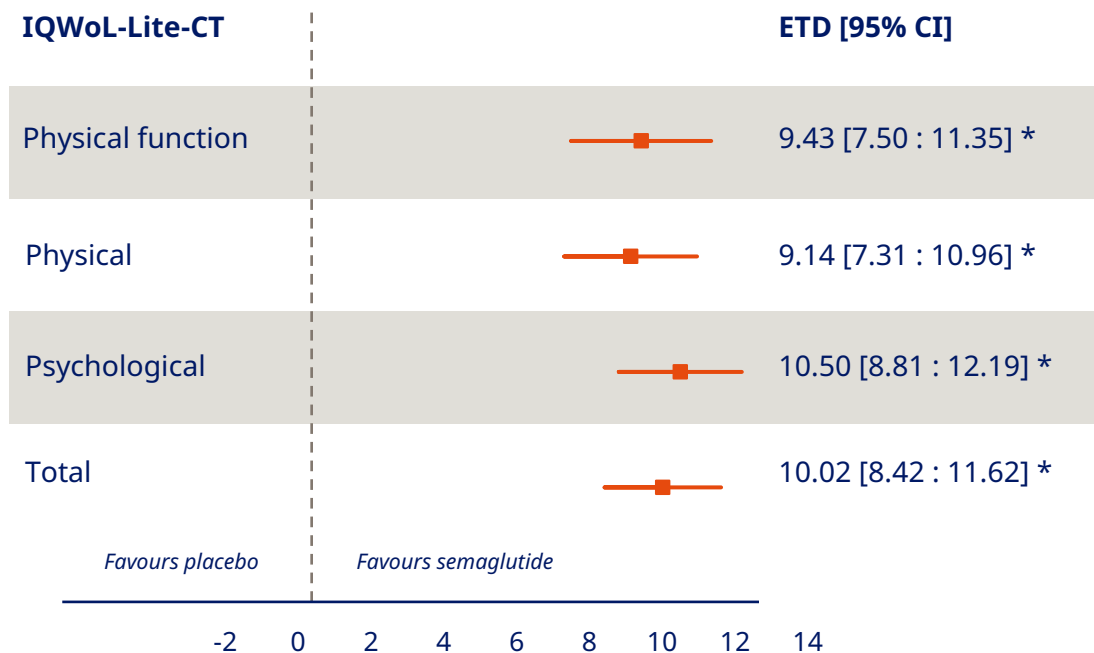
Semaglutide improved health-related quality of life as measured by SF-36 and IWQoL-lite-CT

In STEP 1, 34.8% of patients treated with sema reached $\geq 20\%$ weight loss and reported improved quality of life versus placebo

Categorical weight loss



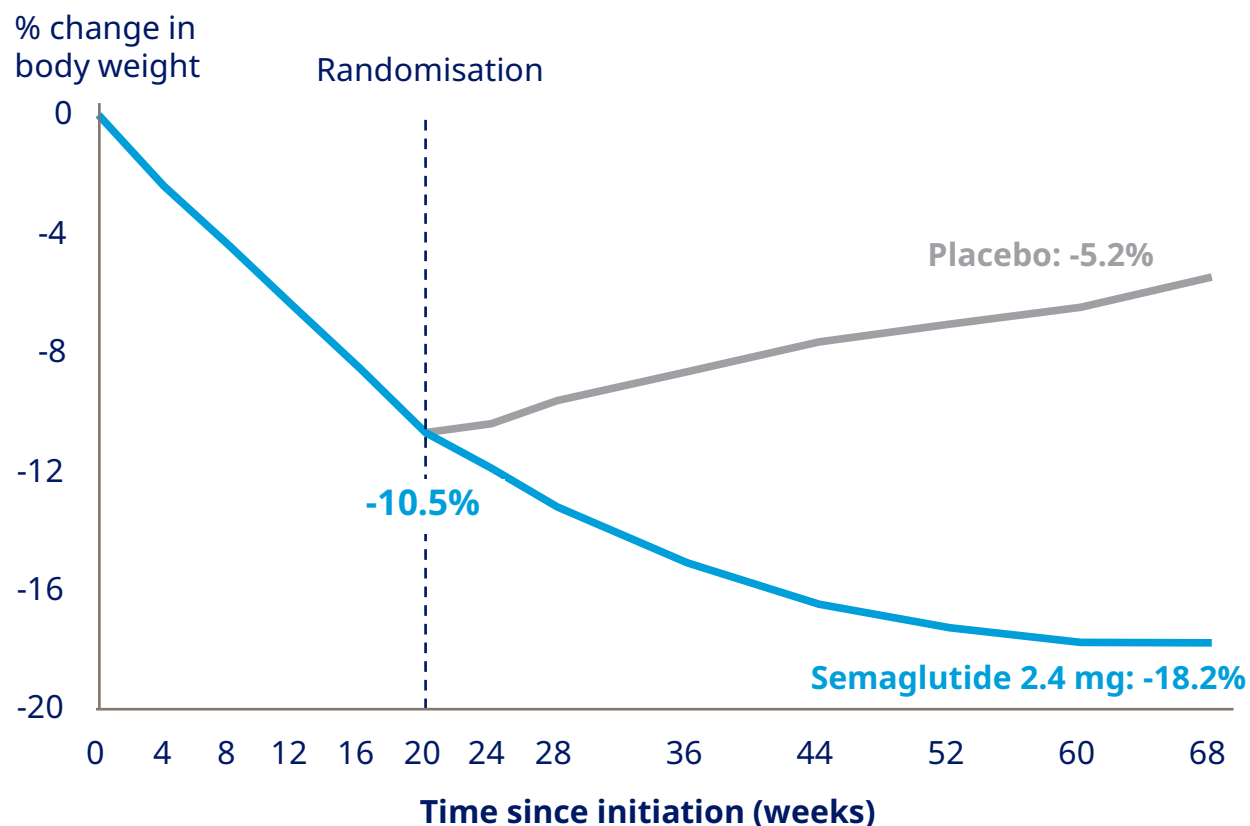
Semaglutide 2.4 mg showed a statistically significant treatment difference versus placebo in the IWQoL-Lite-CT PRO



* statistically significant; p-values other than physical function were not controlled for multiplicity
 PRO: patient reported outcome; CI: confidence interval, ETD: estimated treatment difference, IWQoL-Lite-CT: Impact of Weight on Quality of Life-lite;

In STEP 4, people treated with semaglutide had a superior weight loss of up to 18.2%

STEP 4 showed significantly greater weight loss post run-in than placebo



Data from STEP 4



- Average age 46
- 79% women
- Average BMI – 38.4 kg/m²



Trial highlights that obesity is a chronic disease requiring sustained treatment

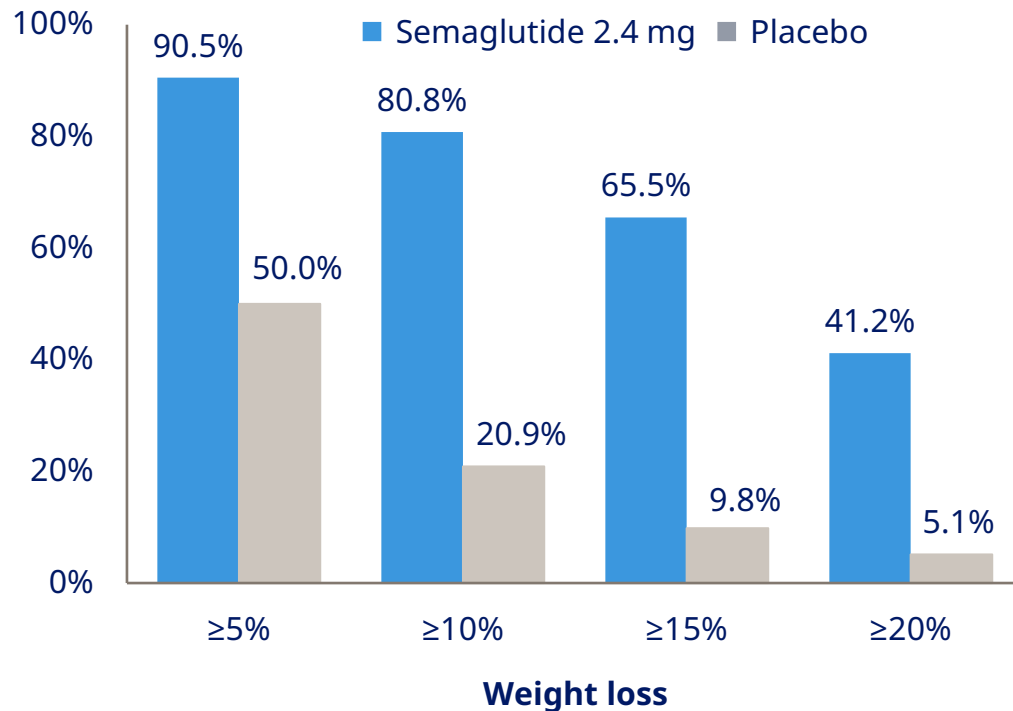


Improvements on a panel of cardiovascular risk markers

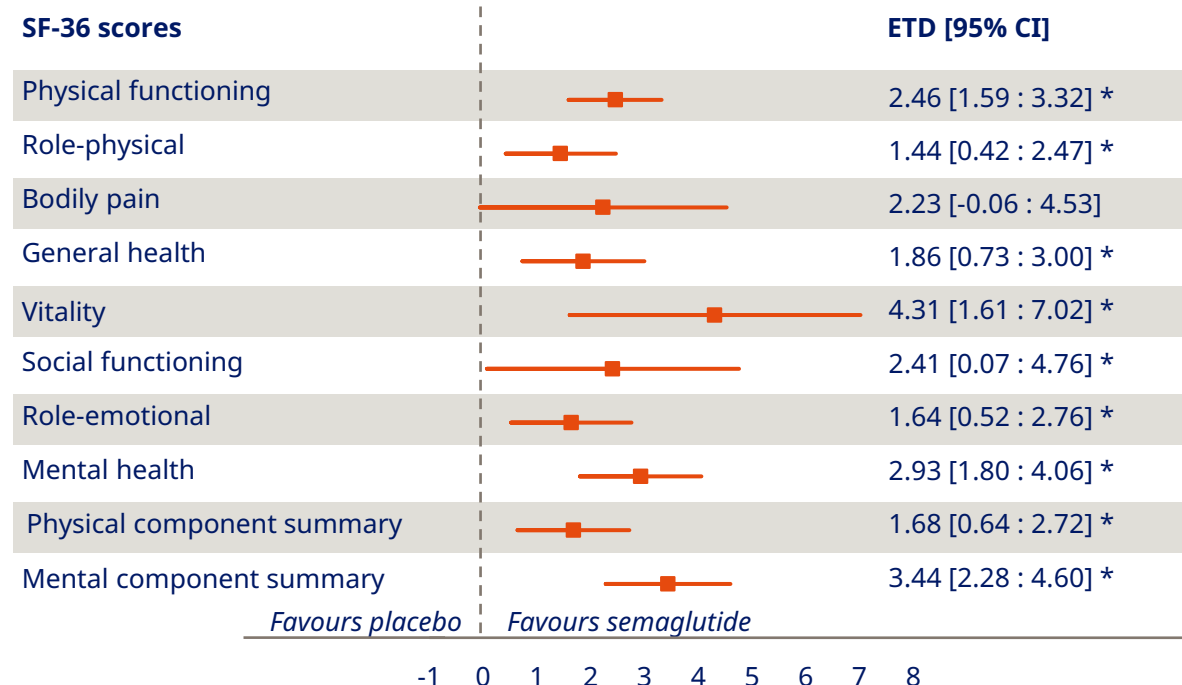
In STEP 4, 41.2% of patients treated with semaglutide reached $\geq 20\%$ weight loss and reported improved quality of life vs placebo

Categorical weight loss

Proportion of patients

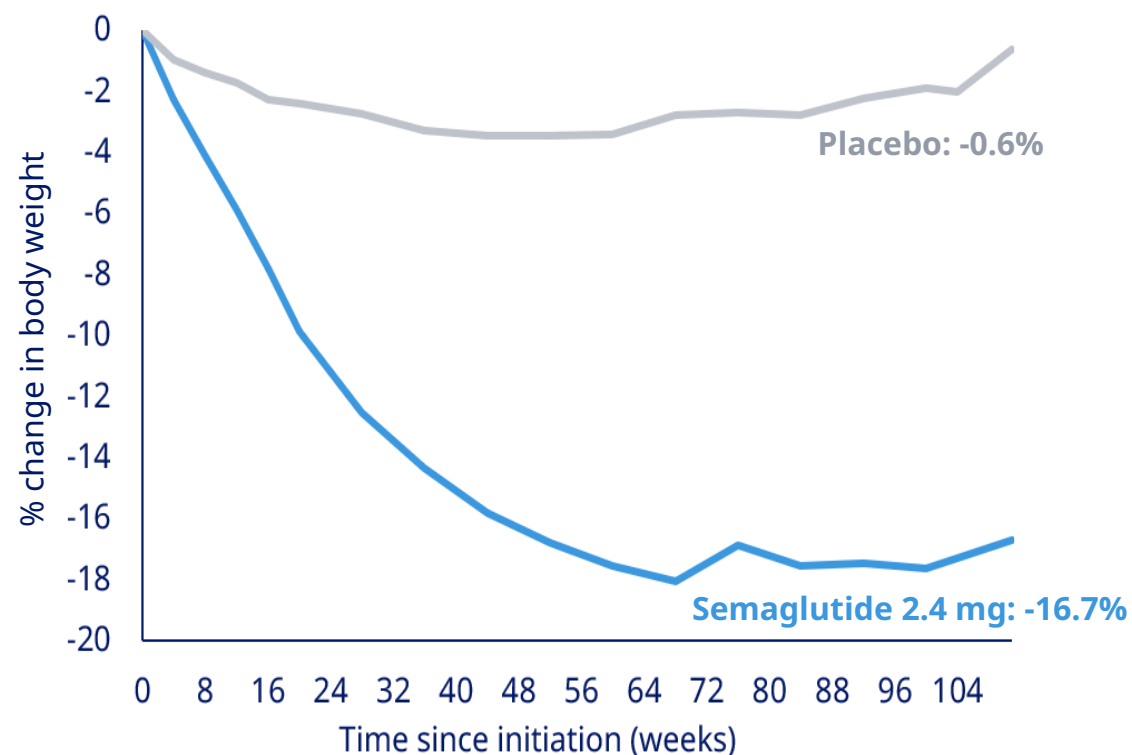


Semaglutide 2.4 mg showed a statistically significant treatment difference versus placebo in the SF-36 patient reported outcome



In STEP 5, people treated with semaglutide 2.4 mg sustained their weight loss over 2 years

Clinically relevant and sustained weight loss in patients with obesity or overweight



Data from STEP 5



40% of patients lost $\geq 20\%$ of their body weight

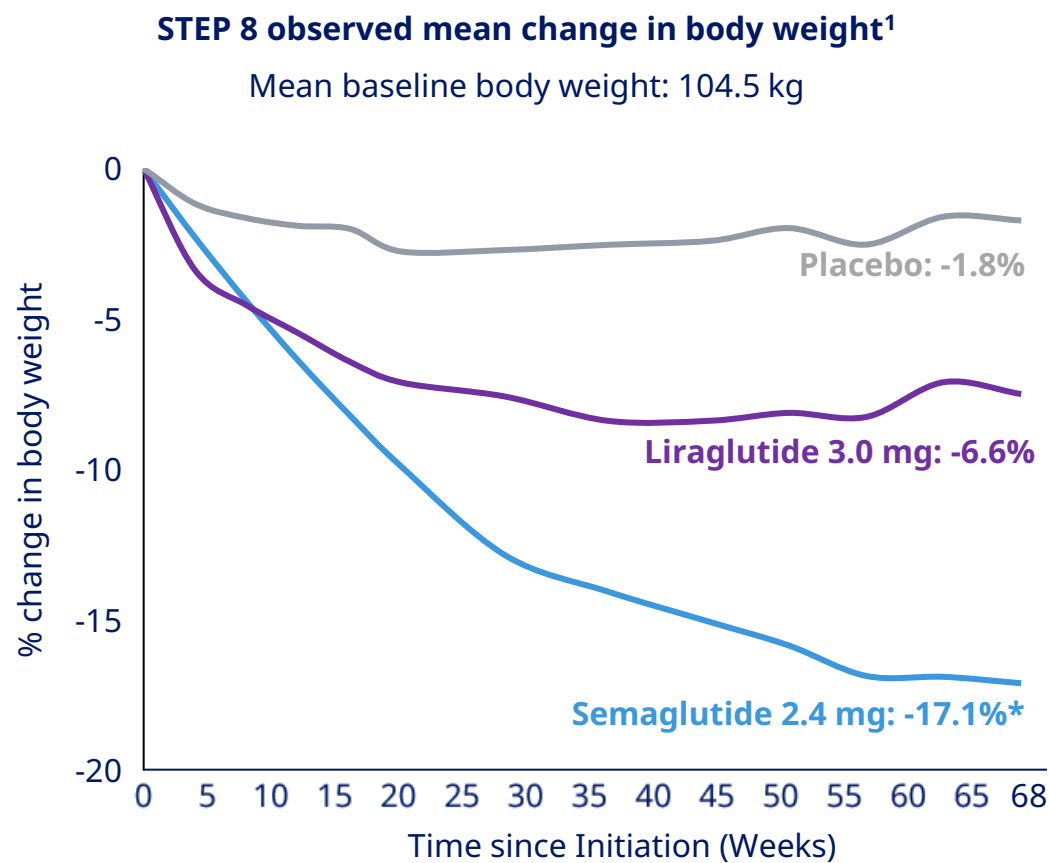


Semaglutide appeared to have a safe and well-tolerated profile



Improvements in lipid profiles as well as C-reactive protein

In STEP 8, semaglutide 2.4 mg showed weight loss of 17.1% compared to 6.6% with liraglutide 3.0 mg



Data from STEP 8



38.5% of patients lost $\geq 20\%$ of their body weight with semaglutide 2.4 mg vs 6.0% with liraglutide 3.0 mg



Liraglutide and semaglutide both appeared to have a safe and well-tolerated profile

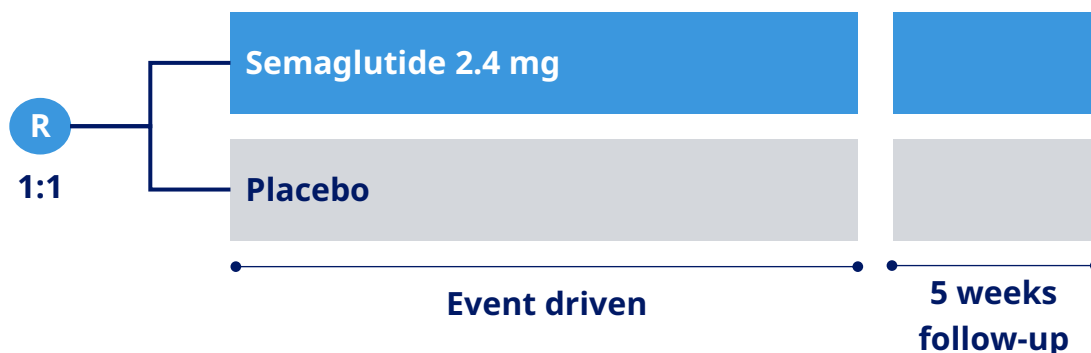


Statistical significant improvements in systolic BP and CRP with semaglutide 2.4 mg vs liraglutide 3.0 mg

¹Observed data for the on-treatment period; *p-value <0.0001 vs lira 3.0 mg; % change in body weight measured as change from baseline
Data shown is the trial product estimand; Sema: Semaglutide; Lira: Liraglutide

Semaglutide 2.4 mg showed 20% MACE reduction in the SELECT trial for people with overweight or obesity and established CVD

SELECT trial with 17,604 people with BMI>27 and established CVD



Primary endpoint

- Time from randomisation to first occurrence of 3-point MACE¹

Secondary confirmatory endpoints

Time from randomisation to first occurrence of:

- CV death
- HF composite endpoint
- All-cause death

Objective

- Demonstrate that semaglutide s.c. 2.4 mg OW lowers the incidence MACE vs. placebo when both added to standard of care in subjects with established CV disease and overweight or obesity.

Headline results

- Semaglutide 2.4 mg demonstrated an 20% reduction in MACE

Safety

- In the trial, once-weekly subcutaneous semaglutide 2.4 mg appeared to have a safe and well-tolerated profile, as seen with previous trials investigating semaglutide 2.4 mg

Next steps

- In March 2024, Wegovy® was approved in the US for CV risk reduction in people with overweight or obesity and established CVD
- In July 2024, Wegovy® was approved in the EU for CV risk reduction in people with overweight or obesity and established CVD

¹MACE includes non-fatal myocardial infarction, non-fatal stroke, and cardiovascular death.

MACE: Major adverse cardiovascular events; HF: Heart failure; CV: Cardiovascular; CVD: Cardiovascular Disease; OW: Once-weekly; s.c.: Subcutaneous; BMI: Body mass index

In SELECT, semaglutide 2.4 mg reduced the risk of a broad composite endpoint by 37%

Key results of the SELECT trial

20% Cardiovascular risk reduction in 3-point MACE

15% Numerical risk reduction of CV death¹

9.4% Sustained weight loss for 4 years

18% Risk reduction of heart failure endpoint²

22% Risk reduction of kidney endpoint

19% Risk reduction on all cause death²

73% Risk reduction of developing diabetes³

Safety

The safety profile of sc semaglutide 2.4 mg in SELECT was similar to that observed in previous clinical trials with semaglutide

Risk reduction in broad composite endpoint

37% Semaglutide 2.4 mg reduces the risk of a broad composite endpoint including:

- Cardiovascular death
- Myocardial infarction
- Stroke
- Other death
- Hospitalisation for UA
- Coronary revascularisation
- Hospitalisation for heart failure
- 5-point Nephropathy
- Diabetes

Number needed to treat to prevent one additional event

Time	Primary endpoint MACE	Broad composite endpoint
1 year	115 people	20 people
4 years	45 people	9 people

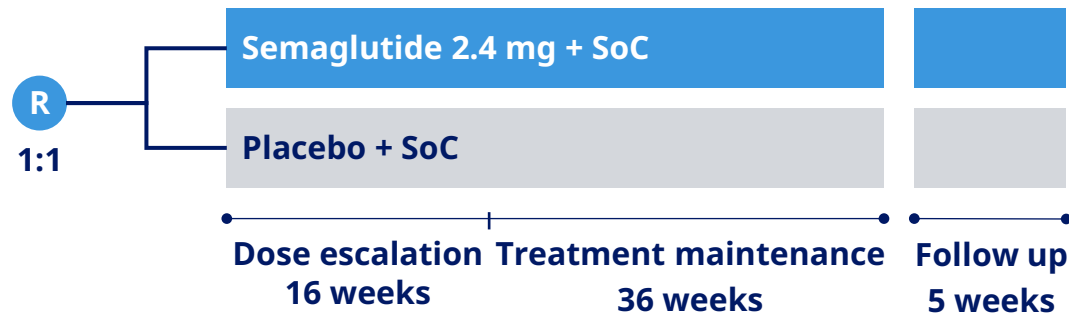
¹Not statistically significant; ²Not tested for superiority; ³73% risk reduction of developing HbA1c ≥ 48 mmol/mol (6.5 %) for semaglutide 2.4 mg vs placebo;

BMI: Body mass index; CI: Confidence interval; CV: Cardiovascular; CVD: Cardiovascular Disease; HR: Hazard ratio; MACE: Major adverse cardiovascular events; sc.: Subcutaneous; UA: Unstable angina

Note: Efficacy analyses based on treatment policy estimand; treatment effect regardless of treatment adherence and changes in background medication. Cumulative incidences of the composite MACE primary endpoint and broad composite endpoint were estimated using the Aalen-Johansen method accounting for non-CV death as competing risk. HRs was estimated using Cox proportional hazards model with treatment as categorical fixed factor

Phase 3 trial STEP HFpEF with semaglutide 2.4 mg was successfully completed in Q2 2023

STEP HFpEF trial with 529 people with obesity and HFpEF



STEP HFpEF

Objective:

- Evaluate the effect on HF specific symptoms, physical function and body weight compared with placebo

Dual primary endpoints:

- Change in KCCQ from baseline to week 52
- Change in body weight from baseline to week 52

Key secondary endpoints:

- Change in 6MWD from baseline to week 52
- Composite endpoint (all cause death, HHF, KCCQ, 6MWD) from baseline to week 52

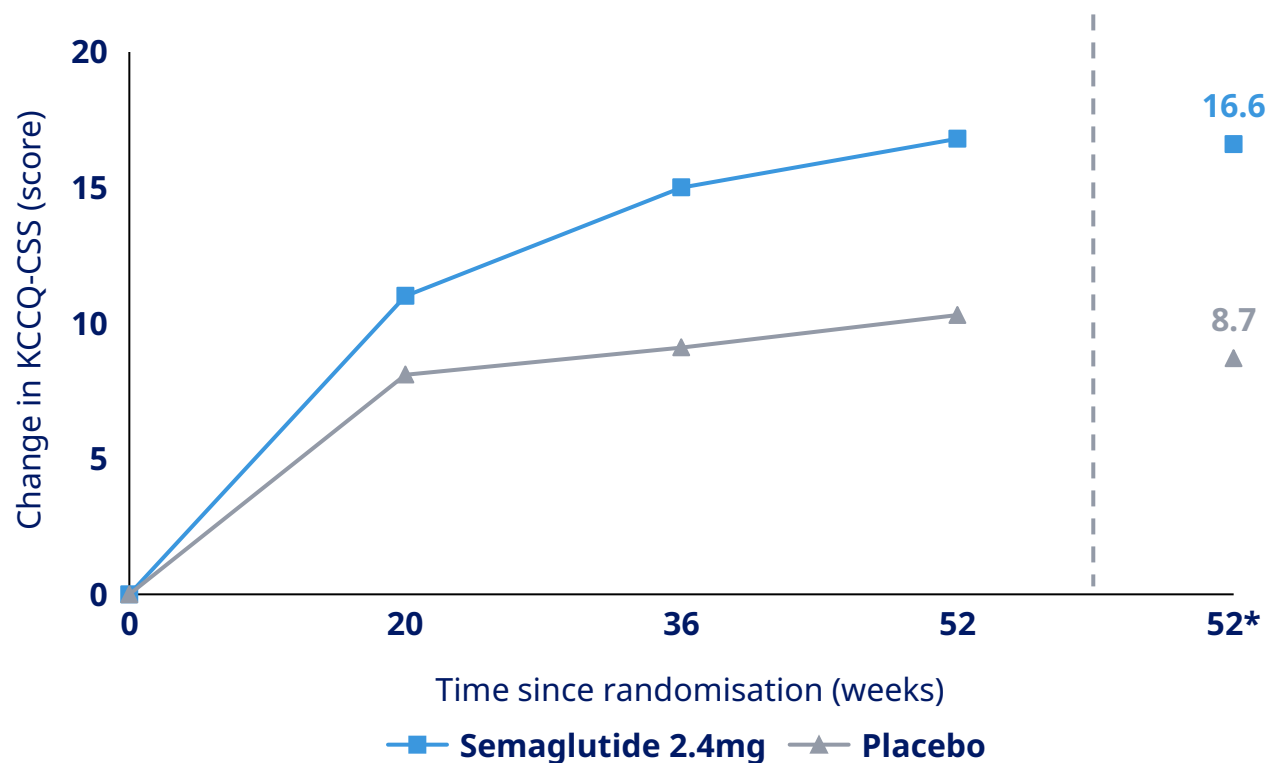
Inclusion criteria:

- BMI ≥ 30 kg/m²
- NYHA II-IV
- Ejection fraction $\geq 45\%$

Semaglutide 2.4 mg demonstrated superior improvement on the primary endpoint of KCCQ-CSS vs placebo in the STEP HFpEF trial

Superior improvement in KCCQ-CSS score in patients treated with semaglutide 2.4 mg

Mean baseline KCCQ-CSS score: 56.7



Key highlights

Primary endpoints:

- KCCQ-CSS estimated treatment difference between semaglutide 2.4 mg and placebo of 7.8

KCCQ in perspective

Clinicians' assessments of clinical change¹:

- Small: ± 5 points
- Moderate-to-large: ± 10 points
- Large-to-very large: ± 20 points

Patients' self-classifications of improvements¹:

- Minimal clinically important difference for 'little improvement': 4.5 points

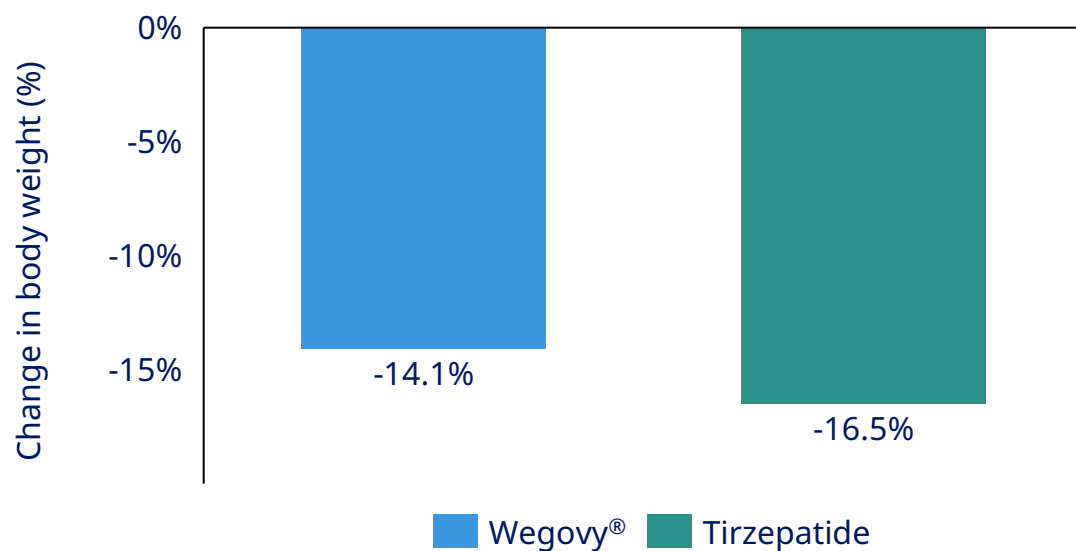
¹Spertus JA, et al. JACC State-of-the-Art Review. J Am Coll Cardiol. 2020 Nov 17;76(20):2379-2390.

Note: Data shown is the treatment policy estimand. *Lines are based on observed data where the value denoted after 52 weeks is estimated mean value derived based on multiple imputation

KCCQ-CSS: Kansas City Cardiomyopathy Questionnaire Clinical summary score

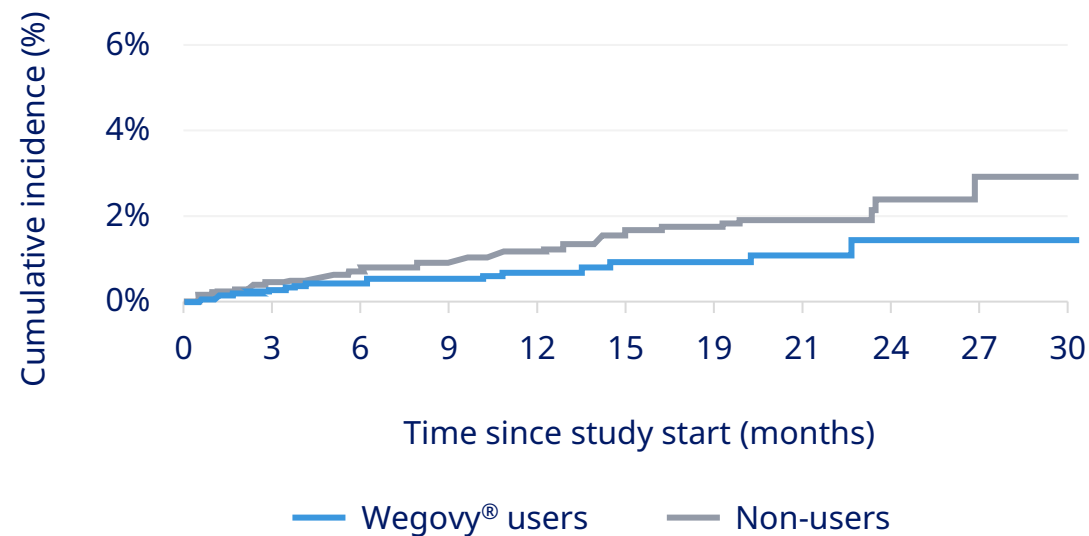
Real world evidence confirms efficacy of Wegovy® and shows 3-point MACE risk reduction of 42%

SHAPE study showed 1-year real-world weight loss in patients with overweight or obesity treated with Wegovy® and tirzepatide



- The SHAPE study included 6,794 patients treated with Wegovy® and 3,122 with tirzepatide
- In a real-world setting, a 2.4%-point weight loss difference between Wegovy® and tirzepatide was seen

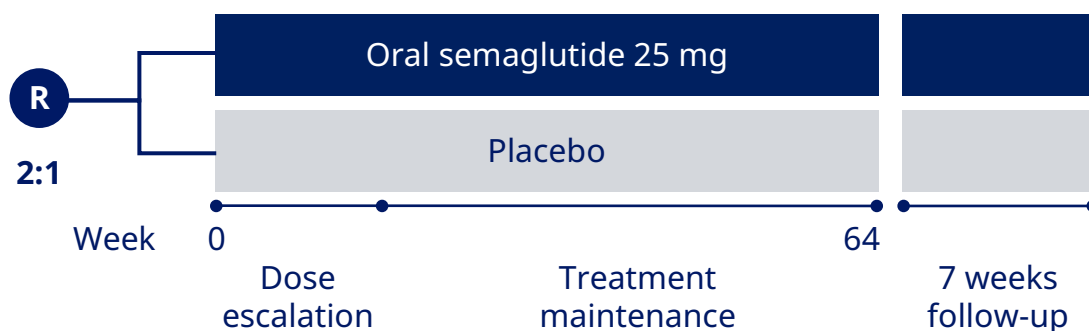
SCORE study showed 42% lower relative risk of 3-point MACE in patients using Wegovy® in routine clinical care vs non-users



- The SCORE study included 9,321 patients treated with Wegovy® and 18,642 non-users
- In the SELECT study, semaglutide 2.4 mg demonstrated an 20% risk reduction in 3-point MACE

Oral semaglutide 25 submitted in the US with efficacy and safety profile broadly similar to Wegovy®

OASIS 4 trial enrolled 306 people with overweight or obesity¹



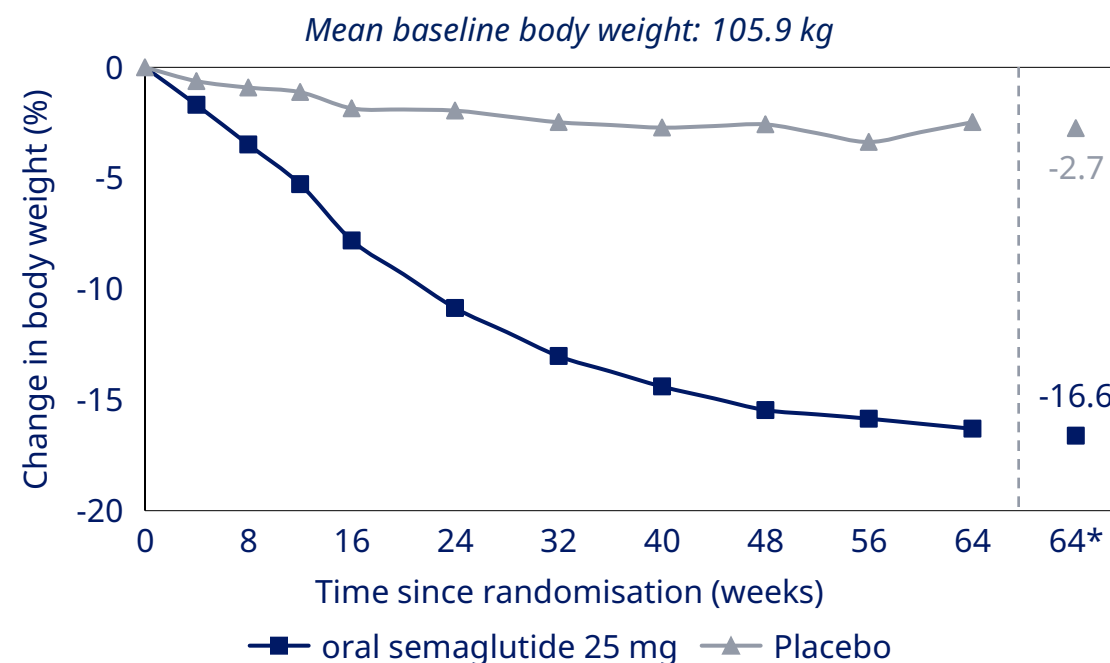
Trial objective

- Confirm superiority of once-daily oral semaglutide 25 mg vs placebo

Co-primary endpoint

- Relative change in body weight (%) from baseline to 64 weeks
- Achievement of $\geq 5\%$ weight loss

Weight loss for oral semaglutide 25 mg in OASIS 4 trial



Categorical weight loss with oral sema 25 mg

$\geq 15\%$ WL reduction

56.1%

$\geq 20\%$ WL reduction

34.4%

¹Estimated means ¹BMI: $\geq 30 \text{ kg/m}^2$ or $\geq 27 \text{ kg/m}^2$ and ≥ 1 comorbidity. Excludes diabetes diagnosis or HbA_{1c} $\geq 6.5\%$

BMI: Body mass index; HbA_{1c}: Haemoglobin A_{1c}; Sema: Semaglutide; US: United States; WL: Weight loss

Note: Trial also included lifestyle intervention, with a 500 kcal/day deficit diet and 150 min/week physical activity. Data shown is trial product estimands

Source: Novo Nordisk data on file

Phase 3 trial programme OASIS for oral semaglutide 50 mg in overweight or obesity

Oral semaglutide characteristics



Oral semaglutide 50mg:

- Semaglutide tablets in overweight or obesity
- Once daily tablet



Phase 3a programme with oral semaglutide 50 mg

- Aims to confirm efficacy and safety
- Oral semaglutide 25 mg submitted to the US FDA

Focused phase 3 trial programme

OASIS 1
50 mg dose

- 667 patients
- 68 week
- Primary endpoint: BW %



OASIS 2
EAST ASIA

- 198 patients incl. T2D
- 68 week
- Primary endpoint: BW %



OASIS 3
China

- 200 patients incl. T2D
- 44 week
- Primary endpoint: BW %



OASIS 4
25 mg dose

- 300 patients
- 64 week
- Primary endpoint: BW %



2022

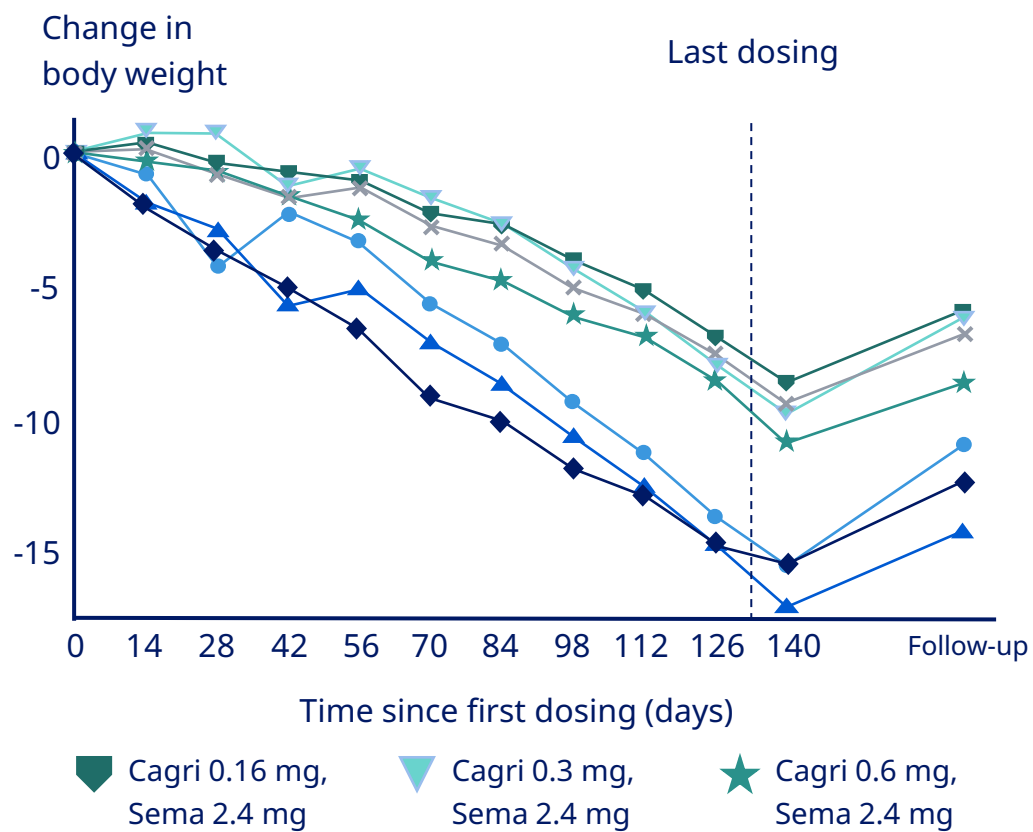
2023

2024

2025

In a 20-week phase 1 trial, CagriSema showed weight loss of 17% and appeared to have a safe and well tolerated profile

Weight loss for different doses of CagriSema in phase 1



The GI profile appeared similar to semaglutide 2.4 monotherapy

	n=12	n=12	n=12	n=12	n=12	n=11	n=24
	N (%)	N (%)	N (%)	N (%)	N (%)	N (%)	N (%)
AEs	11 (92)	12 (100)	11 (92)	12 (100)	12 (100)	11 (100)	23 (96)
SAEs¹	0	0	0	1 (8)	0	0	0
AEs leading to withdrawal	1 (8)	0	0	1 (8)	0	0	0
GI disorders	7 (58)	10 (83)	7 (58)	10 (83)	11 (92)	9 (82)	19 (79)

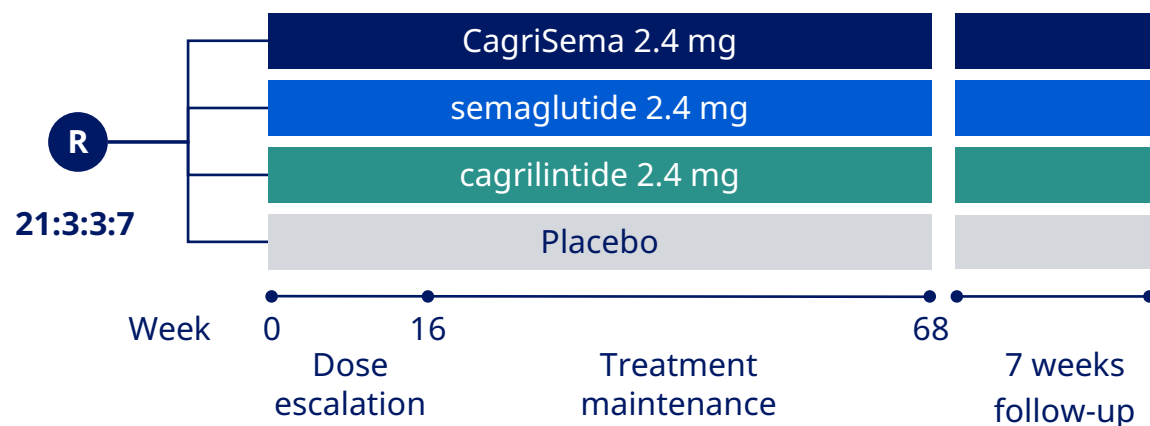
¹The serious adverse event was meningitis

CagriSema: Cagrilintide in combination with semaglutide; Cagri: Cagrilintide; Sema: semaglutide; SAE: Serious adverse events; GI: Gastro-intestinal; Change in body weight is analysed using a mixed model for repeated measurements, where all changes from baseline in body weight measurements enter as the dependent variables and treatment, visit and baseline body weight enter as fixed effects. Treatment and baseline body weight are nested within visit.

Source: Adapted from Enebo et al. Lancet. 2021 May 8;397(10286):1736-1748.

REDEFINE 1 was the first pivotal phase 3 trial to explore CagriSema in people living with overweight or obesity

REDEFINE 1 enrolled 3,417 people with overweight or obesity¹



Trial objective and design considerations

- Confirm superiority of CagriSema 2.4 mg vs placebo, cagrilintide 2.4 mg and semaglutide 2.4 mg
- Flexible trial protocol allowing dose modifications

Co-primary endpoint

- Relative change in body weight (%) from baseline to 68 weeks
- Achievement of $\geq 5\%$ weight loss

Baseline characteristics in REDEFINE 1

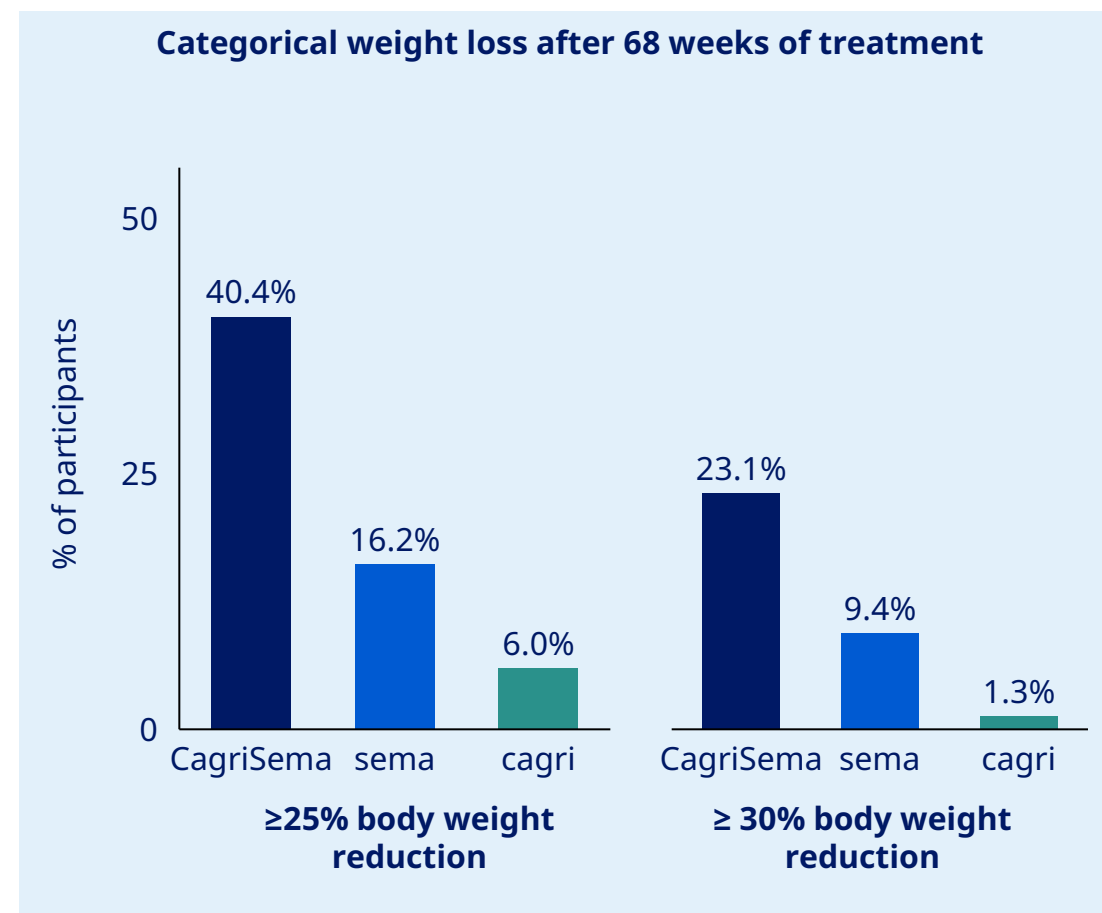
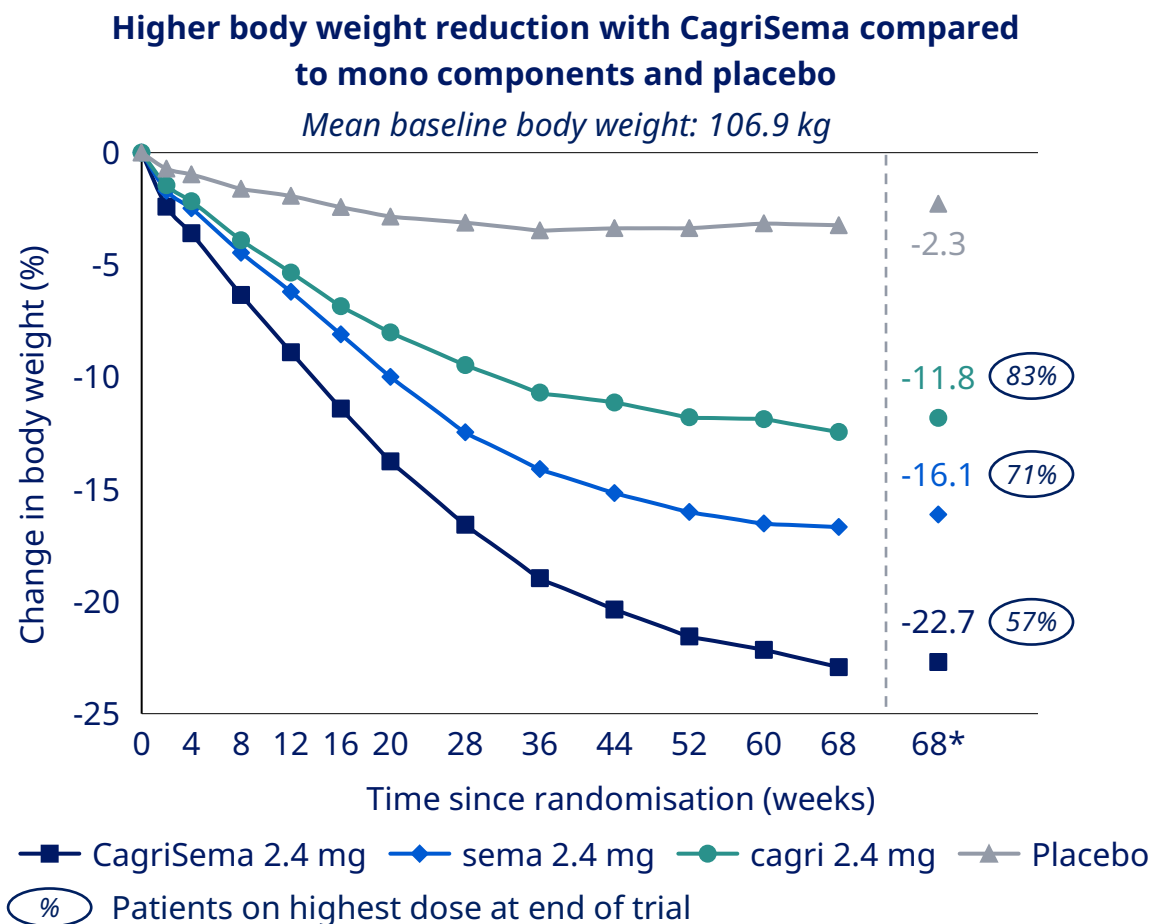
	Female/Male	67.6/32.4%
	Mean age	47 years
	White/Black/Asian/Other	72.0/5.5/18.5/4.0%
	Mean BMI	37.9 kg/m²
	Mean body weight	106.9 kg
	Mean waist circumference	114.7 cm
	Mean HbA _{1c}	5.5%

¹BMI: ≥ 30 kg/m² or ≥ 27 kg/m² and ≥ 1 comorbidity. Excludes diabetes diagnosis or HbA_{1c} $\geq 6.5\%$

BMI: Body mass index; HbA_{1c}: Haemoglobin A_{1c}

Note: CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

In REDEFINE 1, CagriSema achieved 22.7% mean weight loss and more than 40% of participants achieved $\geq 25\%$ weight loss



*Estimated means

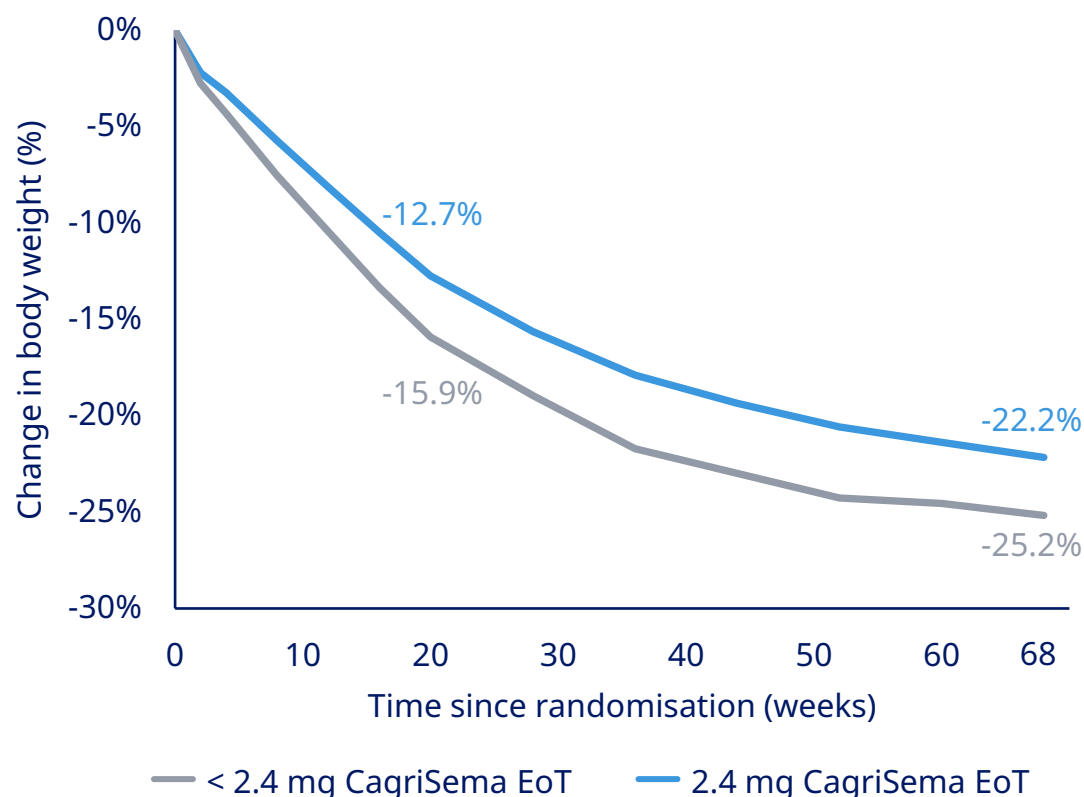
Cagri: cagrilintide; sema: semaglutide

Note: data shown is trial product estimands. CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

Source: Novo Nordisk data on file

Further weight loss potential to be investigated by exploring a longer trial duration and dose re-escalation

Observed weight loss by end of treatment dose in REDEFINE 1¹



Patients treated with the highest dose² at end of treatment

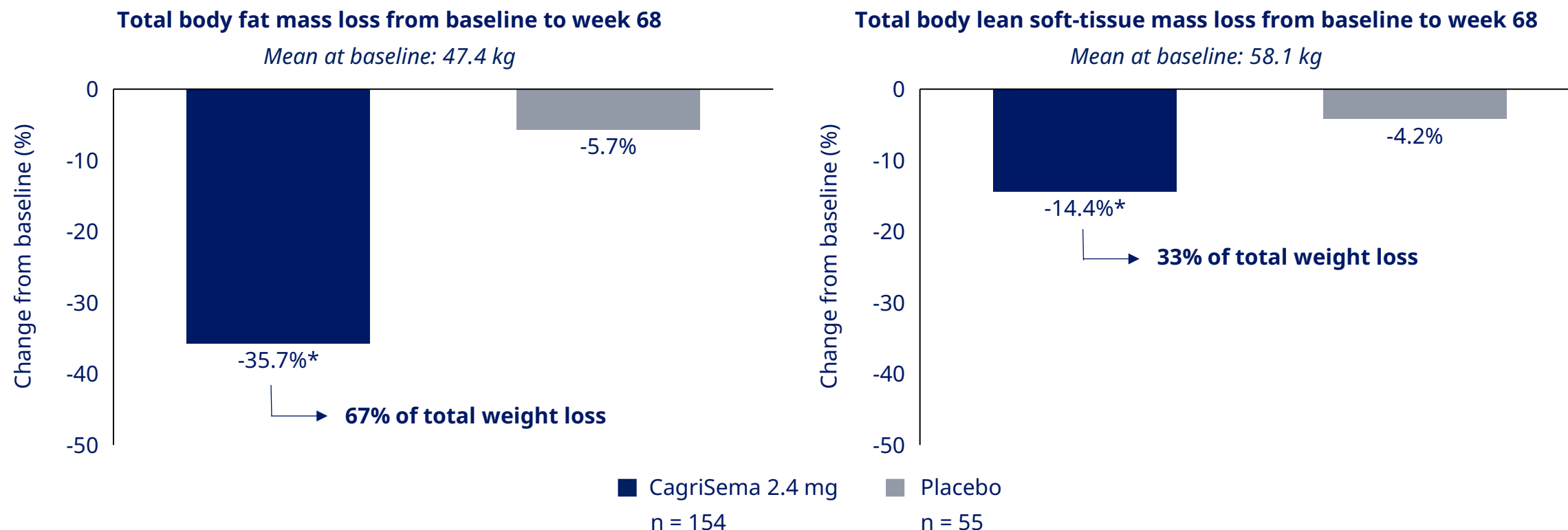
- Weight loss: 12.7% at week 20, 22.2% at week 68
- Tolerability: Average GI AEs per year of 1.9
 - Mean BMI of 30.4 with average dose of 2.4 mg at EoT
- Investigate further weight potential e.g. by longer study duration

Patients treated with lower doses³ at end of treatment

- Weight loss: 15.9% at week 20, 25.2% at week 68
- Tolerability: Average GI AEs per year of 4.0
 - Mean BMI of 26.5 with average dose of 1.1 mg at EoT
- Dose reductions due to: e.g. GI AEs and BMI of lower normal range
- Investigate further weight loss potential e.g. by dose re-escalation

¹Patients are included while on treatment defined until first treatment pause (no trial product for 14 days). A post-hoc analysis of REDEFINE 1. ²Highest dose: 2.4 mg/2.4 mg CagriSema. ³Lower doses: <2.4mg/2.4mg CagriSema. AE: Adverse events; BMI: Body mass index; CagriSema 2.4mg/2.4mg: cagrilintide 2.4 mg and semaglutide 2.4 mg; GI: Gastrointestinal; EoT: End of treatment.

Body composition analysis in REDEFINE 1 showed more than two-thirds body fat mass loss with CagriSema



CagriSema demonstrated an improved body composition at week 68 compared to baseline, with a relative increase of lean soft-tissue mass and decrease of fat mass compared to total body weight

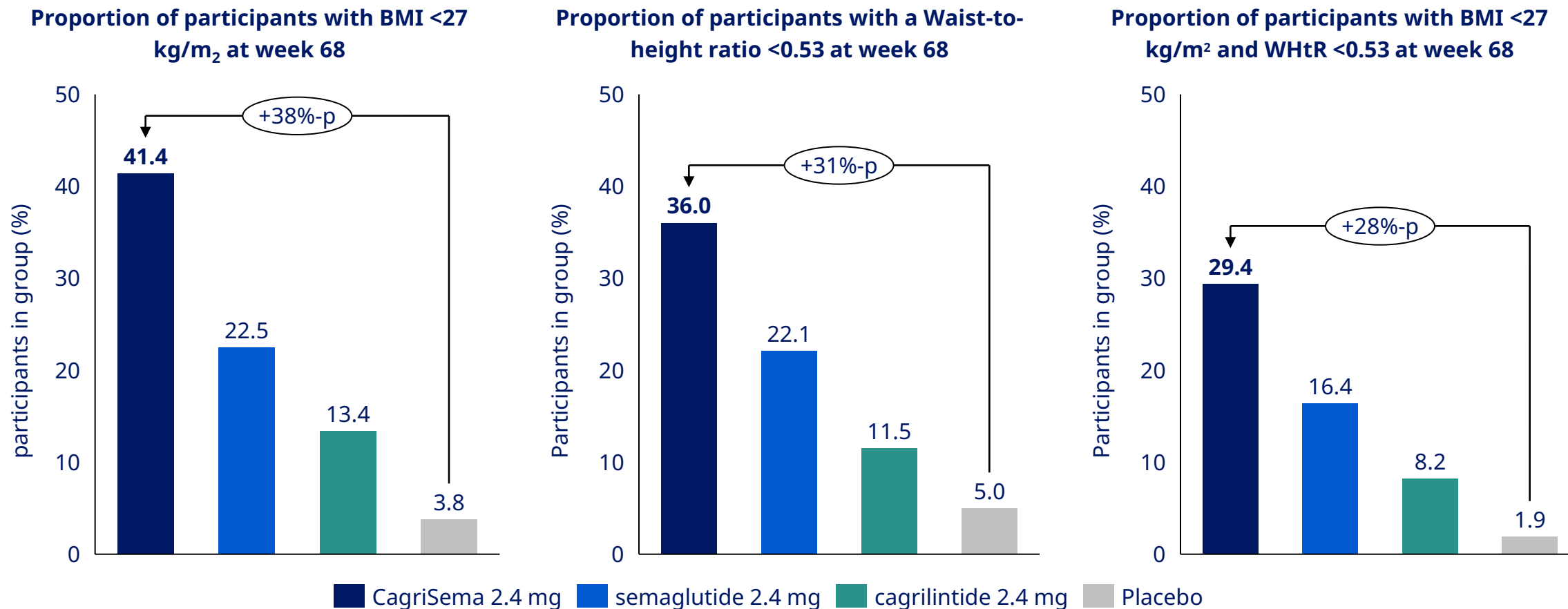
*Significantly more weight loss vs placebo

DXA: dual x-ray absorptiometry

Note: data shown is trial product estimands. CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

Source: Novo Nordisk data on file, CagriSema and placebo DXA subpopulation shown

Treat to target analysis of CagriSema in REDEFINE 1 demonstrates that 41.4% of participants achieve BMI < 27

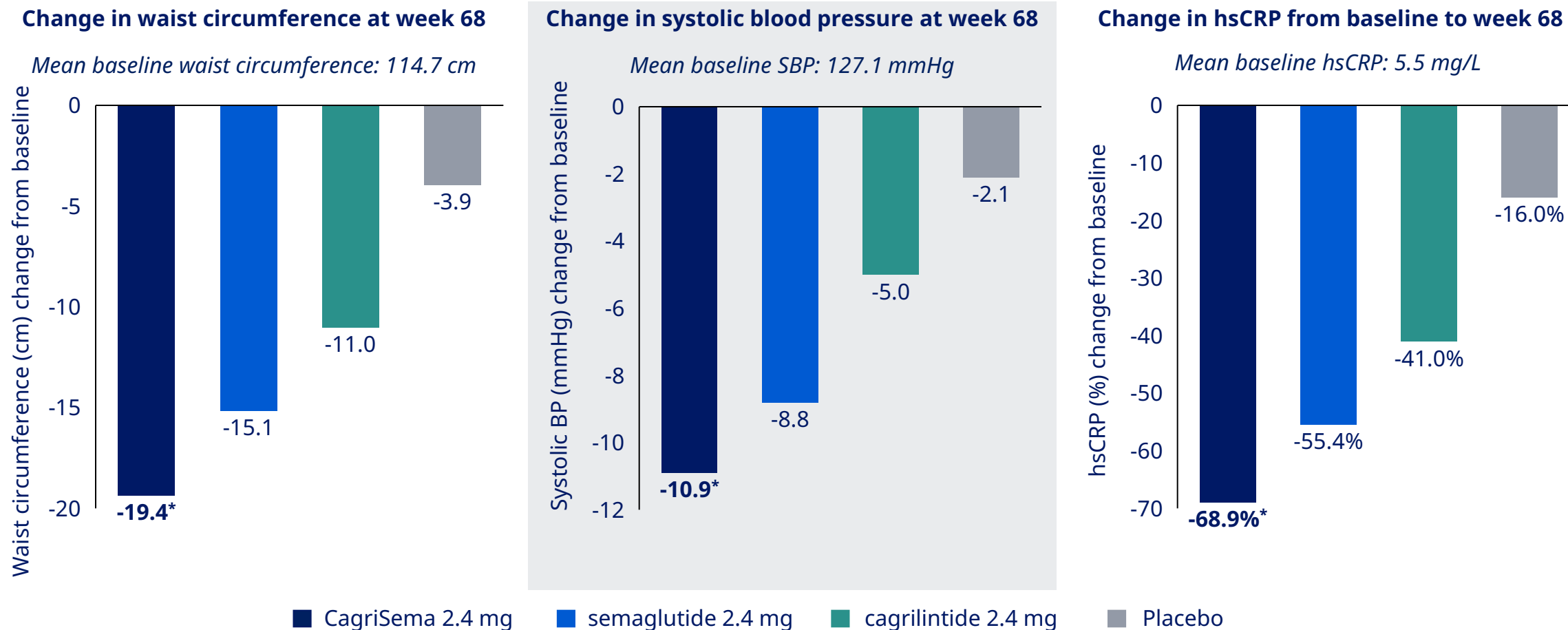


BMI: Body mass index; WHtR: Waist-to-height ratio

Note: Data shown is trial product estimands. CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg; BMI and WHtR indicators of achieving a low 10-year ORC risk, Busetto, Obes Facts 2024;17(suppl 1):7-515 ECO, GC4.158

Source: Novo Nordisk data on file

CagriSema achieved superior reductions in cardiovascular risk factors vs both mono components and placebo in REDEFINE 1



*Statistically significant vs semaglutide 2.4 mg, cagrilintide 2.4 mg, and placebo;

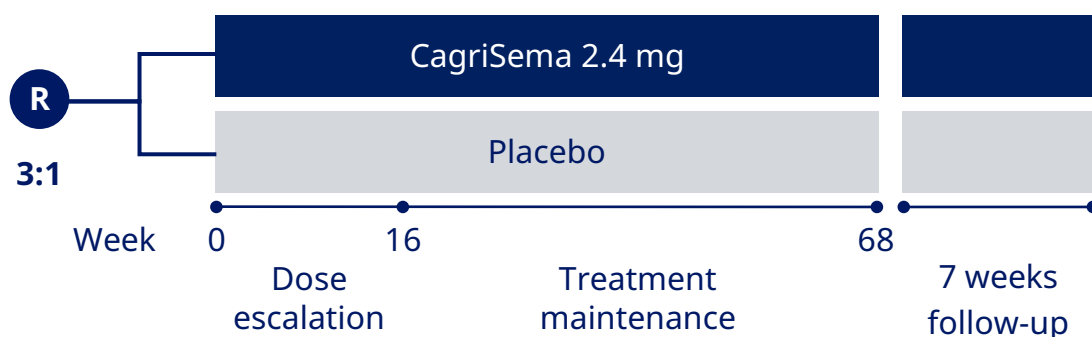
BP: Blood pressure; hsCRP: high-sensitivity C-reactive protein; mmHg: Millimetres of mercury; SBP: Systolic blood pressure

Note: REDEFINE 1 data shown is trial product estimands. CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

Source: Novo Nordisk data on file

In REDEFINE 2, CagriSema achieved 15.7% mean weight loss and more than 29% of participants achieved $\geq 20\%$ weight loss

REDEFINE 2 enrolled 1,206 people with obesity or overweight and T2D¹



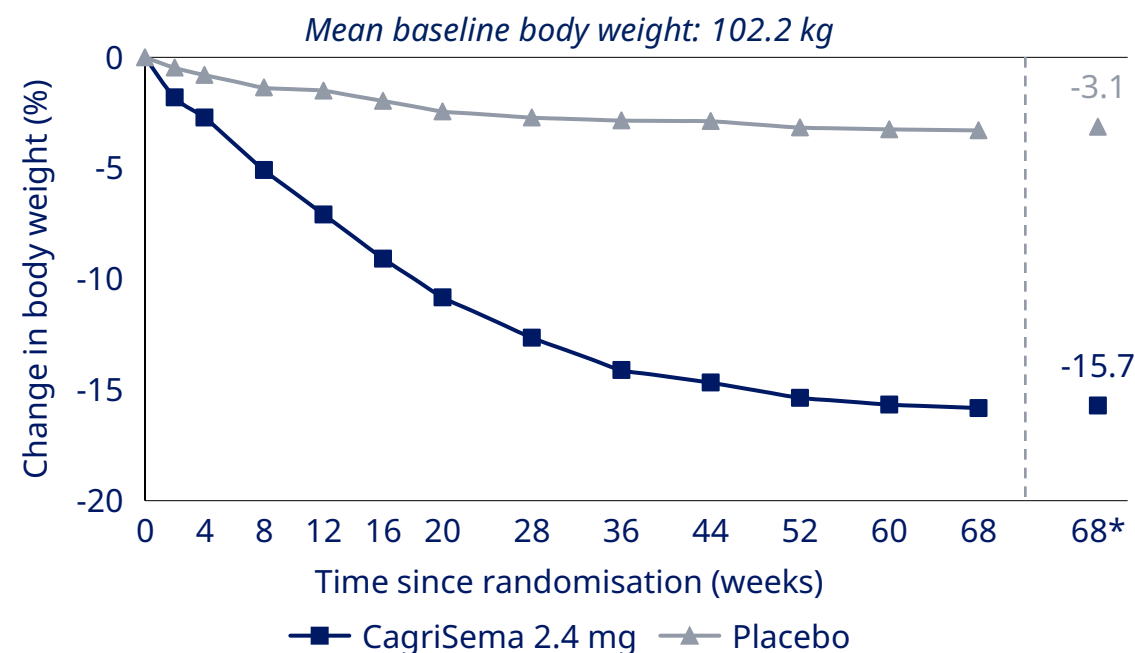
Trial objective and design considerations

- Confirm superiority of CagriSema 2.4 mg vs placebo
- Flexible trial protocol allowing dose modifications

Co-primary endpoint

- Relative change in body weight (%) from baseline to 68 weeks
- Achievement of $\geq 5\%$ weight loss

Weight loss for CagriSema in REDEFINE 2 trial



Categorical weight loss
CagriSema 2.4 mg arm

$\geq 15\%$ WL reduction

51.6%

$\geq 20\%$ WL reduction

29.2%

*Estimated means. ¹BMI: ≥ 27 kg/m² and T2D with HbA1c $\leq 10\%$. 0-3 OADs (no GLP-1 in the last 90 days, no insulin)

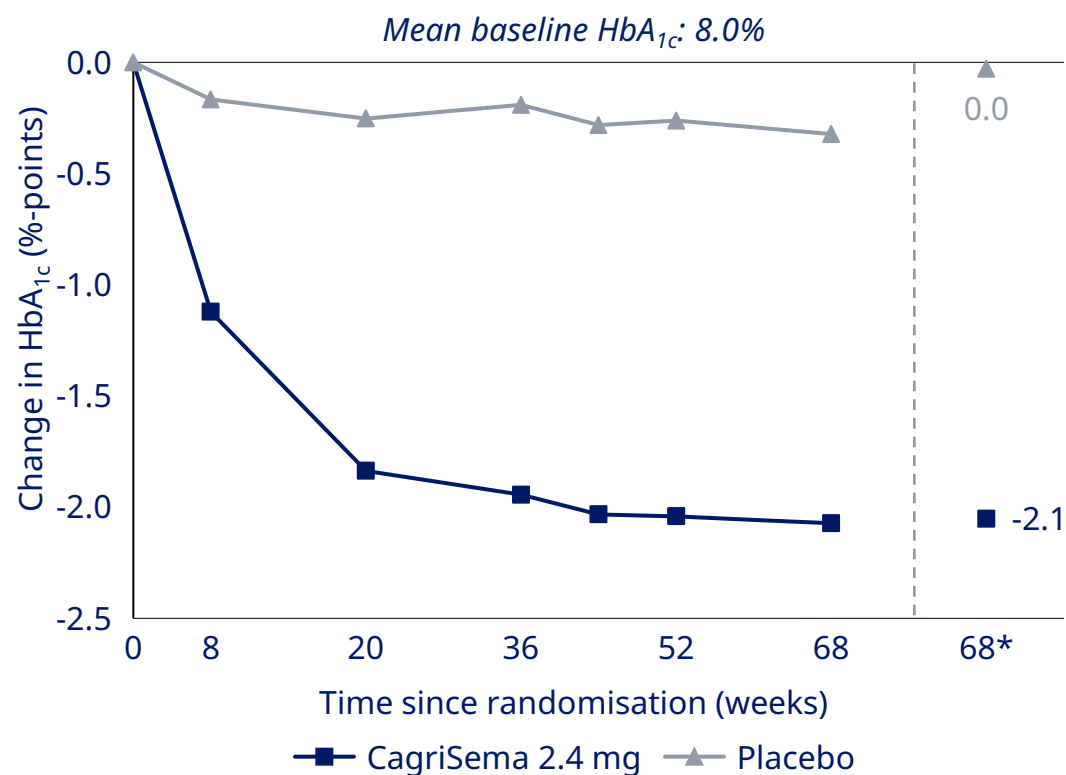
OAD: Oral anti-diabetic; T2D: Type 2 diabetes; WL: Weight loss

Note: data shown is trial product estimands. CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

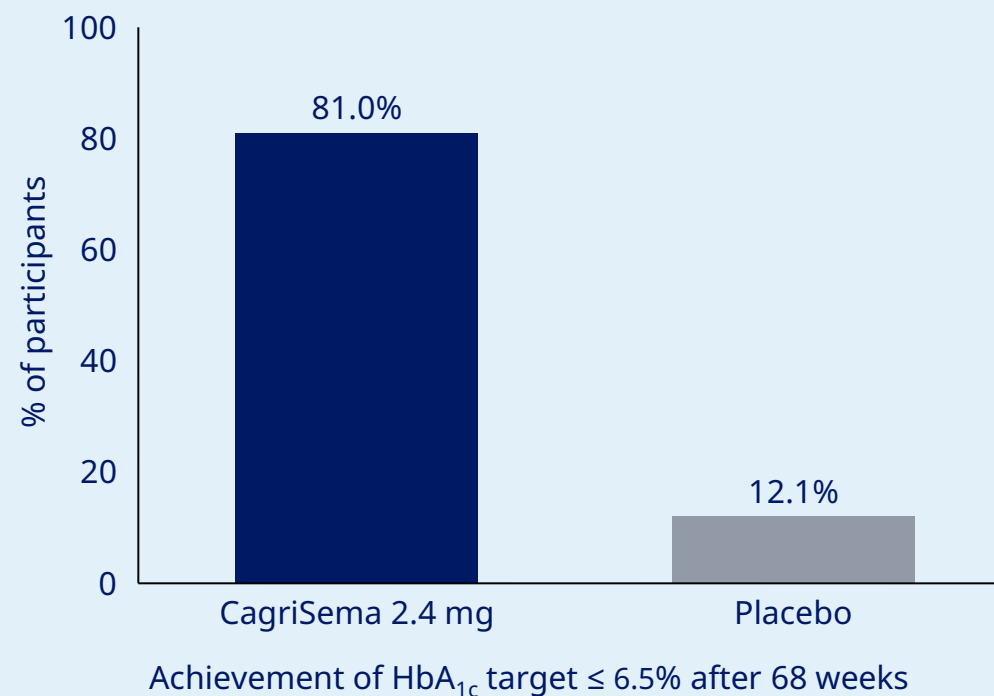
Source: Novo Nordisk data on file

In REDEFINE 2, CagriSema achieved a HbA_{1c} reduction of 2.1%-p, and more than 80% of participants achieved HbA_{1c} target <6.5%

Higher HbA_{1c} reduction with CagriSema compared to placebo



More participants achieved the HbA_{1c} target with CagriSema compared to placebo



*Estimated means

HbA_{1c}: Haemoglobin A_{1c}

Note: data shown is trial product estimands. CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

Source: Novo Nordisk data on file

CagriSema successfully completed pivotal trials and with additional trials ongoing to investigate even further potential

Selected CagriSema phase 3 development trials in Obesity

REDEFINE 3 CVOT

- 7,000 participants
- Primary endpoint: 3-point MACE

REDEFINE 4 H2H vs tirzepatide

- 800 participants
- 84-week vs. tirzepatide
- Primary endpoint: Weight loss

REDEFINE 9 Maintenance doses 1.0 and 1.7 mg

- 300 participants
- 64-week vs. placebo
- Primary endpoint: Weight loss

REDEFINE 11 WL in Obesity

- 600 participants
- 80-week vs. placebo
- Primary endpoint: Weight loss

2024

2025

2026

Pivotal trials

- CagriSema showed substantial weight loss of 22.7%
 - More than 40% of patients achieving BMI < 27
 - Superior reductions in several CV risk factors
- CagriSema appeared to have a safe and well-tolerated profile with overall low discontinuation rates

Further development

- First regulatory submission expected in Q1 2026
- Potential to leverage semaglutide CV effect. In REDEFINE 3 exploring potential complementary amylin effects.
- REDEFINE 9 to explore lower maintenance doses
- REDEFINE 11 initiated to explore further weight loss potential

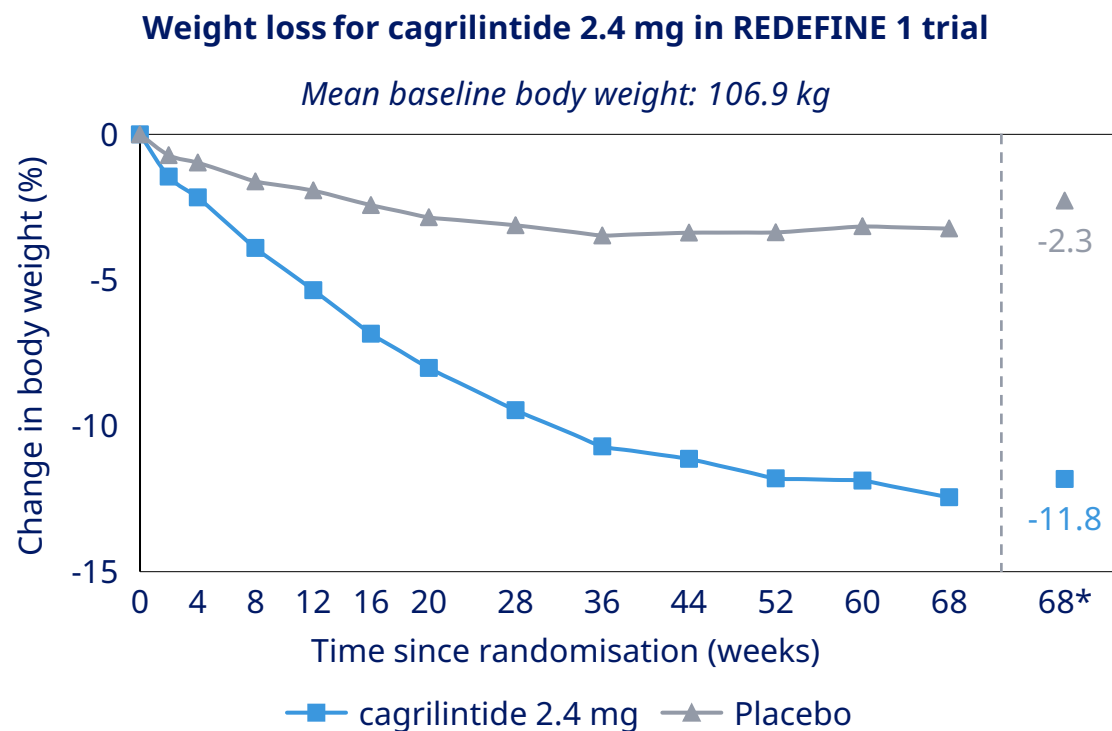
Portfolio

- Pending approvals, US obesity portfolio to include CagriSema, Wegovy® and oral semaglutide 25 mg

CV: Cardiovascular; CVOT: Cardiovascular Outcomes Trial; H2H: Head-to-Head; MACE: Major adverse cardiovascular event; T2D: Type 2 Diabetes; US: United States; WL: Weight Loss

Note: The CagriSema phase 3 development programme also includes REDEFINE 5 (weight loss trial in East Asia with 330 participants) and REDEFINE 6 (weight loss trial in China with 300 participants). CagriSema is a fixed dose combination of injectable cagrilintide 2.4 mg and injectable semaglutide 2.4 mg

Cagrilintide 2.4 mg achieved 11.8% weight loss in the REDEFINE 1 trial with a 1.3% discontinuation rate due to GI adverse events



- In the trial, cagrilintide 2.4 mg appeared to have a safe and well-tolerated profile
- 1.3% discontinuation rate due to gastrointestinal adverse events

	cagrilintide 2.4 mg (n = 302)		Placebo (n = 705)	
	n	%	n	%
Gastrointestinal AEs	165	54.6	287	40.7
Nausea	72	23.8	93	13.2
Diarrhoea	47	15.6	91	12.9
Vomiting	21	7.0	31	4.4
Constipation	63	20.9	87	12.3

Next steps:

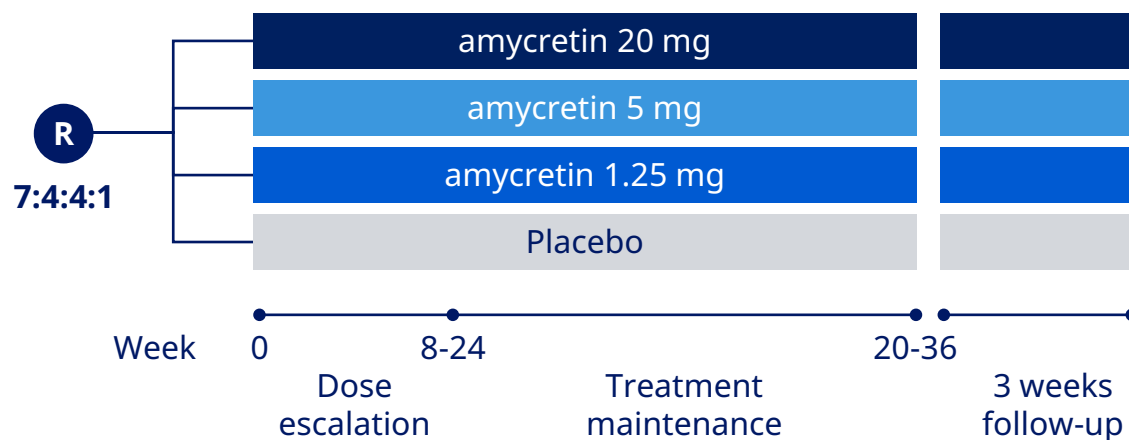
- Phase 3 programme expected to start in Q4 2025

Potential of cagrilintide:

- Once-weekly sc treatment aims to provide effective weight management with a favorable tolerability compared to GLP-1s

The phase 1b/2a trial with subcutaneous amycretin was successfully completed in people with overweight or obesity

Proof of concept part¹ of the sc. amycretin phase 1b/2a trial



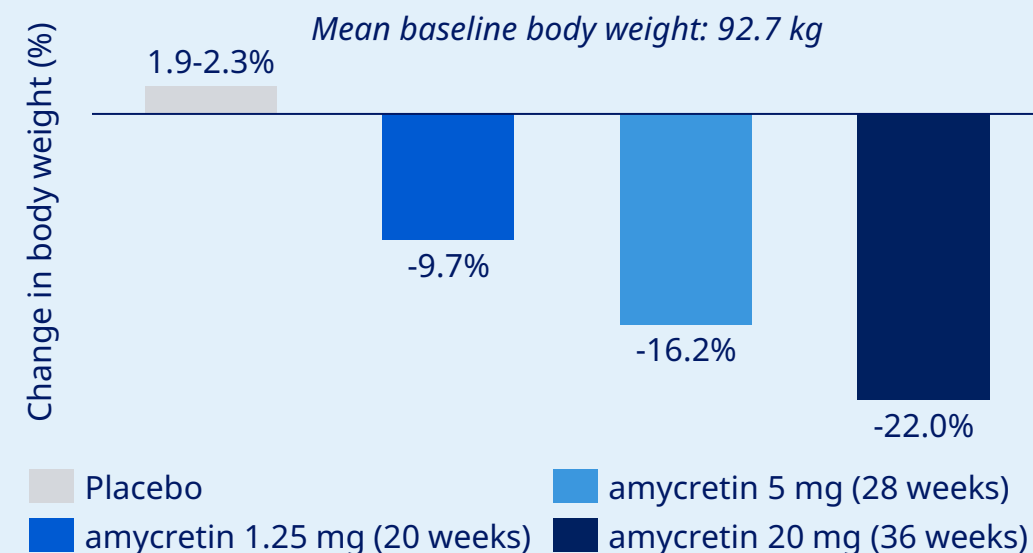
Objective

- Objective: Investigate safety, tolerability, pharmacokinetics and efficacy of amycretin in participants with overweight or obesity

Endpoints

- Primary: Number of treatment emergent adverse events
- Secondary: Relative change in body weight, AUC, c_{max} , t_{max}

Weight loss² in proof of concept part



Safety

- Profile of amycretin was consistent with incretin-based therapies

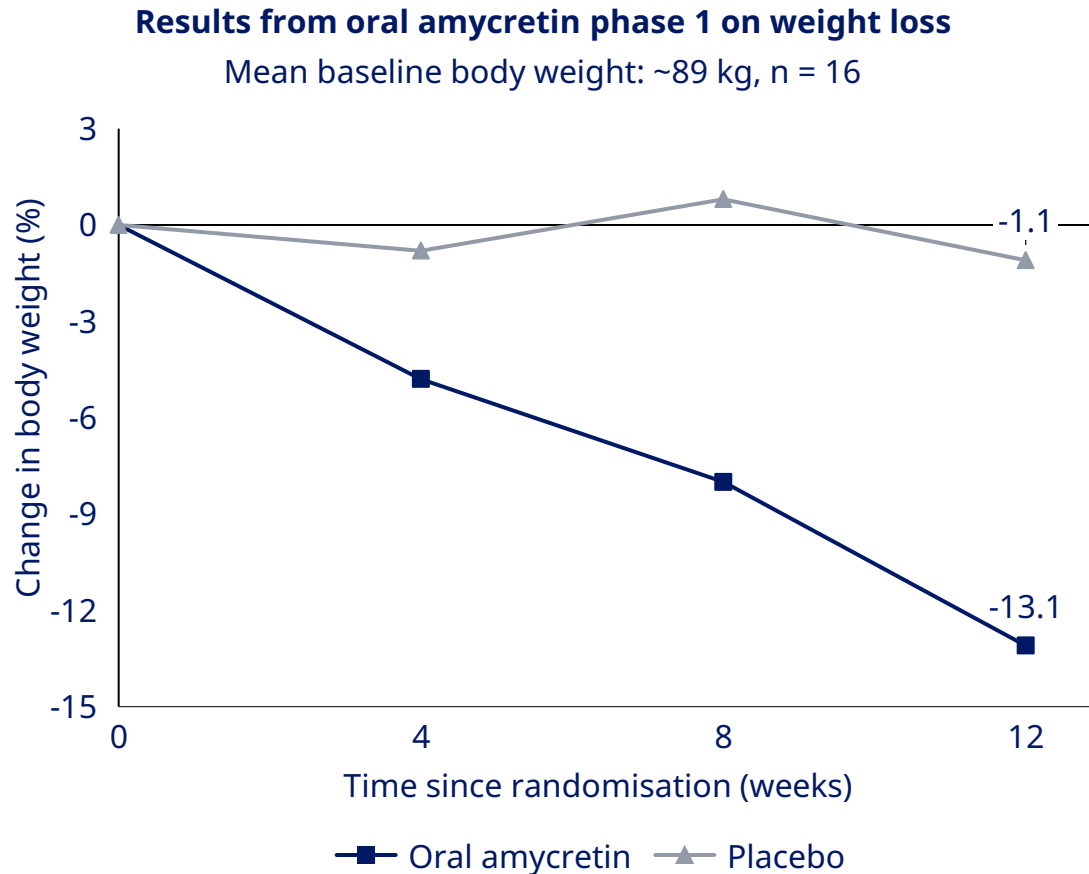
Next steps

- Novo Nordisk is now planning further clinical development of amycretin in adults with overweight or obesity

¹BMI: ≥ 27 -39.9 kg/m². Excludes diabetes diagnosis or HbA1c $\geq 6.5\%$ ²Based on the trial product estimand

Amycretin is a unimolecular GLP-1 and amylin receptor agonist; AUC: Area Under the Curve; c_{max} : maximum (peak) plasma concentration; sc.: subcutaneous; t_{max} : time to reach maximum (peak) plasma concentration

Oral amycretin phase 1 and subcutaneous phase 1b/2a trials have been completed



Amycretin development programme in obesity

Phase 1:

- ✓ Oral amycretin phase 1 completed in 2024
- ✓ Subcutaneous amycretin phase 1b/2a completed in 2025

Next steps:

- Novo Nordisk is now planning further clinical development of amycretin in adults with overweight or obesity

Novo Nordisk is continuing the development of a portfolio of treatment solutions for obesity

Building a leading portfolio

Our key focus areas



Body weight loss



Composition of weight loss



Co-morbidity impact



Safety and tolerability



Dosing frequency

Obesity development pipeline

Obesity

Project	Phase
Saxenda® (liraglutide 3.0 mg)	<i>Marketed</i>
Wegovy® (semaglutide 2.4 mg)	<i>Marketed</i>
oral semaglutide (25 mg)	Submitted in US
semaglutide 7.2 mg	Submitted in EU
CagriSema (2.4 mg/2.4 mg)	Pivotal phase 3 completed
cagrilinitide	Phase 3 planning
monlunabant	Phase 2 ongoing
sc. amycretin OW and oral OD	Phase 3 to be initiated
FUSE¹ - Peripheral focused ultrasound	Phase 2 to be initiated
UBT251² (GGG tri-agonist)	Phase 1b completed
Triple (tri-agonist)	Phase 1 ongoing
amylin 355	Phase 1 ongoing
amylin 1213	Phase 1 ongoing
LX9851 (small molecule)	Phase 1 to be initiated

¹In collaboration with GE Healthcare ²Pending customary closing conditions
CB1R: Cannabinoid receptor 1; GIP: Gastric inhibitory polypeptide; OD: Once-daily; OW: Once-weekly; Sc.: Subcutaneous

Rare disease

Rare disease background

Rare disease innovation

SIERRA CLARK

Sierra lives with Glanzmann-Thrombasthenia
Canada

RareD constitutes an attractive opportunity for Novo Nordisk

Addressing the unmet needs

Patient burdens¹

- Reduced life-expectancy
- Severe co-morbidities and impaired quality of life
- Long diagnostic lead-times
- Broken continuum of care and strong inequalities

A longstanding legacy

Since 1970s in growth disorders

norditropin®
somatotropin (rDNA origin) injection

Since 1980s in haemophilia

NovoSeven®
Recombinant Factor VIII
refixia®
nonacog beta pegol
esperoct®
turoctocog alfa pegol

The Rare disease opportunity for Novo Nordisk

A strategic portfolio play in specialty care



Few patients, high unmet need



Specialised healthcare base



Specialised scientific and commercial teams

A platform to spearhead new trends

Integrated therapeutic solutions
adding diagnostics, digital, data, device and drug (5D)

Innovative access pathways

New operating models

An integrated unit

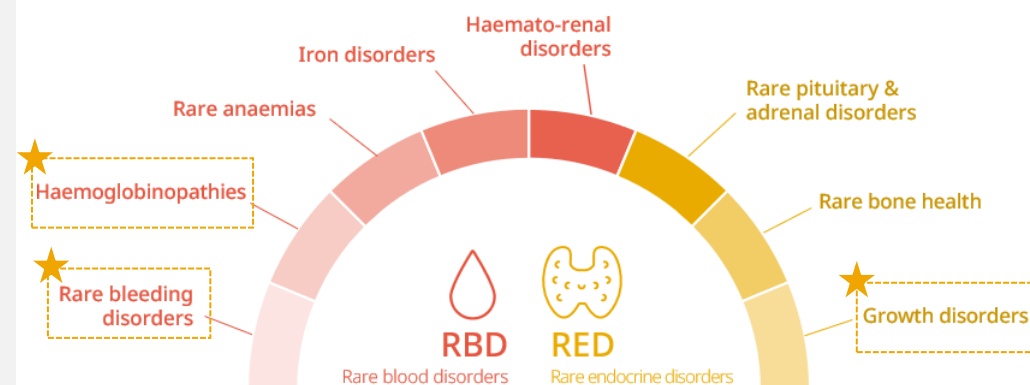
From research to commercial, RareD is operating as an **integrated unit** within Novo Nordisk, with dedicated resources, to provide agility and flexibility

¹Editorial, The Lancet Diabetes & Endocrinology. 2019; 7(2)75
Note: RareD is Novo Nordisk's rare disease unit

Executing on new strategy since 2019 with near-term focus on next generation launches

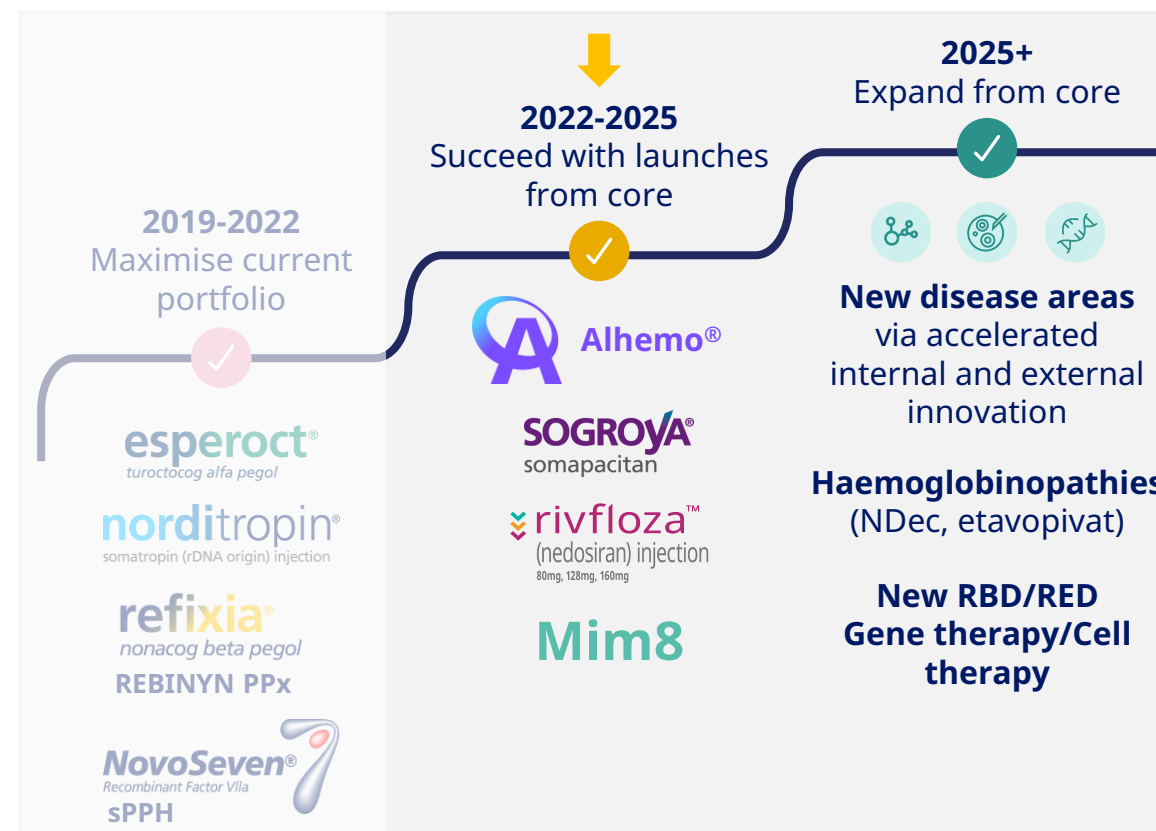
The Rare disease strategy

Strategic focus areas



Out of the 350 million+ rare disease patients globally¹, RareD focuses on a total addressable pool of 20 million (6% of total) today

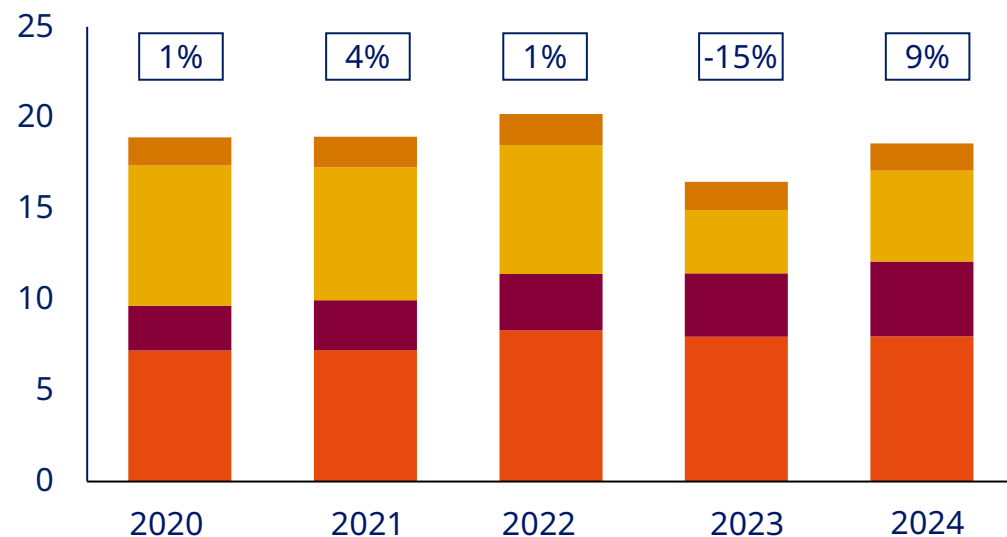
Focus on succeeding with launches from the core



Rare disease sales increased 9% by end of 2024

**NovoSeven® and Norditropin®
account for ~64% of Rare disease sales**

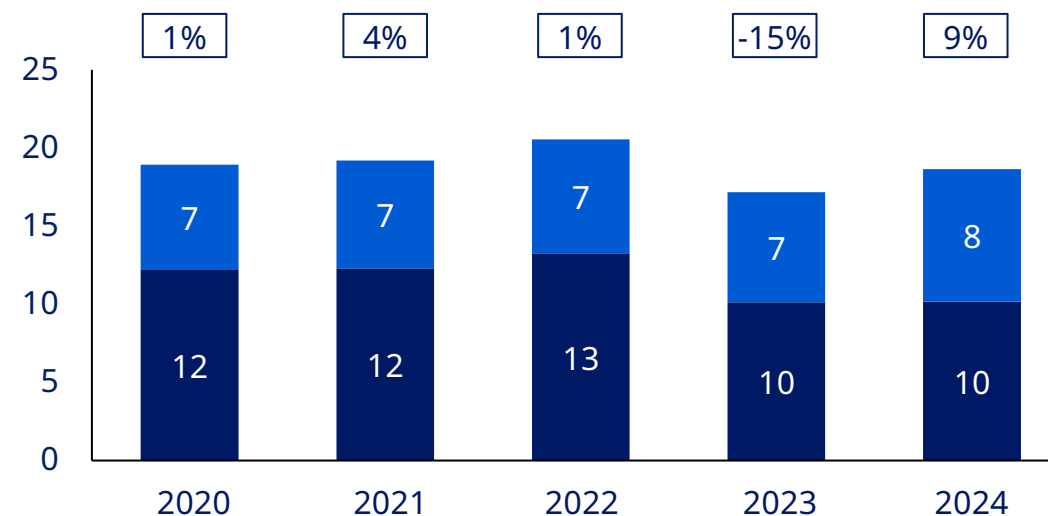
DKK
billion



■ NovoSeven®
■ Other rare blood disorders¹
■ Rare endocrine disorders³
■ Other Rare disease²
 Growth at CER

Global Rare disease franchise

DKK
billion



■ IO ■ US Growth at CER

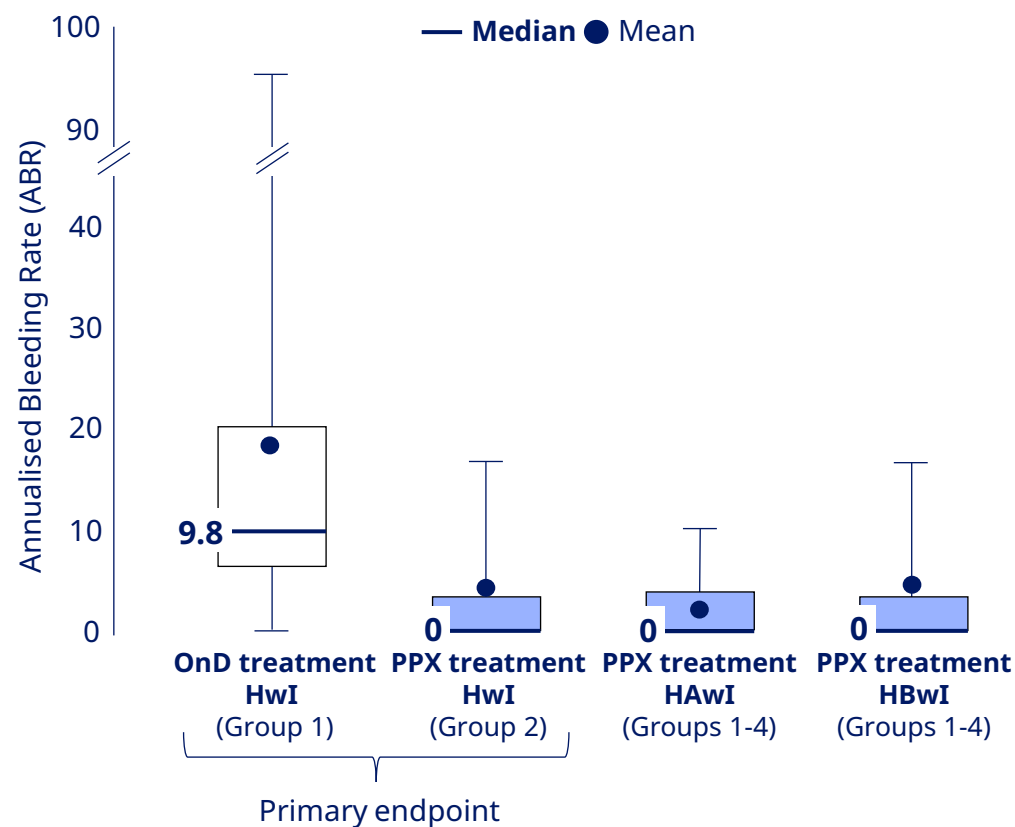
¹Other rare blood disorders primarily consists of NovoEight®, Esperoct®, Refixia® and NovoThirteen® ²Other Rare disease products primarily consists of Vagifem® and Activelle® ³Rare endocrine disorders primarily consists of Primarily Norditropin® and Sogroya®

CER: Constant exchange rates

Note: Company reported sales

In the Explorer 7 trial, concizumab reduced the number of bleeds in adults and adolescents with inhibitors

Explorer 7 trial results: Annualised bleeding rate per patient group



Key highlights

Efficacy

- **Median ABR was 0** for concizumab prophylaxis treatment, compared to 9.8 in the on-demand treatment group
- Estimated mean ABR was 1.7 for concizumab prophylaxis treatment, compared to 11.8 in the on-demand treatment group
- For patients on concizumab prophylaxis, **64% had 0 bleeds** in Group 2

Safety

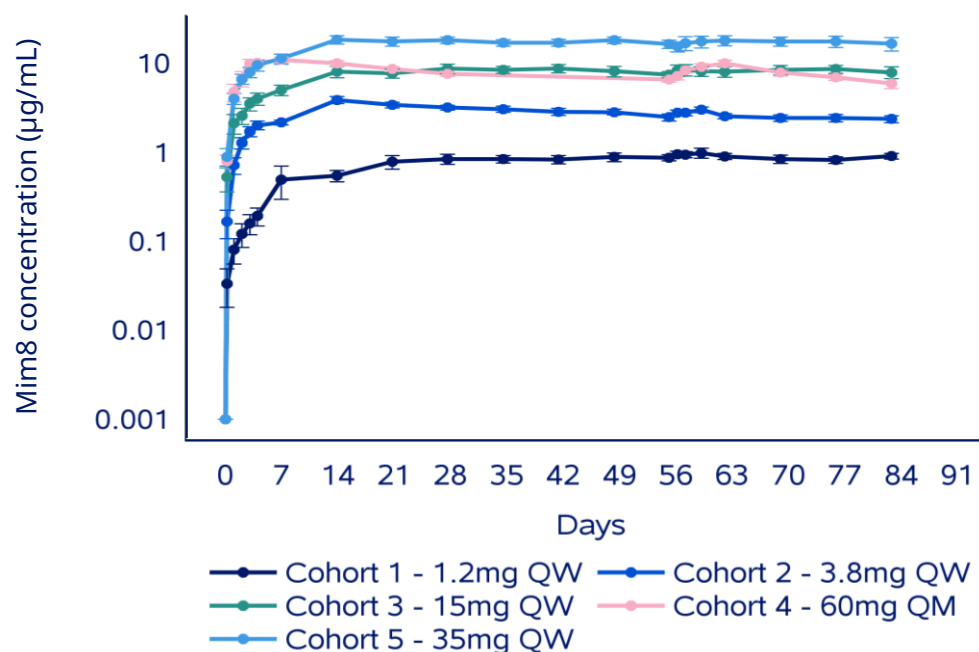
- Concizumab appeared to have a **safe and well tolerated** profile

Status

- Approved in: Canada (HAWI/HBwI), Australia (HAWI/HBwI & HA/HB), Switzerland (HAWI/HBwI), Japan (HAWI/HBwI & HA/HB), EU (HAWI/HBwI) and US (HAWI/HBwI) under brand name Alhemo®
- Alhemo® submitted in the EU for the treatment of haemophilia A and B

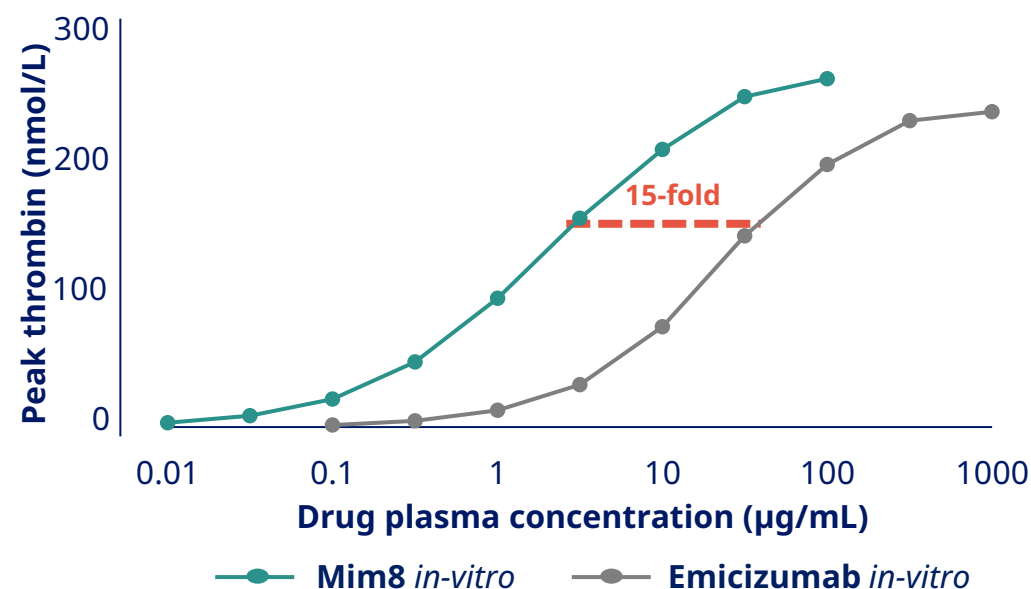
Interim data from Mim8 phase 1/2 show that PK/PD profiles support weekly to monthly low volume dosing

Mim8 pharmacokinetic properties support weekly and monthly dosing



- Mim8 concentration profiles increased with dose
- Mean concentrations at steady state were comparable for Cohort 3 (weekly dosing) and Cohort 4 (monthly dosing)

Higher potency of Mim8 vs emicizumab enabling a low dosing volume



- The PD marker, peak thrombin generation, increased with Mim8 dose
- *In-vitro* exposure-response curves in haemophilia A-like plasma show a 15-fold higher potency of Mim8 compared to emicizumab

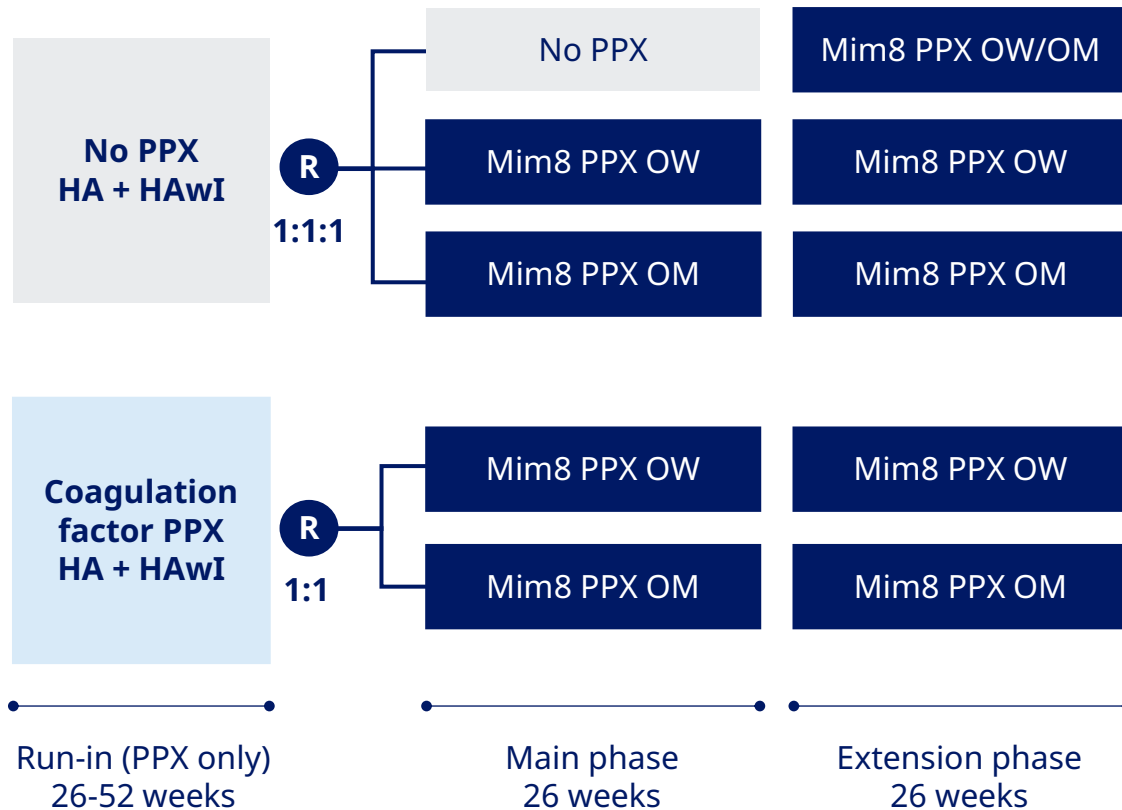
The peak thrombin plot represents *in-vitro* data: human plasma samples from the healthy participants of the SAD cohort were made HA-like with anti-FVIII antibodies, and spiked with different concentrations of Mim8 or commercially available emicizumab.

PK: Pharmacokinetics; PD: Pharmacodynamics; QW: Once-weekly; QM: once-monthly

Reference: FRONTIER 1, 12-week main phase cohort 1-5. Chowdary P, et al. FRONTIER1: A Phase 1/2 Dose Escalation Study of a Novel Factor VIIIA Mimetic Bispecific Antibody, Mim8, for Evaluation of Safety, Pharmacokinetics, and Efficacy. Abstract presented at ISTH 2022; Windyga J, et al. Mim8 is associated with improved thrombin generation vs. emicizumab in patients with haemophilia A, with and without inhibitors. Abstract presented at ISTH 2022; Novo Nordisk data on file

Main part of the FRONTIER 2 trial with Mim8 in people with Haemophilia A has been completed in Q2 2024

Phase 3 trial, FRONTIER 2 trial in 254 adults & adolescents with HA



Trial design

- Novel and accelerated development programme

Trial objective

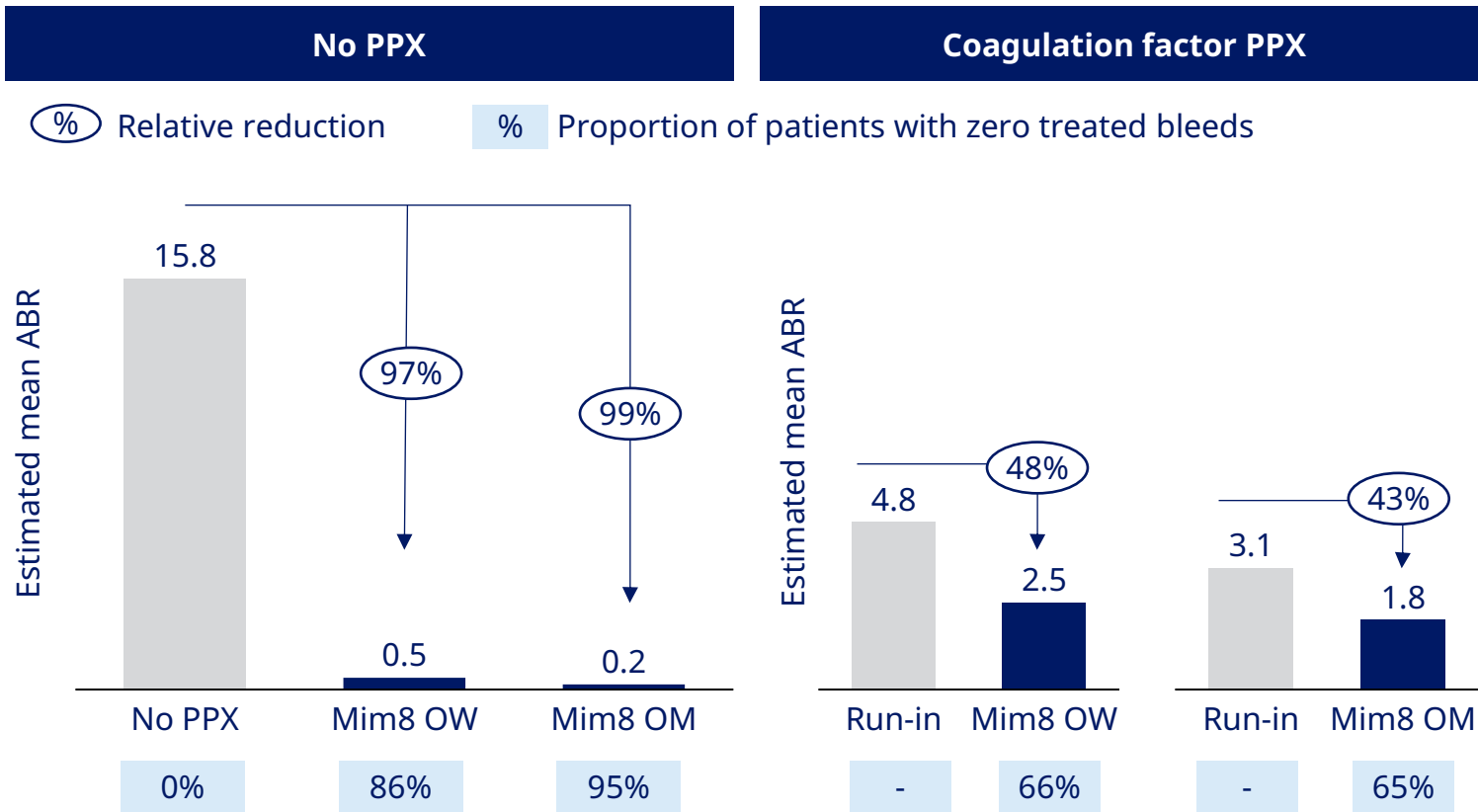
- For people with no prior PPX, the objective was to demonstrate superiority of Mim8 PPX vs no PPX
- For people with prior factor PPX, the objective was to demonstrate non-inferiority of Mim8 PPX vs coagulation factor PPX in run-in period

Key trial endpoints

- ABR for treated bleeds over 26 weeks of treatment
- Overall safety of Mim8 PPX including occurrence of anti-Mim8 antibodies and injection site reactions

Once-weekly and once-monthly Mim8 demonstrated superior reduction of treated bleeding episodes in the FRONTIER 2 trial

Annualised bleeding rate per patient group



FRONTIER 2 safety and next steps

No safety concerns were observed



No thromboembolic events observed



No evidence of neutralising anti-Mim8 antibodies



5-12% of patients with injection site reactions across arms

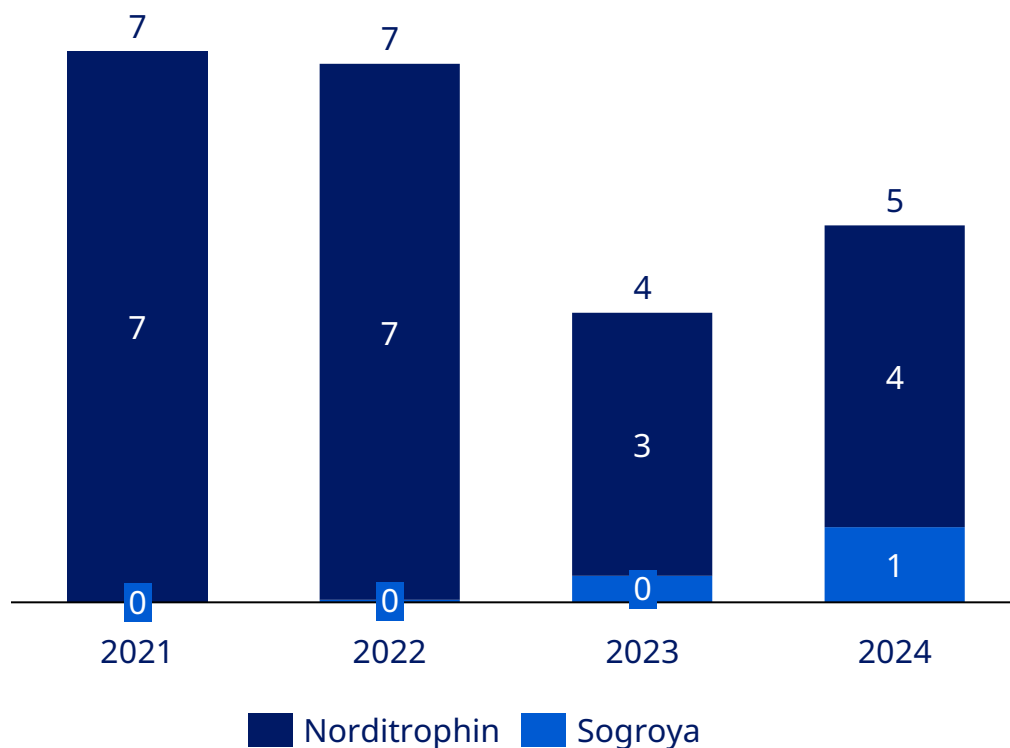
Next steps

- First submission expected in 2025

Growth Hormone sales contribute to 27% of total rare disease sales by end of 2024

Norditropin® and Sogroya® total hGH sales

Sales bDKK



A portfolio offering across markets

Sogroya® strategy

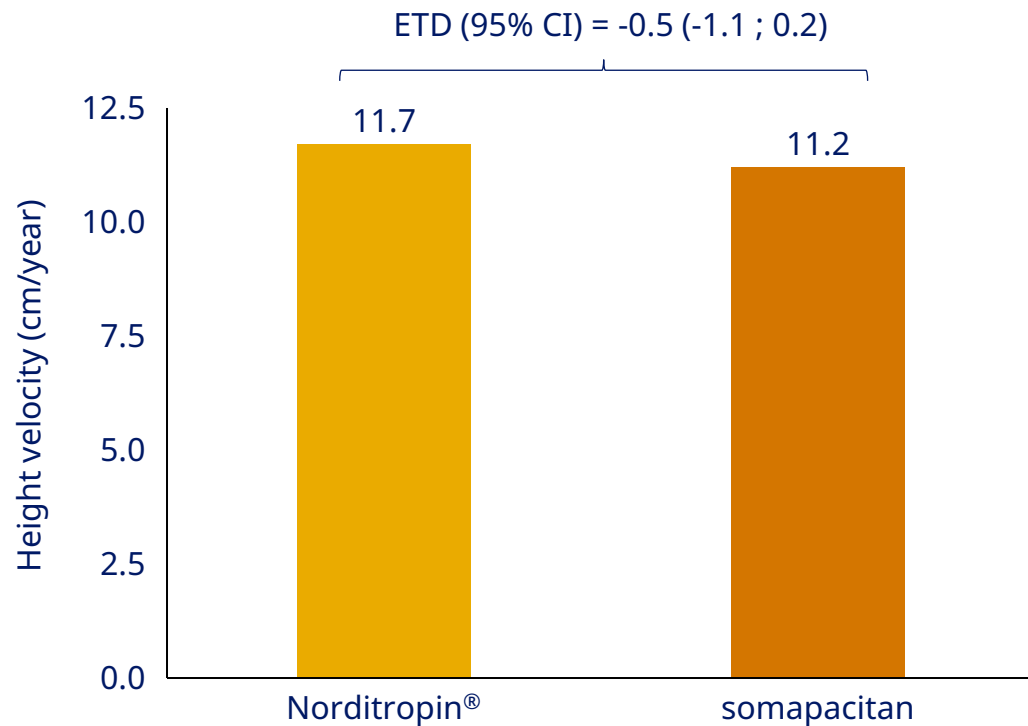
- Once-weekly efficacious treatment on par with Norditropin®
- Simple and easy-to-use device
- Phase 3 trials toward broad range of indications (e.g. SGA, Turner, Noonan, ISS) to expand the market
- Approved for GHD in US, EU and Japan

Norditropin® strategy

- Apply a market-fit approach to support specific markets and patient groups
- Broad label across eight indications

Sogroya® is approved for paediatric growth hormone deficiency in US, EU and Japan

Phase 3a trial results in children with GHD



Key highlights

Efficacy

- Non-inferiority versus Norditropin® for the primary endpoint, height velocity, at week 52 was confirmed
- IGF-I SDS, bone age and glucose metabolism were all similar between Sogroya® (somapacitan) and Norditropin®

Safety and tolerability

- Overall, the safety profile of somapacitan appeared to be similar to the well-known safety profile of daily GHD treatment
- No local tolerability issues were identified

Other treatment parameters

- Significantly reduced treatment burden¹ compared to Norditropin®

Status

- Adult GHD: Approved by the US, EU and JP
- Paediatric GHD: Approved by the US, EU and JP

¹Measured using patient reported outcome TB-CGHD-P (Treatment burden measure - child growth hormone deficiency - parent)

ETD: Estimated treatment difference; IGF-I SDS: Insulin growth factor-1 standard deviation score; GHD: Growth hormone deficiency; IGF-I SDS: Insulin growth factor-1 standard deviation score; US: United States; EU: European Union; JP: Japan

Rare Disease pipeline is leveraging our core expertise to serve more patients through internal and external innovation

Strengthen and progress pipeline

Our key focus areas



Selective expansion from core:

- From haemophilia to rare blood disorders
- From growth disorders to rare endocrine disorders



Faster global patient recruitment



Accelerate pipeline with internal and external innovation



Explore all Novo Nordisk technology platforms

Rare Disease development pipeline

Rare Disease

Project	Phase
Rare Blood Disorders marketed products ¹	<i>Marketed</i>
Rare Endocrine Disorders marketed products ²	<i>Marketed</i>
Refixia ® in Rare Blood Disorders	<i>Marketed</i>
Esperoct ® in Rare Blood Disorders	<i>Marketed</i>
Alhemo ® (concizumab-mtci) in Rare Blood Disorders	<i>Marketed</i>
Rivfloza ® (nedosiran) in Rare Blood Disorders	<i>Marketed</i>
Mim8 in Rare Blood Disorders	Expected submission H2 2025
Etavopivat in Sickle Cell Disease	Phase 3 ongoing
Etavopivat in Thalassemia	Phase 2 ongoing
NDec in Sickle Cell Disease	Phase 2 ongoing
Inno8 in Rare Blood Disorders	Phase 1 ongoing
TMPRSS6 in Rare Blood Disorders	Phase 1 ongoing

¹Includes NovoSeven®, NovoEight®, NovoThirteen® ²Includes Norditropin® and Sogroya®

Cardiovascular & Emerging Therapies

The unmet needs
Cardiovascular disease
MASH
Alzheimer's disease

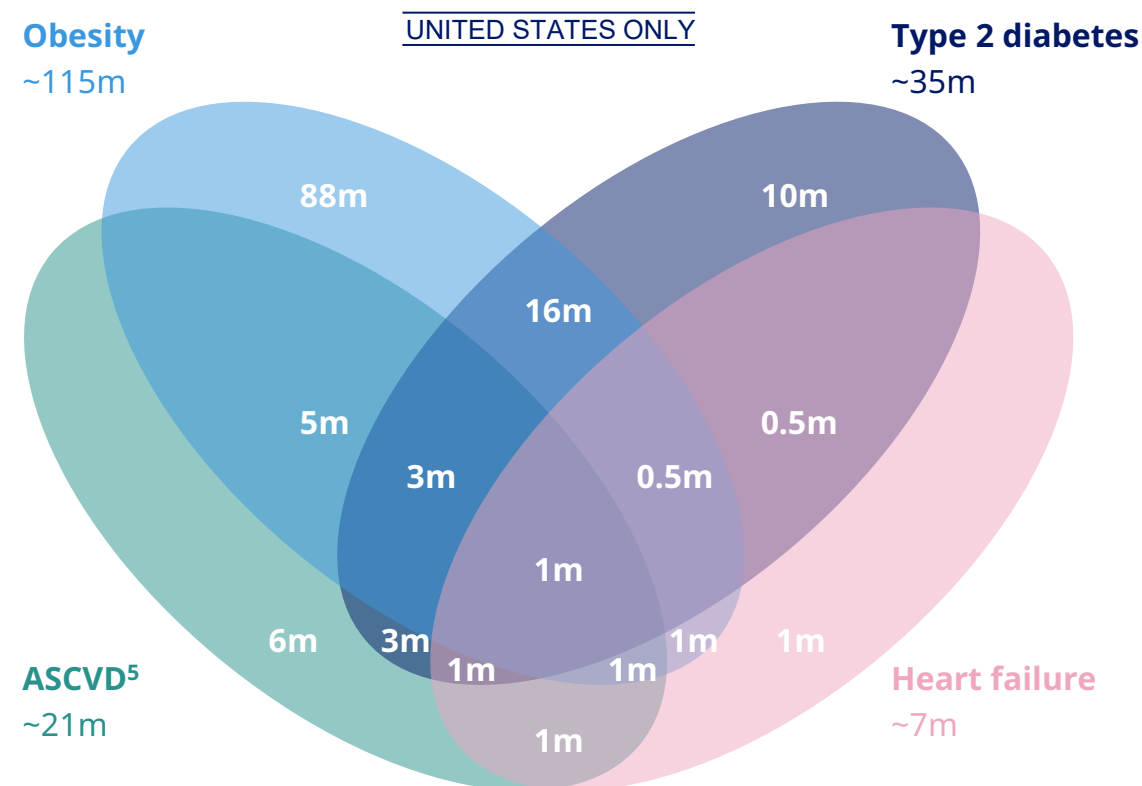


Novo Nordisk is expanding into Cardiovascular and emerging therapy areas

New therapeutic areas have unmet medical needs

Therapy area	Unmet need
1 CVD	32% of global deaths caused by CVD ¹
2 MASH	>250 million people affected by MASH ²
3 CKD	>800 million people affected by CKD ³
4 AD/PD	~70 million people are living with AD worldwide ⁴

Patient overlaps between Novo Nordisk core therapy areas



¹WHO: Cardiovascular Diseases 2023; ²Csaba P. Kovesdy et al. Kidney International Supplements. 2022; 12: 7-11; ³WHO: Dementia key facts 2021; ⁴Alzheimer's Association report: 2020 Alzheimer's disease facts and figures, 2020 (16:391-460);

⁵Myocardial infarction, stroke and coronary heart disease

AD: Alzheimer's disease; ASCVD: Atherosclerotic cardiovascular disease; CKD: Chronic kidney disease; CVD: Cardiovascular disease; MASH: Metabolic dysfunction-associated steatohepatitis; PD: Parkinson's disease; WHO: World Health Organization

Note: Prevalence overlaps have been estimated on patient-level data from NHANES. Post-estimation adjustments have been undertaken to match certain key metrics as reported by publicly available sources. Numbers are rounded

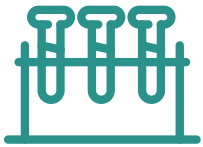
Source: NHANES (waves 2003-2004, 2013-2014, 2015-2016 and 2017-2020); UN World Population Prospects 2022; International Diabetes Federation: Diabetes Atlas 10th edition, 2021; World Obesity Atlas 2023

Novo Nordisk has a focused approach in cardiovascular disease

Focus areas within cardiovascular disease

Atherosclerotic cardiovascular disease

Dyslipidaemia



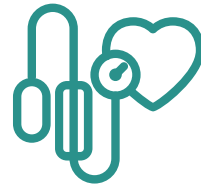
Globally, one third of ischemic heart disease is attributable to high cholesterol¹

Systemic inflammation



Around half of ASCVD patients estimated to have residual inflammatory risk²

Uncontrolled and resistant hypertension



Hypertension is a leading risk factor for CVD, HF, CKD and premature death³

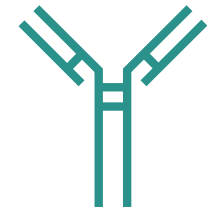
Heart failure

Heart failure with preserved ejection fraction



HFpEF is associated with high morbidity and mortality⁴

Transthyretin amyloid cardiomyopathy

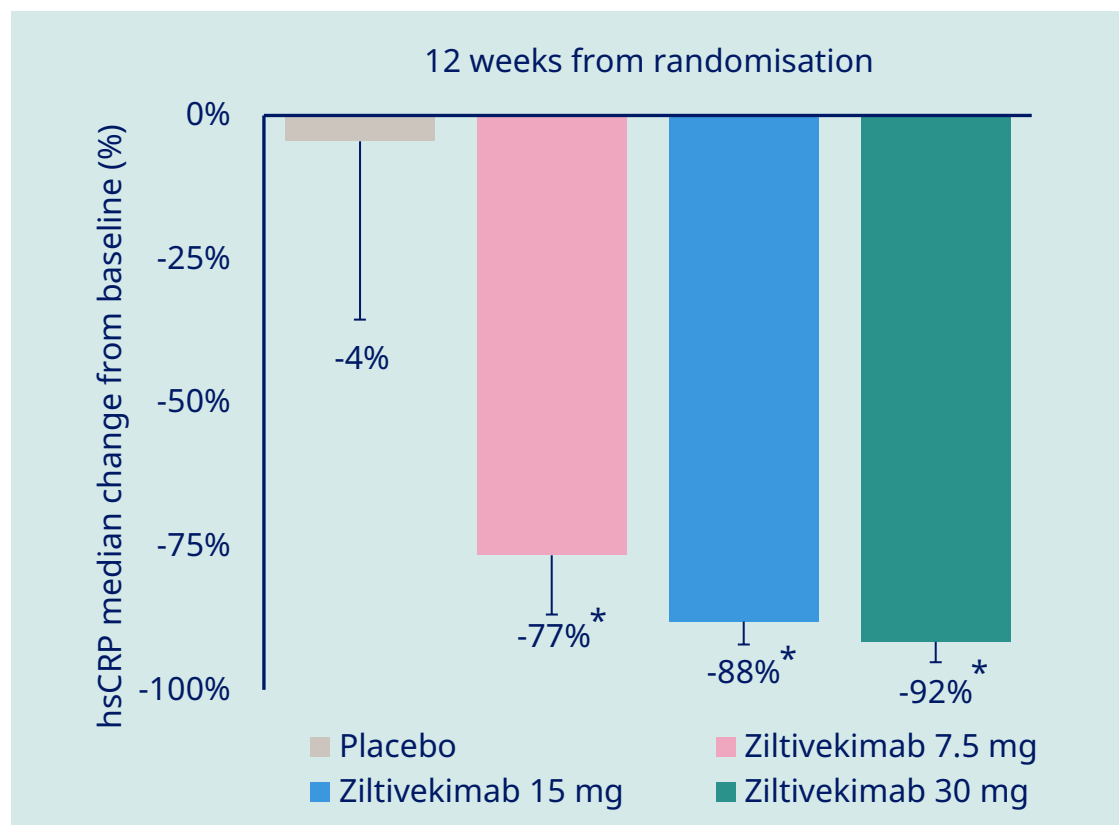


ATTR-CM is a progressive, life-threatening disease⁵

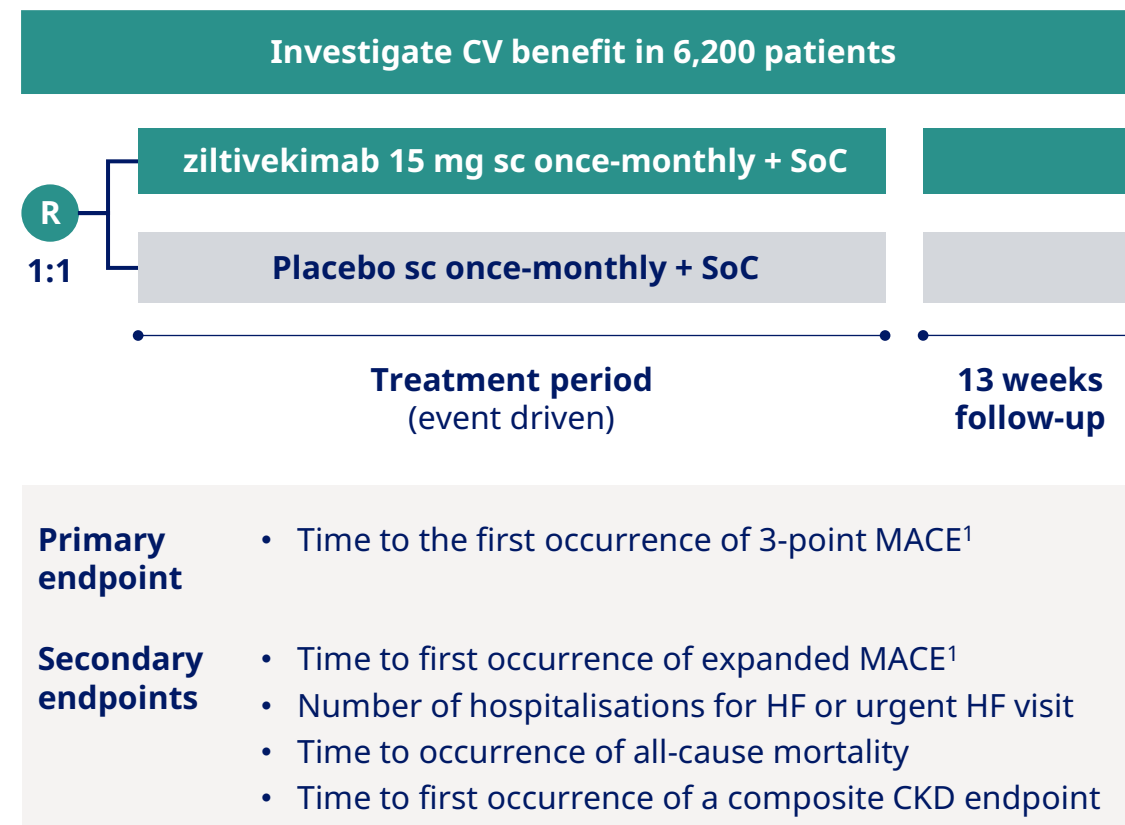
¹WHO: Cardiovascular Diseases (Cholesterol); ²Ridker et. al, J Am Coll 2018;72:3320-3333; ³WHO: Cardiovascular Diseases (Hypertension); ⁴Chioncel O et al. Eur J Heart Fail 2017; 19; 1574; ⁵Singh A. et al. J Am Coll Cardiol 2017; 69:750-759
ASCVD: Atherosclerotic disease; ATTR-CM: Transthyretin amyloid cardiomyopathy; CKD: Chronic kidney disease; CVD: Cardiovascular disease; HF: Heart Failure; HFpEF: Heart failure with preserved ejection fraction; WHO: World Health Organization

ZEUS trial with ziltivekimab aims to validate the link between hsCRP and major adverse cardiovascular events

Results from the phase 2 trial RESCUE with ziltivekimab



Phase 3 CVOT trial ZEUS with ziltivekimab



* Statistically significant; ¹ Inclusion criteria: Age ≥18 years, History of ASCVD, eGFR ≥15 and <60 mL/min/1.73 m², Serum hsCRP ≥2 mg/L

¹ MACE includes CV death, non-fatal MI or non-fatal stroke, Expanded MACE includes: (CV death, non-fatal MI, non-fatal stroke or hospitalisation for unstable angina pectoris requiring urgent coronary revascularisation)

hsCRP: High-sensitivity C-reactive protein; CVOT: Cardiovascular outcome trial; CV: Cardiovascular; sc: Subcutaneous; SoC: Standard of care; HF: Heart failure; CKD: Chronic kidney disease

Source: Ridker PM, et al., IL-6 inhibition with ziltivekimab in patients at high atherosclerotic risk (RESCUE): a double-blind, randomised, placebo-controlled, phase 2 trial, 17 May 2021

Ziltivekimab phase 3 development programme targets high unmet need populations within CVD

ZEUS

ziltivekimab cardiovascular outcomes trial

Atherosclerosis and chronic kidney disease

n = 6,400



2021 • ————— • ~2026

Event driven
~ 4 years

Primary Endpoint:

Time to the first occurrence of 3-point MACE

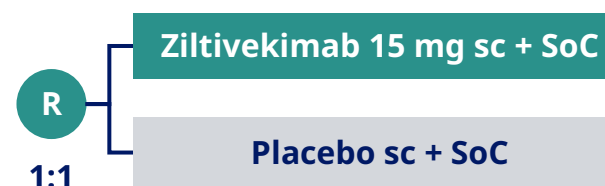
- Cardiovascular death
- Non-fatal myocardial infarction
- Non-fatal stroke

HERMES

ziltivekimab in patients with heart failure with mildly reduced or preserved ejection fraction

HFmrEF and HFpEF

n = 5,600



2023 • ————— • ~2027

Event driven
~ 4 years

Primary Endpoint:

Time to the first occurrence of

- Cardiovascular death
- Hospitalisation for heart failure
- Urgent heart failure visit

ARTEMIS

ziltivekimab in patients with acute myocardial infarction

Acute myocardial infarction

n = 10,000



2024 • ————— • ~2027

Event driven
~ 2.5 years

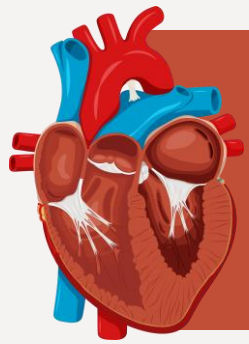
Primary Endpoint:

Time to the first occurrence of 3-point MACE

- Cardiovascular death
- Non-fatal myocardial infarction
- Non-fatal stroke

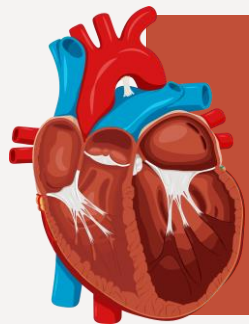
For patients with heart failure, the goal is to bring disease modifying and curative treatments to the market

Heart failure at a glance



Diastolic dysfunction (HFpEF)

- Impaired filling capacity
- Stiff and thick ventricle



Systolic dysfunction (HFrEF)

- Impaired contractility
- Stretched and thin ventricle

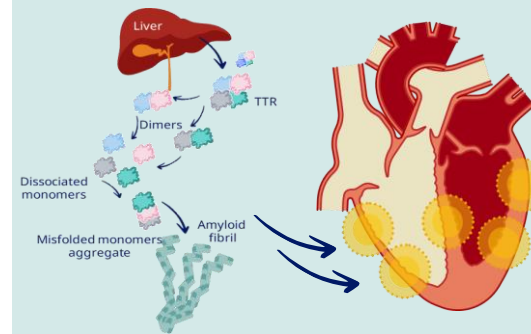
Pipeline includes potential disease modifying and curative treatments

Symptom relief

Today's marketed treatments

Disease modifying

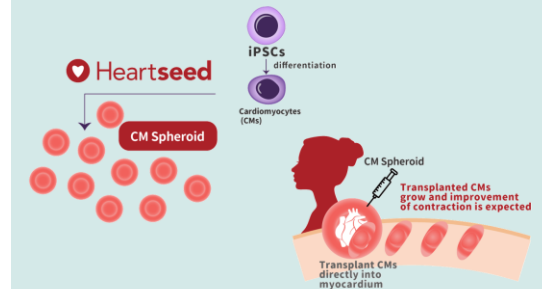
Coramitug



A monoclonal antibody designed to deplete the amyloid plaques associated with ATTR-CM in a niche population

Curative

Heartseed



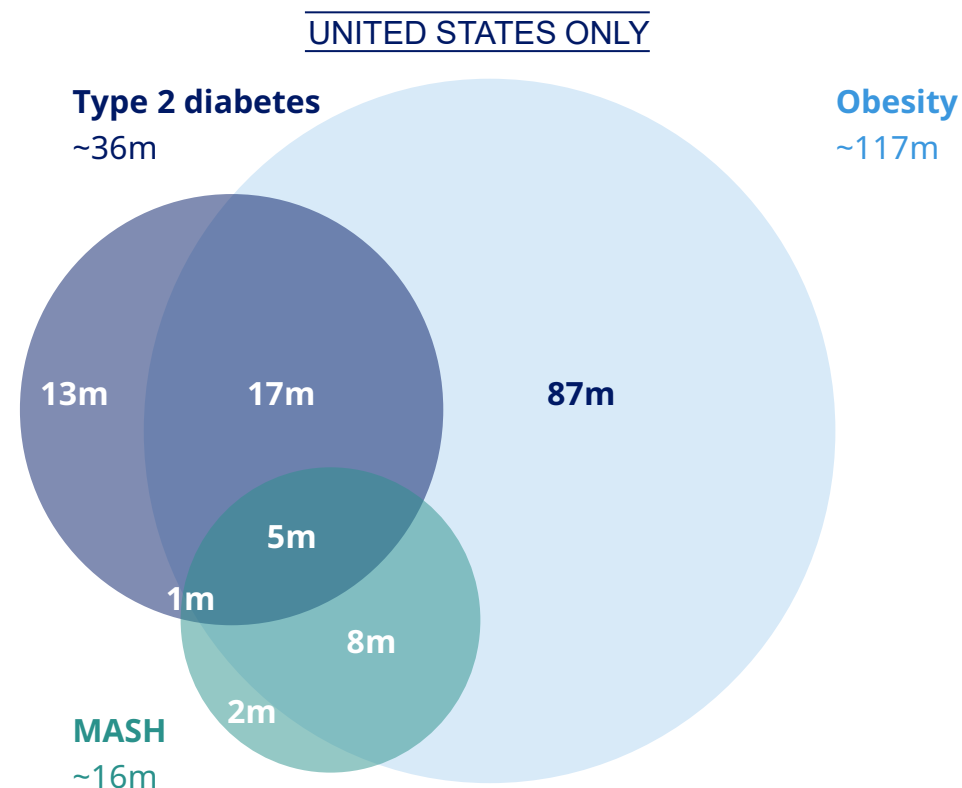
- HS-001 use iPSC-derived cardiomyocytes to treat HF
- The cells are treated in a solution to enhance survival and/or engraftment

Metabolic dysfunction-associated steatohepatitis shares a large patient population with Novo Nordisk's core therapy areas

New therapeutic areas have high unmet medical needs

Therapy area	Unmet need
1 CVD	32% of global deaths caused by CVD ¹
2 MASH	>250 million people affected by MASH ²
3 CKD	>800 million people affected by CKD ³
4 AD/PD	~70 million people are living with AD worldwide ⁴

Patient overlap between Novo Nordisk core therapy areas and MASH



¹WHO: Cardiovascular Diseases 2023; ²Csaba P. Kovesdy et al. Kidney International Supplements. 2022; 12: 7-11; ³WHO Dementia key facts 2021; ⁴Alzheimer's Association report: 2020 Alzheimer's disease facts and figures, 2020 (16:391-460)

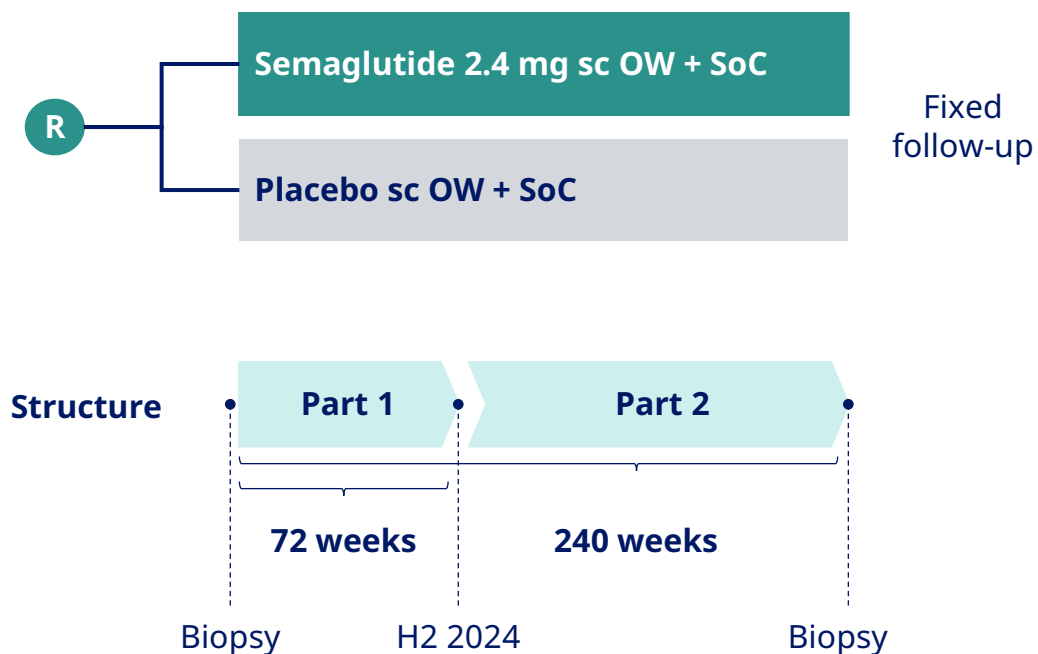
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Note: Prevalence overlaps have been estimated on patient-level data from NHANES. Post-estimation adjustments have been undertaken to match certain key metrics as reported by publicly available sources. Numbers are rounded

Source: NHANES (waves 2003-2004, 2013-2014, 2015-2016 and 2017-2020); UN World Population Prospects 2022; International Diabetes Federation: Diabetes Atlas 10th edition, 2021; World Obesity Atlas 2023

Part 1 of the ESSENCE trial investigated semaglutide 2.4 mg compared to placebo in people with MASH

ESSENCE trial with 1,200 patients with MASH F2–F3



Primary objectives and endpoints for Part 1 and 2

Part 1 | Improvement in liver tissue (histology)

Two binary histology endpoints at week 72 in 800 patients:

- Resolution of MASH and no worsening of liver fibrosis
- Improvement in liver fibrosis and no worsening of MASH

Part 2 | Reduction of liver-related clinical events

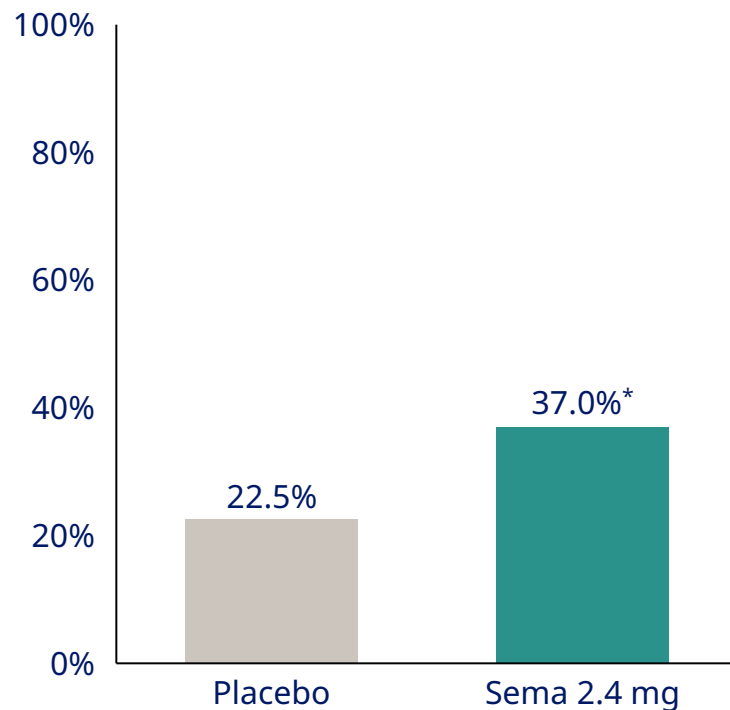
Composite endpoint at week 240 in 1,200 patients:

- Histological progression to cirrhosis
- Death (all cause)
- Liver-induced MELD score ≥ 15
- Liver transplant
- Hepatic decompensation events

Semaglutide 2.4 mg demonstrates superior improvement in both liver fibrosis and MASH resolution in the ESSENCE trial

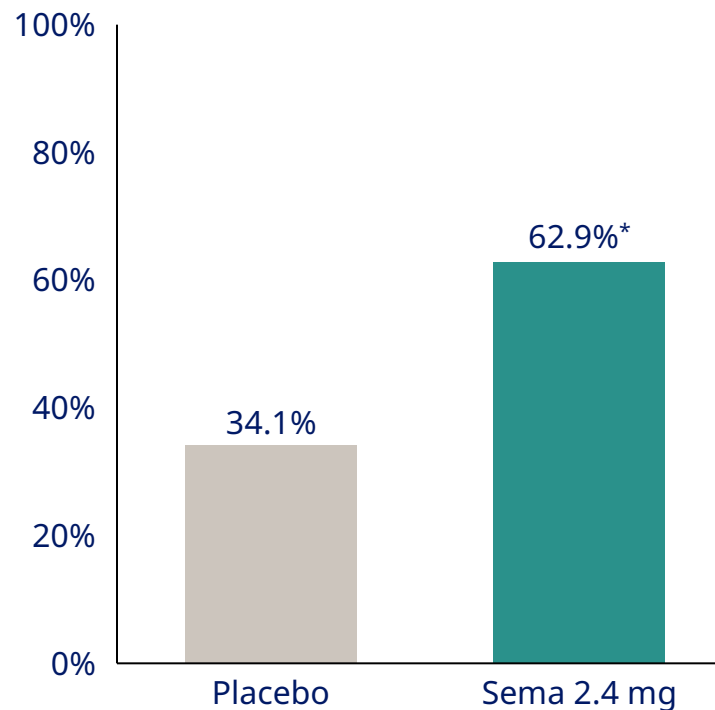
Improvement in fibrosis with no worsening in steatohepatitis

Proportion of patients



Resolution of steatohepatitis with no worsening of fibrosis

Proportion of patients



Addressing unmet need in MASH

Headline results

- The trial achieved its primary endpoints
- In the trial, semaglutide 2.4 mg appeared to have a safe and well-tolerated profile

Unmet need in MASH remains

- ~16 million live with F2-F4c MASH¹ in US
- Only one approved treatment

Next steps

- Submitted for regulatory approval in the EU and US in Q1 2025 - FDA priority review granted in the US
- Part 2 of the ESSENCE trial will continue, completion expected in 2029

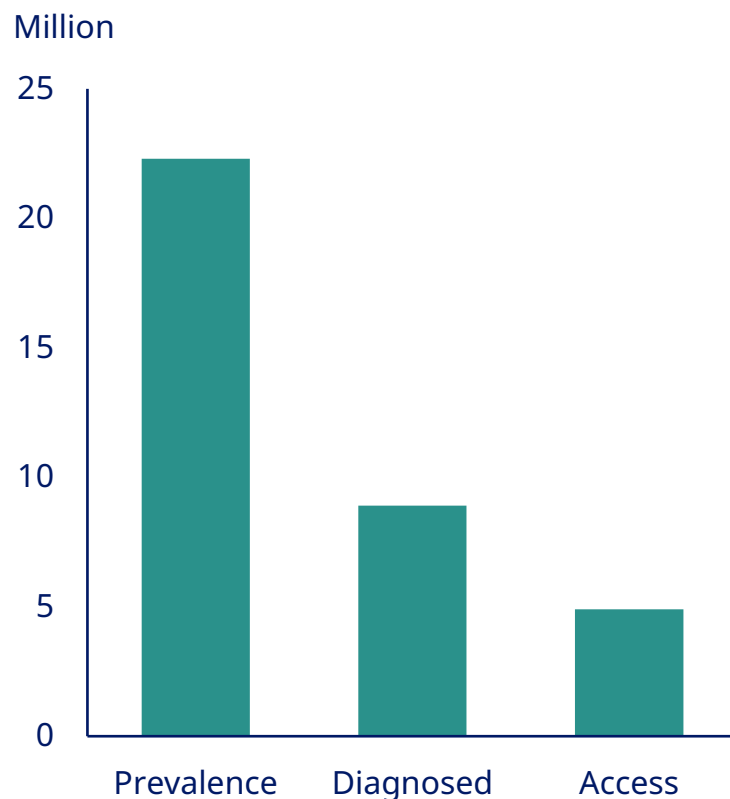
*Statistically significant

¹NHANES (waves 2003-2004, 2013-2014, 2015-2016 and 2017-2020); UN World Population Prospects 2022; International Diabetes Federation: Diabetes Atlas 10th edition, 2021; World Obesity Atlas 2023

F: Fibrosis stage; Sema: Semaglutide; MASH: Metabolic dysfunction-associated steatohepatitis

Novo Nordisk will focus on F2-F4c with commercial efforts related to awareness, referrals and diagnosis

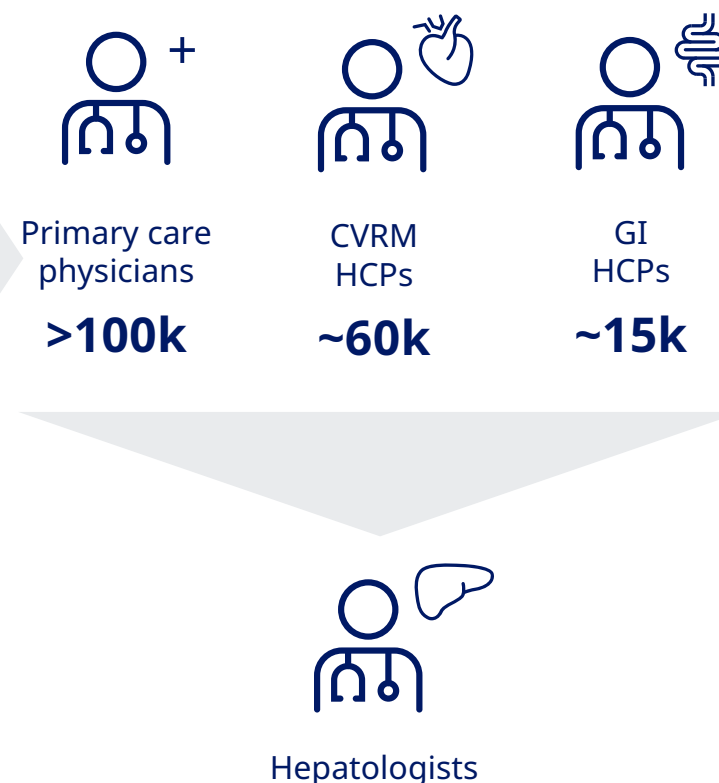
~22 million people are expected to live with MASH F2-F4c by 2030¹



Focus areas to establish presence in MASH

- **Awareness**
Recognise liver health as additional risk factor and increase patient screening at scale
- **Referrals**
Ensure high risk patient referral and support guideline changes
- **Diagnosis**
Ensure sequential NITs are used in diagnosis
- **Treatment**
Semaglutide as foundation; Liver-specific MoAs as add-on in F2-F3c; Multi-MoA anti-fibrotics in F3-F4c

MASH referrals to hepatologists in the US



¹Estes C, Modelling the epidemic of non-alcoholic fatty liver disease demonstrates an exponential increase in burden of disease, Hepatology, 2018

CVRM: Cardiovascular, renal, metabolic; F: Fibrosis stage; (F0-F1: no or mild fibrosis; F2 significant fibrosis; F3-4 advanced fibrosis); GI: Gastrointestinal; HCPs: Healthcare professionals; MASH: Metabolic dysfunction-associated steatohepatitis; MoA: Mode of action; NIT: Non-invasive tests

Note: Advanced fibrosis (F3-4) defined as per Kleiner DE. Hepatology. 2005;41:1313-21 and Brunt EM. Hepatology. 2011;53: 810-20.

Novo Nordisk enters partnerships to enhance diagnosis in MASH

Partnerships across relevant non-invasive tests

Blood test		
Pro-C3	ELF test	OW Liver

Blood test score		
NIS4	FIB-4	Fibro Sure

Scan			
SWE	MRE/MRI-PDFF	Liver MultiScan	TE FibroScan

Novo Nordisk supports NIT for MASH screening and diagnosis



Clinical guideline development recommending screening for MASH in type 2 diabetes



Disease education activities to enable screening, diagnosis and evidence generation

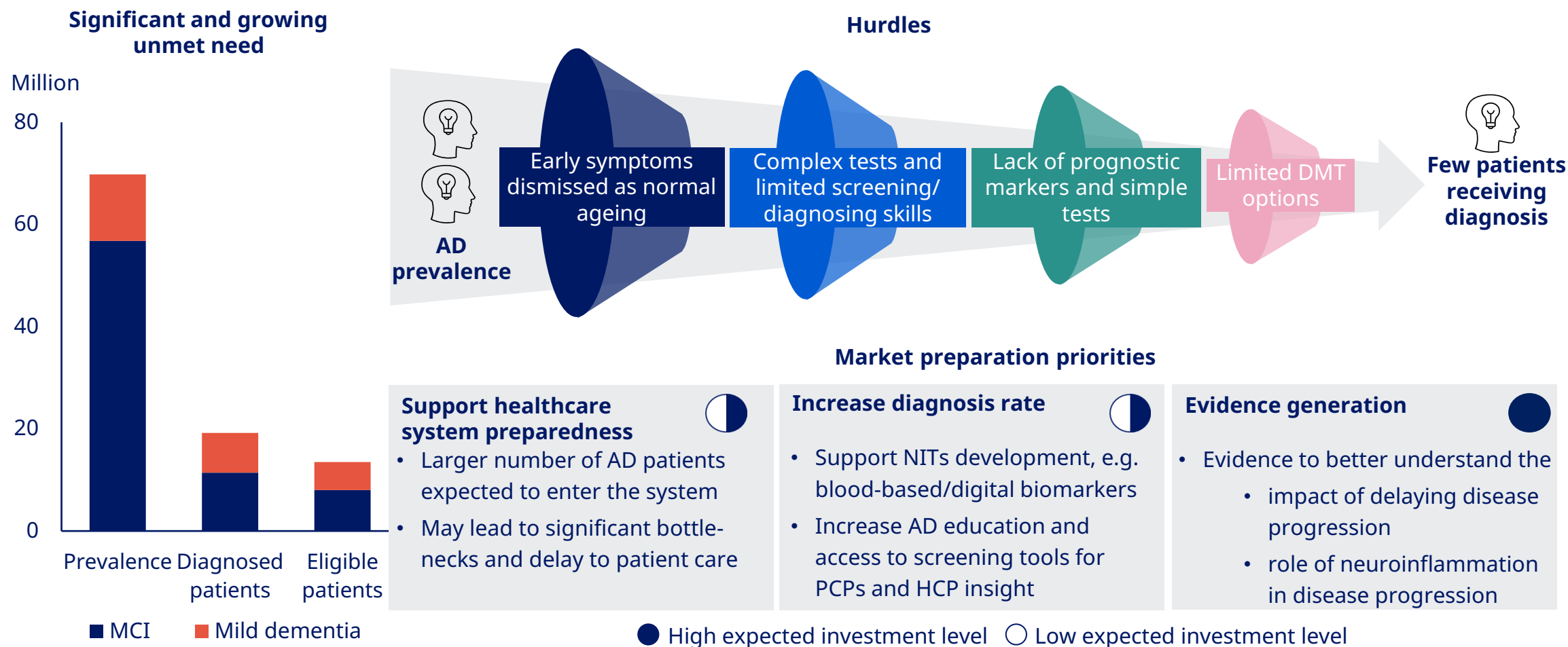


Engaging in consortia (Litmus, Nimble, Liver Forum)



Engaging with larger diagnostic companies to ensure NIT capacity

Alzheimer's disease patient journey is complex and underscores key barriers to overcome for Novo Nordisk to be successful



AD: Alzheimer's disease; QD: Once-daily; MCI: mild cognitive impairment; DMT: Disease-modifying treatment; PCP: primary care physicians; NITs: Non-invasive diagnostics; HCP: Healthcare professional

Note: MCI and Mild dementia in the graph are both *due to AD*.

Source: Alzheimer's Association report: 2020 Alzheimer's disease facts and figures, 2020 (16:391-460)

Entering phase 3 development of semaglutide in Alzheimer's disease was based on a number of data points



Real world evidence trials

Four RWE studies show reduced risk of dementia or AD with GLP-1

Danish registry¹

- **11%** lower risk of dementia per year of GLP-1 exposure

TRUVEN claims database¹

- **31%** lower risk of dementia after >2 years of GLP-1 exposure

Danish registry²

- **42%** lower odds of dementia after GLP-1 exposure

FAERS (FDA database)³

- **64%** lower odds of Alzheimer's disease after liraglutide exposure



Randomised controlled trials

53% lower risk of dementia diagnosis with liraglutide/semaglutide in NN's CVOTs in T2D⁴

Less decline in cerebral glucose metabolism (FDG-PET) with liraglutide in AD⁵

Reduced incidence of **major adverse CV events** in T2D with semaglutide incl. stroke⁶

Systemic anti-inflammatory effects with semaglutide^{7,8}

Short-term **memory improvement** with liraglutide in people with obesity⁹

Reduced cognitive decline with dulaglutide in patients with T2D¹⁰



Pre-clinical studies

Improved memory function with GLP-1¹¹ incl. semaglutide¹²

Reduced phospho-tau accumulation¹³

Reduced neuroinflammation with GLP-1^{14,15} incl. semaglutide¹⁶

Reduced atherosclerosis with liraglutide and semaglutide¹⁷

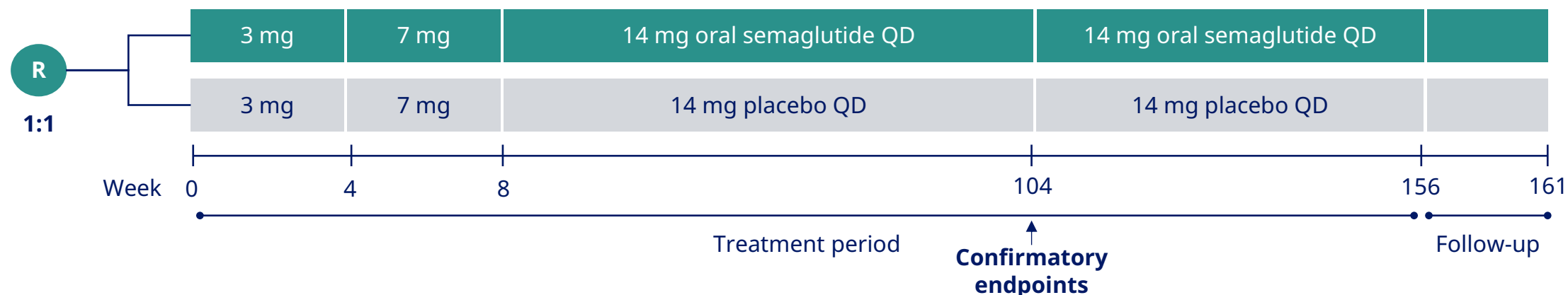
Systemic **anti-inflammatory** effects with semaglutide¹⁷

¹NN data on file, Danish register: Dementia cases based on diagnosis (ICD10) or treatment (anticholinesterases, memantine) codes; TRUVEN: Dementia cases based on SNOMED ids for all diagnoses (ICD-10) or treatment (anticholinesterases, memantine); ²Wium-Andersen IK et al. Eur J Endocrinol. 2019;181(5):499-507; ³Akimoto H et al. Am J Alzheimers Dis Other Dement. 2020;35:1-11; ⁴Ballard et al. Presented online at the Alzheimer's Association International Conference (AAIC), 27-31 July 2020; ⁵Gejl M et al. Front Aging Neurosci 2016;8:108; ⁶Husain M et al. Diabetes Obes Metab 2020;22:442-451; ⁷Aroda VR et al. Diabetes Care 2019;42:1724-1732; ⁸Rodbard HW et al. Diabetes Care 2019;42:2272-2281; ⁹Vadini F et al. Int J Obes (Lond) 2020;44:1254-1263; ¹⁰Cukierman-Yaffe T et al. Lancet Neurol 2020;19:582-590 ¹¹Hansen HH et al. J Alzheimers Dis 2015;46:877-888; ¹²Preliminary data in NN ongoing pre-clinical studies; ¹³Hansen HH et al. Brain Res 2016;1634:158-170; ¹⁴Brundin L et al. Nature Med 2018;24:900-902; ¹⁵Yun SP et al. Nature Med 2018;24:931-938; ¹⁶Secher A et al. Oral presentation at Virtual Alzheimer's Disease/Parkinson's Disease International Conference, 9-14 March 2021; ¹⁷Rakipovski G et al. JACC Basic Transl Sci 2018;3:844-857

AD: Alzheimer's disease; CI: confidence interval; RWE: Real world evidence

evoke and evoke+ trials are ongoing with expected completion in 2025

evoke and evoke+ trials have been initiated with 1,840 patients in each trial with a total of 3,680 patients



Objective

To confirm superiority of oral semaglutide vs placebo on the change in cognition and function in people with early Alzheimer's disease

Primary endpoint

Change in the Clinical Dementia Rating – Sum of Boxes (CDR-SB) score from baseline to end of 104 weeks of treatment

Inclusion criteria

- Early Alzheimer's disease (mild cognitive impairment or mild dementia)
- Mini-Mental State Examination (MMSE) $\geq 22/30$
- Age between 55-85 years
- evoke+ has at least 20% with small vessel pathology

AD: Alzheimer's disease; QD: Once-daily; MCI: mild cognitive impairment; QD: once-daily.

Note: CDR-SB ratings are utilising in six domains are summed to provide a clinical measure = Sum of Boxes. These are: memory, orientation, judgment and problem solving, community affairs, home and hobbies, personal care.

CDR-SB Scores range from 0 to 18 with higher scores representing greater impairment

CETA clinical pipeline has expanded, leveraging internal and external innovation and synergies

Addressing significant unmet needs

Cardiovascular disease



Pursue innovative mechanisms of action



Combine internal and external innovation

MASH



Aim for effect on resolution of MASH and improvement or no worsening of fibrosis



Prioritise multi-MoA anti-fibrotics in F3-F4c to secure a best-in-class profile

Alzheimer's disease



Opportunistic trial to slow clinical progression in people with early AD

Cardiovascular and emerging therapy areas development pipeline

CETA	Therapy area	Project	Phase
	Cardiovascular disease	Ziltivekimab , ASCVD and CKD	Phase 3 ongoing
		Ziltivekimab , HFpEF	Phase 3 ongoing
		Ziltivekimab , AMI	Phase 3 ongoing
		Coramitug , ATTR-Cardiomyopathy	Phase 2 completed
		CDR132L , Heart failure	Phase 2 ongoing
		NLRP3i , Atherosclerosis	Phase 1 ongoing
		CNP , Heart failure	Phase 1 ongoing
		Stem Cells , Heart failure	Phase 1 ongoing
	MASH	ESSENCE (semaglutide 2.4 mg), F2-F3c	Submitted in EU/US
		LXR(a) , F2-F3c	Phase 1 ongoing
		MARC1 , F3-F4c	Phase 1 ongoing
		NLRP3 , MASH	Phase 1 ongoing
	AD/PD	EVOKE (semaglutide 14 mg), AD	Phase 3 ongoing
		Stem Cells , Parkinson's disease	Phase 1 ongoing

US Operations

US health care system

US at a glance

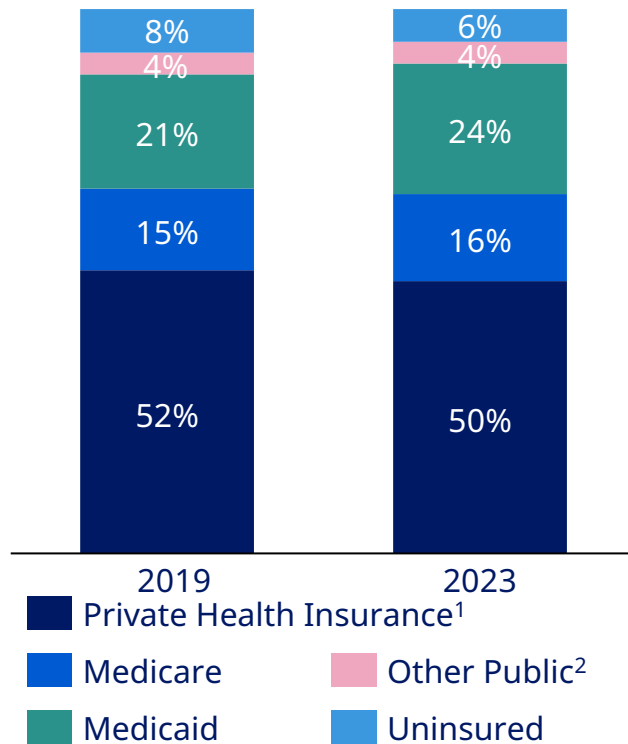
Leonard
Thompson
1922

novo nordisk

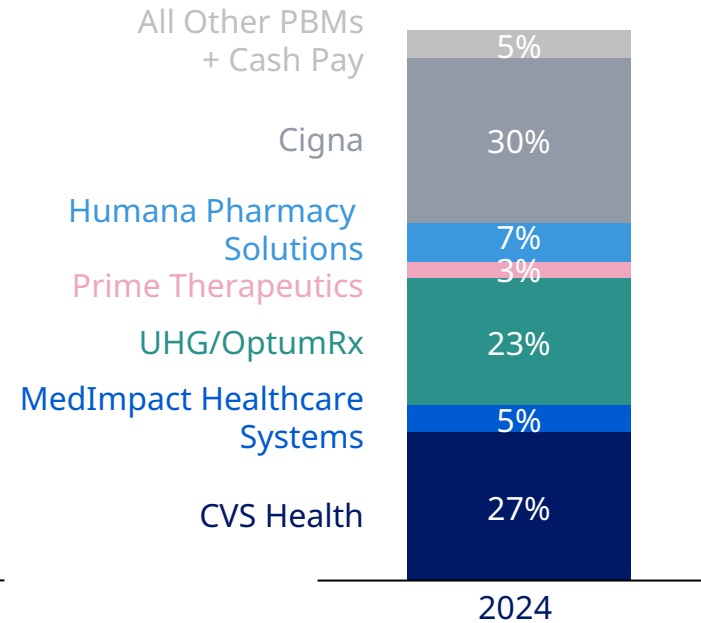


US healthcare is a mix of private and public health insurance, dominated by a few large PBMs

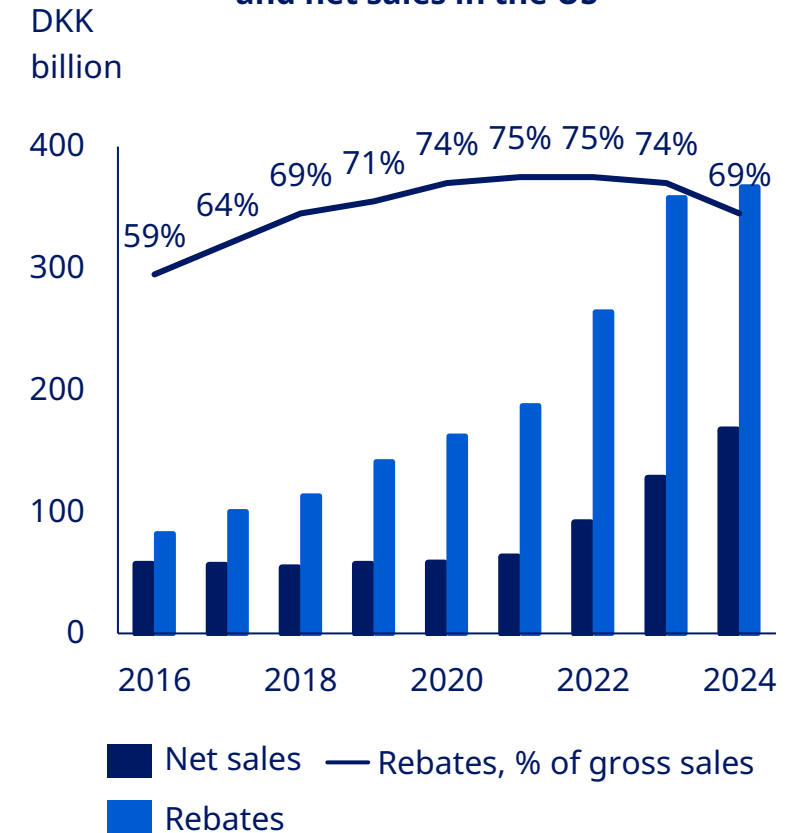
US health insurance enrollment and uninsured



US PBMs market shares



Development of Novo Nordisk rebates and net sales in the US

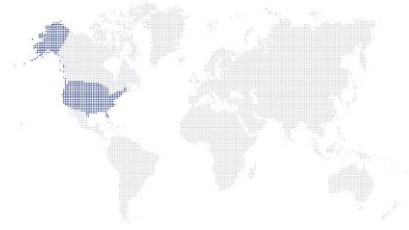


¹Private insurance includes employer sponsored insurance, health exchanges, and direct purchase insurance by individuals

²Other Public includes health insurance coverage provided by the Department of Veterans Affairs and the Department of Defense
Source: Centers for Medicare & Medicaid Services, National Health Expenditure, Historical Data. [Historical | CMS](#) (table 22)

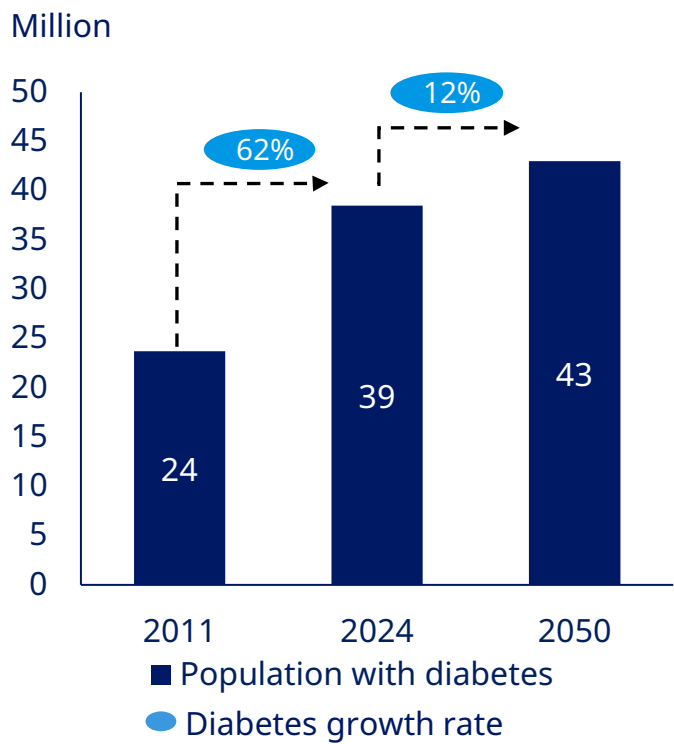
PBM: Pharmacy Benefit Manager; UHG: UnitedHealth Group
Source: Drug Channels Institute research and estimates. Calculated based on total equivalent prescription claims. 2024 data from The 2025 Economic Report on U.S. Pharmacies and Pharmacy Benefit Managers

Source: Novo Nordisk Annual Report 2024

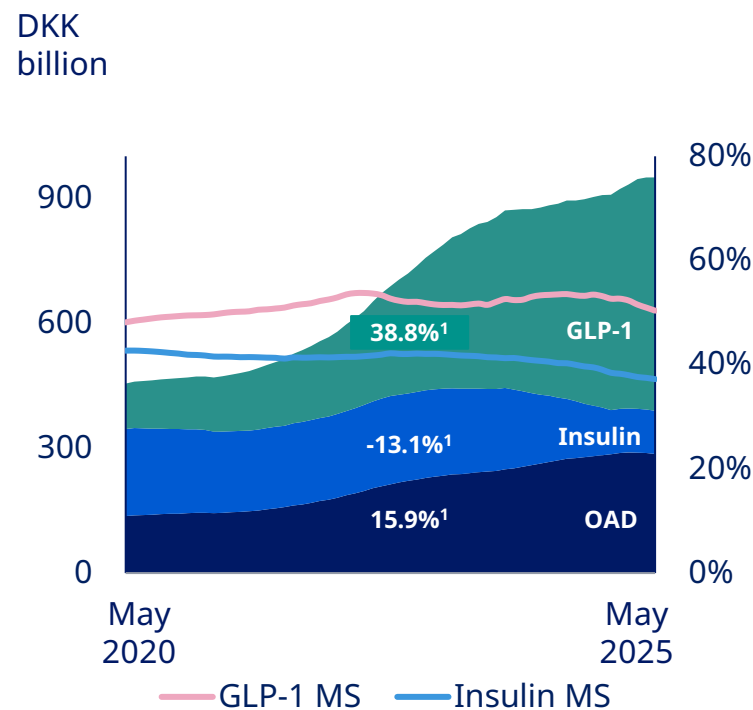


US Operations at a glance

Diabetes trend in population



Diabetes market by value and Novo Nordisk market share



Novo Nordisk H1 2025 reported sales

H1 2025	Sales (mDKK)	Growth ²
Injectable GLP-1 ³	45,273	12%
Rybelsus®	4,671	-10%
Total GLP-1	49,944	9%
Total insulin ⁴	8,081	17%
Other Diabetes care ⁵	81	-23%
Diabetes care	58,106	10%
Obesity care ⁶	24,899	36%
Diabetes & Obesity care	83,005	17%
Rare disease ⁷	4,274	23%
Total	87,279	17%

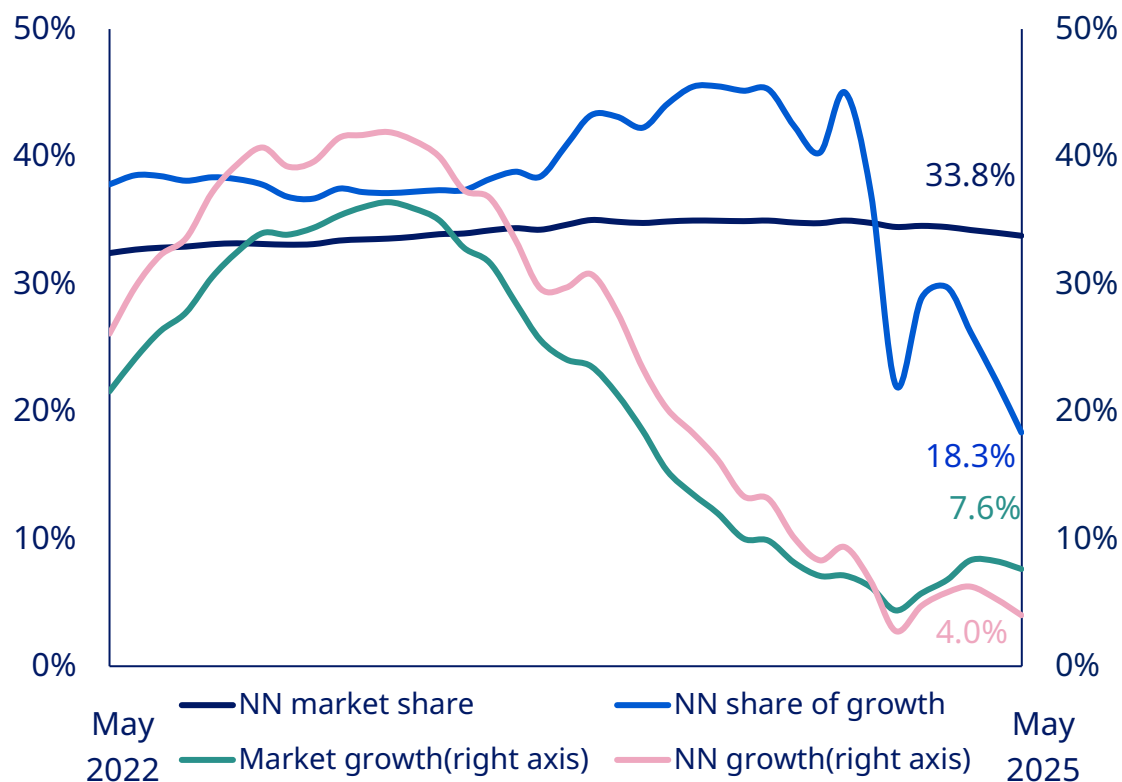
¹CAGR calculated for 5-year period
Competitor insulin value market shares, as of May 2025: Novo Nordisk 37%, Others 63%; Competitor GLP-1 value market shares, as of May 2025: Novo Nordisk 50%, Others 50%. OAD: Oral anti-diabetic; MS: Market Share; Note: Market values are based on list prices; Source: IQVIA MAT, May 2025 value figures

²At constant exchange rates ³Comprises Victoza®, Ozempic®
⁴Comprises Tresiba®, Xultophy®, Levemir®, NovoMix®, Fiasp®, Ryzodeg® and NovoRapid® ⁵Comprises NovoNorm® and needles ⁶Comprises Saxenda® and Wegovy® ⁷Comprises primarily NovoSeven®, NovoEight®, Esperoct®, NovoThirteen®, Refixia®, Norditropin®, Vagifem® and Activelife®

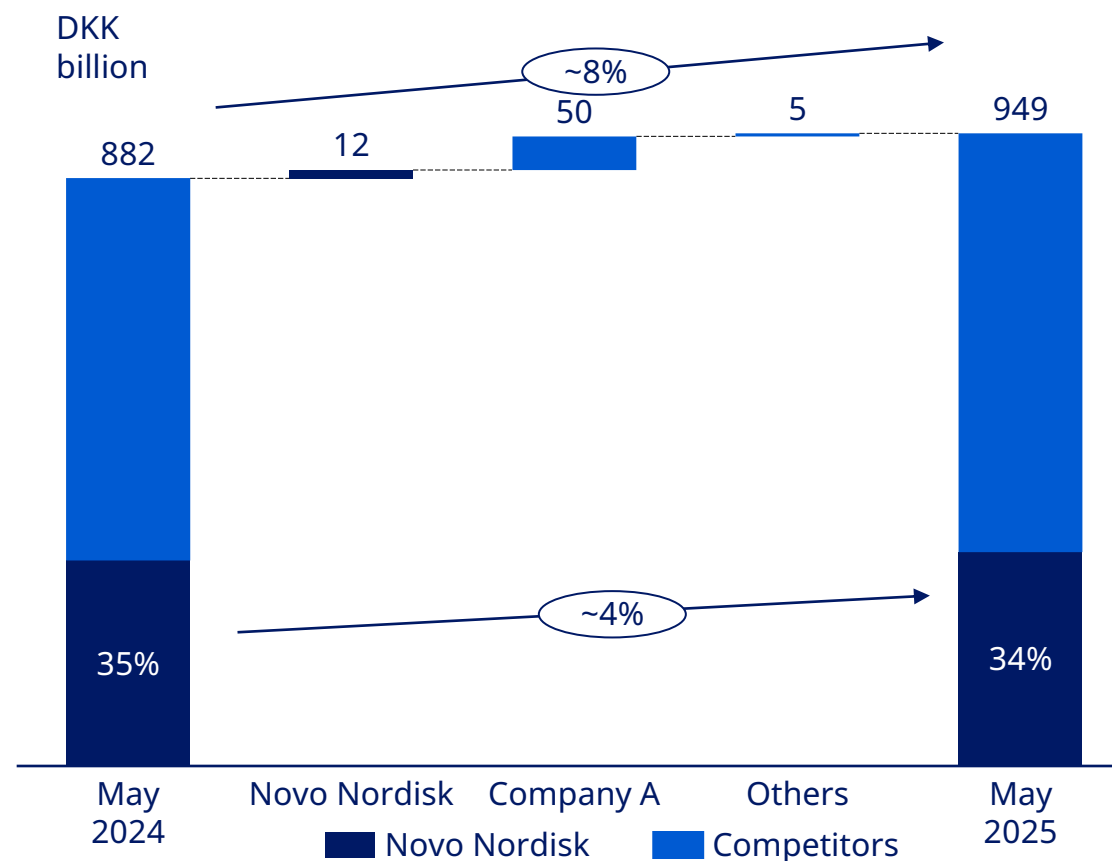


Diabetes market share and market growth in US Operations

Diabetes market growth and Novo Nordisk market share



Diabetes market size and growth



NN: Novo Nordisk

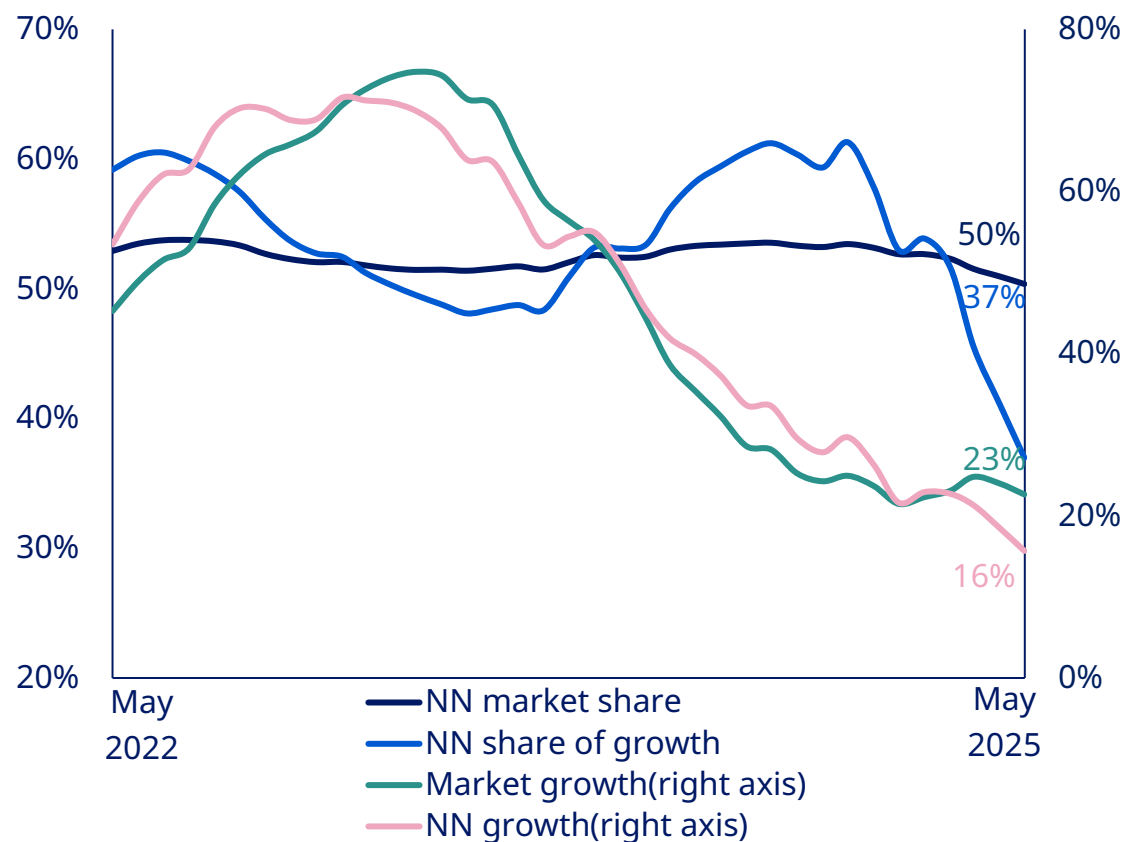
Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company; Market values are based on the list prices

Source: IQVIA, May 2025, value, MAT

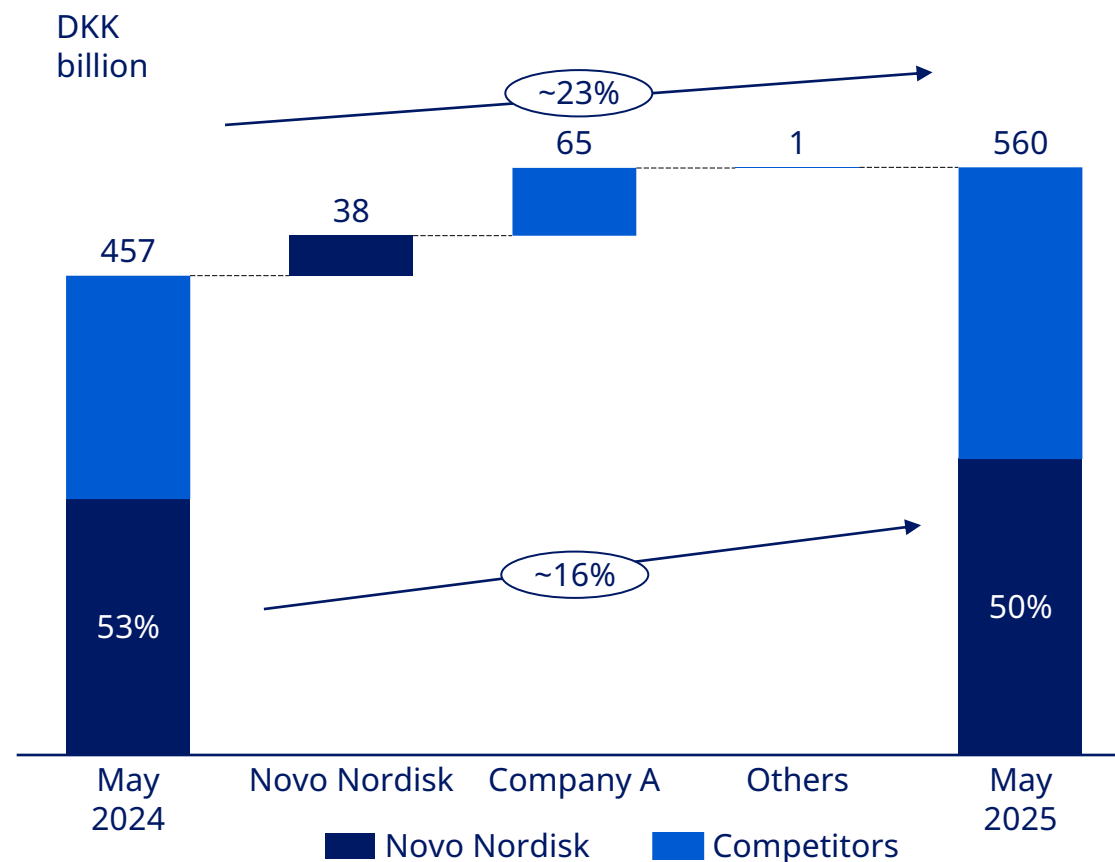


GLP-1 market share and market growth in US Operations

GLP-1 market growth and Novo Nordisk market share



GLP-1 market size and growth



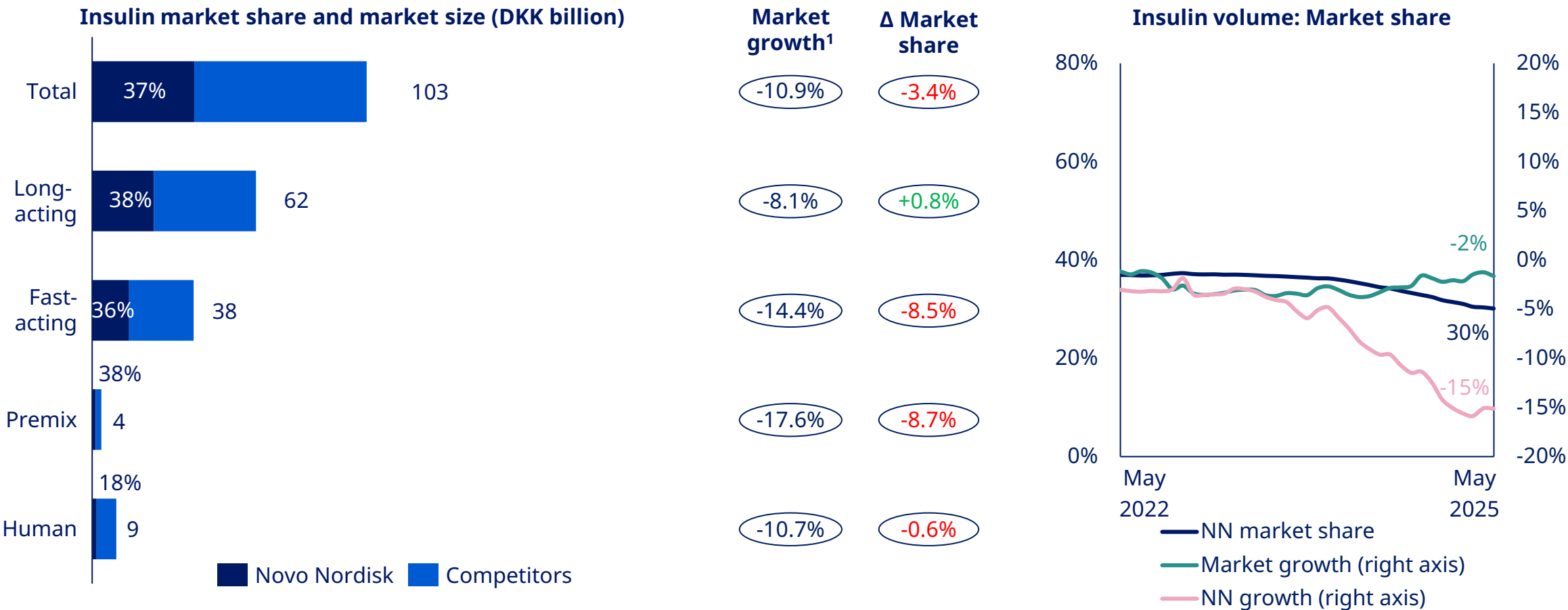
NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company; Market values are based on the list prices

Source: IQVIA, May 2025, value, MAT



Insulin market size and volume market share in US Operations

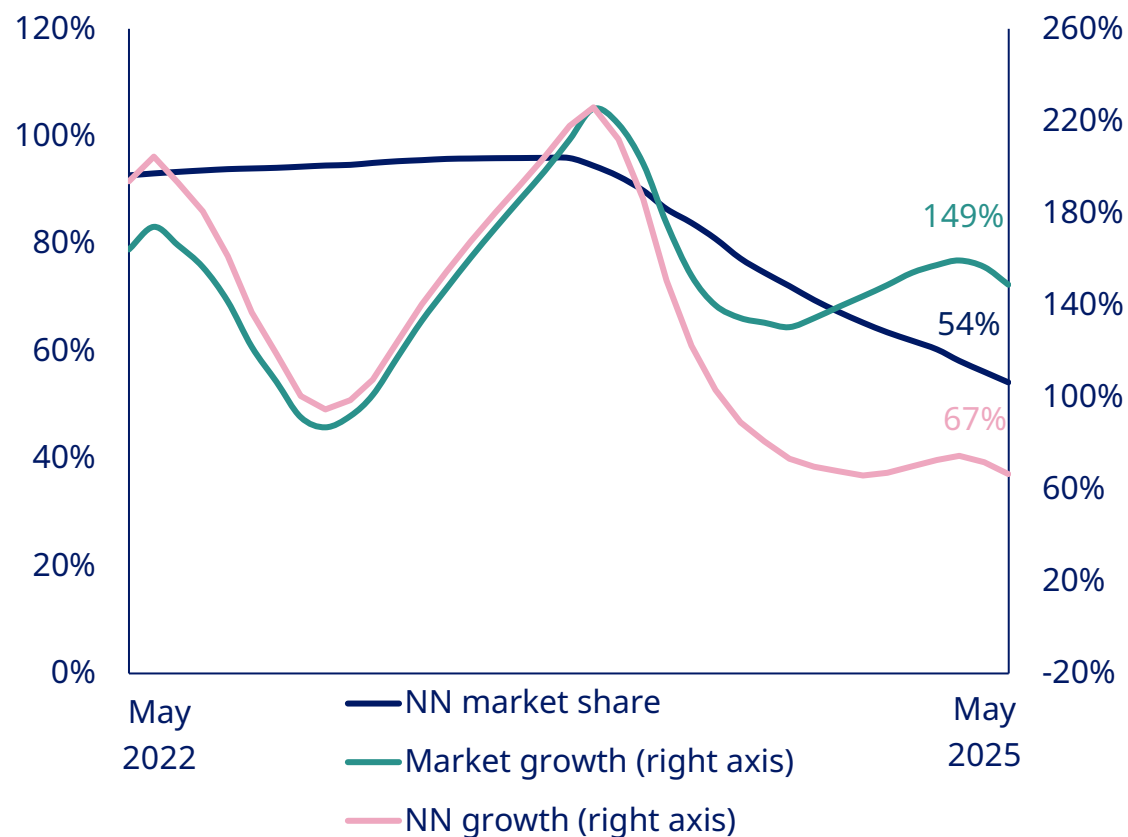


¹Market growth is YTD current vs YTD previous year
NN: Novo Nordisk; Note: Insulin market numbers do not reflect rebates. Share of growth not depicted due to too high numbers. Market values are based on the list prices
Source: IQVIA, May 2025, LHS graph – Value, RHS Graph - Volume, MAT, all countries

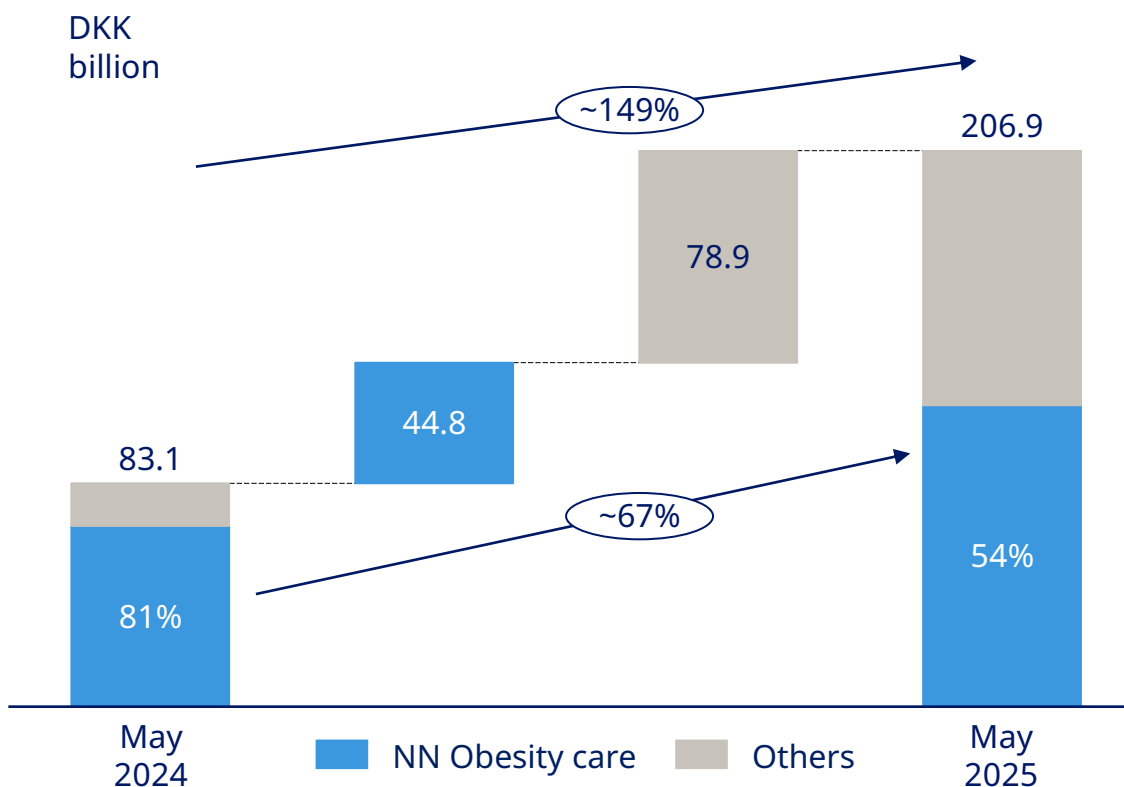


Obesity market share and market growth in US Operations

Obesity market growth and Novo Nordisk market share



Obesity market size and growth



NN: Novo Nordisk

Note: Share of growth not depicted due to too high numbers; Market values are based on the list prices

Source: IQVIA, May 2025, value, MAT, all countries

International Operations

International Operations

EUCAN

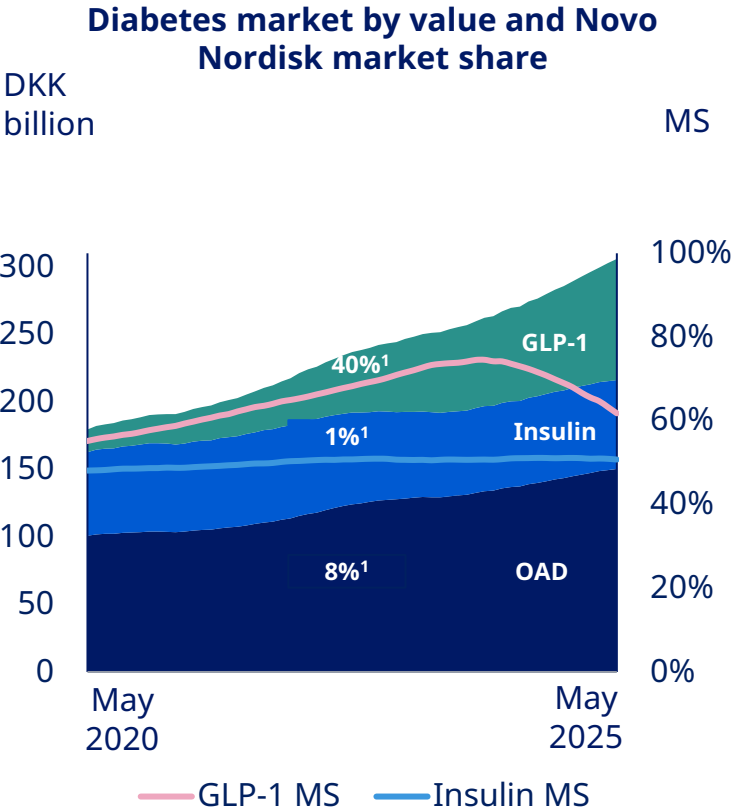
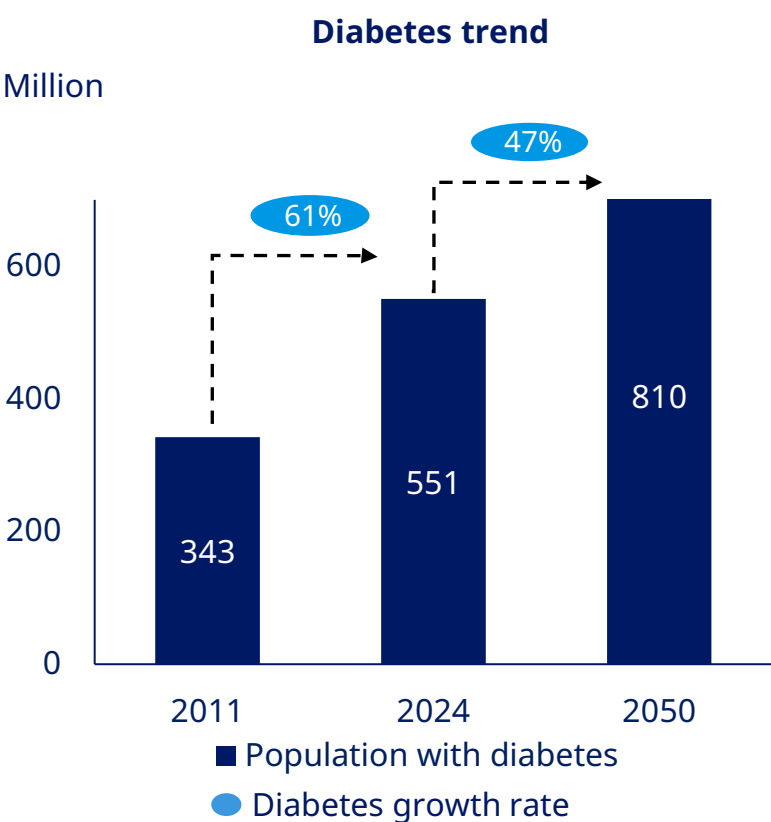
Emerging Markets

APAC

Region China



International Operations at a glance



Novo Nordisk H1 2025 reported sales

H1 2025	Sales (mDKK)	Growth ²
Injectable GLP-1 ³	21,319	8%
Rybelsus®	6,677	20%
Total GLP-1	27,996	10%
Total insulin⁴	19,662	-1%
Other Diabetes care ⁵	846	-15%
Diabetes care	48,504	5%
Obesity care ⁶	13,897	125%
Diabetes & Obesity care	62,401	19%
Rare disease ⁷	5,264	10%
Total	67,665	19%

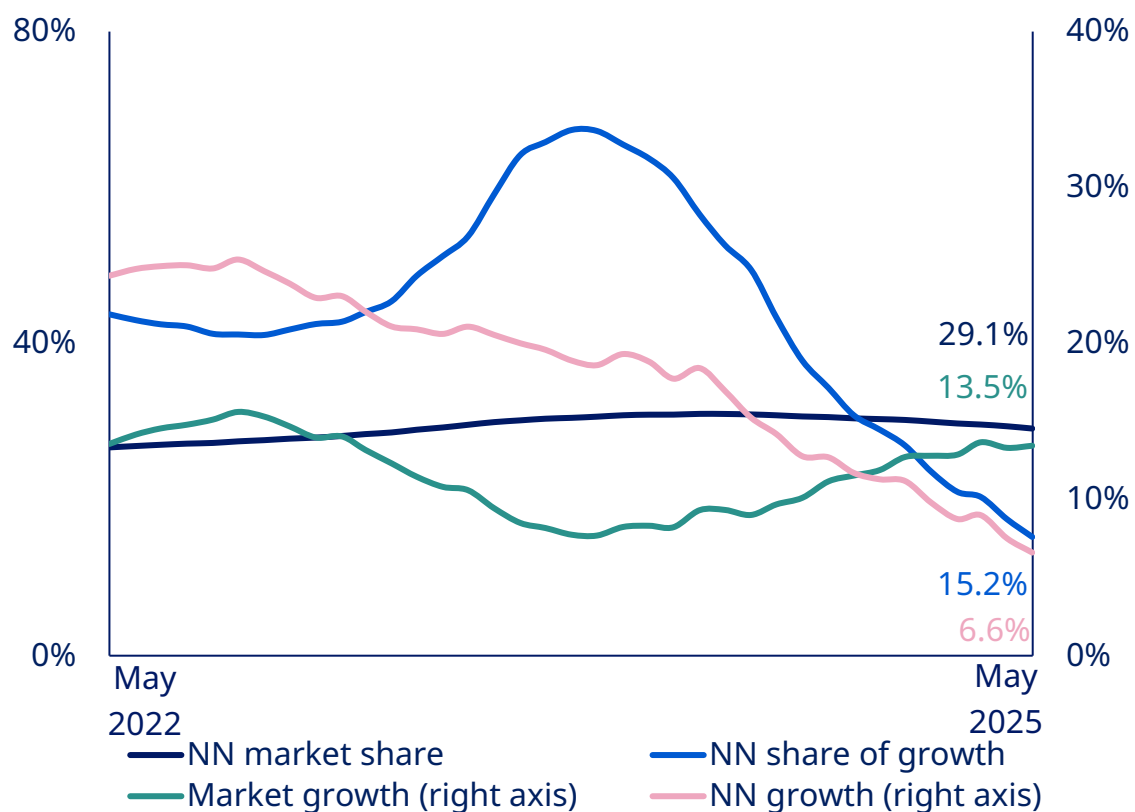
Source: International Diabetes Federation: Diabetes Atlas 11th edition, 2025

¹ CAGR calculated for 5-year period; Competitor insulin value market shares, as of May 2025: Novo Nordisk 51%, Others 49%; Competitor GLP-1 value market shares, as of May 2025: Novo Nordisk 63%, Other 37%; OAD: Oral anti-diabetic; MS: Market share; Note: Market values are based on the list prices; Source: IQVIA MAT, May 2025 value figures

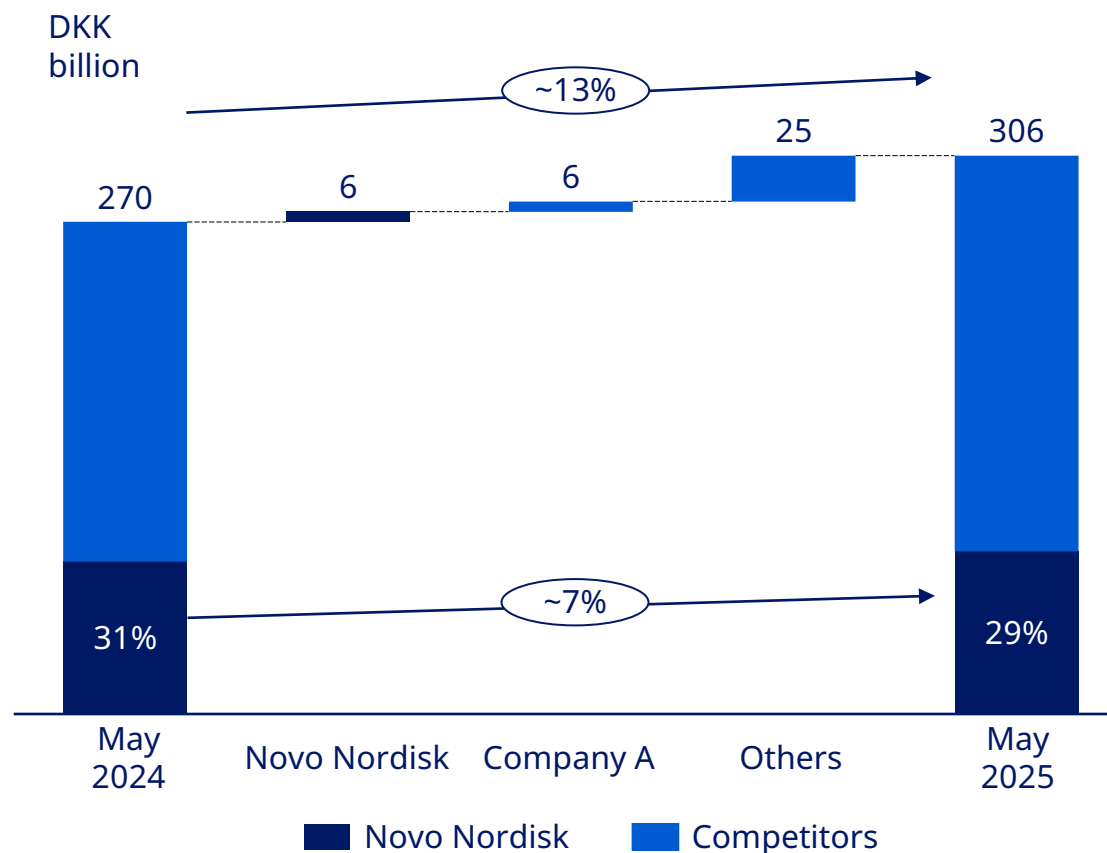
² At Constant exchange rates; ³ Comprises Victoza®, Ozempic®; ⁴ Comprises Tresiba®, Xultophy®, Levemir®, Ryzodeg®, NovoMix®, Fiasp®, Awiqli®, Ryzodeg® and NovoRapid®; ⁵ Comprises NovoNorm® and needles; ⁶ Obesity care comprises Saxenda® and Wegovy®; ⁷ Comprises primarily NovoSeven®, NovoEight®, NovoThirteen®, Refixia®, Esperoct®, Norditropin®, Vagifem® and Activello®

Diabetes market share and market growth in International Operations

Diabetes market growth and Novo Nordisk market share



Diabetes market size and growth



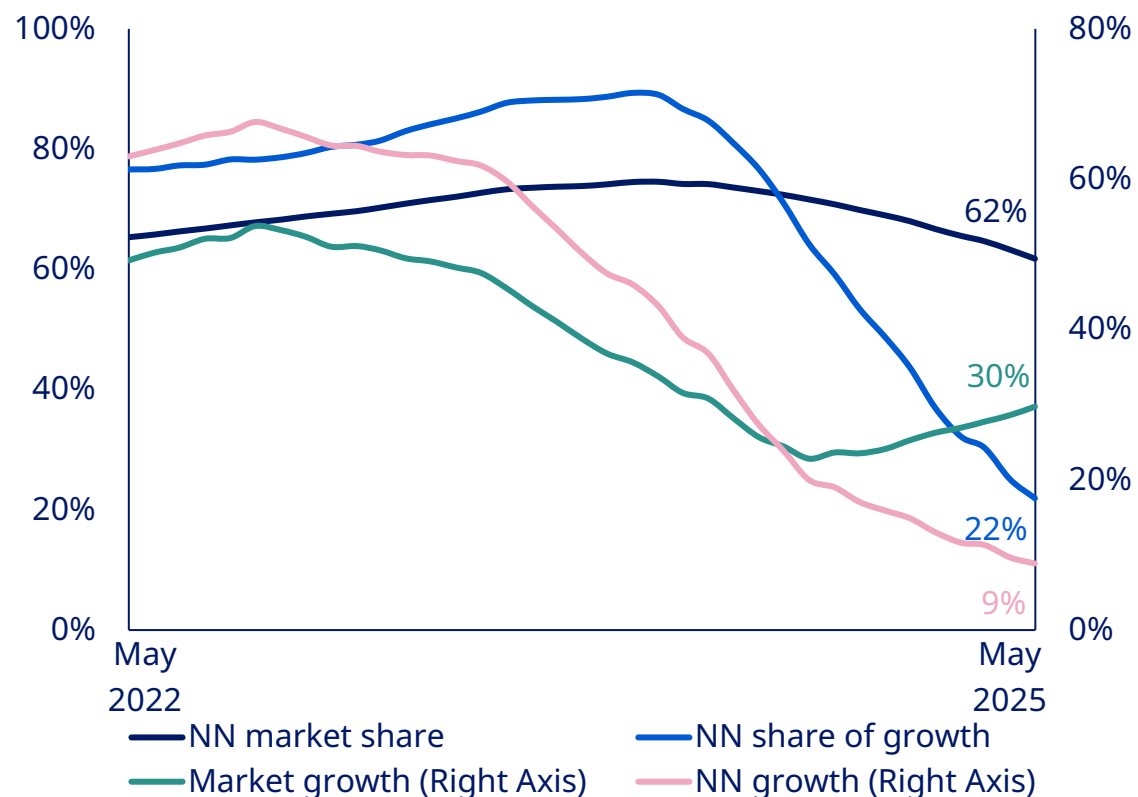
NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company. Market values are based on the list prices

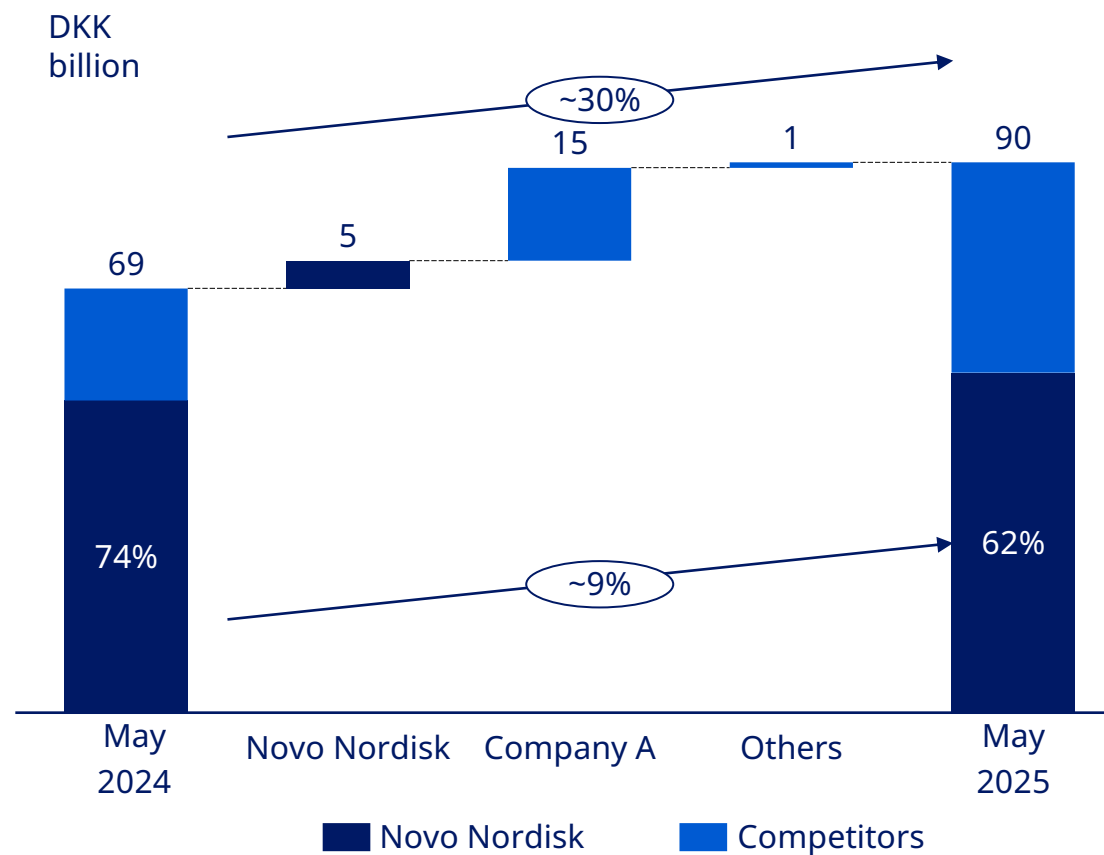
Source: IQVIA, May 2025, Value MAT

GLP-1 market share and market growth

GLP-1 market growth and Novo Nordisk market share



GLP-1 market size and growth

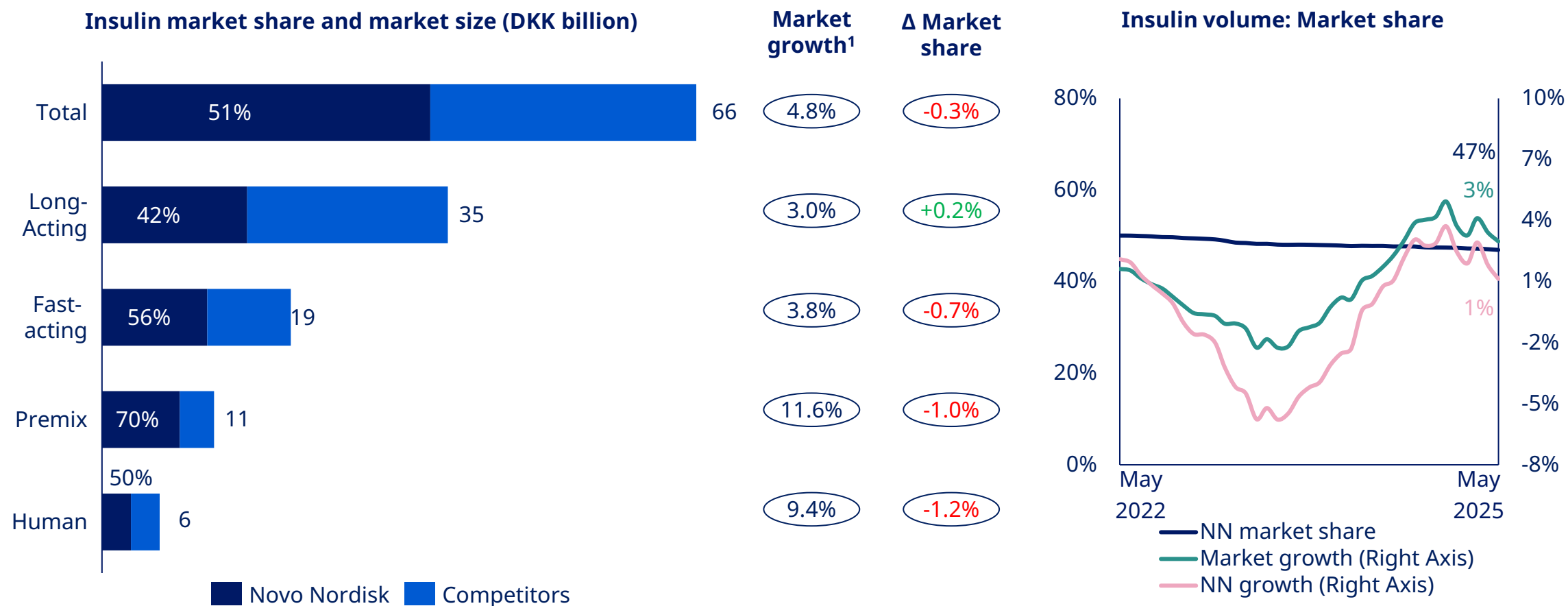


NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company. Market values are based on the list prices

Source: IQVIA, May 2025, Value MAT, all countries

Insulin market size and volume share of growth and market share in International Operations



¹Market growth is YTD current vs YTD previous year

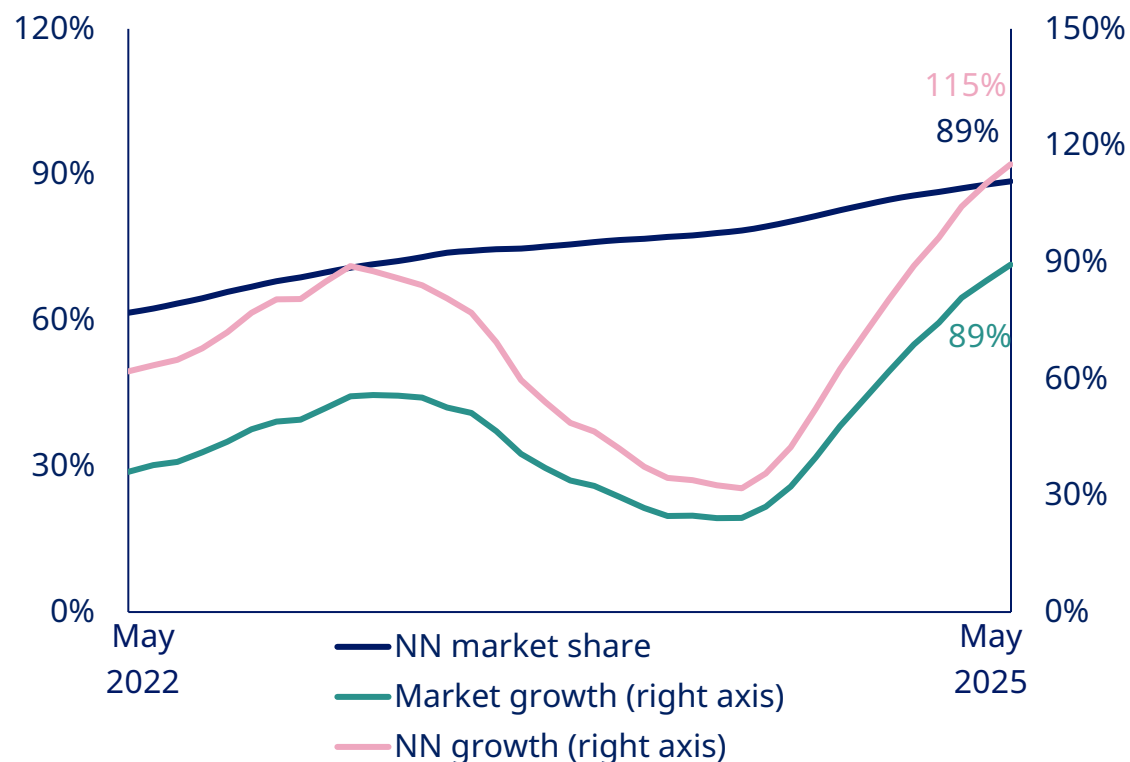
NN: Novo Nordisk

Note: Share of growth not depicted due to too high numbers; Market values are based on the list prices

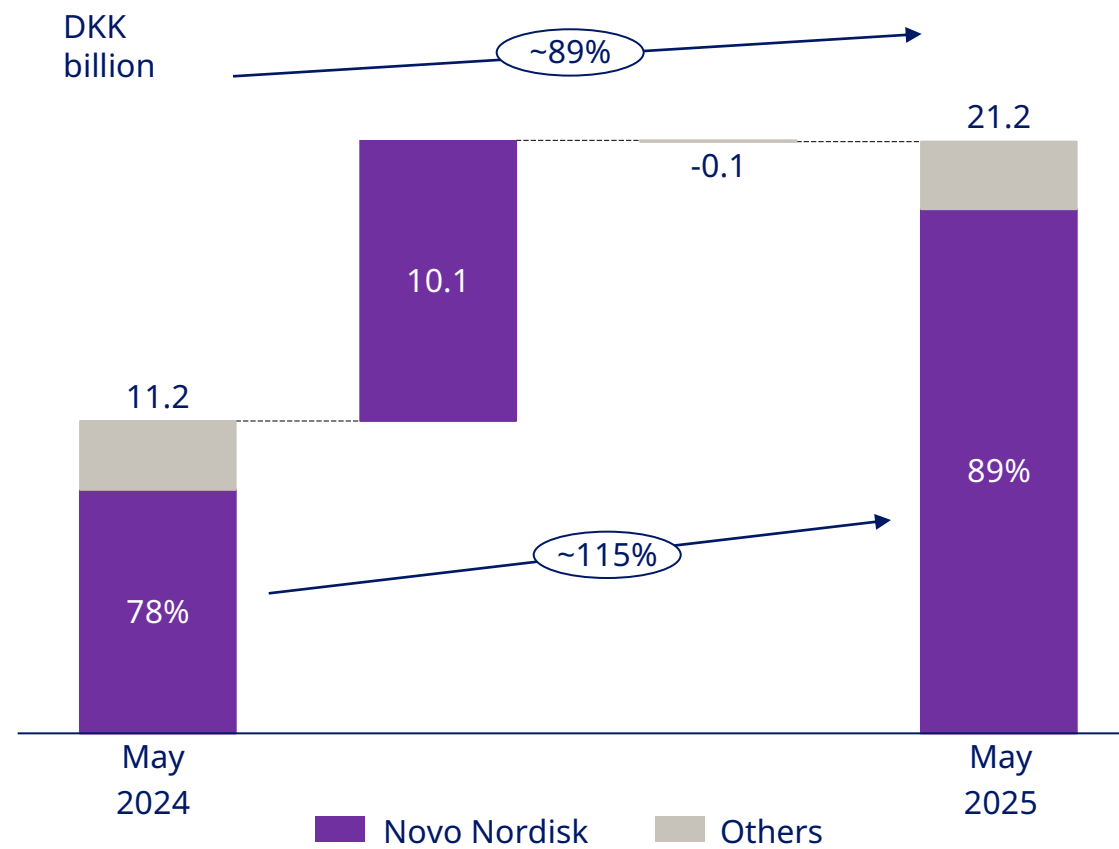
Source: IQVIA, May 2025, LHS graph – Value, RHS Graph – Volume, MAT, all countries

Obesity market share and market growth in International Operations

Obesity market growth and Novo Nordisk market share



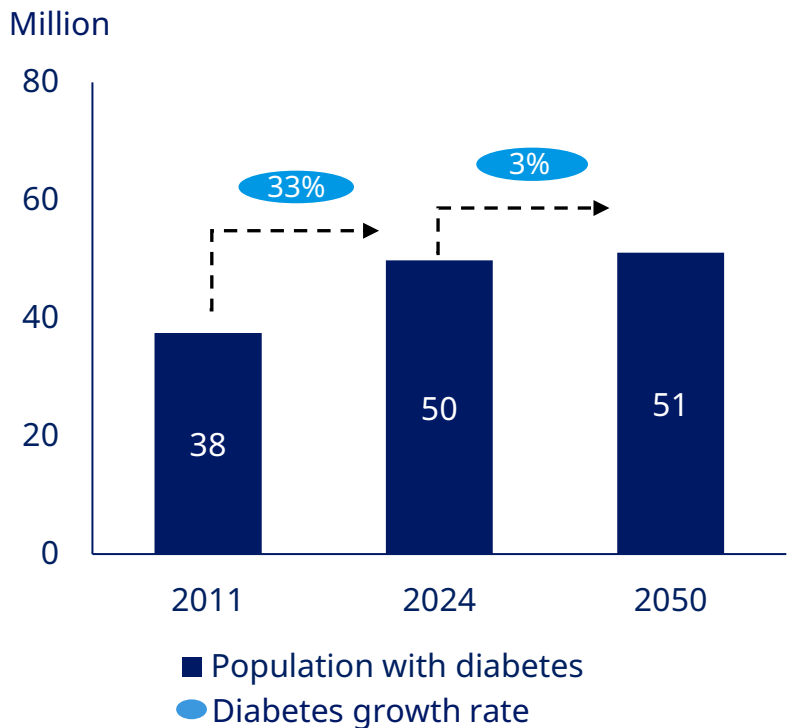
Obesity market size and growth



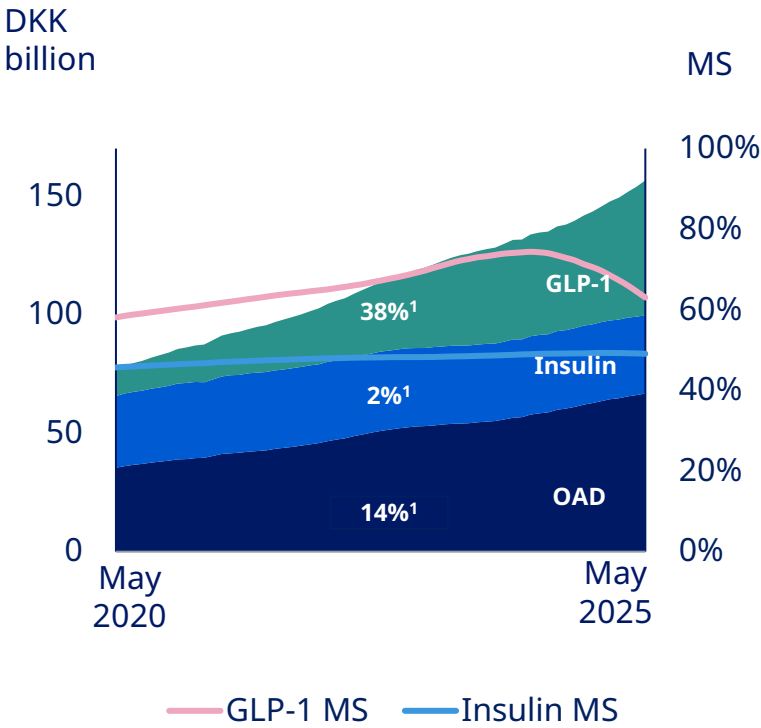


EUCAN at a glance

Diabetes trend



Diabetes market by value and Novo Nordisk market share



Novo Nordisk H1 2025 reported sales

H1 2025	Sales (mDKK)	Growth ²
Injectable GLP-1 ³	11,261	13%
Rybelsus®	3,734	23%
Total GLP-1	14,995	15%
Total insulin ⁴	6,361	-5%
Other Diabetes care ⁵	263	-5%
Diabetes care	21,619	8%
Obesity care ⁶	7,060	64%
Diabetes & Obesity care	28,679	18%
Rare disease ⁷	2,533	1%
Total	31,212	16%

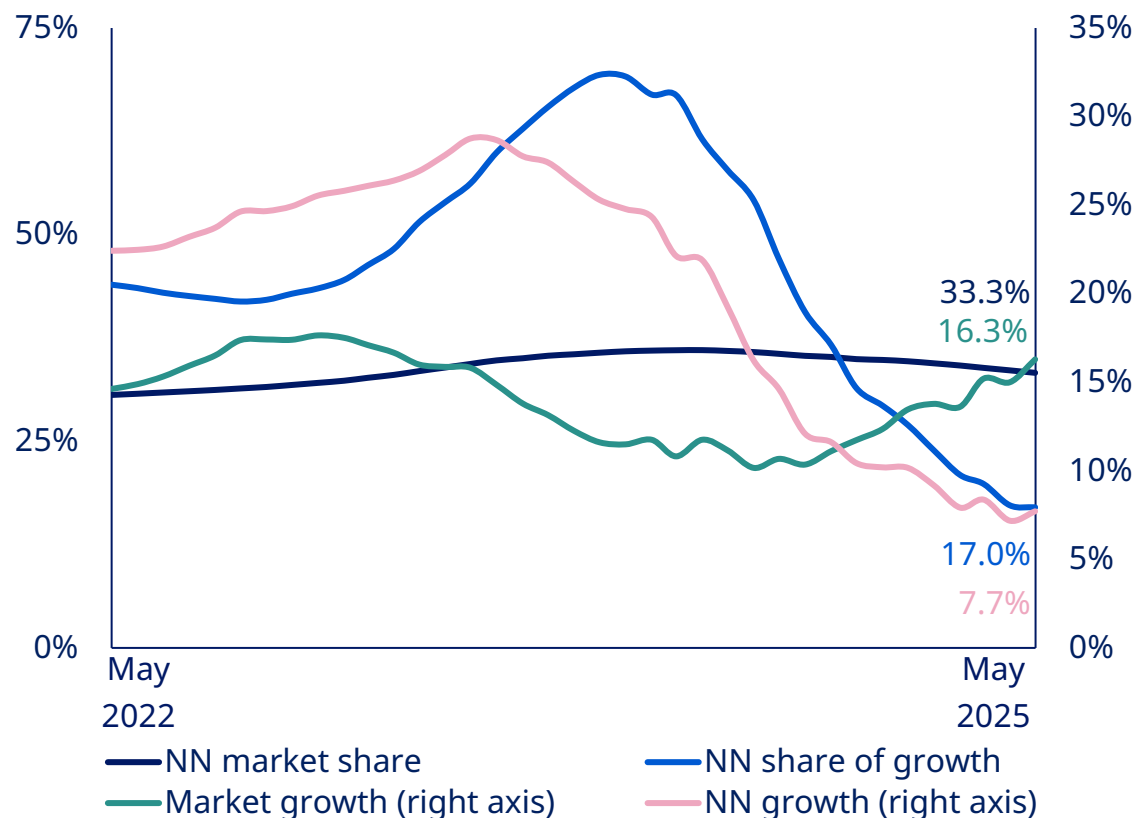
¹ CAGR calculated for 5-year period; Competitor insulin value market shares, as of May 2025: Novo Nordisk 49%, Others 51%; Competitor GLP-1 value market shares, as of May 2025: Novo Nordisk 63%, Others 37%. OAD: Oral anti-diabetic; MS: Market share; Note: Market values are based on the list prices; Source: IQVIA May 2025 value figures

² At Constant exchange rates; ³ Comprises Victoza®, Ozempic®; ⁴ Comprises Tresiba®, Xultophy®, Levemir®, Ryzodeg®, Awiqli®, NovoMix®, Fiasp® and NovoRapid®; ⁵ Comprises NovoNorm® and needles; ⁶ Obesity care comprises Saxenda® and Wegovy®; ⁷ Comprises primarily NovoSeven®, NovoEight®, NovoThirteen®, Esperoct®, Refixia®, Norditropin®, Vagifem® and Activellé®

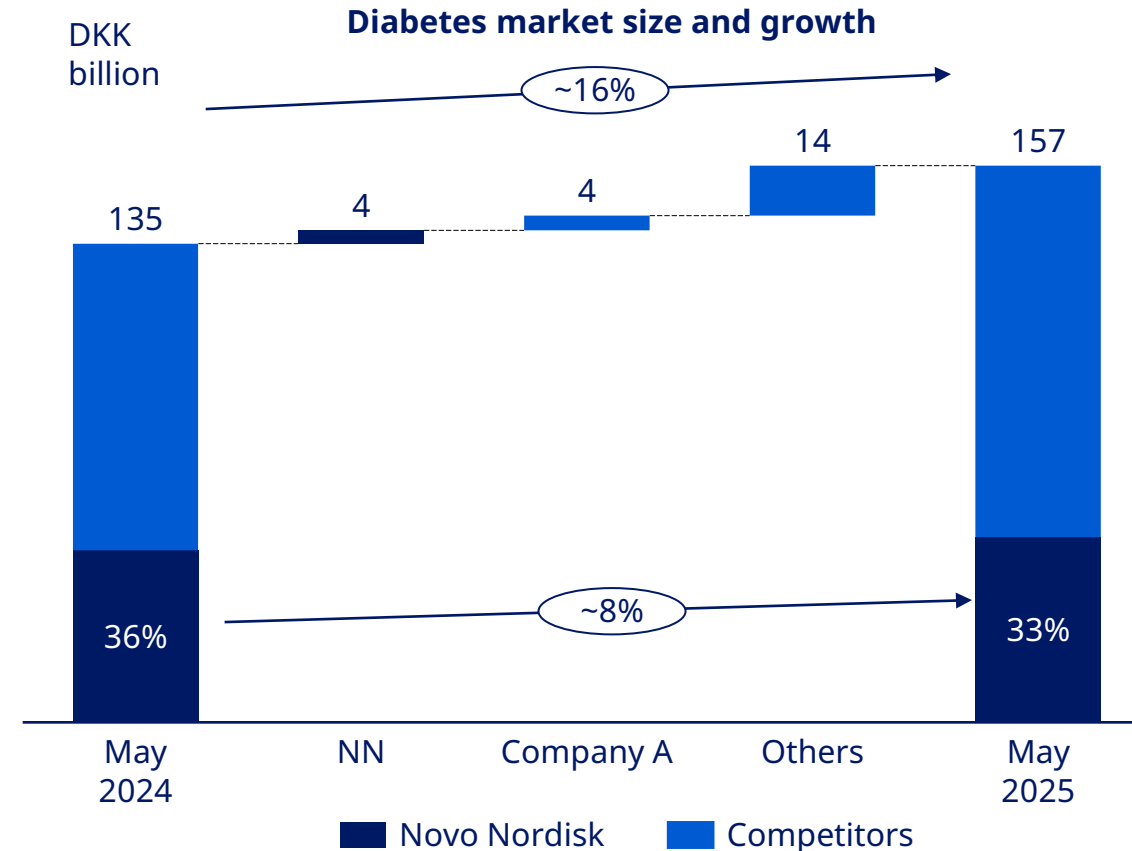


Diabetes market share and market growth in EUCAN

Diabetes market growth and Novo Nordisk market share



Diabetes market size and growth



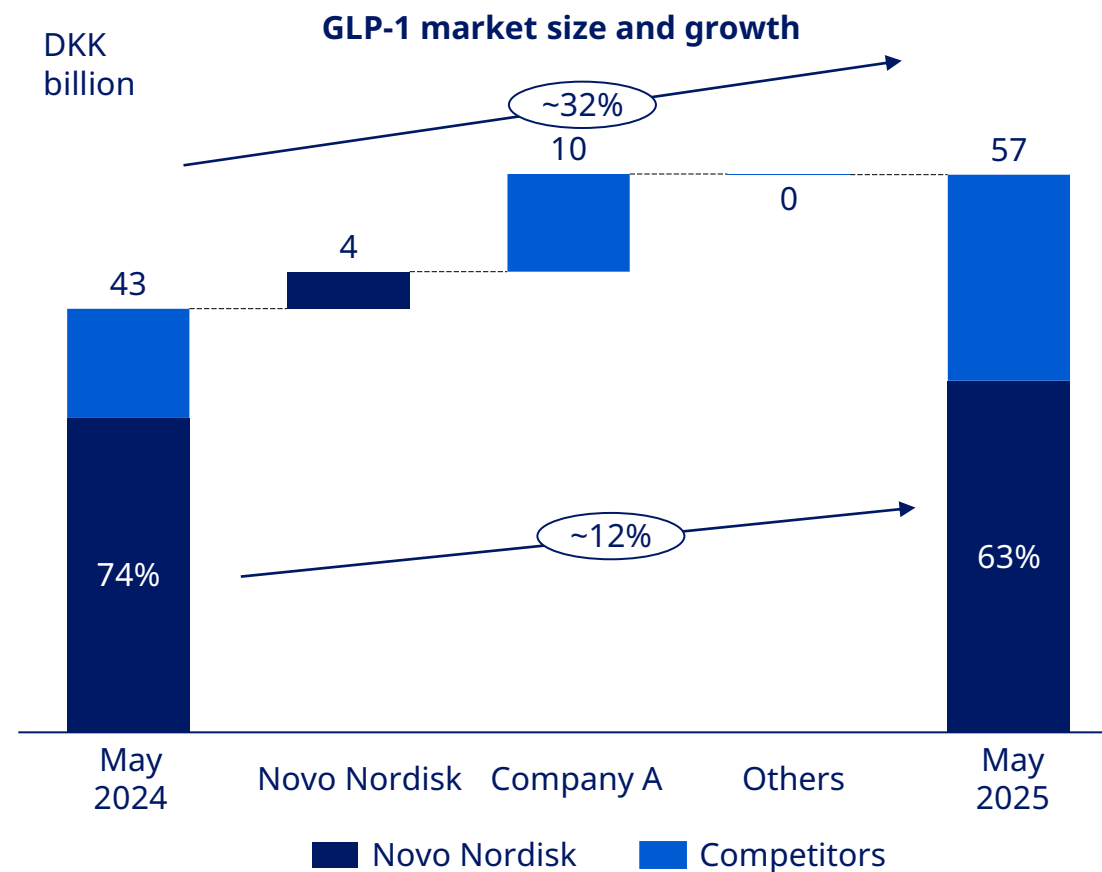
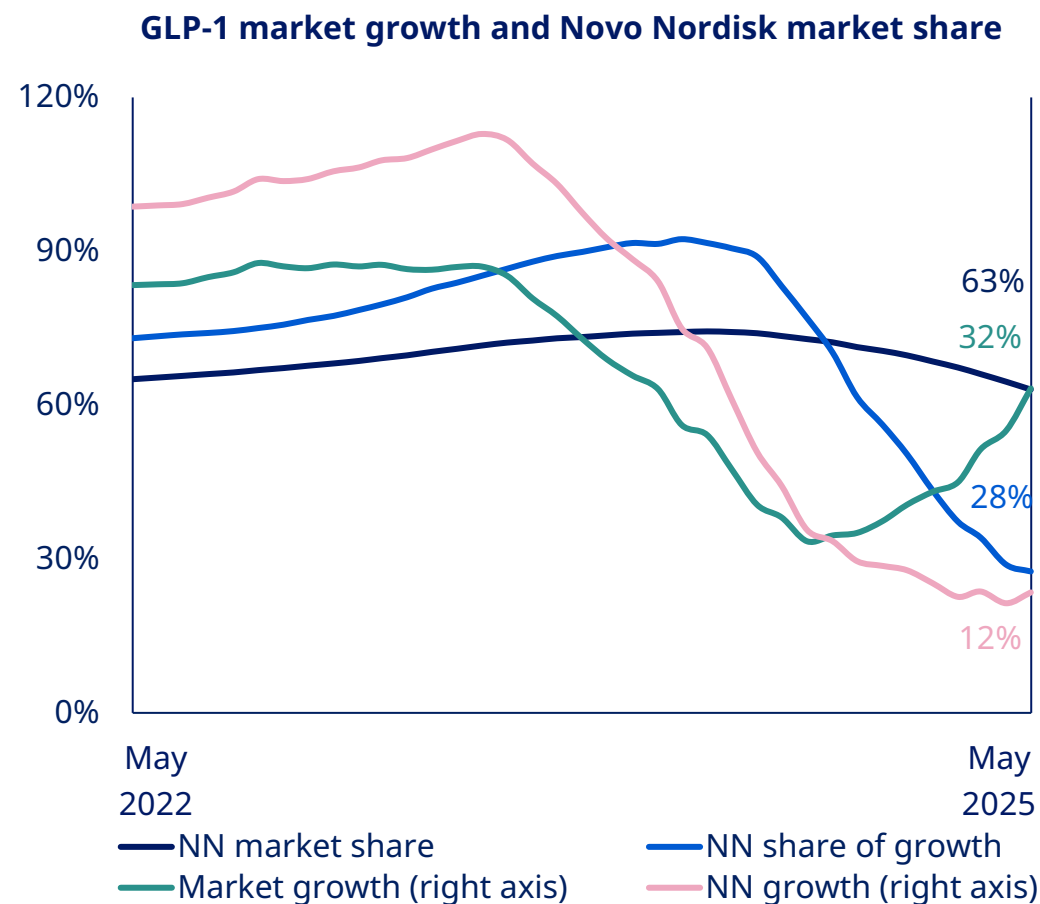
EUCAN: Europe and Canada; IO: International Operations; NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company; Market values are based on the list prices

Source: IQVIA, May 2025, Value, MAT



GLP-1 market share and market growth in EUCAN



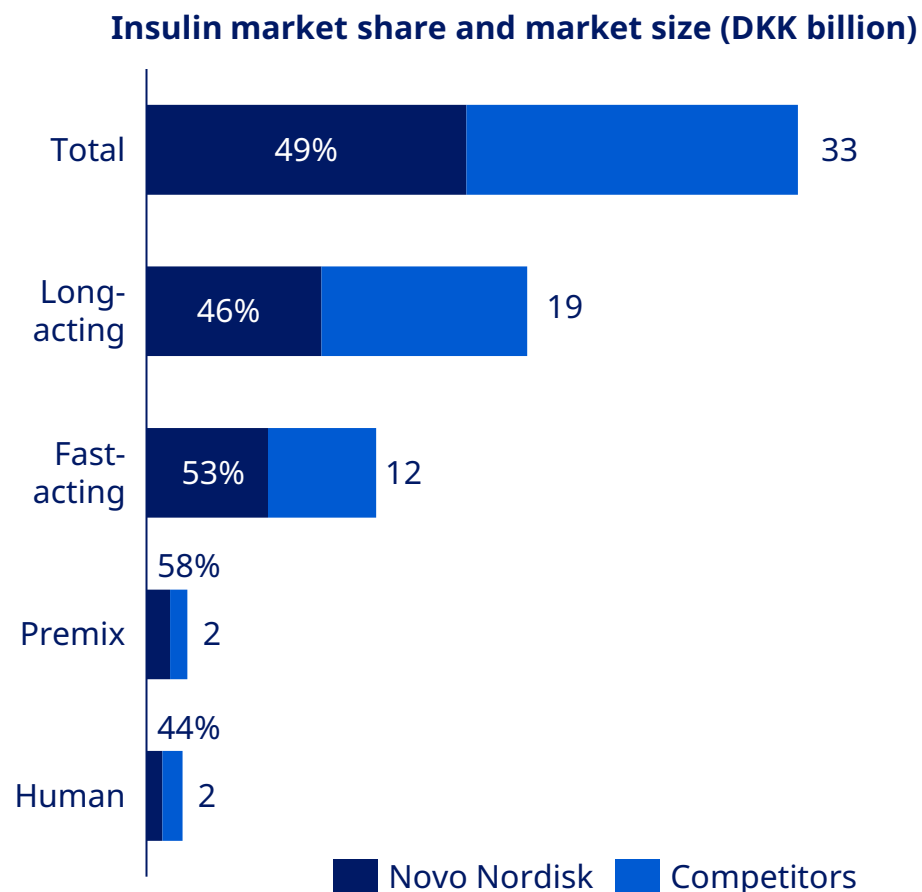
EUCAN: Europe and Canada; NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company; Market values are based on the list prices

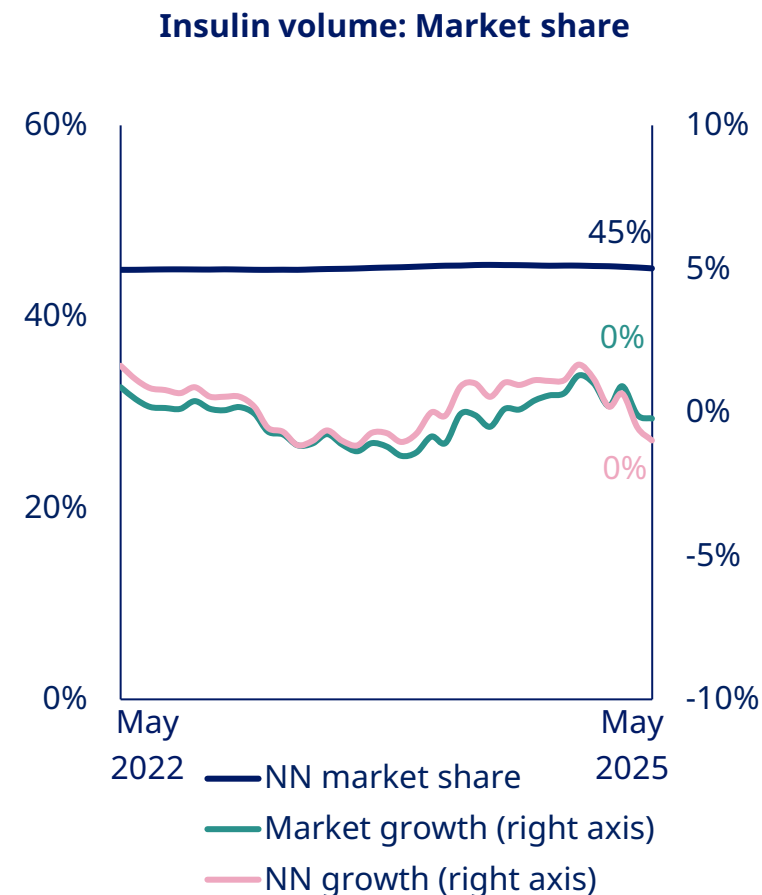
Source: IQVIA, May 2025, Value, MAT



Insulin market size and volume market share in EUCAN



Market growth ¹	Δ Market share
-0.6%	+0.0%
-0.7%	+0.3%
-0.5%	-0.2%
-0.3%	+0.1%
1.5%	-1.9%



¹Market growth is YTD current vs YTD previous year

EUCAN: Europe and Canada; NN: Novo Nordisk

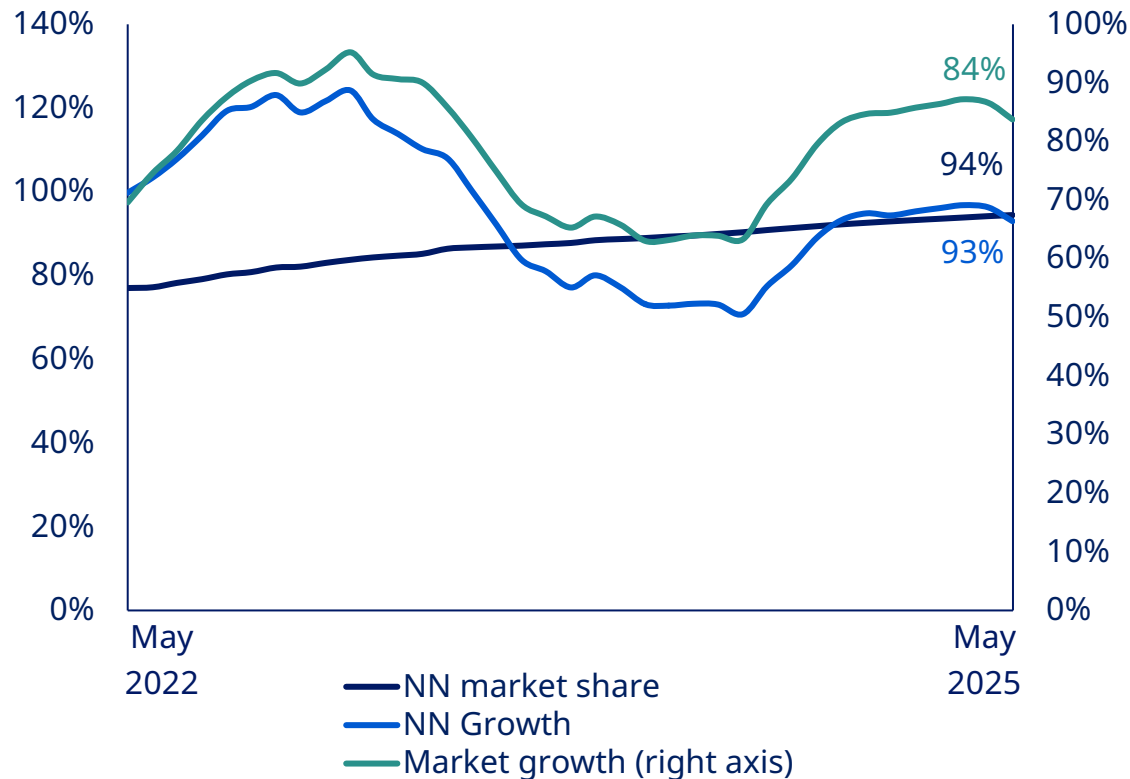
Note: Share of growth not depicted due to too high numbers; Market values are based on the list prices

Source: IQVIA, May 2025 LHS graph - Value, RHS Graph - Volume, MAT

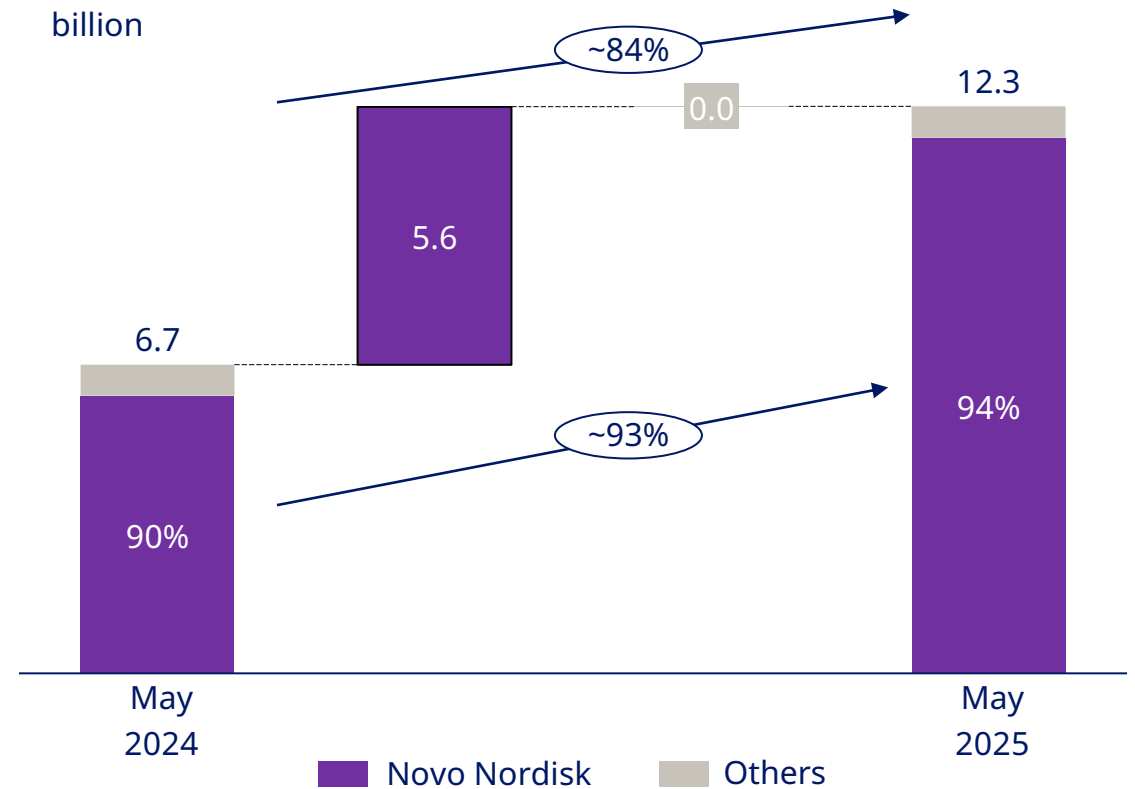


Obesity market share and market growth in EUCAN

Obesity market growth and Novo Nordisk market share

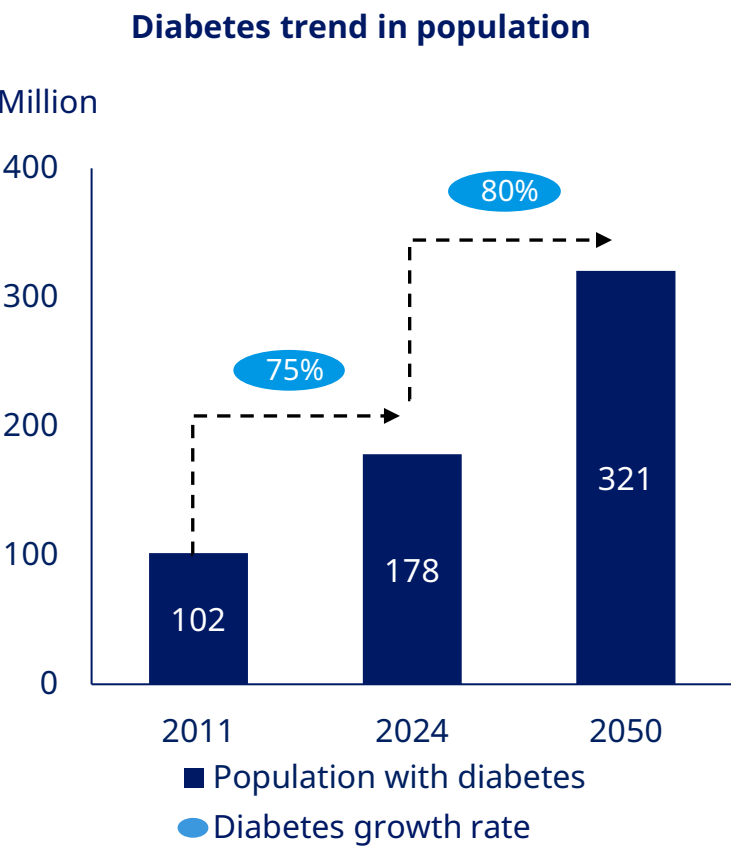


Obesity market size and growth

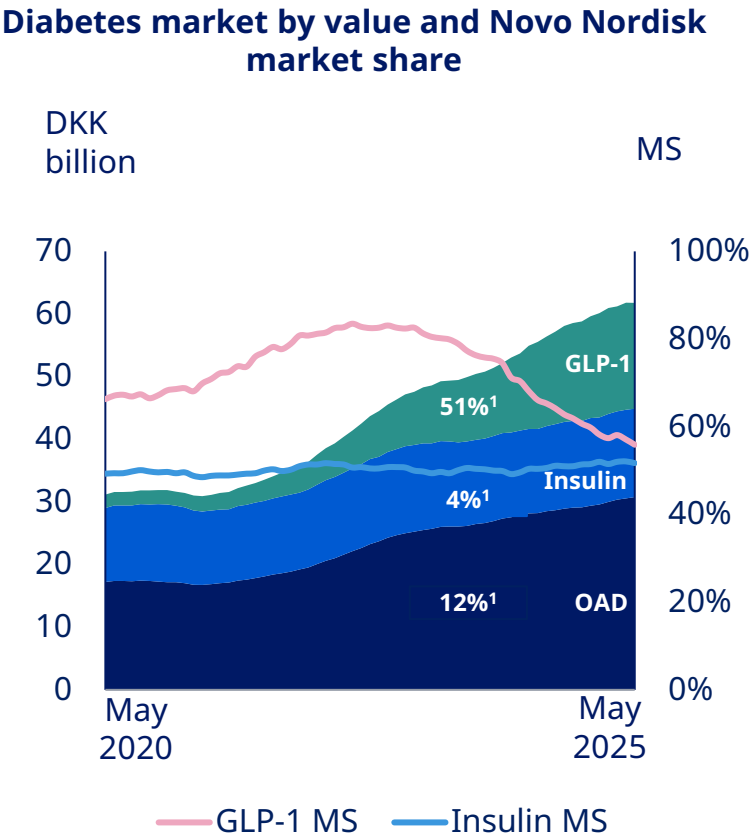




Emerging Markets at a glance



Emerging Markets: mainly Latin America, Middle East and Africa
Source: International Diabetes Federation: Diabetes Atlas 11th edition, 2025



¹ CAGR calculated for last 5-year period
Competitor insulin value market shares, as of May 2025: Novo Nordisk 52%, Others 48%; Competitor GLP-1 value market shares, as of May 2025: Novo Nordisk 56%, Others 44%. OAD: Oral anti-diabetic; MS: Market Share; Note: Market values are based on list prices; Source: IQVIA MAT, May 2025 value figures

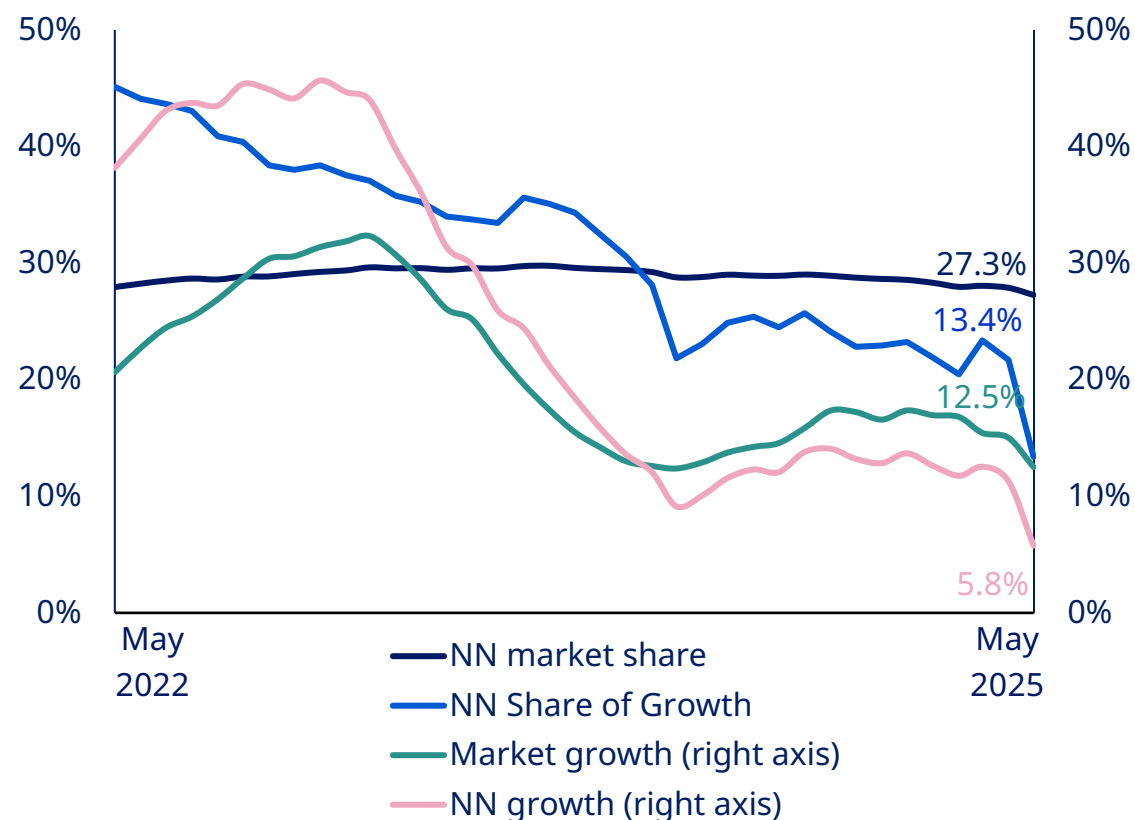
Novo Nordisk H1 2025 reported sales		
H1 2025	Sales (mDKK)	Growth ²
Injectable GLP-1³	5,138	14%
Rybelsus®	1,066	8%
Total GLP-1	6,204	13%
Total insulin⁴	5,357	2%
Other Diabetes care ⁵	144	-1%
Diabetes care	11,705	7%
Obesity care ⁶	3,260	157%
Diabetes & Obesity care	14,965	24%
Rare disease ⁷	1,369	6%
Total	16,334	22%

² At constant exchange rates; ³ Comprises Victoza®, Ozempic®; ⁴ Comprises Tresiba®, Xultophy®, Levemir®, Awiqli®, NovoMix®, Ryzodeg®, NovoRapid® and Fiasp®; ⁵ Comprises NovoNorm® and needles; ⁶ Comprises Saxenda® and Wegovy®; ⁷ Comprises primarily Esperoct®, Refixia®, NovoSeven®, NovoEight® and Norditropin®

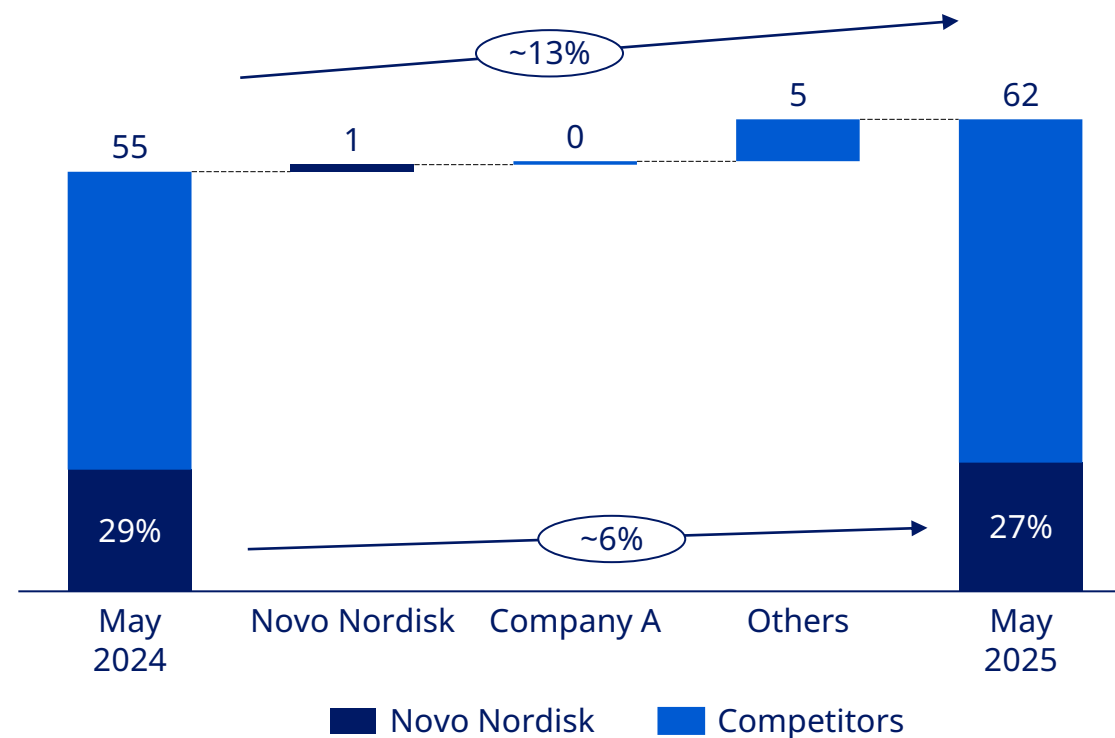


Diabetes market share and market growth in Emerging Markets

Diabetes market growth and Novo Nordisk market share



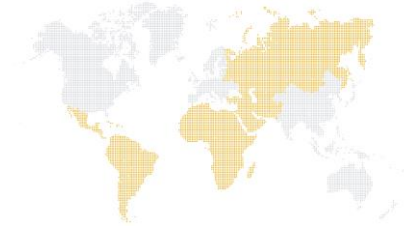
DKK billion Diabetes market size and growth



Emerging Markets: mainly Latin America, Middle East and Africa; NN: Novo Nordisk

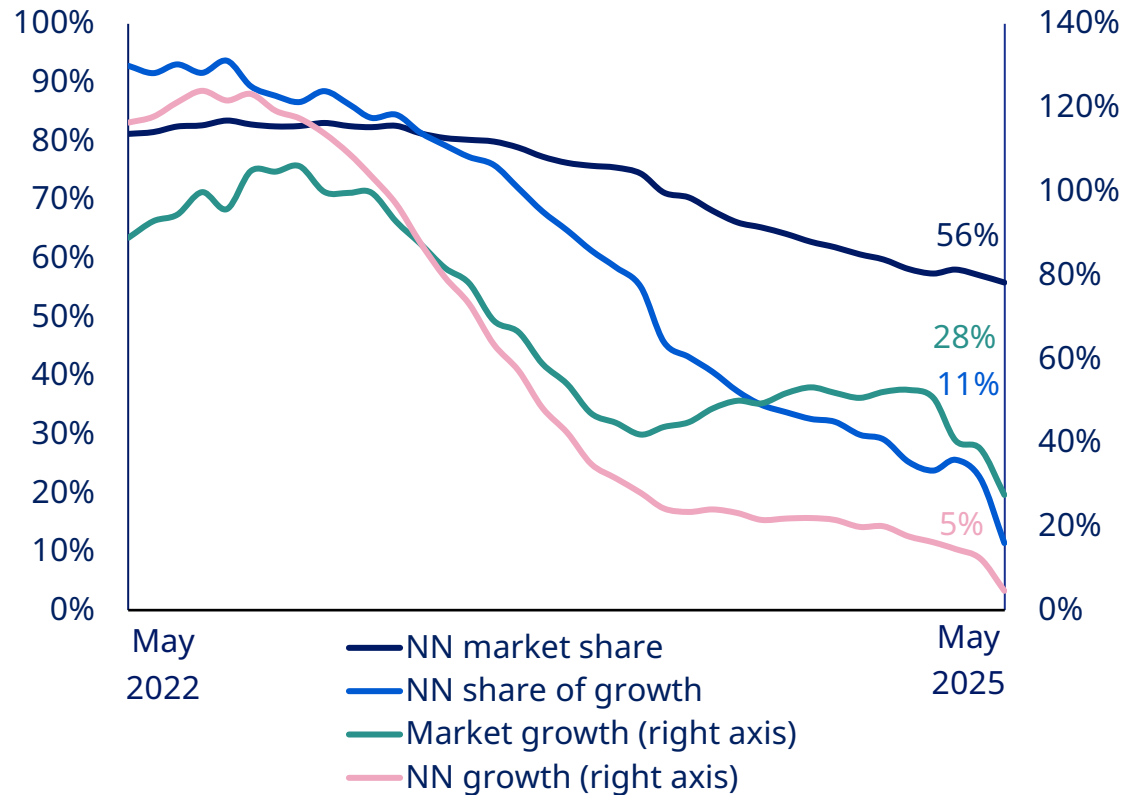
Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company. Rest of world Market values are based on the list prices

Source: IQVIA, May 2025, value, MAT

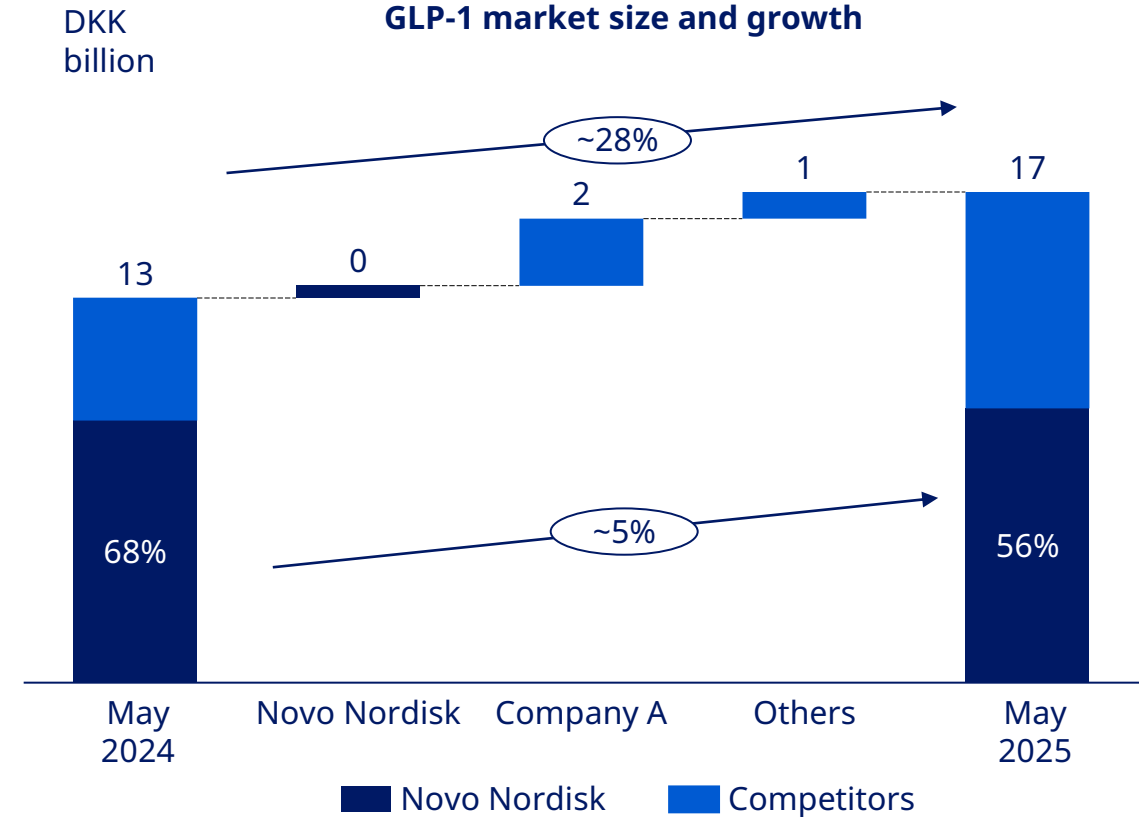


GLP-1 market share and market growth in Emerging Markets

GLP-1 market growth and Novo Nordisk market share



GLP-1 market size and growth



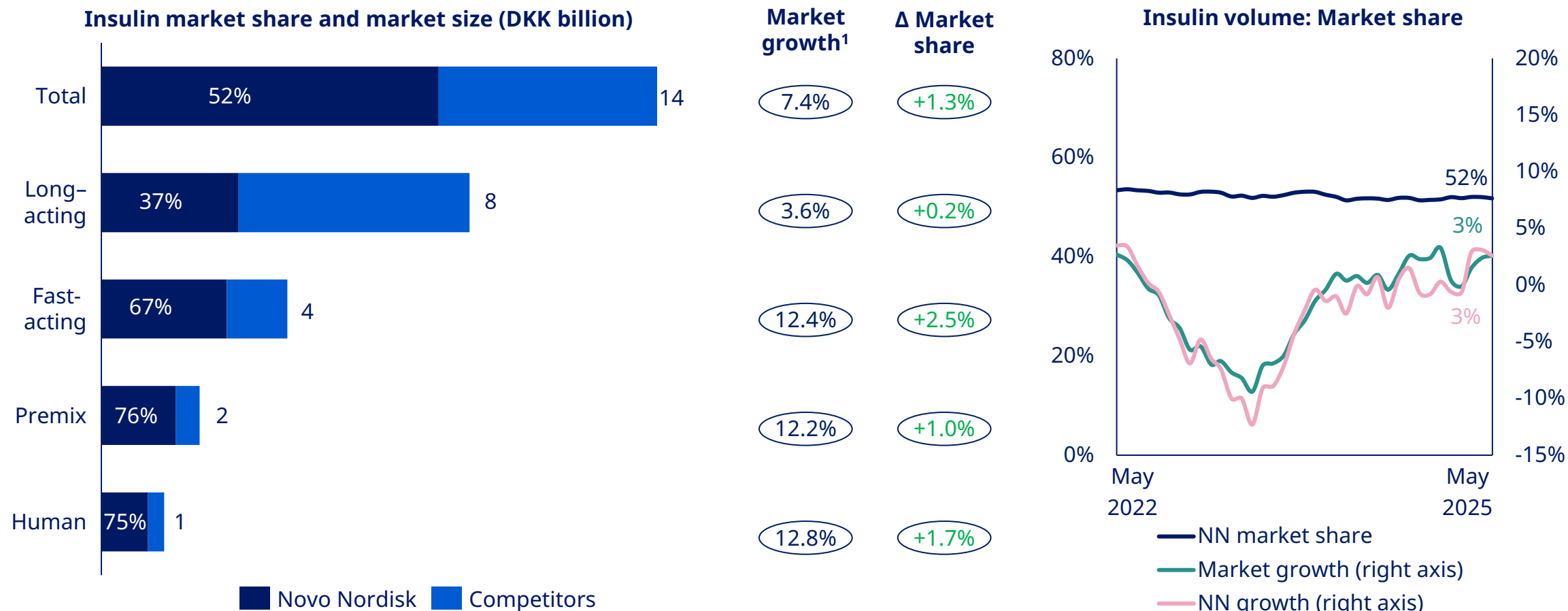
Emerging Markets: mainly Latin America, Middle East and Africa; NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company.; Market values are based on the list prices

Source: IQVIA, May 2025, value, MAT



Insulin market size and volume market share in Emerging Markets



¹Market growth is YTD current vs YTD previous year

Emerging Markets: mainly Latin America, Middle East and Africa; NN: Novo Nordisk

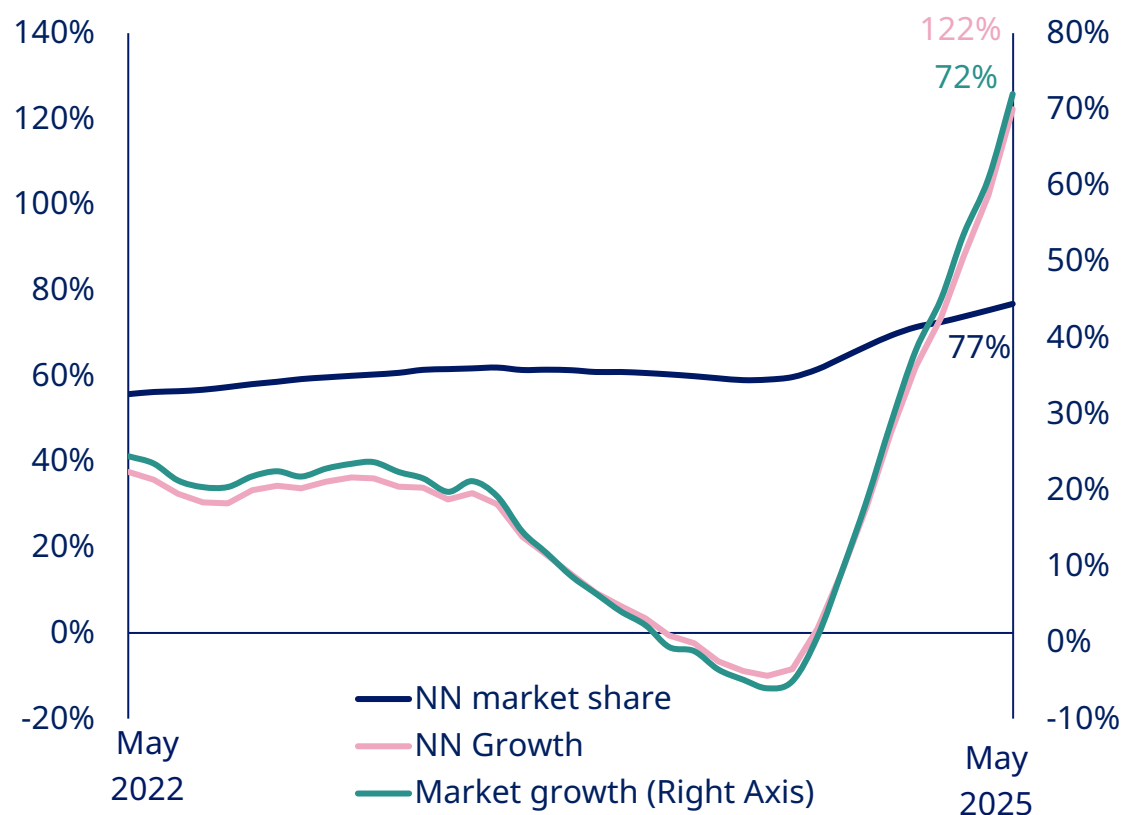
Note: Share of growth not depicted due to too high numbers;; Market values are based on the list prices

Source: IQVIA, May 2025; LHS graph – Value, RHS Graph – Volume, MAT

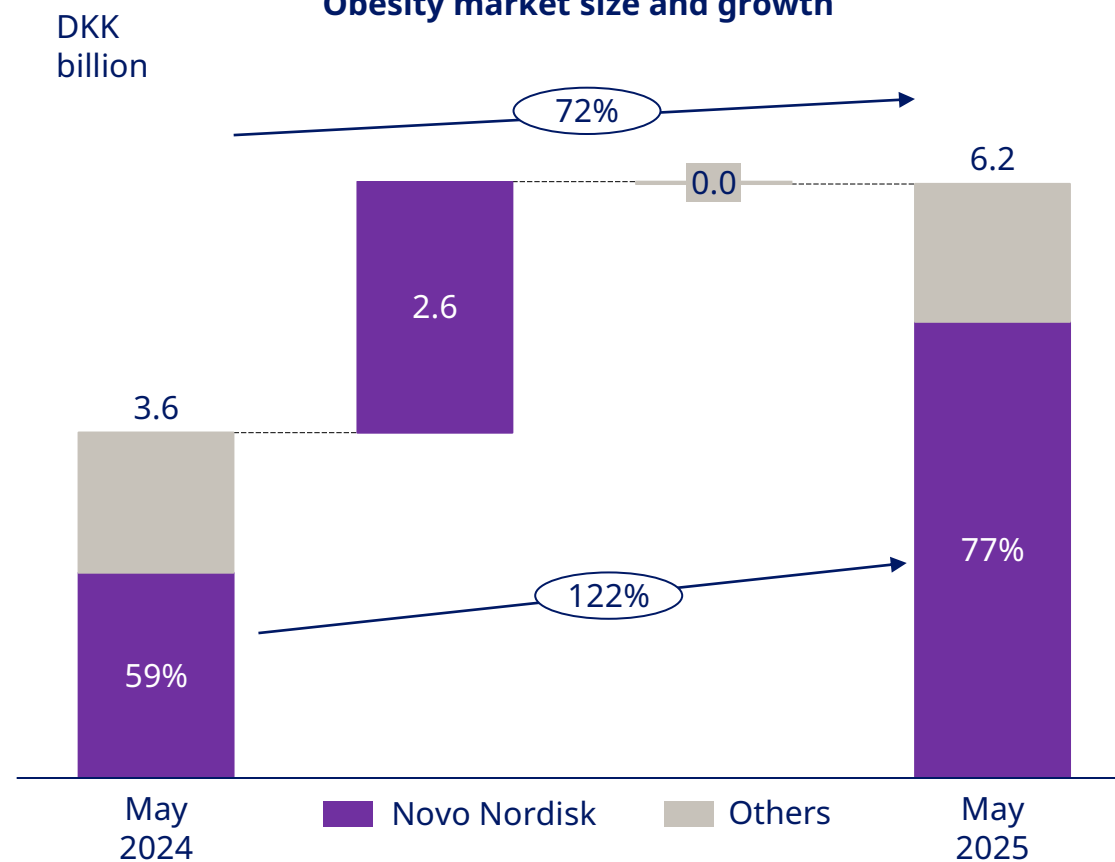


Obesity market share and market growth in Emerging Markets

Obesity market growth and Novo Nordisk market share

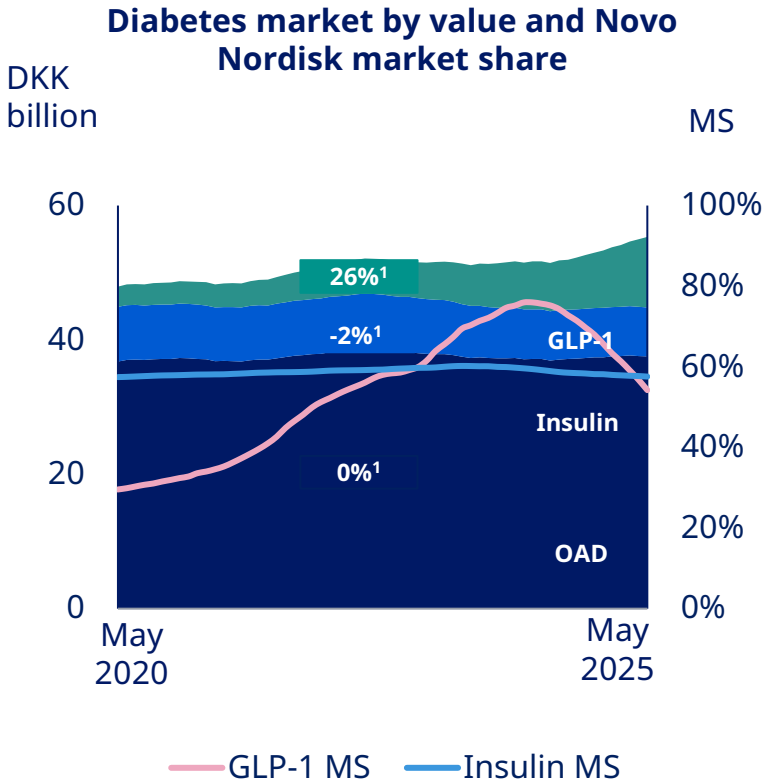
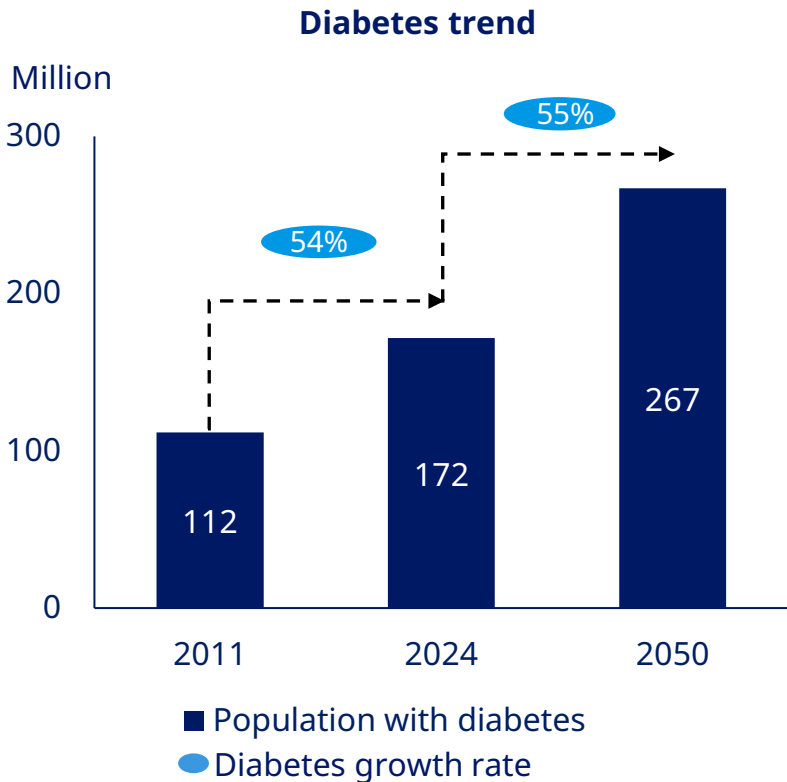


Obesity market size and growth





APAC at a glance



Novo Nordisk H1 2025 reported sales		
H1 2025	Sales (mDKK)	Growth ²
Injectable GLP-1 ³	1,814	2%
Rybelsus®	1,761	22%
Total GLP-1	3,575	11%
Total insulin ⁴	2,754	-3%
Other Diabetes care ⁵	138	0%
Diabetes care	6,467	4%
Obesity care ⁶	2,715	361%
Diabetes & Obesity care	9,182	37%
Rare disease ⁷	1,027	22%
Total	10,209	35%

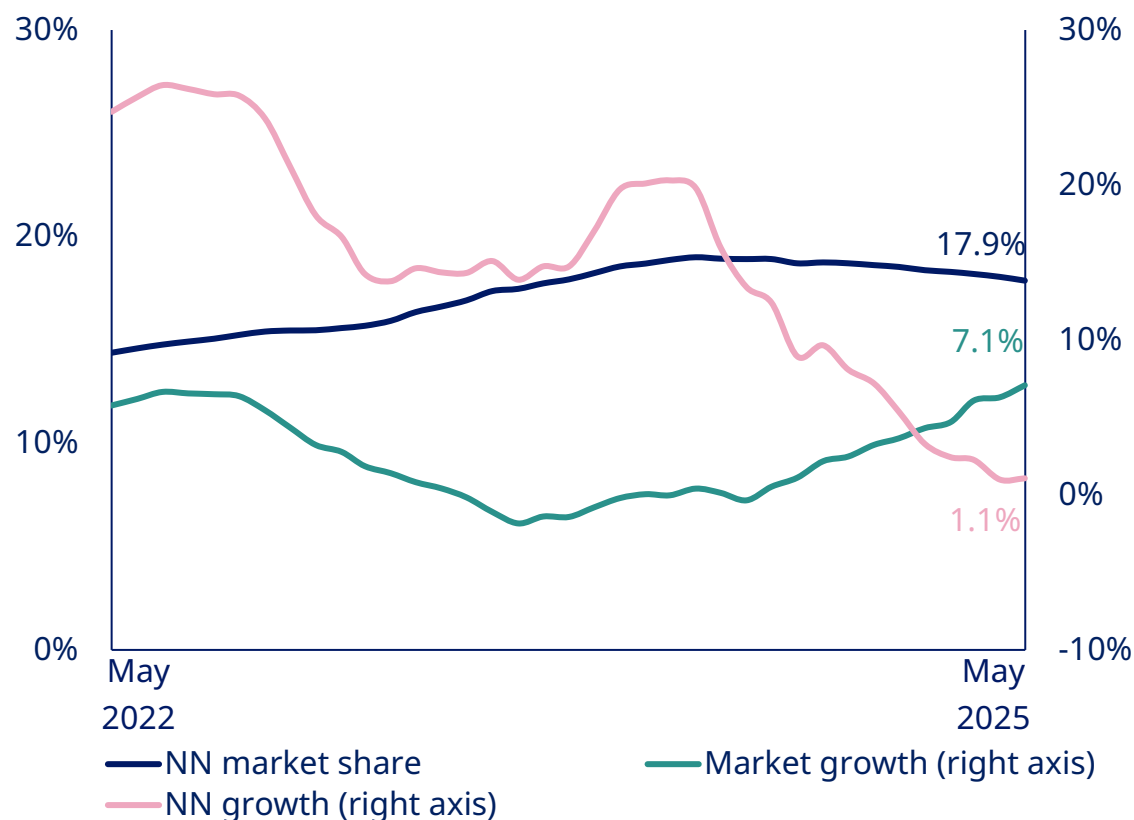
¹ CAGR calculated for 5-year period; Competitor insulin value market shares, as of May 2025: Novo Nordisk 58%, Others 42%; Competitor GLP-1 value market shares, as of May 2025: Novo Nordisk 54%, Others 46%. OAD: Oral anti-diabetic; MS: Market share; Note: Market values are based on the list prices; Source: IQVIA May 2025 value figures

² At Constant exchange rates; ³ Comprises Victoza®, Ozempic®; ⁴ Comprises Tresiba®, Xultophy®, Levemir®, Ryzodeg®, Awiqli®, NovoMix®, Fiasp® and NovoRapid®; ⁵ Comprises NovoNorm® and needles; ⁶ Obesity care comprises Saxenda® and Wegovy®; ⁷ Comprises primarily NovoSeven®, NovoEight®, NovoThirteen®, Esperoct®, Refixia®, Norditropin®, Vagifem® and Activelle®

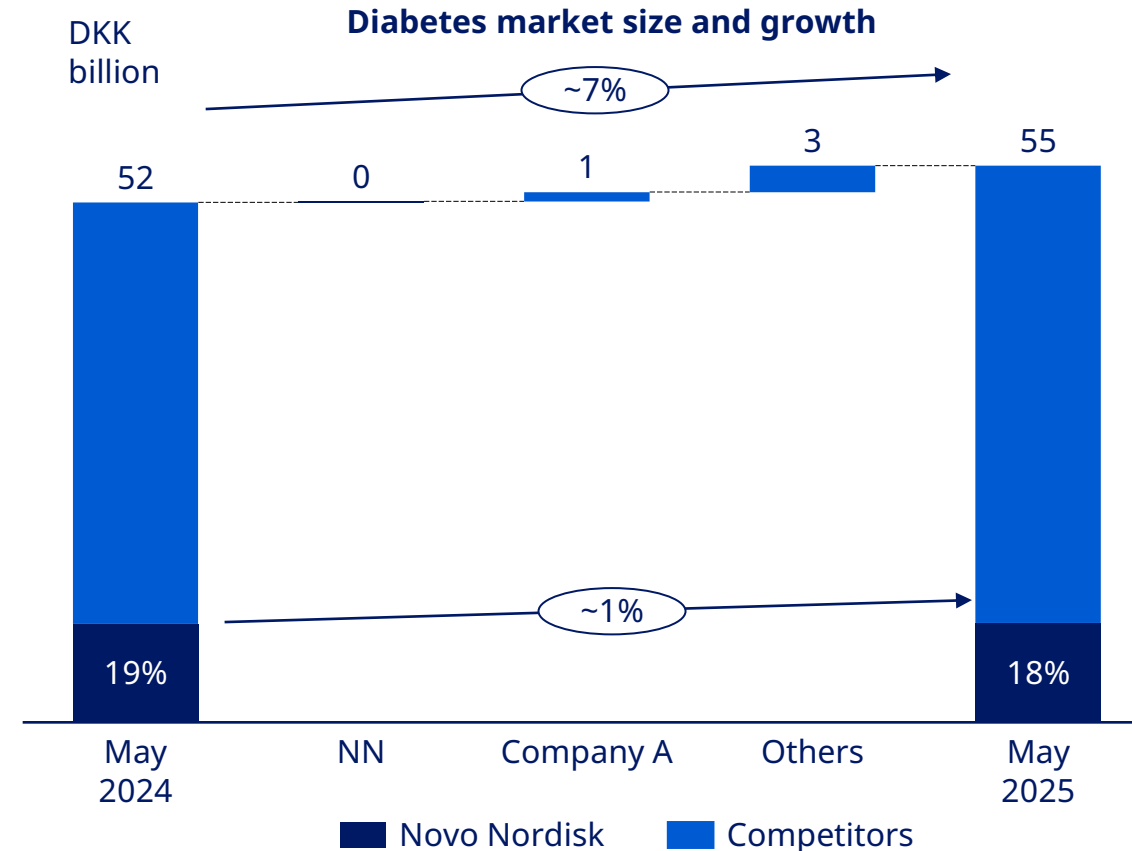


Diabetes market share and market growth in APAC

Diabetes market growth and Novo Nordisk market share



Diabetes market size and growth



APAC: Japan, Korea, Oceania and Southeast Asia; NN: Novo Nordisk

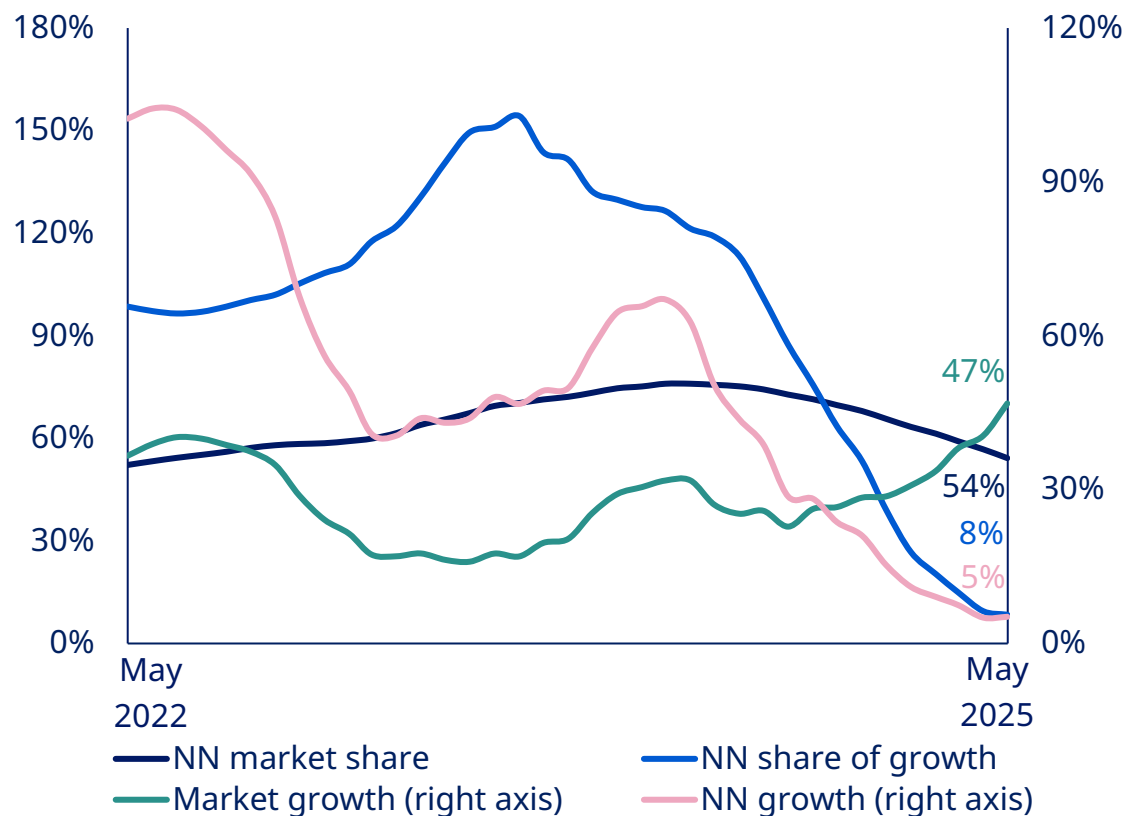
Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company; Market values are based on the list prices

Source: IQVIA, May 2025, Value, MAT

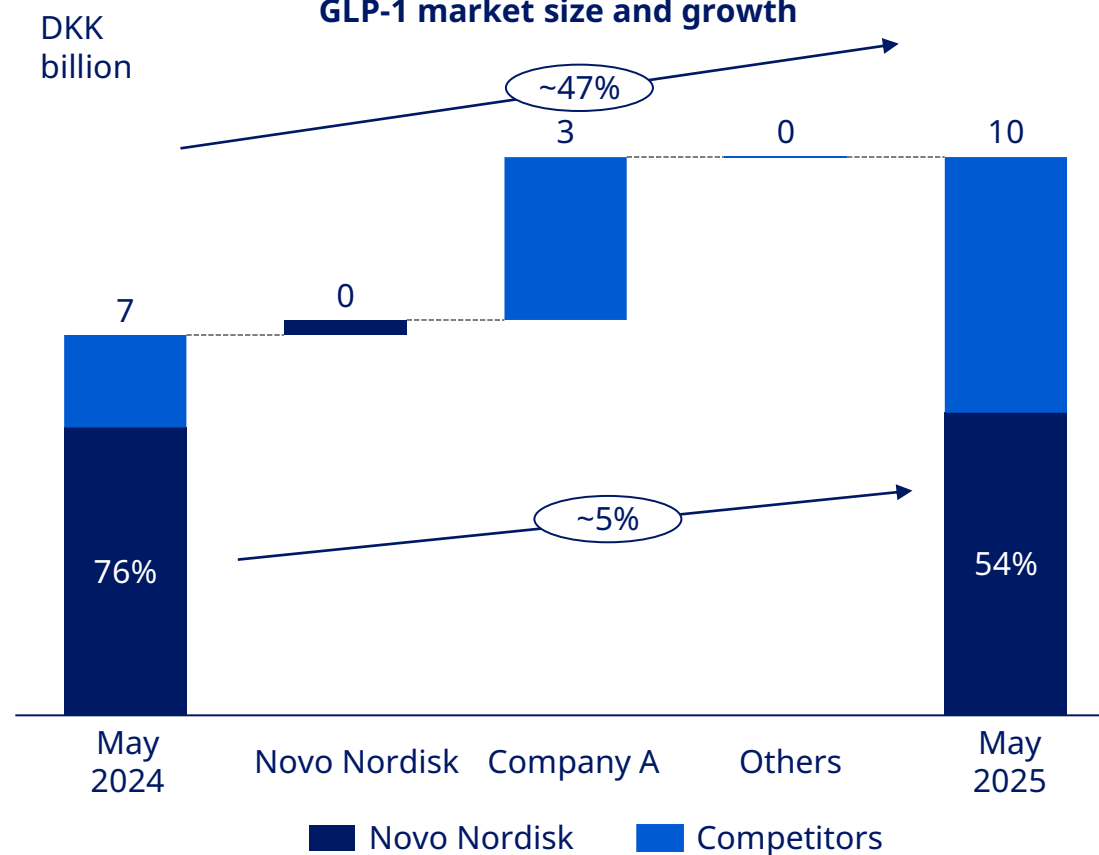


GLP-1 market share and market growth in APAC

GLP-1 market growth and Novo Nordisk market share



GLP-1 market size and growth



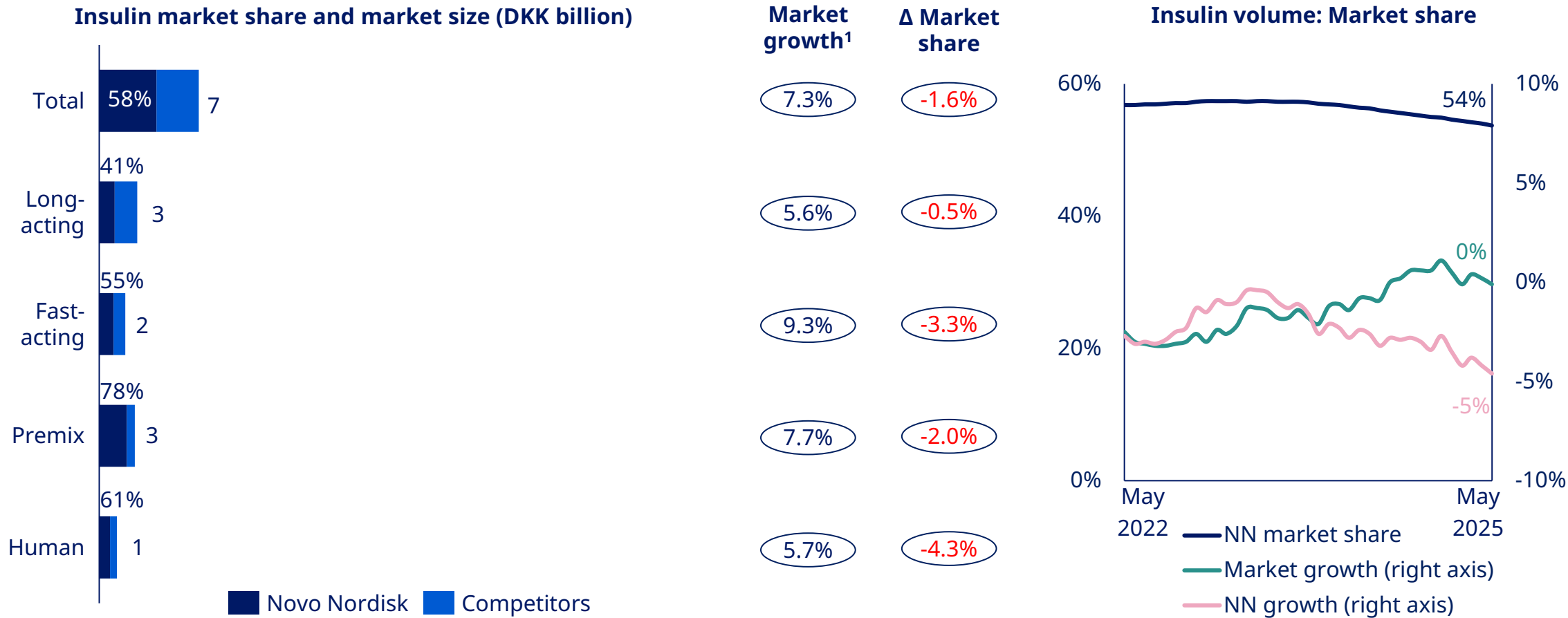
APAC: Japan, Korea, Oceania and Southeast Asia; NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company; Market values are based on the list prices

Source: IQVIA, May 2025, Value, MAT



Insulin market size and volume market share in APAC

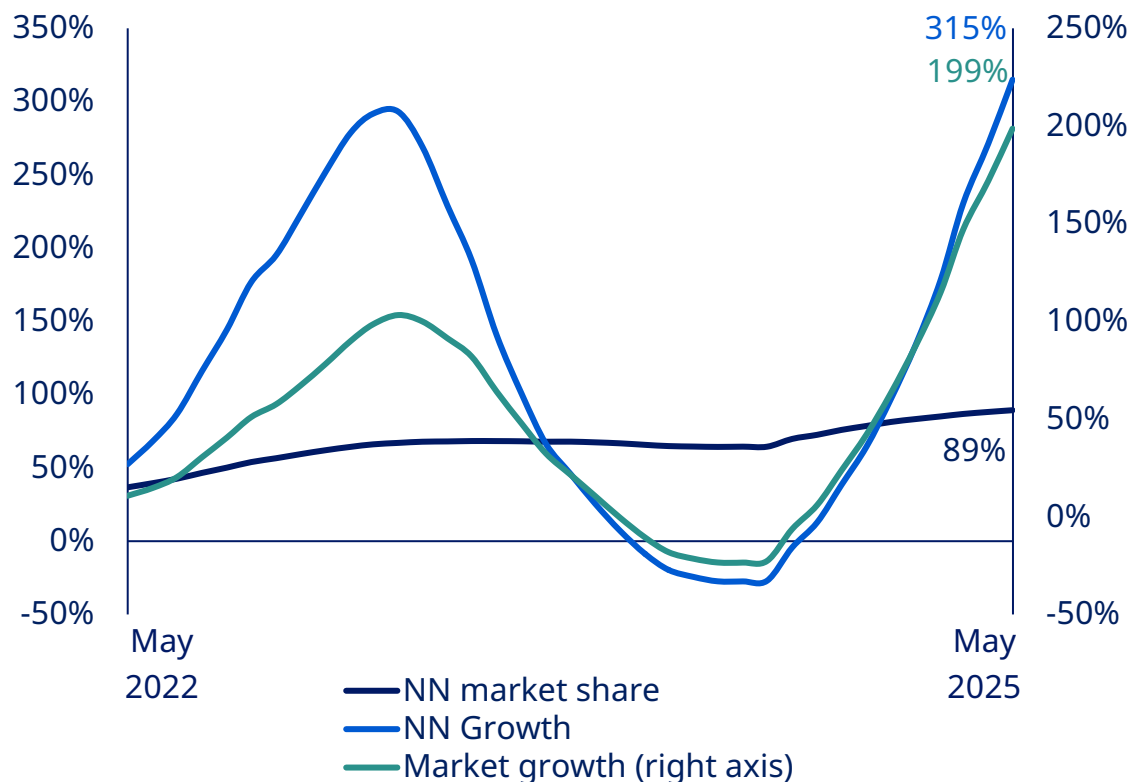


¹Market growth is YTD current vs YTD previous year
APAC: Japan, Korea, Oceania and Southeast Asia; NN: Novo Nordisk
Note: Share of growth not depicted due to too high numbers; Market values are based on the list prices
Source: IQVIA, May 2025 LHS graph - Value, RHS Graph - Volume, MAT

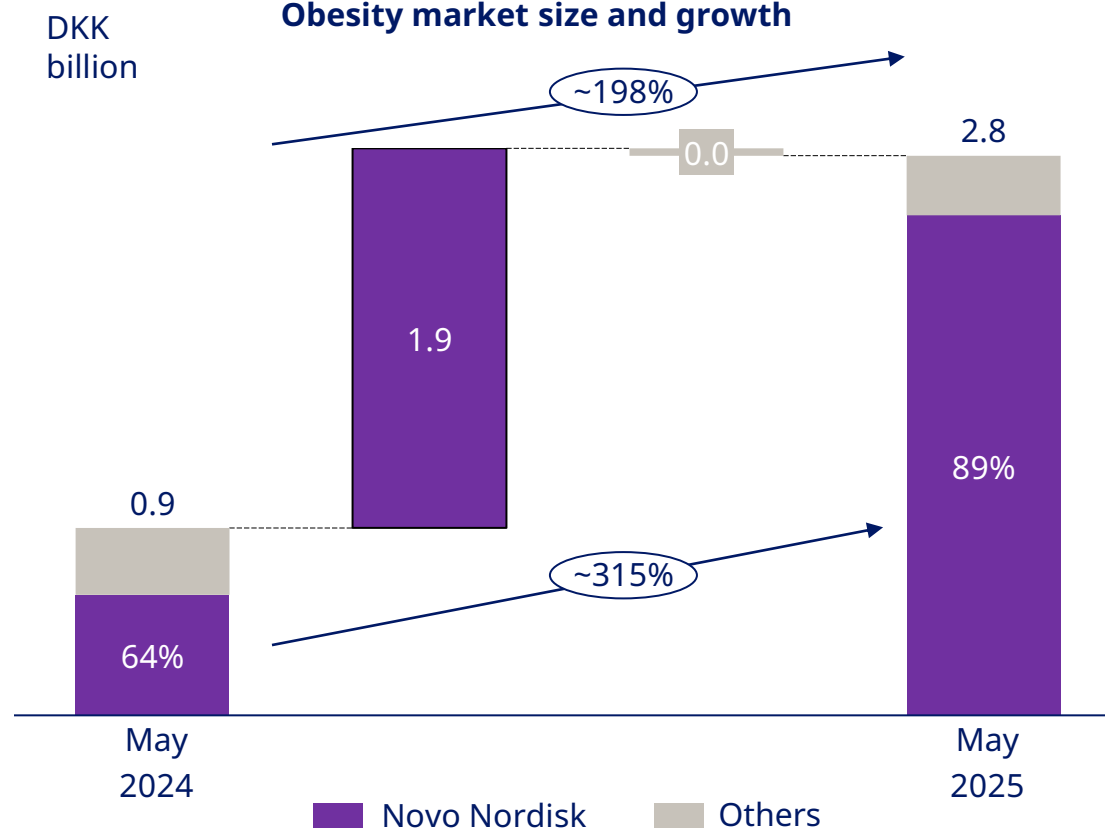
Obesity market share and market growth in APAC



Obesity market growth and Novo Nordisk market share

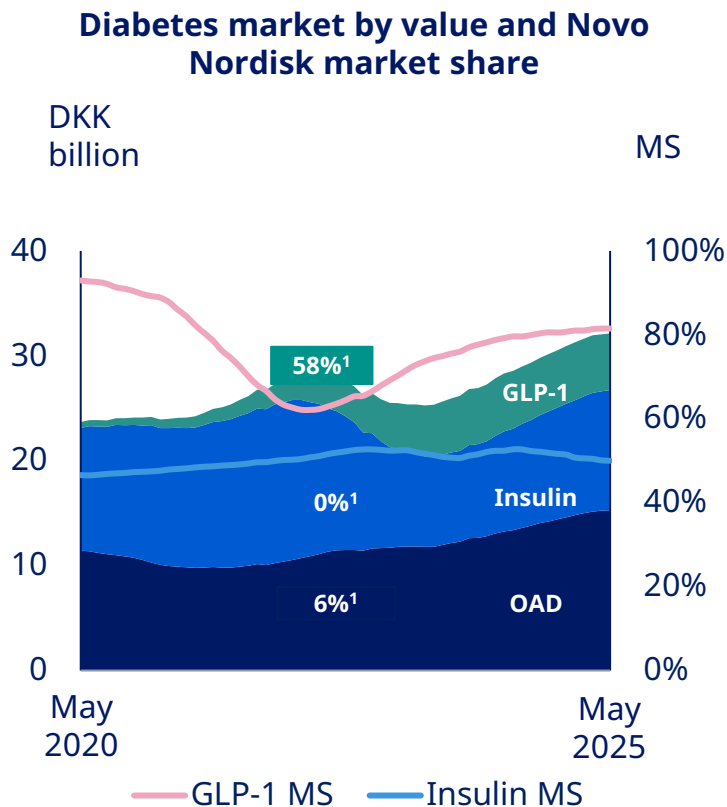
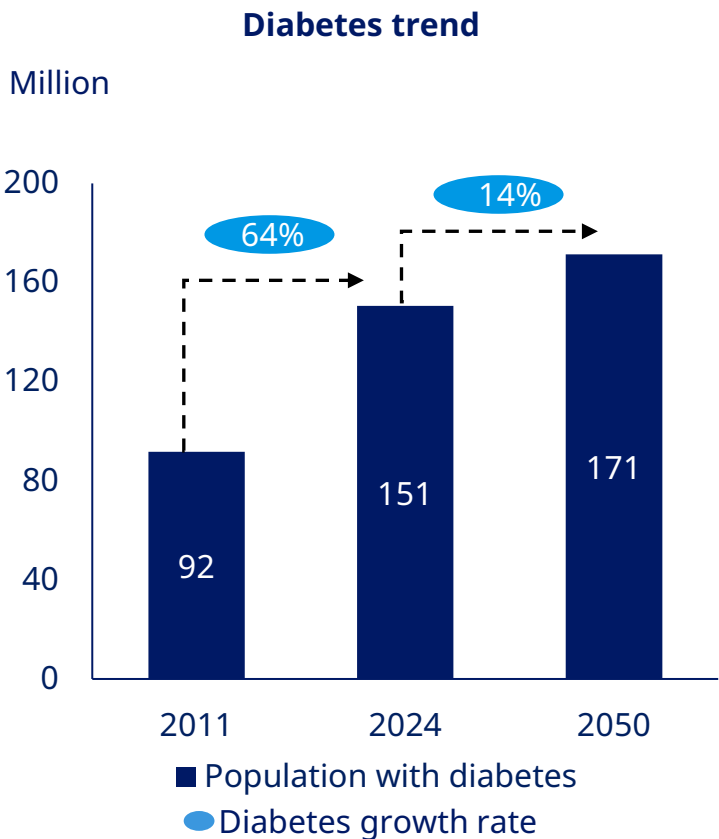


Obesity market size and growth





Region China at a glance



Novo Nordisk H1 2025 reported sales

H1 2025	Sales (mDKK)	Growth ²
Injectable GLP-1 ³	3,106	-12%
Rybelsus®	116	7%
Total GLP-1	3,222	-11%
Total insulin ⁴	5,190	3%
Other Diabetes care ⁵	301	-31%
Diabetes care	8,713	-4%
Obesity care ⁶	862	0%
Diabetes & Obesity care	9,575	4%
Rare disease ⁷	335	93%
Total	9,910	6%

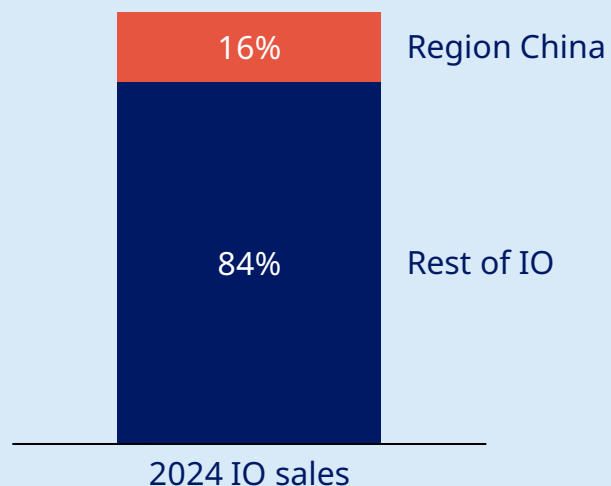
Note: Region China covers mainland China, Hong Kong, and Taiwan
Source: International Diabetes Federation: Diabetes Atlas 11th edition, 2025

¹CAGR calculated for last 5-year period
Competitor insulin value market shares, as of May 2025: Novo Nordisk 50%, Others 50%; Competitor GLP-1 value market shares, as of May 2025: Novo Nordisk 81% and Others 19% OAD: Oral anti-diabetic; MS: Market Share;
Note: Market values are based on list prices; Source: IQVIA MAT, May 2025 value figures

²At constant exchange rates; ³Comprises Victoza® and Ozempic®; ⁴Comprises Tresiba®, Xultophy®, Levemir®, NovoMix®, Awiqli®, Ryzodeg®, NovoRapid®; ⁵Comprises NovoNorm® and needles; ⁶Comprises Wegovy® & Saxenda®; ⁷Comprises primarily NovoSeven®, NovoEight® and Norditropin®

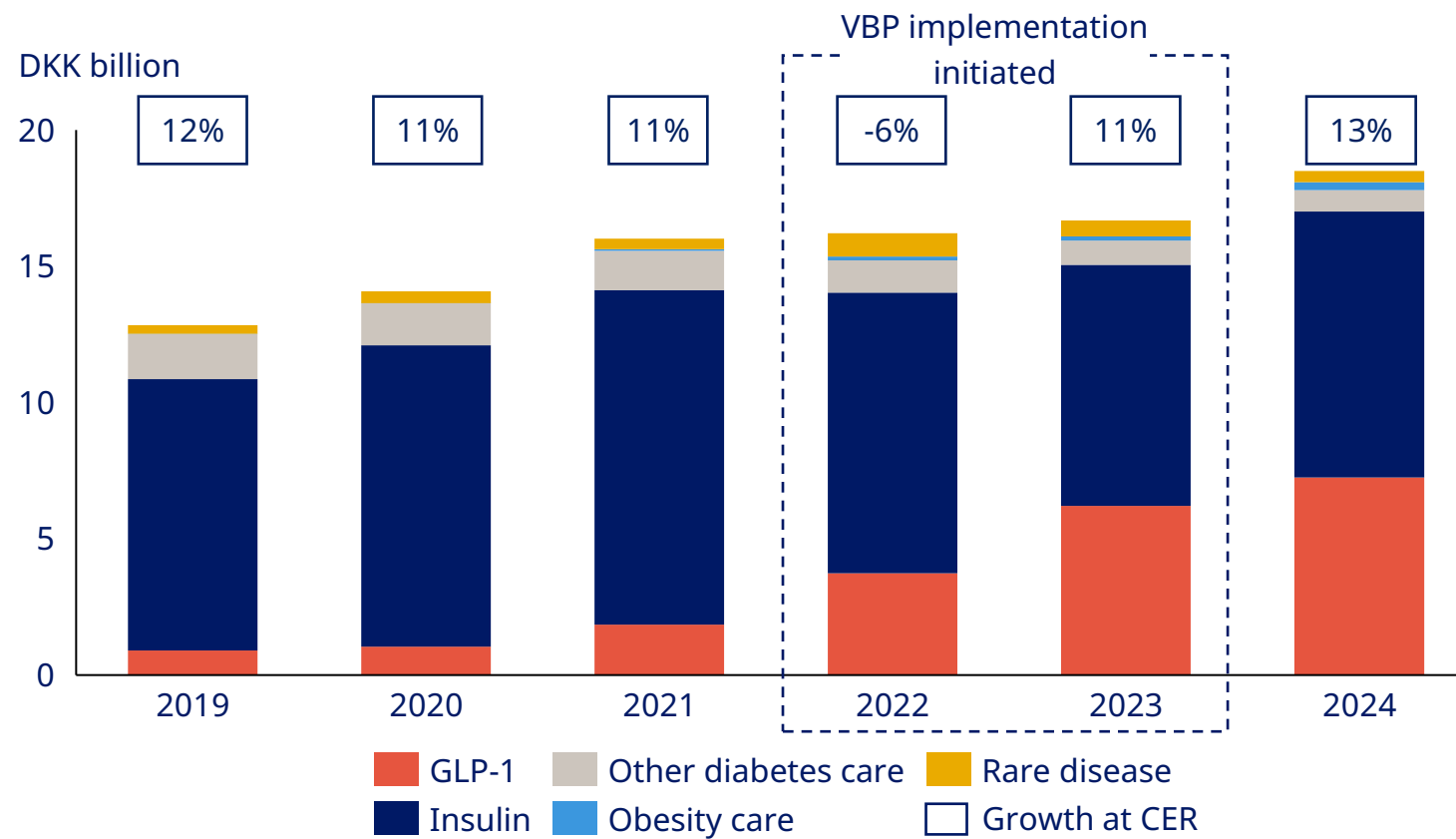
Region China remains a key market for Novo Nordisk and the established presence offers growth opportunities

Region China is the largest market within IO



52.6% Insulin market share¹ **79.6%** GLP-1 market share¹

Novo Nordisk Region China sales



¹Only mainland China

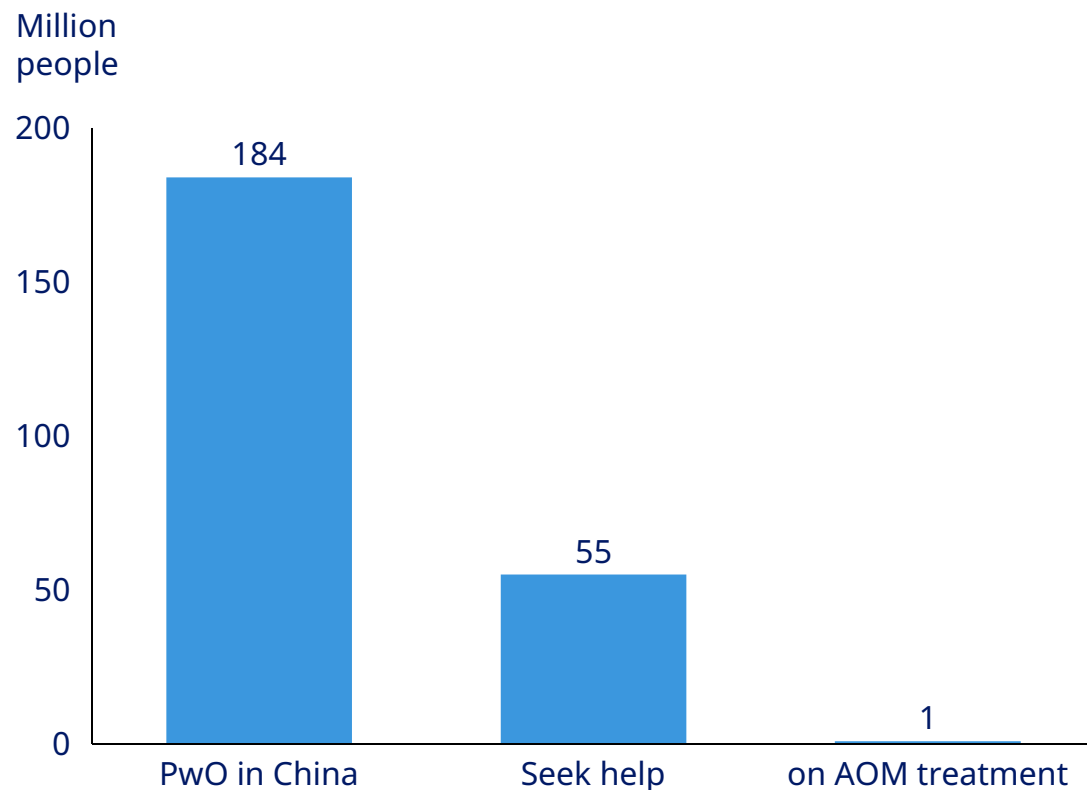
CER: Constant exchange rates; IO: International Operations; VBP: Volume-based procurement

Note: Region China covers mainland China, Hong Kong, and Taiwan

Sources: NN reported sales; IQVIA MAT CHPA data, Nov 2024

Wegovy[®] was launched in Nov 24 and is expected to address the high unmet need for anti-obesity medications in Region China

High unmet need for anti-obesity medications in mainland China



AOM: Anti-obesity medication; PwO: People with obesity
Note: Obesity in China defined as BMI ≥ 28 ; Region China covers mainland China, Hong Kong, and Taiwan
Source: Lancet Diabetes Endocrinol 2021, Goldman Sachs Global investment research, Data from 2019

Wegovy[®] launch out-of-pocket initially

Nov 2024

Launched in
mainland China

ONCE-WEEKLY
wegovy[®]
semaglutide injection **2.4 mg**

Wegovy[®] launch strategy

- Volume-capped launch
- Out-of-pocket market is initial focus of launch

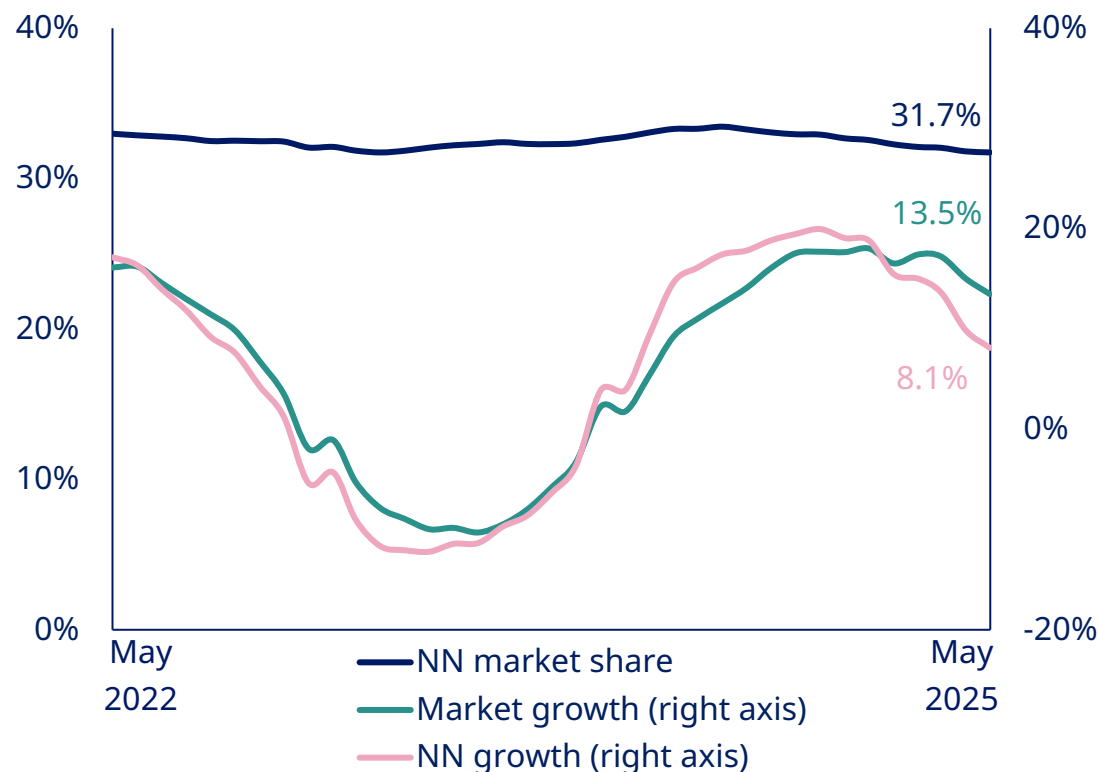
Access strategy

- Achieve hospital listing for Wegovy[®] at selected hospitals
- Explore commercial health insurance for selected sub-populations

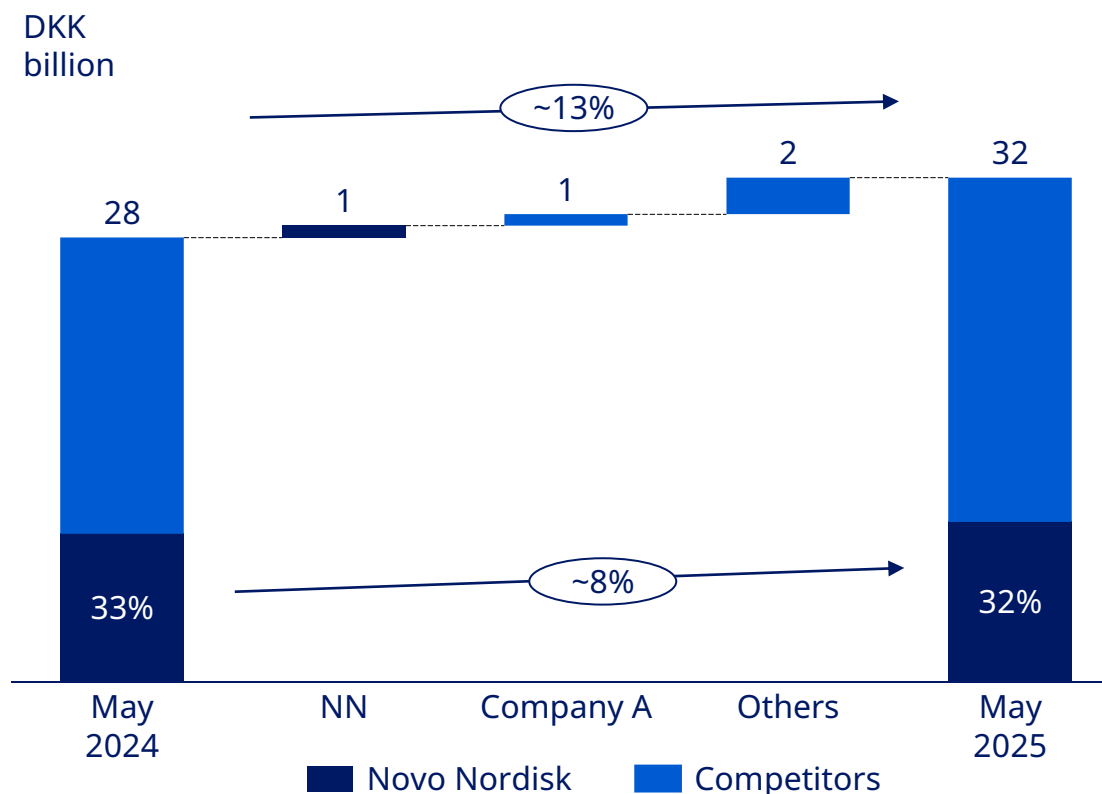


Diabetes market share and market growth in Region China

Diabetes market growth and Novo Nordisk market share



Diabetes market size and growth



NN: Novo Nordisk

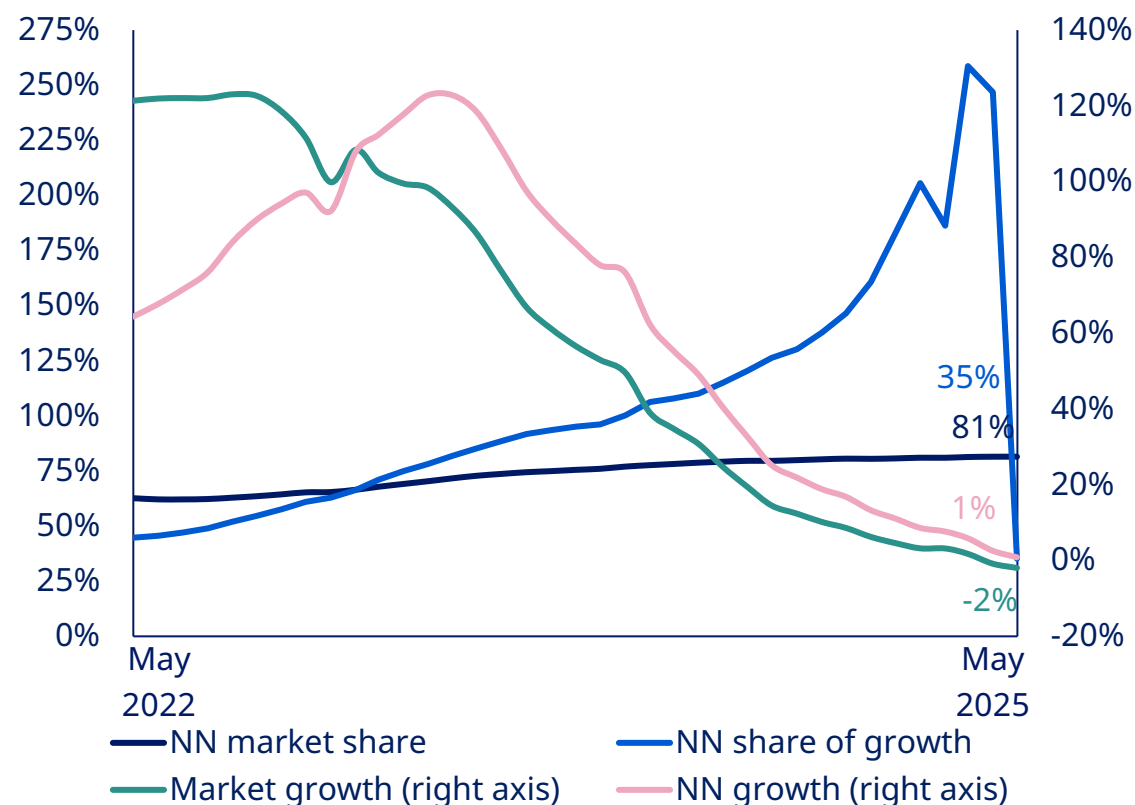
Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company. Region China covers Mainland China, Taiwan, and Hong Kong; Market values are based on the list prices

Source: IQVIA, May 2025, Value, MAT

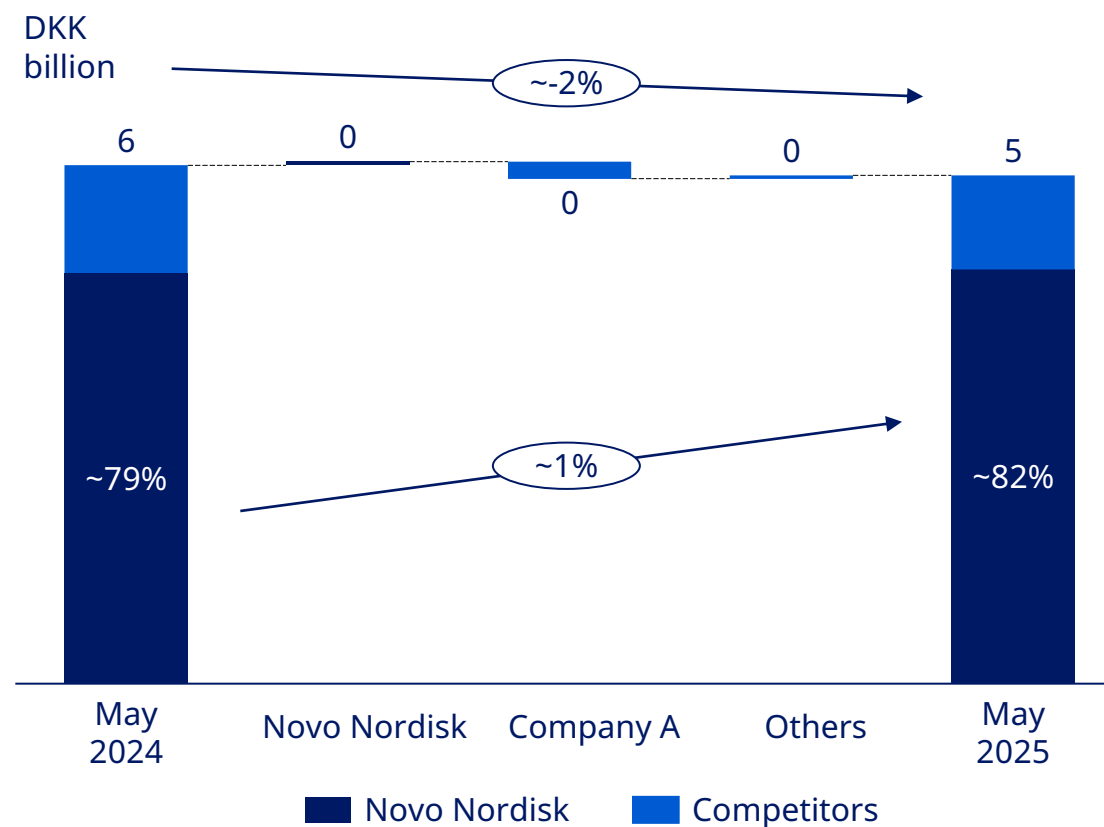


GLP-1 market share and market growth in Region China

GLP-1 market growth and Novo Nordisk market share



GLP-1 market size and growth



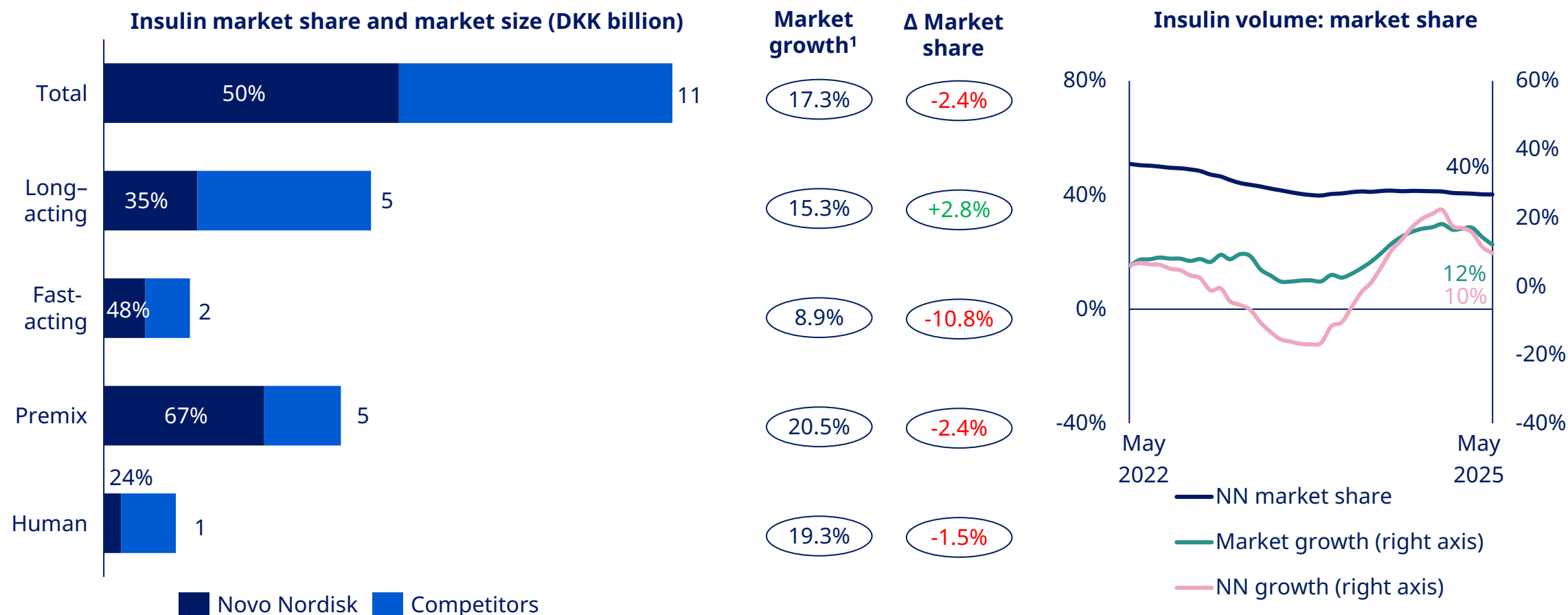
NN: Novo Nordisk

Note: Due to contractual obligations competitor names are not disclosed. Company A represents an actual company.; Region China covers Mainland China, Taiwan, and Hong Kong; Market values are based on the list prices

Source: IQVIA, May 2025, Value, MAT



Insulin market size and volume share of growth and market share in Region China



¹Market growth is YTD current vs YTD previous year

NN: Novo Nordisk; Note: Region China covers Mainland China, Taiwan, and Hong Kong; Market values are based on the list prices

Source: IQVIA, May 2025, LHS graph – Value, RHS Graph – Volume, MAT

Financials and Product Supply

Profit and loss, resource allocation

Product supply

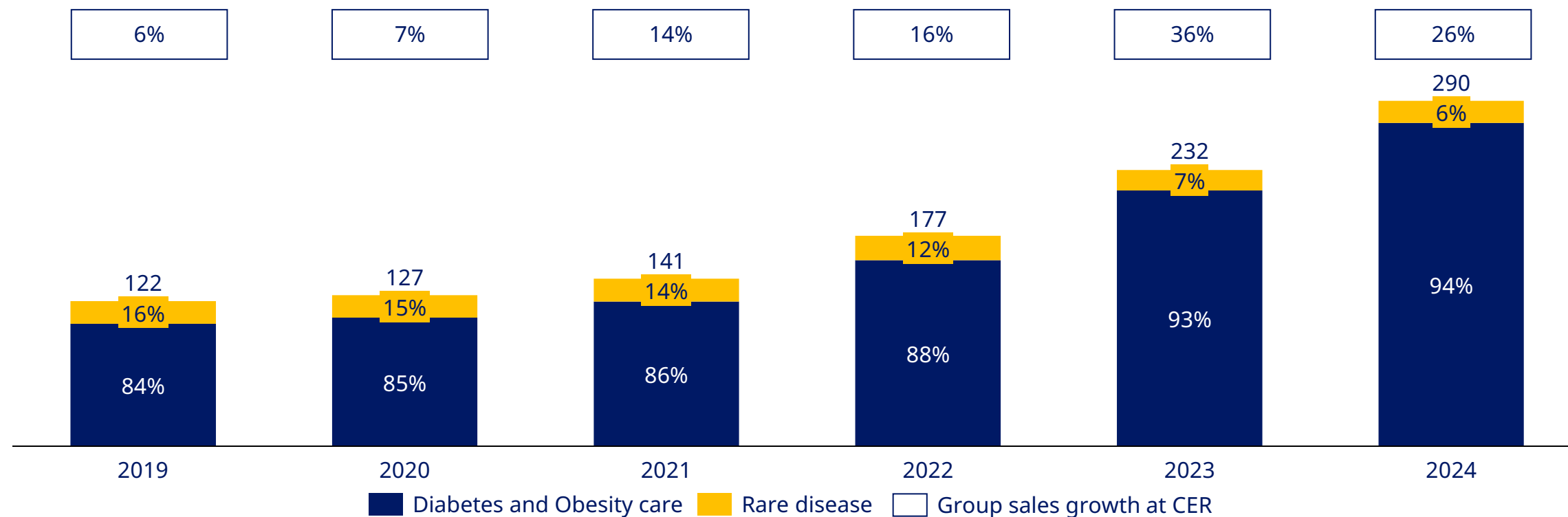
Margin development & capital allocation

Currencies

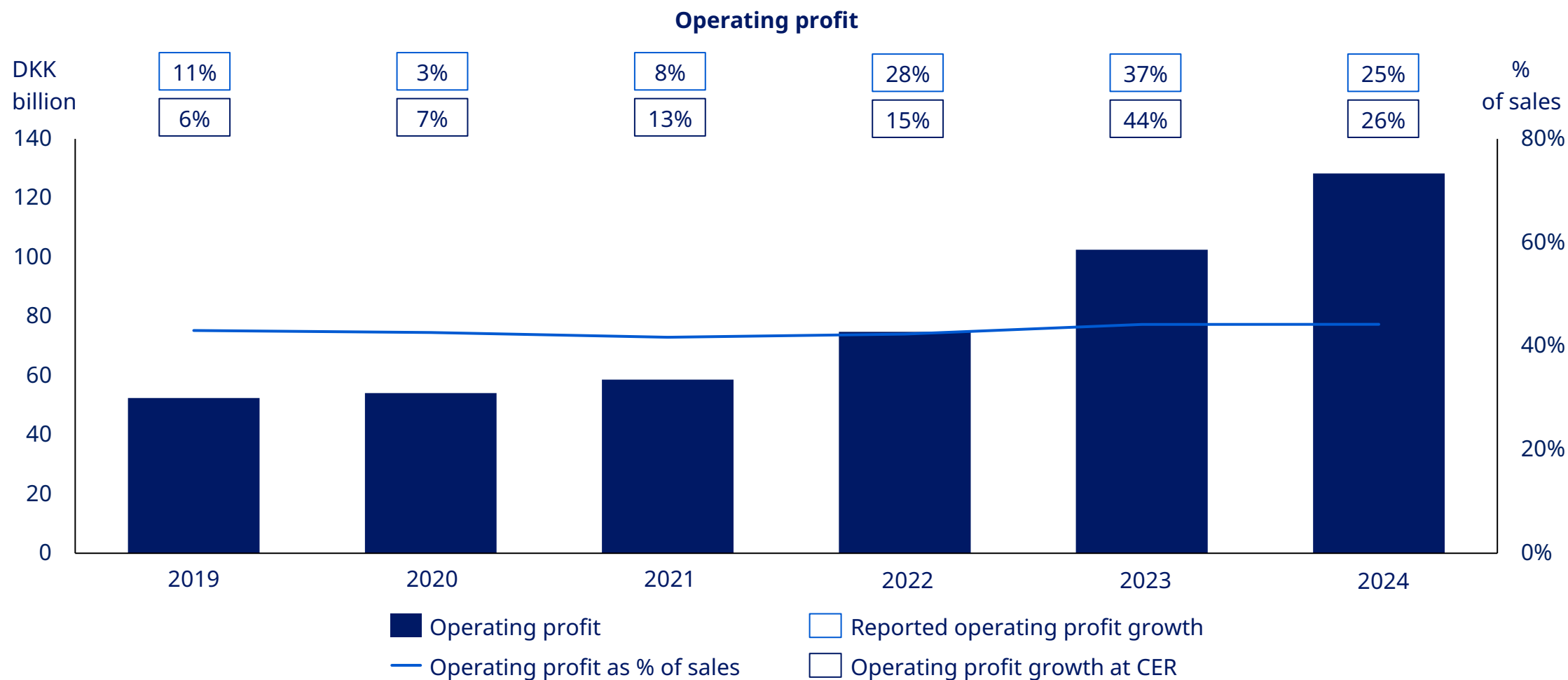
Solid sales growth driven by Diabetes and Obesity care

Reported annual sales 2019-2024

DKK billion, % of total sales



Solid operating profit growth



Resource allocation in Novo Nordisk is guided by investing in future growth while delivering attractive shareholder returns

Corporate strategy guides resource allocation



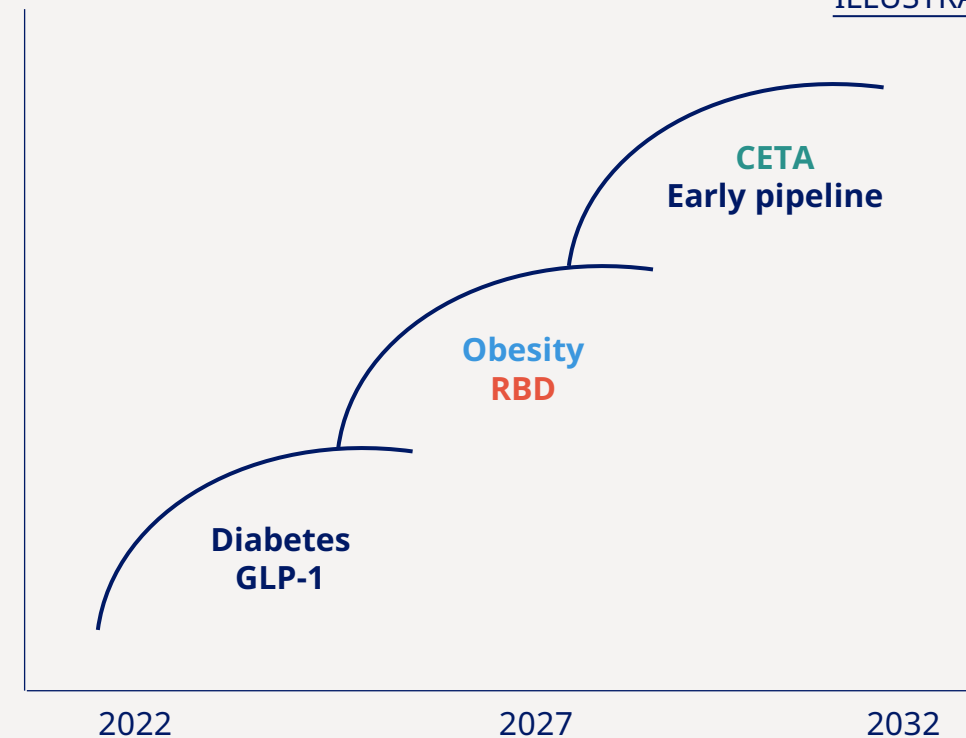
Focus on driving sustained sales growth

- Build obesity care market
- Expand manufacturing capacity
- Expand R&D pipeline

Expected primary sales growth drivers towards 2032

Waves of growth

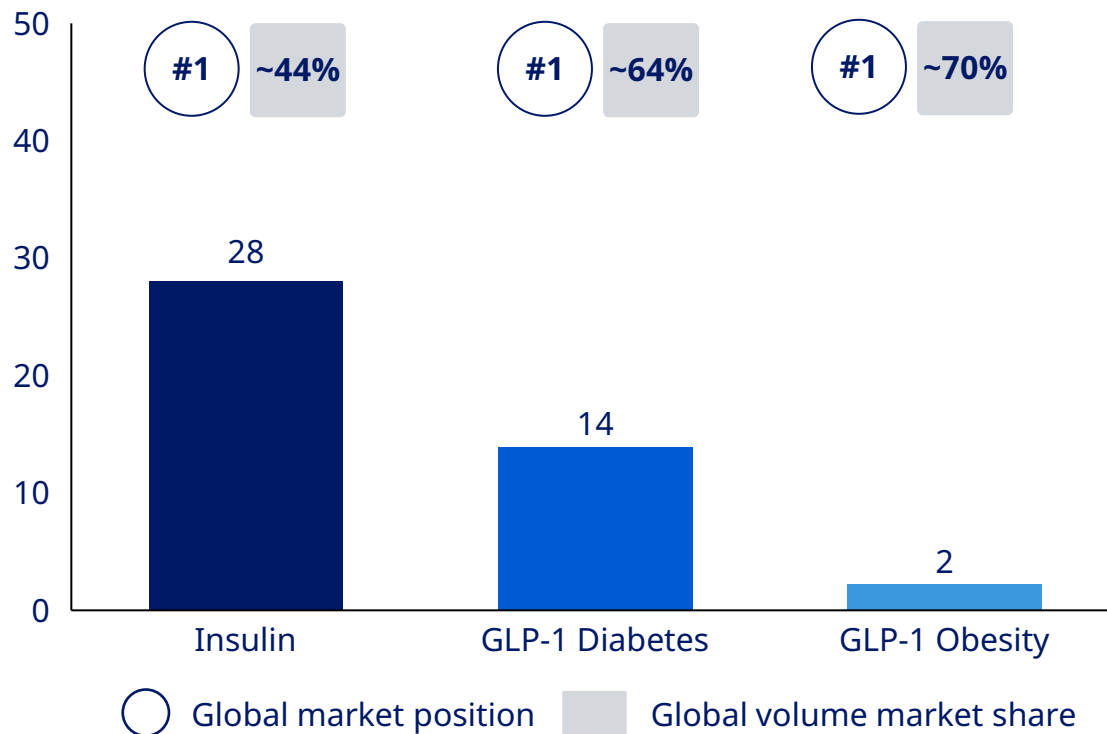
ILLUSTRATIVE



Manufacturing scale and expertise within biologics is a competitive advantage for Novo Nordisk

The world's largest manufacturer of insulin and GLP-1¹

Million patients on NN products in 2024



Novo Nordisk competitive advantages in manufacturing



Decades of experience with high volume production of core yeast and mammalian API platforms

API scalability and yield optimisation driven by continuous production technology



High volume installed capacity for biologics

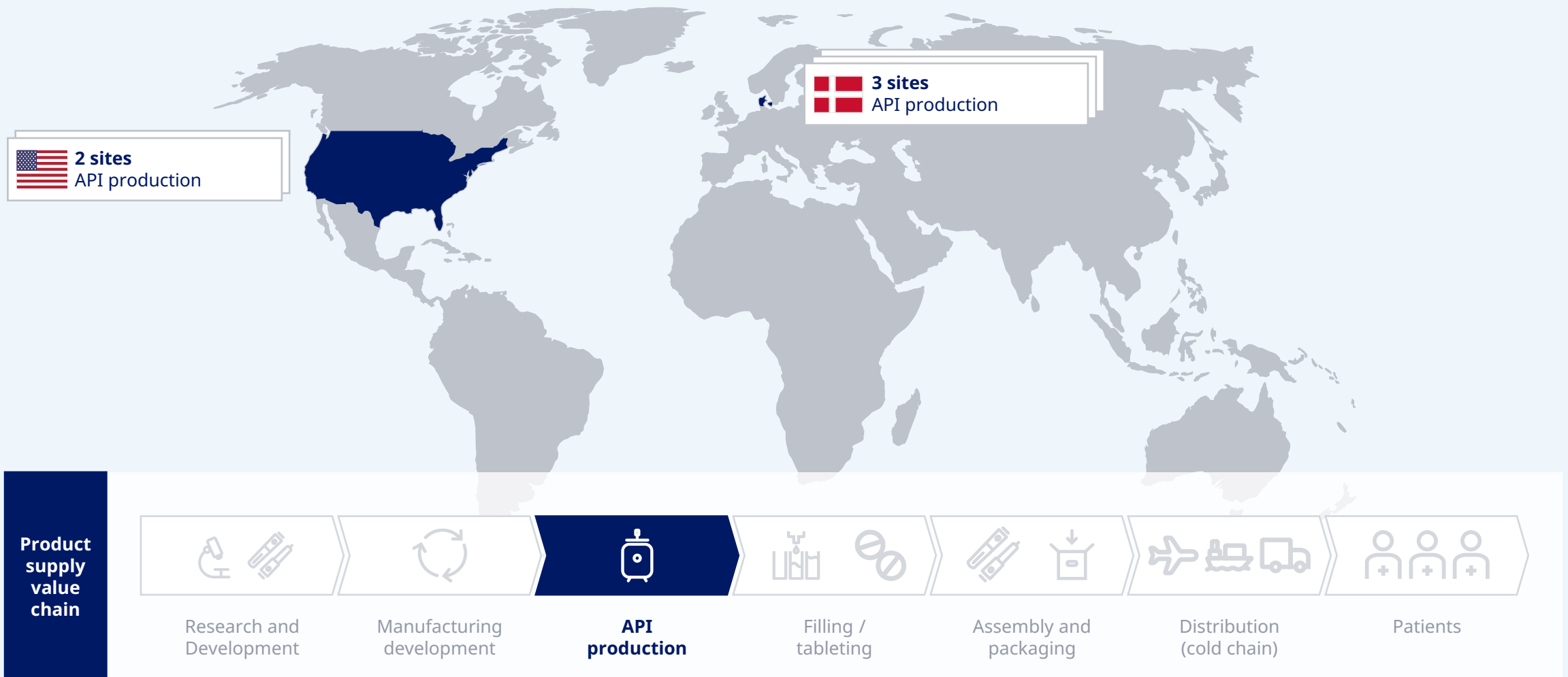
In-house expertise in the development and manufacturing of devices

¹In addition to the above-mentioned product classes, other diabetes care constitutes the remainder of people treated with Novo Nordisk products

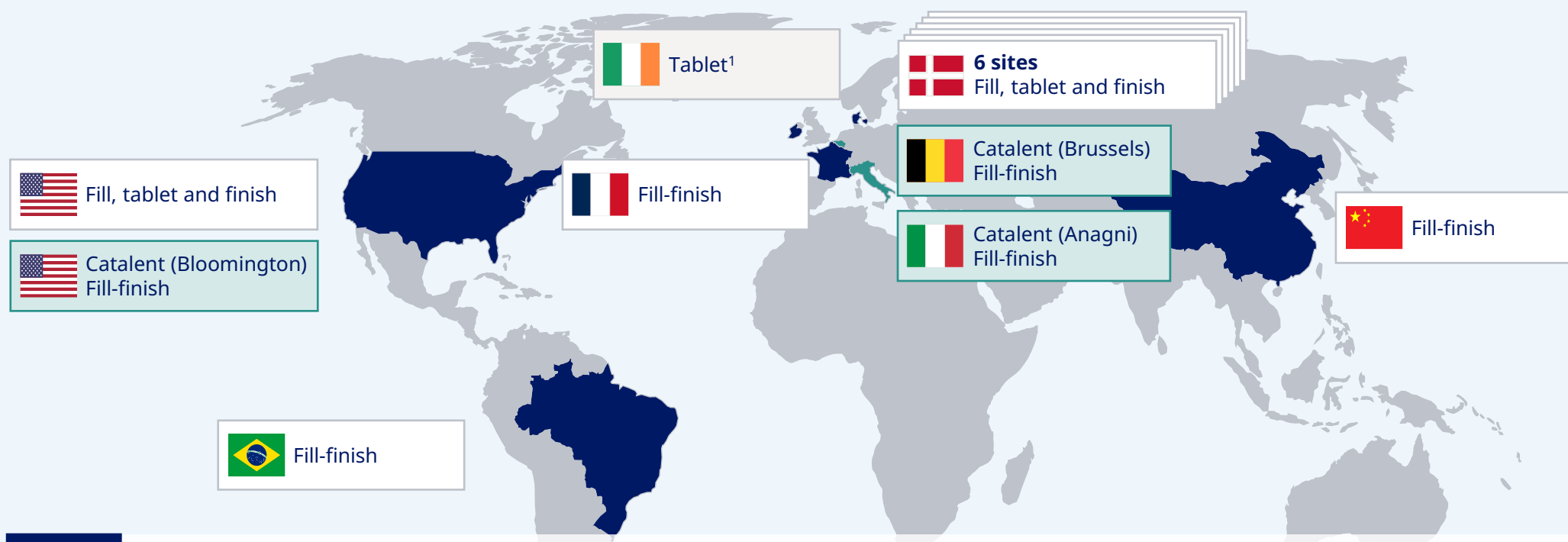
API: Active pharmaceutical ingredient; NN: Novo Nordisk

Sources: Volume market share and position based on IQVIA Moving Annual Total (MAT), Nov 2024 (Spot rate); Novo Nordisk Annual Report 2024

Active pharmaceutical ingredient | The strategically important sites in Novo Nordisk are based in Denmark and the US



Fill-finish | The global footprint has expanded from 11 to 14 sites with the closing of the Catalent acquisition in December 2024



Product supply value chain



¹The Alkermes transaction (Dec 2023): Expected to close in mid-2024

API: Active pharmaceutical ingredient

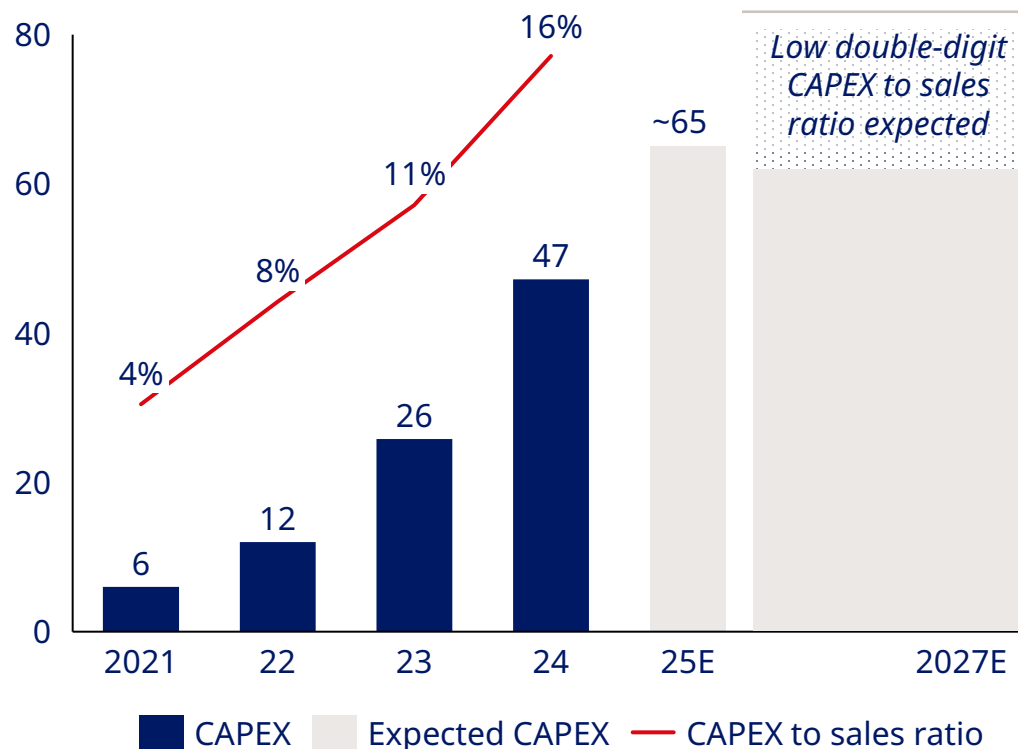
Note: There are local production facilities in Algeria, Iran, Japan, and Russia

New sites following closing of the Catalent transaction in December 2024

Significant step-up in CAPEX investments across the full value chain to enable growth for current and future products

CAPEX investments

DKK billion



Several large investments announced since 2021

Announced	Site	Scope	Investment
2021 December	Kalundborg Denmark	Mainly API	17 bDKK
2022 November	Bagsværd Denmark	Clinical API	5 bDKK
2023 June	Hillerød Denmark	API for CETA	16 bDKK
2023 November	Kalundborg Denmark	Mainly API	42 bDKK
2023 November	Chartres France	Fill-Finish	16 bDKK
2023 December	Athlone Ireland	Oral portfolio	1 bDKK
2024 June	Clayton US	Fill-Finish	27 bDKK
2024 December	Odense Denmark	Not specified	9 bDKK

Typical construction timelines: API: 5+ years | Fill-finish: 3+ year

Catalent fill-finish sites are expected to start adding additional capacity from 2026

Successfully closed the acquisition of three fill-finish sites



Bloomington site (Indiana, US)



Brussels site (Belgium)



Anagni site (Italy)



Novo Nordisk will honour all customer obligations at these sites

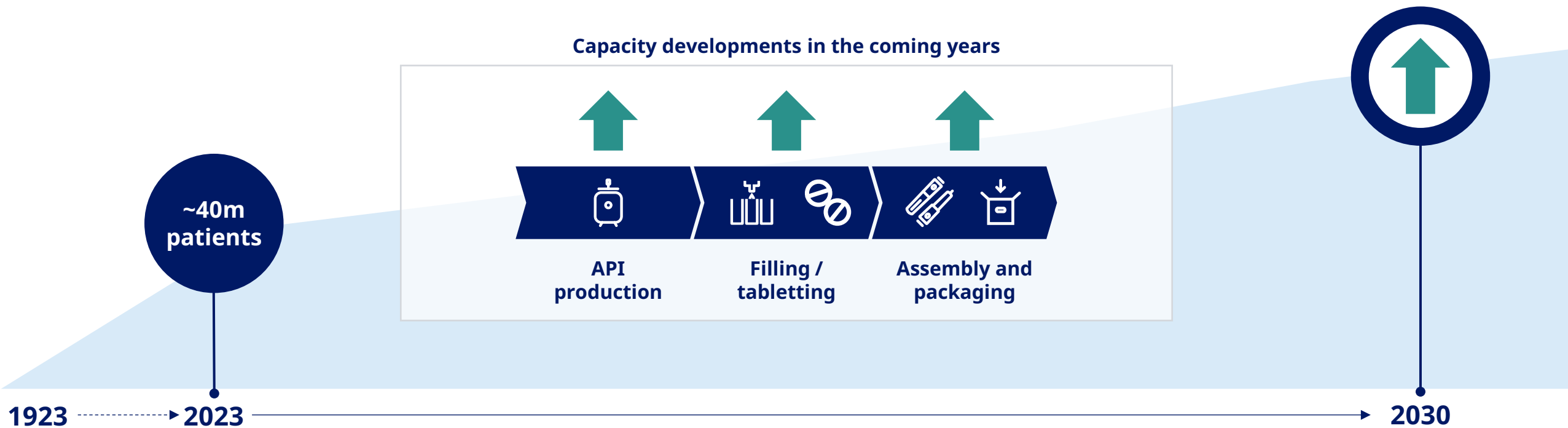
The acquisition will help expand capacity faster

- Will help reach more patients with current and future treatments
- Enables faster expansion of manufacturing capacity at scale, while providing future optionality and flexibility
- The three sites are fully operational and employ >3,000 people
- The acquisition is expected to gradually increase Novo Nordisk's fill-finish capacity from 2026 and onwards

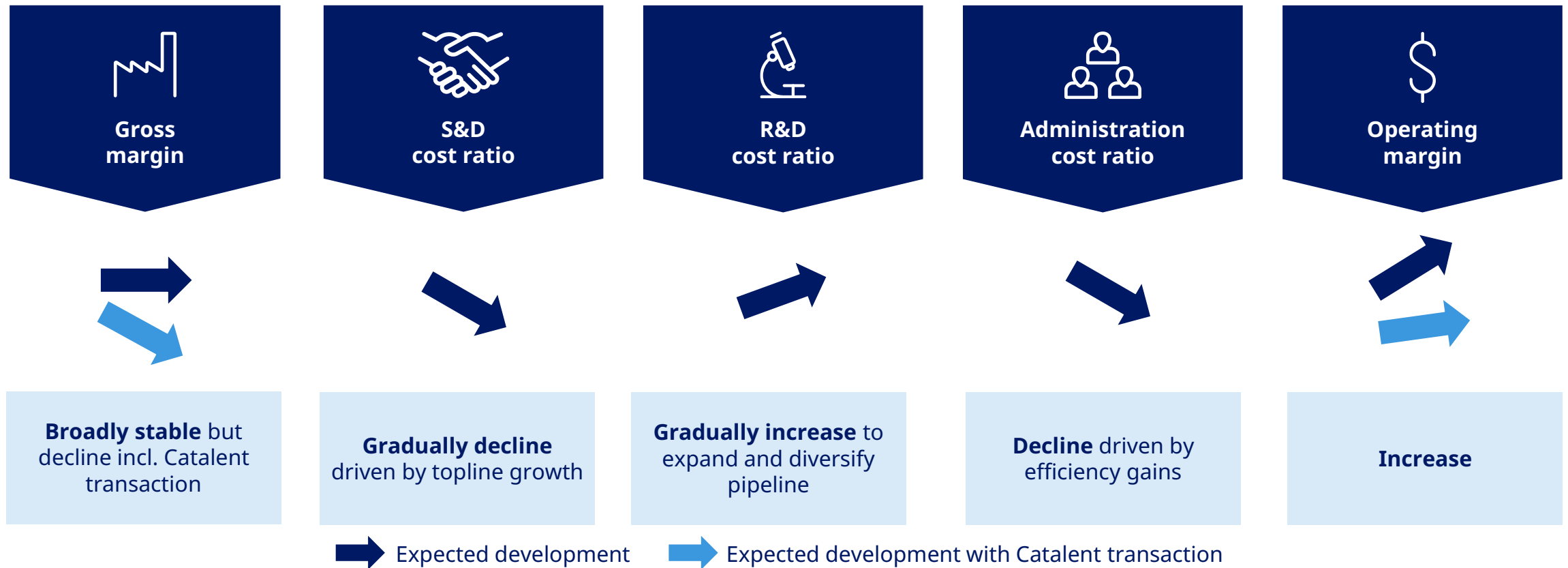
The acquisition of the three sites was completed on the 18th Dec

Investments across the full manufacturing value chain to significantly increase patient reach towards 2030

ILLUSTRATIVE



Expected margin developments in the coming years compared to 2023 are reflecting strategic resource allocation

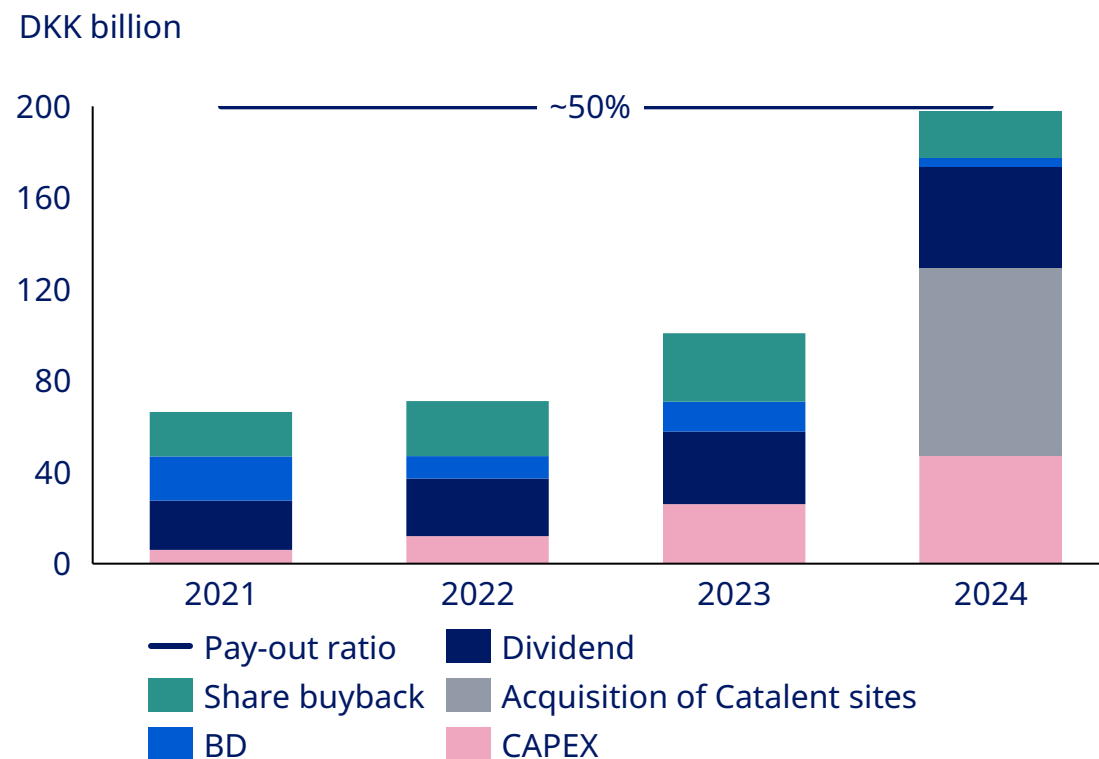


Novo Nordisk's capital allocation allows for investing in the business while maintaining attractive shareholder returns

Strategic capital allocation priorities

- 1 Internal growth opportunities: R&D and PS investments
- 2 Attractive annual dividend
- 3 BD investments to enhance R&D pipeline
- 4 Flexible share buybacks to distribute excess cash

Stable dividend pay-out ratio despite increased CAPEX and BD



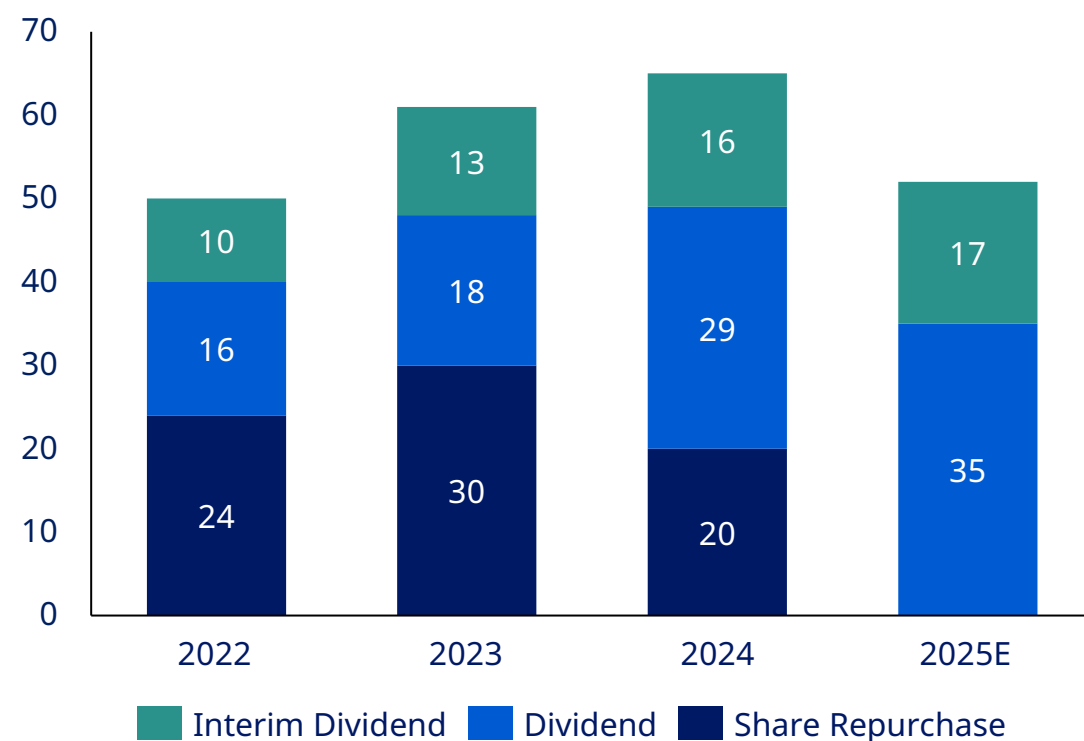
BD: Business development; CAPEX: Capital expenditure; E: Estimated; PS: Product supply; R&D: Research and development

Note: All numbers except for pay-out ratio are based on cash flow statement. Pay-out ratio calculated as total dividends for the year as a percentage of net profit for the same year

Attractive capital allocation to shareholders

Annual cash return to shareholders

DKK billion



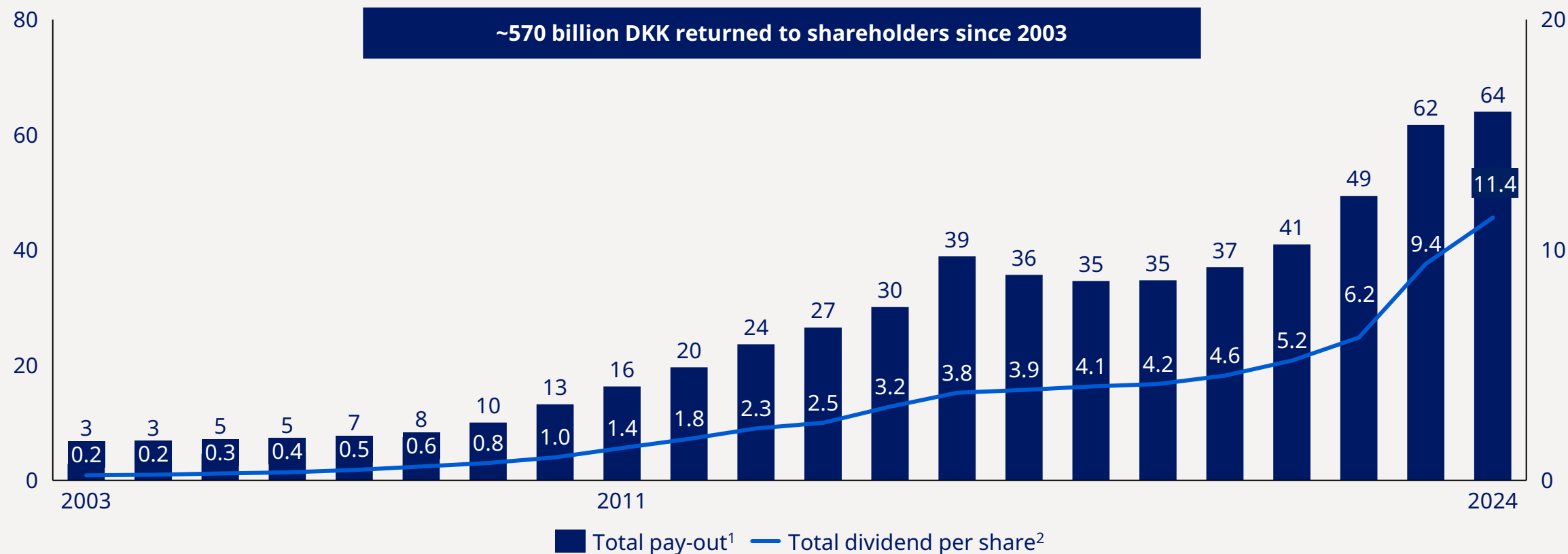
Capital allocation

- For 2024, the total dividend per share increased 21.3% to 11.40 DKK, comprised of an interim dividend of 3.50 DKK paid in August 2024 and a final dividend of 7.90 DKK paid in April 2025
- For 2025, the interim dividend of 3.75 DKK per share will be paid in August 2025
- Following Novo Nordisk's capital allocation principles, no share buyback programme has been initiated for 2025.

Two decades of consistent cash distribution to shareholders

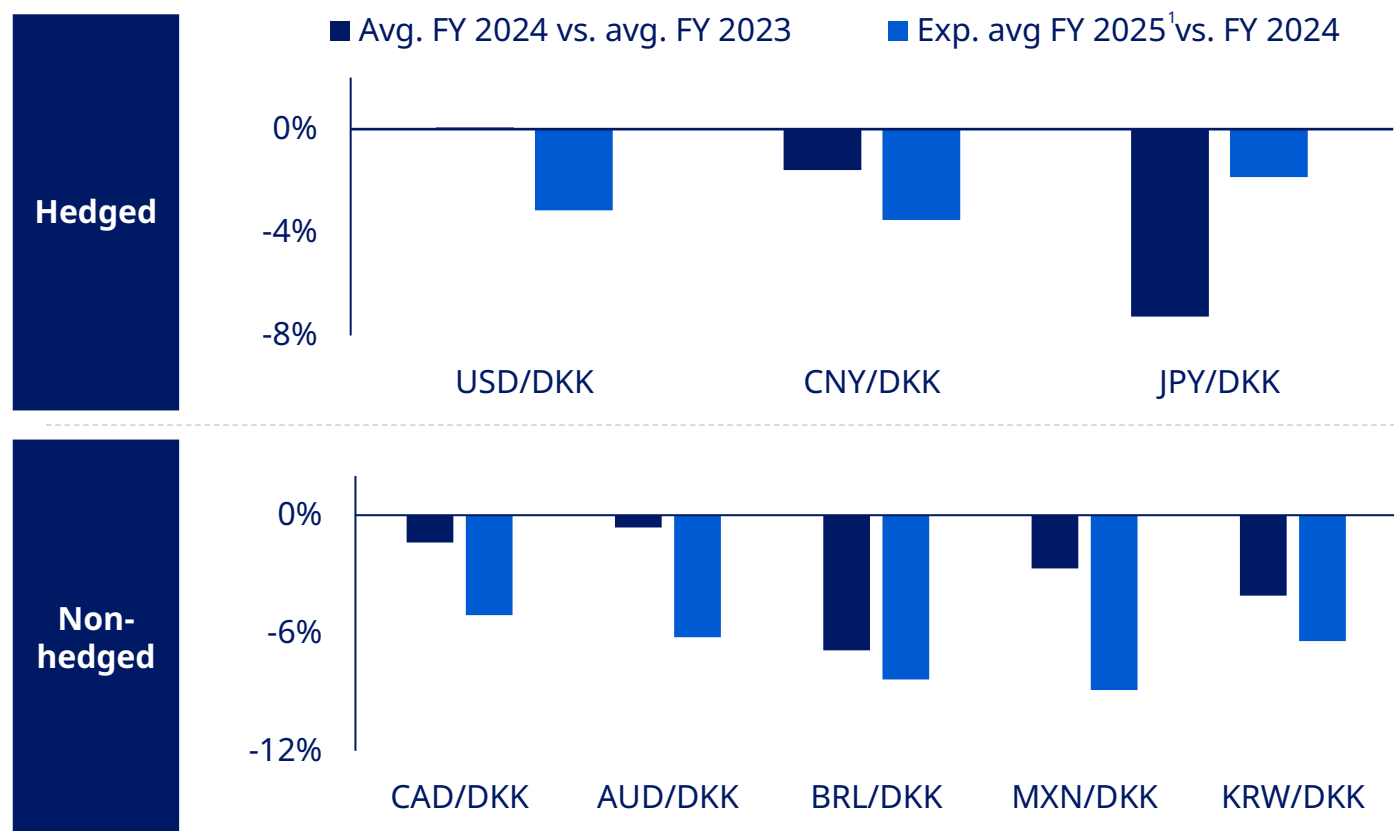
Share buybacks and dividends (bDKK)

Total dividends per share (DKK)



¹Dividends and share buybacks in the year of pay-out; ²Reflects year of earnings
Source: Novo Nordisk annual Reports

Net financials expected to be positively impacted by currencies in 2025 – offset by currency impact on operating profit



FY 2024

- Negative FX impact on operating profit of 1.1 bDKK
- Negative FX impact on net financials of 1.0 bDKK
- Net foreign exchange loss of 2.1 bDKK

FY 2025 outlook

- Currency impact on operating profit is expected to be around -5%-points
- Net financial items is expected to be a gain of around 1.6 bDKK mainly driven by:
 - **FX** - Gains on USD hedging contracts
 - Partially offset by **net interest expenses** relating to funding of the three fill and finish sites acquired from Catalent

¹ Year-to-date realised data and remainder expected flat currency development based on the spot rate as of 31 July 2025

USD: United States Dollar; DKK: Danish Kroner; CNY: Chinese Yuan Renminbi; JPY: Japanese Yen; CAD: Canadian Dollar; AUD: Australian Dollar; BRL: Brazilian Real; MXN: Mexican Peso; KRW: Korean Won

Purpose & Sustainability

Sustainable business

Environmental responsibility

Social responsibility

Ethics and compliance



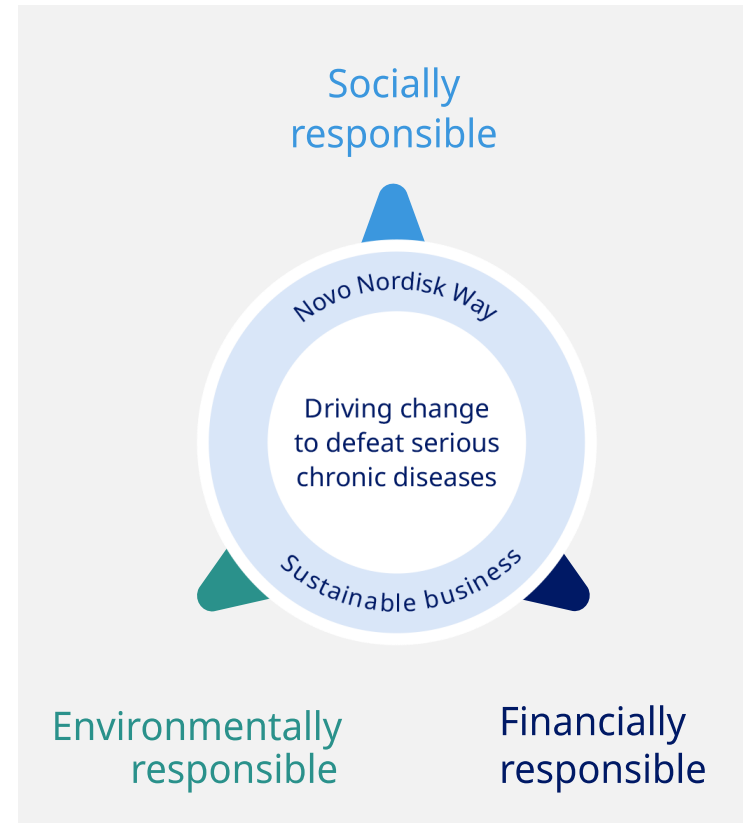
RANJITH S.
Ranjith lives with type 1 diabetes
India

Being a responsible business drives long-term value

Ownership structure creates long-term value



Commitment to lead a sustainable business¹



¹Environmental, Social and Governance responsibility has been anchored in Articles of Association since 2004; ²Consists of 1,075 million shares; ³Consists of 3,390 million shares
Note: Ownership structure as of 30 June 2025

Novo Nordisk's ambition is zero environmental impact



CO₂ emissions

- 2024** Emissions increased due to growth and CAPEX investments
- 2030** Target: Zero emissions from own operations and transportation
- 2045** Target: Net zero emissions across full value chain



Plastic

- 2020** ReMed™, Novo Nordisk's plastic take-back programme initiated
- 2023** 2+ million used NN pens returned¹
- 2023** Lilly, Sanofi and Merck joined the initiative in Denmark



Biodiversity

- Committed to start making nature-related disclosures
- Nature and biodiversity strategy being developed
- Novo Nordisk early adopter of TNFD²

¹Since 2020 ²As TNFD early adopter, Novo Nordisk has committed to report according to TNFD by 2025
CAPEX: Capital expenditure; NN: Novo Nordisk; TNFD: Taskforce on Nature-related Financial Disclosures

Social responsibility is core to Novo Nordisk and initiatives focus on prevention, access and innovation



Prevention

- Cities Changing Diabetes to build healthier environments in cities
- Partnership with UNICEF to reduce childhood obesity
- Obesity transformational prevention unit created in 2023



Access

- ~8 million people reached through our initiatives in 2024
- Aspen partnership to produce human insulin for Africa
- Changing Diabetes® in Children to provide care in low-and middle-income countries



Innovation

- Transformative treatments to raise the innovation bar

Integrating ethics and compliance into every aspect of our business

Ethics and compliance are at the core of Novo Nordisk

Novo Nordisk

Way

10

We never compromise on **quality** and **ethics**

Core elements of our compliance set-up

Mandatory ethics training

Global Code of Conduct

Audits

Trends, monitoring and risk management

Steps taken to strengthen ethics and compliance setup



Communication: Letters shared with HCPs reinforcing approved indication included in product label



Training: Enhanced training and processes around KOL engagements, HCPs, partners, patients etc



Resources: Dedicated obesity ethics, legal and compliance teams established to further increase compliance when launching Wegovy®

2024 statement of ESG performance

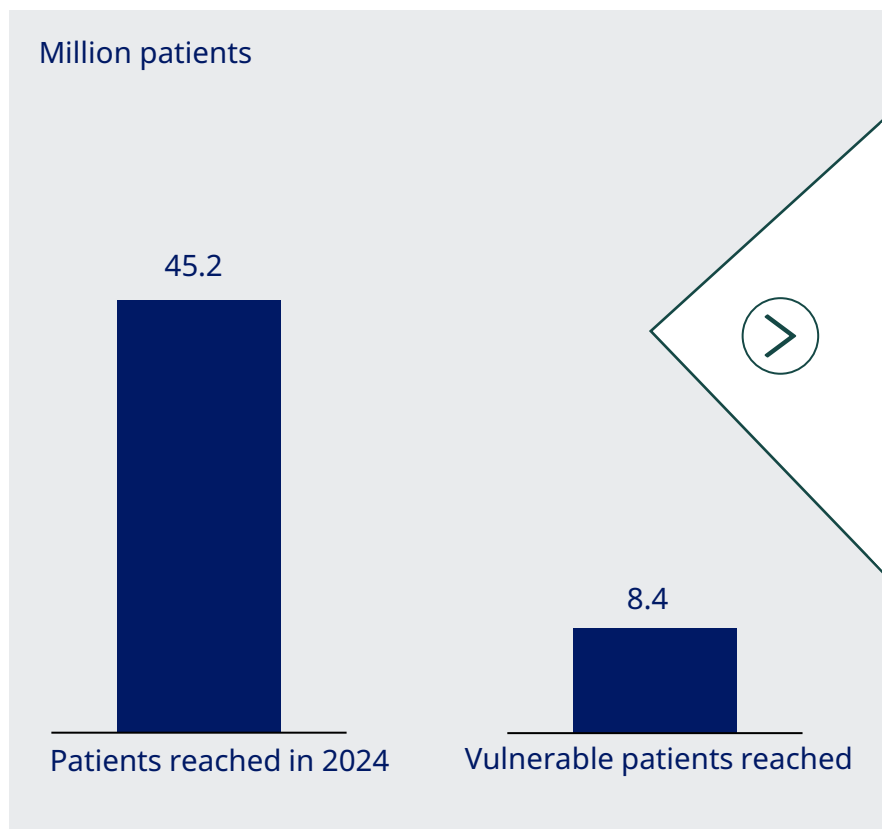
		Units	2024	2023	2022
Essential sustainability topics					
Patient protection and quality of life	Patients reached with Diabetes and Obesity care products	Number in millions	45.2	41.6	36.9
	Vulnerable patients reached with Diabetes care products ¹	Number in millions	8.4	8.8	-
	Children reached through Changing Diabetes® in Children programme (cumulative)	Number	64,743	52,249	41,033
	Product recalls	Number	3	2	3
	Failed inspections	Number	0	0	0
Climate change	Scope 1 GHG emissions	1,000 tonnes CO ₂ e	85	78	76
	Scope 2 GHG emissions (market-based)	1,000 tonnes CO ₂ e	16	15	16
	Scope 3 GHG emissions ²	1,000 tonnes CO ₂ e	2,160	1,743	-
Resource use and circular economy	Plastic footprint (absolute)	Tonnes	15,654	-	-
	Plastic footprint per patient	Kg/patient	0.35	-	-
Own workforce	Employees (headcount) – excluding Catalent ³	Number	74,156	64,319	55,185
	Gender in senior leadership positions	% men: women	58:42	59:41	61:39
	Rate of recordable work-related accidents for own workforce ⁴	Accidents per million hours worked	1.2	1.3	1.3
	Employees reporting symptoms of stress	%	13.8	13.8	13.8
	Employees reporting symptoms of work-related physical pain	%	6.8	7.1	7.8
Important sustainability topics					
Business conduct	Substantiated cases reported within accounting issues, fraud and business ethics matters via the Compliance Hotline ⁵	Number	242	221	227
	Animals purchased for research	Number	49,284	56,508	79,750
Water	Total Water consumption	1000 m ³	630	-	-
Pollution	Total amount of substances of very high concern that leave facilities	Tonnes	1	-	-
	Total amount of substances of concern that leave facilities	Tonnes	10	-	-

¹2023 figure has been restated ²2023 figure has been restated ³Total headcount of 77,349 in the Consolidated Financial Statement. The variance of 3,913 employees is due to Catalent Employees not included ⁴2023 and 2022 figures have been restated

⁵2023 and 2022 figures have been restated

In 2024, more than 8.4 million people with diabetes were reached with access and affordability initiatives

8.4 out of 45.2 million people were reached with access and affordability initiatives



A number of focused programmes (as of full year 2024)

Patients reached with NN diabetes and obesity care products

- Patients treated with our Diabetes products increased 6% from 40.5 million in 2023 to 43 million in 2024 primarily driven by the increase in Diabetes GLP-1-based products
- Patients reached with Obesity treatments increased from 1.1 million in 2023 to 2.2 million in 2024 primarily driven by the launch of Wegovy® in +10 additional countries in International Operations

Changing Diabetes® in Children¹

- 64,743 children reached at the end of 2024 across 30 countries
- More than half of the 12,494 newly enrolled children reached through expansion in Asian countries mainly India, Pakistan, Indonesia and Malaysia

Vulnerable patients reached

- Vulnerable patients treated with our Diabetes care products decreased 5% from 8.8 million in 2023 to 8.4 million in 2024 due to fewer vulnerable patients reached through human insulin tender sales and access and affordability initiatives.

US affordability offerings

- In 2024, 80% of US patients with insurance coverage for Ozempic® or Wegovy® paid USD 25 or less for each prescription, and almost 90% of US patients paid USD 50 or less.
- Continued commitment of long-standing patient assistance program to support eligible patients.

¹Changing Diabetes® in Children is a public-private partnership between the International Society for Paediatric and Adolescent Diabetes, the World Diabetes Foundation, Roche, and Novo Nordisk

Investor contact information

Share information

Novo Nordisk's B shares are listed on the stock exchange in Copenhagen under the symbol 'NOVO B'. Its ADRs are listed on the New York Stock Exchange under the symbol 'NVO'.

For further company information, visit Novo Nordisk on:
www.novonordisk.com

Access the full investor presentation here:



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